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Medplus Health Services Limited

040-6724 6724

Regd. off. H. No: 11 -6-56, Survey No: 257 & 258/1, Opp: IDPL Railway Siding Road, Moosap et, Kukatpally, Hyderabad – 500037, Telangana, India

CIN No: L85110TG2006P LC051845 | Website: www.medplusindia.com | Email: medplus@medplusindia.com

Date: 10th February , 2023

BSE Limited National Stock Exchange of India Limited
Phiroze Jeejeebhoy Towers, Exchange Plaza , 5th Floor
Dalal Street, Fort, Plot No. C/1, G Block,
Mumbai – 4000 01 Bandra - Kurla Complex Bandra
(East), Mumbai – 400051

SUBJECT: Post -Earning Calls

Dear Sir/ Madam,

This is to inform you that pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 (as amended), the transcript of the Post earnings call held on 04:00 PM of the 06th day of February 2023 , has been made available at the Investor Presentations section of our website :
<https://www.medplusindia.com/corporate.jsp>
You are requested to take the same on record.

For and on behalf of MedPlus Health Services Limited

Shilpi Keswani
Company Secretary

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“MedPlus Health Services Limited Q3 FY23 Earnings
Conference Call”

February 06, 2023

MANAGEMENT : MR. MADHUKAR REDDY – CHIEF EXECUTIVE
OFFICER , MEDPLUS HEALTH SERVICES LIMITED
MR. SUJIT KUMAR MAHATO – CHIEF FINANCIAL
OFFICER , MEDPLUS HEALTH SERVICES LIMITED
MR. CHETAN DIKSHIT – CHIEF STRATEGY OFFICER ,
MEDPLUS HEALTH SERVICES LIMITED

MR. PRASAD REDDY – MEDPLUS HEALTH SERVICES
LIMITED

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Moderator: Ladies and gentlemen, good evening and a warm welcome to the MedPlus Health Services Limited Q 3 FY23 Earnings Conference Call. As a reminder, all participant lines will be in the listen only mode and there will be an opportunity for you to ask questions after the presentation concludes. If you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Prasad Reddy from Med Plus. Thank you and over to you, Prasad.

Prasad Reddy: Thank you, Tanvi. Good evening everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q3 Earnings Conference to discuss the Financial Results of MedPlus for the Third Quarter of Financial Year 2023, which was announced on 3rd February 2023.

We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi – CEO and MD, Mr. Sujit Mahato – CFO and Mr. Chetan Dikshit – CSO. Before we begin, I would like to mention that some of the statements made in today's discussion may be forward looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning the risk and uncertainties on Slide # 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier and these have also been posted on our corporate website. Please note that we have uploaded the revised presentation to the stock exchanges today, including correction of few typos on Page #8 and #21.

I will now hand over the call to Mr. Madhukar for his opening comments, thank you and over to you Madhukar.

Madhukar R. Gangadi: Thank you, Prasad. Good evening everyone. At MedPlus today we are over 21,000 colleagues. I would like to thank my team for their discipline and hard work that goes into providing a vital service to our customers.

As on 31st December we cater to the healthcare and household needs of neighborhoods in 497 cities, across seven states through a network of 3557 pharmacy stores. We have now expanded into 43 new cities during the current quarter. We are continuing with our store expansion program. We have added 1080 stores in the last twelve months that's called med stores. In Q3 we have opened 246 stores and the highest additions of 65 and 58 were in Tamil Nadu and Maharashtra, respectively. In Q3 58% of our store openings have been in Tier-2 cities and beyond. We have 1523 of 3557 stores in Tier-2 cities and beyond. These are good markets from our store economics standpoint and MedPlus continues to expand in these markets because of the maturity of its operations and supply chain capability. There were 17 closures in Q3 versus 14 closures in Q2. Overall, there are net additions of 229 in Q3 versus 348 in Q2. For the last twelve months, net additions have been 1080.

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To give you an idea of the way our network is split, 32% of all of our stores are less than a year, 17% are between one and two years and 51% of the stores are two years and older. So, we ended Q3 with 49% of our stores being in the less than two years age bracket. In comparison for Q3 FY22, 36% of our stores are less than two-year-old. All stores below two-year age bracket are still in the ramp-up phase and from a financial point they are a drag on the operating EBITDA. However, as they mature we expect these stores to contribute in our profits. We closely track the time to break even for our new stores. The stores opened between July 21 and June 22, 69% of the stores achieved breakeven within six months of operations. To give you a sense of how rapid the ramp up to breakeven is and to six months and beyond level -79% of

stores broke even in the 7th month, and more than 10% broke even in just one month after that. At the end of the quarter, December 31, 2022 our network has now grown to 3557 stores spread across 2 million + square foot compared to 2477 stores spread across 1.5 million square foot at the end of December 2021. The average store size is 558 square foot. To give you a sense of spec, in store sizes, we have 2474 stores less than 600 square foot and 1083 that are greater than 600 square foot.

With our scale, it is now better for us to increase our overall share of private label and to increase our overall share of revenue from the private label side and our scale allows us to add new products for which we did not have the minimum order quantity earlier and our scale also

allows us to establish a brand which makes all the products much more easier for customers to accept. Our private label range is intended to provide quality products at affordable prices. Thus today we have over 900 curated products across pharma and non-pharma. Private label sales at the end of last quarter were 13.8% of our overall revenue. Overall, the trajectory of increasing share of our private labels in our customer markets continues. Through private label, our pharma range has also been gaining share, 8.6% of pharmacy revenues is from private label pharma. Our increasing presence in Tier-2 cities and beyond is reflected in our revenue mix. Sales from Tier-2 cities and beyond contributed 30% of our pharmacy revenues this quarter. This has increased from 29% in Q3 of the last quarter.

We continue to extend our range of Pincodes for our online orders. This complements we have with our physical stores and our thrust will continue to focus on increasing the coverage of our two-hour delivery offering. Store pickups as a share of online orders continue to maintain a higher share than home delivery. Reflective of the convenience and accessibility of our store network, our online strategy remains unchanged. We have not spent heavily to acquire customers online. In fact, in the last quarter we have not spent at all, I would say, on increase of online customers. We will continue to maintain our Omnichannel as a profitable channel. The way we think of our Omnichannel when we set up a store and we enter a neighborhood, is that we will automatically draw 100 customers per day kind of footfall. All of these people will be immediately connected, we let all these customers know that we also have the same service online and we let the customers, according to their convenience, choose to either go online, offline or in a lot of cases go between both. We don't see and given that we are the only one distributing the products only in that particular neighborhood, for us, every single transaction becomes a profitable transaction.

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Neither is their cost of acquisition, nor is their extra cost or extra discounting which we give, nor do we spend too much on the delivery side. For us, while we believe that online is a very critical part of the overall supply chain, overall selling process out there, we think that the customers will pick it based on their convenience and there is actually no need to spend a lot of money to acquire customers or at least increasing 10 customers into going online.

With that, I will actually let our CFO give you an update on our numbers.

Sujit Kumar Mahato: Thank you, Madhukar. Now, on the quarter's performance:

Our consolidated revenue was 11,903 million, a growth of 27.5% year on year and 6.2% quarter on quarter. Our consolidated operating EBITDA stood at 371 million, representing 3.1%. This is a 1.2% year on year and 31% quarter on quarter improvement. Around 99% of our revenues is from our pharmacy operations. The pharmacy operating EBITDA was 406 million representing 3.5%.

On our

store performance, I would like to update you on our stores older than 12 months, revenue from these stores in Q3 was 9895 million or 85% of our total pharmacy revenue. These stores had a store level EBITDA margin of 10%. The store level operating ROCE of these stores stood at 60.8%, a word here on the store level EBITDA margin by age, while stores greater than 12 months had a margin of 10%, for stores in the greater than 24 months category margin was 10.6% and for stores in the 13 months to 24 months category margin was 7.2%. If we allocated non-store-related cost, then the operating EBITDA of stores are greater than 12 months which would be 490 million, which translates to a margin of 4.9%. On working capital, our net working capital for Quarter 3 was 63 days, the inventory in our warehouse was 37 days. As you are aware, because of the sales trajectory of new stores where inventory turnover is lower in the first year. In Q3, the inventory level of our first year stores was 109 days. In comparison to our stores older than 12 months, the inventory was 38 days. On our segmental data I would like to add an important note, in page 17 of our earnings update we have presented the business segments which are different from the regulatory filings. For example, opticals have been grouped under others in the presentation, whereas in the regulatory filings, opticals is grouped under retail. We hope this information will be useful to you.

Now, I request Chetan to update us on the diagnostic business, over to you, Chetan.

Chetan Dikshit: Thanks Sujit. Good afternoon, everyone. I'm Chetan Dikshit and I'm the Chief Strategy Officer of MedPlus. We currently have three full-service diagnostic centers in Hyderabad and over 100 collection centers. Any member can avail themselves of the full range of radiology tests. Example MRI, CT and pathology tests at 75% discount to MRP.

To recap, there are four differences in our models versus our typical tier:

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- Firstly, we do not operate via franchisees and so there is no revenue sharing.
 - Secondly, our collection centers are housed within our pharmacies, so there are only marginal incremental establishment costs at a consolidated level.
 - Thirdly, our plan is designed such, that we do not depend on the referral network for patient walk-ins and lastly we expect our centers to achieve scale faster than peers.
- As an indication, the capacity utilization for our advanced radiology machines in six months of operations is nearly twice that of a typical new center in the marketplace. Up to 31st of December, we have sold 68,000 plans with 119,000 underlying lives. In October, November and December, we sold 250, 281 and 275 plans per day respectively.

That's our update on diagnostics, handing it back to Madhukar.

Madhukar R. Gangadi: Thanks, Chetan. So, what can you expect from MedPlus going forward? We operate in the attractive pharmacy space first for growth on the back of our store expansion. Our cluster-based network enables profitable Omnichannel service and scale allows a larger share of our private label basket. Our diagnostic project has proven that we can use our pharmacy stores to cross sell other health care solutions. We'd like to explore other avenues that can add incremental sales without increasing costs. With this, I'd like to open up the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. First question is from the line of Arshita Jain from Nuvama Group. Please go ahead.

Arshita Jain: Good evening. My first question is on your ramp up of new stores. About 69% of your stores achieve breakeven within six months versus 75%, 80% seen in the past. Seems like new stores are taking longer to break even, is the reason of that related more with the opening of new

stores in Tier-2, tier 3 cities or it's just a competition in general? What is your take on the current competitive scenario?

Madhukar R. Gangadi: As I said, 69% of our stores achieved breakeven in six months. No, I don't think opening them in Tier-2 is actually a cause for slowing down or even speeding up or whatever I don't think there's any difference out there. While stores in Tier-2, the top line for them is slightly lower. Typically their rents and other costs are also much lower and the share will apparently will, which makes for our profitable sales to be actually higher. They end up breaking even at the same time. But there is some variation once in a while it goes between 65% to 75% and sometimes goes higher than 83%. This is not something which we're too concerned about. As I said in my call, if you look at when we look at the 7th month, the overall breakeven number actually jumped to 79%. Yeah, it's just, so for us breakeven is exactly meeting all the operational expenses of the store to the last thing. Sometimes there are a few stores which fall short by 500 or go over the 500. I won't stress too much about that actual number.

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Arshita Jain: Sure, this will be helpful. Secondly, on your expansion plans, any new states that you are targeting apart from the one already disclosed on your expansion plans in terms of number of stores for FY24 and 25?

Madhukar R. Gangadi: Yeah, we are actually going to be looking at Kerala, Chhattisgarh and Madhya Pradesh. We'll be seeding stores in Indore, Raipur and Cochin actually, this will either happen this quarter, will probably end up opening some stores this quarter or definitely in the next quarter after that. This year, definitely we will be looking at least three new states to kind of start operations. Take the store count starting with maybe 20 or 30 to 50 or 60 later and get, as you would say, our feet wet in the water out there. Once we are comfortable, then we'll start expanding. That's the plan.

Arshita Jain: Your expansion plans in terms of number of stores remains intact. 1000 stores for 24 and 25?

Madhukar R. Gangadi: More or less. We will have at least 2000 stores, I would say. Yeah fall in line with our current numbers.

Arshita Jain: Okay. Lastly on your discounts, while your blended discounts have remained the same, is there any changes in the discount buckets in any of the regions or the ranges are same, order value?

Madhukar R. Gangadi: The model is the same. We have not seen any big difference in the above thousand or below thousand kind of bill sizes, number of bills. It is more between 16.5 to 17 kind of percentage, but not a significant increase. We have not changed the discount bucket.

Arshita Jain: Thank you so much and congratulations on the good set.

Moderator: Thank you. The next question is from the line of Avnish Khara from VT Capital. Please go ahead.

Avnish Khara: Thank you for taking my question. If I look at the contribution from lower to a city over the past six to eight quarters it's been slowly inching up. What I wanted to understand is, are there any difference in the store economics in the metro cities versus the lower tiers and you are also saying that number is continuously going to be go up. I also wanted to gauge what is the psychological consumers in lower tiers versus metro cities and what our strategy is for those regions.

Madhukar R. Gangadi: If I understood you right, you're asking if there's any particular reason for us to actually go to Tier-2 and Tier 3 right?

Avnish Khara: Correct.

Madhukar R. Gangadi: We have been in the seven states at least in three or four states for quite a while. We're fully established in AP, Telangana, Karnataka, Tamil Nadu and West Bengal where we have our main warehouses in the capital cities of these states. I would

not say we are saturated, but we
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have a fairly good presence in each of these cities. Hyderabad, Bangalore and Chennai we have got over 400 stores and Calcutta, we are now nearing 300. To sweat the asset out there, which is our warehouse, and the supply chain out there, it is right for us to actually go deeper into Tier-2, tier 3 and tier 4, which is what we're doing, because the setting up of warehouse here cost once that is amortized, then you would want to basically do as much as possible in the state and that's the reason why we are going deeper into the Tier-2 and tier 3. Another reason is, we started the Company when we said that there's a lot of fake drugs in the market. 30% of all fake drugs in the market are actually fake is what I had read in a WHO report when we actually started the Company. We think the value proposition which MedPlus has of providing genuine medicines and also of providing a great price driven by our scale of course, is even more that value proposition is even more relevant in a small town where there are a lot of small operators, they don't give as much discounts and there certainly I would say is more prevalence of non-standard or non-quality kind of drugs. On both our main mission of actually doing this to the value proposition resonating with the local public and to making the best use of our asset, I think we are best set we would be a best served by putting in as many stores as possible cater to Tier-2 and tier 3. Now looking from the economics perspective, absolutely while the top line is slightly lower than a metro city in a small town, given that the rents and staff costs and the private label mix are all favorable to us in a small town, the EBITDA actually in a small town is much better.

Avnish Khara: Right. Also, I just wanted to understand that is it I mean, from your past experience, do you think that in the lower tier cities it's more easy to push a private label product because they might be a little lower price than the branded ones?

Madhukar R. Gangadi: Our private label is not necessarily lower price. It's almost the same price. No, it is easier because a lot of people do come for OTC kind of drugs and a lot of the OTC kind of medication is sold. A lot of the medication that you sell in private label is OTC. No, it is not about the price as much as the type of customer who walks into a store in a smaller town.

Avnish Khara: Right. I will just squeeze in one last one. You had amalgamated MHS Pharma this quarter. So, what exactly happened over there? Could you just throw some light on that?

Sujit Kumar Mahato: I'll explain that. Thank you for asking this question. The amalgamation was part of an old transaction. In FY20, the Company had disclosed a BT arrangement between MHS and through a stock 22.24 sale, the Company had acquired the entire business of MHS. It is now only being formally amalgamated. So, there is net impact, there is a payable in the books of MedPlus of 17.5 crores to MHS, which would be set off against this amalgamation. Net-net at a group level, MedPlus would have 17.5 crores of extra cash.

Avnish Khara: Right. Okay, thank you so much. That's all from my side.

Moderator: Thank you. The next question is from the line of Bino Pathiparampil from Incred Capital. Please go ahead.

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Bino Pathiparampil: Hi, good afternoon and congrats on continued good performance. Just a few questions on the diagnostic side. The revenue or the cash that you get by selling this membership, how is it recognized? Is it right to recognize as revenue or over a period of year or so?

Chetan Dikshit: Hi this is Chetan I'll just take it up. So, here's what we don't do. When we sell a plan and the cash comes into our system, we don't recognize it as revenue. That

is probably the first part, I would like to say. The second question is of specific in nature, but how does it move from the balance sheet into revenue recognition? Well, there are really two buckets to it. There's a 75% bucket and 25% bucket, 25% and 75% bucket. The 25% bucket gets recognized on the first instance of use of the plan. The balance 75% of the bucket is then amortized over the course of one year. You see that the membership plan is a one year plan. It basically gets recognized as deferred revenue.

Bino Pathiparampil: Okay, great. Thank you. Thank you very much.

Moderator: Thank you. The next question is from the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal: Yeah, hi, good evening. Just one question on the discounting, I know it is already discussed but how is the discounting in the last few quarters? Has it improved, has the pricing improved or the discount remains very high? I hear that there has been some month end kind of discount with increases and then it fades away to 20%, if you could just share some thoughts on the competitive landscape that you are seeing?

Madhukar R. Gangadi: Yeah, for us, of course, it's a standard discount which we operate through the year. If we change it, then we change it for good out there in that state, as we have a 10% discount for all bills under 1,000 and 20% above 1,000. For us, it doesn't really change much. The only way the overall discount can change for us is if a higher portion of people are buying for bill sizes above 1,000. It more or less remains the same for us across all the states. In some states, of course, in West Bengal it's slightly higher, whereas in Telangana it is slightly lower. The bill sizes are slightly lower but on the competition side, we see that competition is in the game of, I would say, offering a discount, which is not really viable, which is 25% discount in the initial stage, which means they can only offer it for a certain amount of time or for a certain number of times. Either they go around saying that okay, we'll give it to you on the first three bills, or sometimes they come in and do a flash sale where they say, okay, we'll do it in the first year or second year or third year, which is all very great. Our only problem is the amount of money required to communicate this kind of message is actually more expensive than the discount itself and it often gets misused most of the time. We don't see much benefit I would say, of doing that. Ours is more like an everyday low price. We continually make the offer better and better for the customers. But we have been kind of steady at the same number for a while now.

Prakash Agarwal: What I understand is you're saying competition still is at 25%. There is not much of a breather there. Despite that, you are growing (+25%).

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Madhukar R. Gangadi: The question is, is the competition still doing 25%? Yes, absolutely. In fact, sometimes even I myself get text messages of various competitors saying that they are willing to offer 30% once in a while. So, yeah, I don't think we have really let up on that.

Prakash Agarwal: Okay. Some color on the growth outlook and margin outlook, because this is the first time where your store addition has been a little softer, which in the right spirit, has also helped in improving some EBITDA and EBITDA. Would this strategy be followed and would it be at the expense of growth or what is the right balance you want to follow from the next few quarters and years?

Madhukar R. Gangadi: So, Prakash, if you look at our last quarter was around 348, this quarter was around 248. Once in a while these numbers go up and down. The plan is to grow a grade of anywhere between 250 to 300 stores per quarter, could go up or down a little bit. A lot of times it is, this quarter I think were affected by

little bit by the rate at which the licenses came in. Because of the front

28:12, they did not come in time in some states. So, there's a process of inspection. Sometimes the inspection itself takes a while and then the license is issued. So, sometimes it could take a while that's the only reason. We are not calibrating that right now. We plan to remain steady between the 1000 and 1200 kind of number for the year. That's going to be the plan for next year. On the revenue side, I think we can basically say anywhere from 20% to 25% is what we're thinking would be the growth will be our revenue estimate for the next year growth over this year.

Prakash Agarwal: Okay, fair enough. Lastly, in terms of using cash, is there a thought of buying out some of the smaller chains? Are there actually some in the market? If you could give some color there.

Madhukar R. Gangadi: We come across some chains, but we have never found anyone which is priced right honestly, given that we are able to open mostly at the rate of around 100 stores a month, and given that the next largest change is only around 300 stores, we really don't see too much benefit of paying a big premium for that. We have kind of shied away from that given that our ability to open source is pretty good.

Prakash Agarwal: Okay, and one more question, if I may, ask on the diagnostic side. Earlier thought was to have the pilot and still doing okay, but is there any key take away in terms of next 6-12 months plan of adding more pilot projects itself in Hyderabad and how will you take it forward?

Madhukar R. Gangadi: Okay, so it remains a pilot. It is not going to go beyond Hyderabad. The pilot, of course, was an integrated radiology and a back lap kind of service. So, that will always remain only in Hyderabad for now at least. The plan is to open 12 stores. 12 diagnostic centers, four of which will be large ones with MRI and CT and 8 would be smaller ones, so that continues. We need to see at certain number of subscriptions in activity before we can take column opening in other places but of course, I do not expect us to make that decision in this quarter or next

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Prakash Agarwal: Okay and of the large four ones stores, three are already open of two are already open and then?

Madhukar R. Gangadi: Right now, we have three large centers open and I think we have at least two smaller ones open out there, currently running.

Prakash Agarwal: Okay, so you have five and your plan is to take it 10.

Madhukar R. Gangadi: Yes, the plan is 12 overall four plus eight.

Moderator: Thank you. The next question is from the line of Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji: Sir, my first question is on other expenses, which has remained flat quarter -on-quarter and I believe that the key components of other expenses are packing and forwarding charges, debit card commission charges and electricity charges which are mostly variable in nature. So, what has cost this other expenses run rate to be the same quarter -on-quarter and do you expect it to increase going ahead?

Madhukar R. Gangadi: No, I do not see any reason to think that will actually too much over there and part of it could be variable and not all of it is variable over there for us the moving expenses do not really change if you add let us say, 5 stores or 10 stores in the same territory and all. The credit card charge is of course a function of the number of bills which go over there, not a significant change, it is a small percentage overall. So, I do not see any difference out there, beside.

Sayantan Maji: And my second question is on this store addition, run rate so we continue to maintain a guidance of 1000 to 1200 stores per year so while we are entering new states, do you see enough clusters for growth in our key state of Andhra P

radesh, Telangana, Tamil Nadu,

Karnataka as well where we see sufficient headroom in tier 2, tier 3 towns in these states.

Madhukar R. Gangadi: Yes, see in three of our current states we are very big in the main cities and tier 2 and tier 3, is completely open for us even on those states. In the other states, we are not even big in the tier I, for instance in West Bengal except for Calcutta and even there, we only have slightly under 300 stores compared to the 400 plus stores in Bangalore, Chennai and Hyderabad. I think there is room even for growth in those cities. In almost all of Maharashtra, we are not there at the full prudential even for the larger cities forget about the tier 2, tier 3. So, answer to your question, if there are enough scope of growth in the seven states, absolutely. We are only feeding three new states. Just to make sure, or not to just to make sure. It is just a logical extension; I would say of our overall growth. We are growing in contiguous states. We are leveraging the warehouses in the states before, we just want to add a few, figure out how the market is, get the supply chain all set up and then grow rapidly after that. So, for us, it is not for lack of opportunity in the current states, that is going outside.

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Sayantan Maji: And what would be your key focus districts in Maharashtra apart from Mumbai and Pune?

Madhukar R. Gangadi: For us we look at it as two different regions one is the overall Vidarbha region and then there is Marathwada region and then there is region around Bombay and Pune. So, in the Vidarbha region, we are pretty strong in Nagpur. We are now actually extending rapidly into Amravati, Akola and a bunch of other districts around that and similarly around Nanded we have a bunch of stores coming up. Apart from these two, then there are districts around Pune, Bombay of course being place where, we expect that we will go very cautiously with slightly small stores a month. There is lot of different areas of growth for us in Maharashtra.

Sayantan Maji: So, what was the average store size, I missed that, for this quarter?

Madhukar R. Gangadi: 558.

Sayantan Maji: 558 and basically the declining trend is mainly due to smaller stores coming up in Maharashtra.

Madhukar R. Gangadi: Smaller not just in Maharashtra but also in the smaller town. We do not necessarily need a very large store in a very small town obviously with the town kind of population and lot of times those locations are also not really available when we go into the Tier-2, Tier-3 kind of locations.

Sayantan Maji: And my last question is on diagnostics. Chetan, you mentioned that you guys are witnessing a fastest scale of operation compared to peers. So, what could be the primary reason for that and basically related question is how have we spend so far, I remember that you had allocated Rs.90 crores for this pilot. So, how much have you spent and what would be the spends in the next one or two quarters?

Chetan Dikshit: So, first on the, why our utilization rates are more. The prices are incomparable specially at the high -end radiology machines like MRI and CT. 75% discount, when we say that it is to be comparable diagnostic centers. But if you compare it to the rates for these machines within the hospital which is even more stark. So, the price is rising the utilization in all our three centers right up to midnight and we do have patients coming in even after 9 p.m. So, price and hours are two contributors to high utilization of high -end machine. Next question from your side was on how much have we spent? Our budgeted allocation for the financial year April to March was Rs.85 -90 crores and we are pretty much on that track. I will just check with Sujith if he wants to add anything further on the capex side. No that is it Sayantan, and we are fine, unless you have a more detail

ed follow -up on this question.

Sayantan Maji: So, this Rs.85 -90 crores is only for FY 2023?

Chetan Dikshit: Yes, that was our guidance for the financial year.

Sayantan Maji: Only for FY 2023, so FY 2024 could have additional spending on top of this.

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Chetan Dikshit: As Madhukar said, what we have is an outlay for the project in Hyderabad. So, there really is no material capex that is intended for FY 2024 but within centers we may, for example if we have two Ultrasounds or three Ultrasounds, we may go to a fourth one. So, think about more as regular capex rather than capex which is going to fuel new center or expand the network.

Moderator: Thank you the next question is from the line Kunal Randeria from Nuvama. Please go ahead.

Kunal Randeria: My first question is in the last middle year even a half year, there is (+1000) stores. So, have you, may be because of this, given and size now, have you started to see some procurement benefits?

Madhukar R. Gangadi: See for a while now we have buying directly from the companies while there is still a small set of companies for whom we end up actually purchasing to distributors at a very small check and those companies we are approaching, get it through now and we will end up considering having the 10% in the next two quarters or so. I do not think there is going to be a significant procurement benefit as far as buying from companies is going to be concern. But that said the scale is want to help us benefit on the private label purchases one and also in getting a new products that were otherwise not available to us because we were not able to meet the minimum mode of quantity for purchase of that product. So, those are all coming in with scale more than the purchase benefit. Going forward, as we continue to work with more close limit companies. We expect with sharing of data and we expect with looking at healthy minimum, the new launches and all. We will be able to get some kind of fees out there. We already do get a significant portion on that but we expect to benefit as we move forward. But on the direct purchase I really do not see too much happening.

Kunal Randeria: Then would it be fair to understand that lets say you know may be double within three years, your procurement cost per let say now lot of companies remain the same despite of clear sky, let's say talking of some XYZ or does it get better. May be there is couple of us enjoy discount because of their size.

Madhukar R. Gangadi: I am sorry the question was not very clear, the audio did not come through very well but if I understand it.

Kunal Randeria: Madhukar sir, what I wanted to ask was so may as you increase your sizes on scale will the same company from which you are buying will be offering you slightly higher discounts because of the size.

Madhukar R. Gangadi: No, I do not think anyone has any reason to offer as more because of that, because to be quite honest we do not really influence the overall sales out there. It is the doctors who have to write the prescriptions and companies typically end up spending their marketing money out there but on the supply chain side, when they actually want to launch a product across the country, let's say they want to launch it across 4000 stores. In 7 states, we will be person they will approach and for that we will definitely end up getting slightly better margin and the overall thing. So, in terms of managing expiry. In terms of managing supply chain, in terms of making sure that

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their products are launched across uniformly in all. I think we would be able to do a better job as we go bigger and bigger and we will definitely be able to get a slight benefit from those companies, that is one. Second as you may have noticed from our presentation, today 13.7% of

our overall sales is from private label. Now, here we can definitely understand as the scale continues to grow this is where we continue to benefit from, so when the margins are good

right now on these products. I see no reason why they cannot do better we continue to increase ourselves.

Kunal Randeria: Perfect and my second question, I think one of the comments that you made was some of the discounts given by online clearly unsustainable. So, when I take your point, one of the online players has now may be you stated the intention of handling around 2000 pharmacies across India. So, you believe the discount would worsen from here.

Madhukar R. Gangadi: What are the questions, I know one of them is doing 2000 stores but what are the questions you have?

Kunal Randeria: You believe that discounting could may be from 16.5, do you think it could may be go through 17%-18% or you think this discount level is manageable.

Madhukar R. Gangadi: No for us I do not think so, today we are close to 3700 odd stores as of now or slightly more. It is not as if we have taken the whole market. I do not know how the addition of another 2000 stores is really going to affect the overall market. I do not really expect with them coming in that, rate will suddenly change. I do not think that is going to be the case at all. As it is, I am pretty sure whatever advantages we have on scale. They will also get the same but whatever difficulties or whatever cost they are in running stores. Everyone is going to have that, I do not really see anyone else being able to sustainably beat our prices out there and even if they did for a little while we are not too concerned because as far as we are concerned the tool aspects to it. Every country in the world has at least 2 or 3 or 4 large players like US has Walgreen, Eyegate, CVS along with Walmart and all doing their own pharmacy and all that certain stuff. So, there is no equivalent of a super store do it, there are also a large chain of pharmacies and most of these countries will require more than one as we have been saying over the last several quarters. It is as Apollo right now, I am pretty sure Netmeds and Tata 1mg and a couple of other guys may come in, that will be at least a fourth player kind of thing, if not for some more super stores also coming in and doing this. So, we fully expect they will come, I do not think the market will completely be upset just because one or two players have entered out there. I do not think it will push us to give more discounts immediately. For us, given that what we are looking at in the Indian market is a shift from unorganized to organized and given that organized is so small, even if one or two new players are going to enter. There is enough market share for us to gain from the unorganized side and that is what we will be focusing on. So, as long as we are ahead of the other 85% of the market which is there in the hands of non-corporate dealers. I do not see any problem to continue to actually expand the way we wanted to. So, short answer to the Question, Do you want to expand the discount I do not think so.

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Kunal Randeria: And just one last one, you have around Rs.320 crores of cash lying on your books, we would have negative free cash of Rs.73 crores in the quarter. So, to fund your expansion you think this is enough or maybe you need to go for funding at some stage?

Management: So, on the cash balance and cash in bank, the store expansion would continue in the same manner and we would use this cash only for our store expansion the way as explained by Madhukar earlier.

Kunal Randeria: Do you think this is enough or you would need to raise some more because you are still investing diagnostics right?

Madhukar R. Gangadi: Yes, as on date, as you are aware, we are debt free, so we have a good headroom if required. So, I t

hink as of now, there is no plan to take up any debt on the balance sheet but as and when required we will definitely consider.

Moderator: Thank you. The next question is from the line of Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee: Sir, on the growth that you report for mature stores just above 10% is that something which is a sort of a line with your expectation or slower than your expectation and are there any geographical differences that in certain geographies this is faster than the other geographies or Tier-1 versus smaller cities. Any color you can give on the mature store growth rate.

Madhukar R. Gangadi: No, it is actually in line with what we actually thought. They had been slightly lower than what we thought at the beginning of the year. Last year, the competitive intensity has been fairly high, but other than that now it has kind of become stable. For us, we expect to grow at a rate of inflation on the old stores and we also expect to take some share from the local mono top retailers over there as we continue to play on the value proposition which we have for our customers, great franchise, great availability, and guaranteed return and medicines. So, no one would say we are completely in line with what we are thinking right now. Is there a significant difference between small and large towns? No, not really, it will be a function of the number of stores we have in that area and that is all there is to it honestly. May be a little bit of cannibalization in some of the larger cities where we set up for the first one month or so. For instance, if we have a store in our neighborhood and we go and open a store which is 500

meters away or 700 meters away. All the people who are coming to that store from that catchment will probably shift to the new store in the first month but after that its kind of sales down. So, nothing which is outside of what we expected.

Saion Mukherjee: And sir, similar question on the private label, on an average numbers are going higher, is again there something, how is the dispersion like across your clusters and towns that you operate.

This 13.7 is there a wide variation that you see across regions in India or it is more or less on average?

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Madhukar R. Gangadi: Smaller towns definitely has slightly more private label penetration out there. So, the nature of kind of customers who walk-in and the value proposition of our private label non-pharma products resonates more sometimes with the smaller thought by. So, it could possibly have slightly higher ones out there. The range though is most of the stores tend to be between, little bit to say, very few stores have less than 8% -8.5% overall private label. There are some stores which have as high as 20% but the average falls in the 12% -14% kind of in a month.

Saion Mukherjee: And sir, you had guided for this number going up 100 basis point every year. So, you continue to maintain that?

Madhukar R. Gangadi: On the overall, I do not really see any problem at all, 100 basis point should be easy. On the pharmacy side, we will be able to go ahead and give you guidance, may be in the next quarter or so. On the pharma, private label in the whole 13.7% there is a composition of devices, FMCG non-medical products and medicines.

Moderator: Thank you. The next question is from the line of Utkarsh Maheshwari from Reliance General Insurance. Please go ahead.

Utkarsh Maheshwari: Sir, just a housekeeping question, Is there a possibility where you can share the average sales per day for the stores which are like more than two years in operation and which are like less than two year operation?

Madhukar R. Gangadi: Right now, I think we gave you as much data as we can, It is little bit more tougher, for us to share anything out there. So, I believe we are in line with the other retail operators

right now.

We are trying to stick to the formats which are currently in prevalence across the Indian retail markets.

Utkarsh Maheshwari: And just to reconfirm one thing, as of now I think the diagnostic is that Rs.90 crores odd which we have planned, right? nothing beyond that has been thought about, right? Because that was a pilot for the Hyderabad.

Madhukar R. Gangadi: Absolutely, no further capex outside of the routine maintenance kind of stuff is plan for the integrated cath and radiology kind of centers in Hyderabad in the pilot as we started it.

Utkarsh Maheshwari: Sir, I think the run rate of adding the new people has actually gone little bit slow I think, we had to plan for 100K plus, right? as the plan or something of that sort and we added something 25,000 -30,000 in the first month itself and now we are at 67 -70 odd. So, is the equation slow in this particular thing? The new membership additions.

Madhukar R. Gangadi: Not really, so we expected to get all our 12 or if these majority of our 12 stores are up and running, that has taken slightly longer than what we expected probably we delayed by a month or so. I do not see any big difference other than that.

Utkarsh Maheshwari: What is the target for us in terms of number of members we look to enroll.

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Madhukar R. Gangadi: The first number which we are kind of looking at is 100,000 members then after that we will see, depending on the pace of expansion of that number from 100,000 to 200,000 we will take a call on whether we plan to go to other states and all.

Moderator: Thank you. The next question is from the line of Amit Kadam from Canara Robeco Mutual Fund. Please go ahead.

Amit Kadam: I have two questions. Just in the initial commentary on the gross margins, sir, Madhukar sir, what is that particular commentary narrative that why gross margin improved sequentially was it a lower discount or mix?

Madhukar R. Gangadi: Sorry, is the question, why is the number sequentially going up gross.

Amit Kadam: Yes, gross margins improved sequentially so the reason for it?

Madhukar R. Gangadi: It is the private label mix; we have not really reduced the overall discount. The discount is made and if anything, it has gone up slightly I would by a fraction of there but it is mainly the mix of products and maybe there is possibly a slight betterment in the procurement side but yes it has not to do with the redemption of discount.

Amit Kadam: Because private label what I see, was 13.9 in Q2 FY 2023 which is 13.7 % this time. So, hence I thought that sequentially that cannot be the reason for that particular 50 basis point of

improvement when the mix or the private label is down 20 basis point sequentially.

Madhukar R. Gangadi: Private label is also a broad basket of products out there with margins ranging anywhere from 30% to 90% -95%. So, it is not a 1:1 mapping of 1% being exactly that, it depends on exactly what we sold in private label that is one. Second, as I said, there could also be a small improvement in the procurement side and third, the other thing was that we also had a COVID loss earlier which is no longer there this time and that will also basically increase the overall margins slightly.

Amit Kadam: But sir, sequentially that will not be the third one, will not be the reason. Sequentially, in the quarter 2 to quarter 3.

Madhukar R. Gangadi: Yes, we took a loss on inventory which was already there bought during COVID and not sold. There is some small inventory that happened in the last quarter and that could be one difference between Q2 and Q3.

Chetan Dikshit: Hi, this Chetan, I just thought I would add to Madhukar's question, if you look at slide no.9 of investor presentation. It gives the gross margins split between the consolidated business and pharmacy. So, in pharmacy from Q2 to Q3. It has only

been a 0.3 increase in gross margin and

that is in line with the gradual increase in private label that we had seen. I just thought that I would add that one.

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Amit Kadam: Okay, and just because I wanted to just check this aspect of the business also like one of the participants also did ask is that this growth in size does that economy of scale or size really starts benefiting us in terms of better procurement and since helping us in terms of margins in terms of gross.

Madhukar R. Gangadi: Sorry, was there a question?

Amit Kadam: No, I will move to the second question, on the SSG where you mentioned that it is 10%. I just wanted to know the difference between price and volume so what would be the inflation part in that SSG?

Madhukar R. Gangadi: It is difficult to split the exact number between price and inflation. Inflation for pharma is typically around 4%.

Amit Kadam: But this year, Madhukar sir, because WPI had been on a higher end where both NLEM and non-NLEM got a good price hike for this particular year hence they were all upwards away odd plus hence I was just checking specifically for this particular quarter what was the price and volume.

Madhukar R. Gangadi: No for us, difficult to split between volume and inflation.

Moderator: Thank you. The next question is from the line of Pathanjali Srinivasan from Mirabilis Investment Trust. Please go ahead.

Pathanjali Srinivasan: Sir, how do we make customer choose our own brand with respect to private label and you had also mentioned that we are not necessarily lower the price. So, how does the customer choose from our brand over a branded pharma?

Madhukar R. Gangadi: See as you are well aware in the Indian pharmaceutical industry there are 100s of 1000s of marketing companies which are selling the same products. Every single molecule which we sell has got over a few thousand different marketing company selling the product. The result is this, despite the best efforts on our side, to stock the right product in the right store to make the exact demand with our supply. We often find that out of our prescription which has got 7 or 8 products, we find ourselves not having one or two of them. In the past we would basically this is a product which you have asked for and this product is from a company like Cipla or Ranbaxy or whatever. I do not have it right now and I will be able to give you something from Sun Pharma or someone like that, so we would that they would just give them substitute over there. But over a period of time what we have started doing is basically offering a substitute of the unavailable product with our own product. Customers because they believe us they are able to actually just pick it up and go and that is the one place where private label comes into play and that is why we are not really looking at doing a price substitution. We are not saying that take something else from us, it is actually a lesser cost. Basically, just saying this is not available instead this is something else that is available, that is one of course. And second quite a significant number of people basically come in for pure generic standard, basically come in

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asking for a headache or cough or cold kind of medication with an OTC products and where if the customer has not asked for Crocin or Calpol. He will also get paracetamol from our company. So, in neither case do we basically use price to sell more of our products.

Pathanjali Srinivasan: So, what would be the mix difference in our private label between chronic and OTG?

Madhukar R. Gangadi: Chronic and this one will be 50:50 as of now.

Pathanjali Srinivasan: And sir did you mention some sales growth guidance during the call from 30% -35% or something for the next year?

Madhukar R. Gangadi: What were you saying?

aying?

Pathanjali Srinivasan: Did you give any sale guidance during this call like I could not hear you properly sometime during the call.

Madhukar R. Gangadi: Our overall sales for the next year could be anywhere from 20% -25% over the current affairs.

Pathanjali Srinivasan: 20%-25% okay and what is the store mix Tier-wise?

Madhukar R. Gangadi: Tier wise? Okay let me just pull it up and then I will just give the number. It is there in our presentation.

Pathanjali Srinivasan: Mix is there, I wanted the store mix Tier-wise.

Madhukar R. Gangadi: I think 32% of our stores are in Tier-2 and Tier-3 rest are metro and Tier 1

Pathanjali Srinivasan: 32% are in Tier-2 and Tier-3 and rest are in metro.

Madhukar R. Gangadi: If you are asking about the location of the stores, right?

Pathanjali Srinivasan: Yes, that is right.

Madhukar R. Gangadi: So, non-metro would be 44% versus metro 56%.

Moderator: Thank you. Ladies and gentlemen due to time constraints that was the last question for today. I now hand it over to the management for closing comments.

Madhukar R. Gangadi: Thank you ladies and gentlemen, I thank all participants in this call for your interest in MedPlus journey. Our Investor Relation team can be contacted at ir@medplusindia.com Thank you.

Moderator: Thank you members of the management. Ladies and gentlemen on behalf of MedPlus Health Services that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: May 2023

(no extractable text — likely a scanned image PDF)

Call: Aug 2023

(no extractable text — likely a scanned image PDF)

Call: Nov 2023

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040-6724 6724

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November 10 , 2023

The Listing Department
BSE Limited The Listing Department
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Phiroze Jeejeebhoy Towers Exchange Plaza , 5th Floor
Dalal Street, Fort, Plot No. C/1, G Block,
Mumbai – 400001 Bandra - Kurla Complex
Scrip code: 543427 Bandra (East), Mumbai - 400051
Symbol: MEDPLUS
Dear Sir/ Madam,

Sub: Submission of earning call Transcripts for the quarter & half year ended September 30, 2023 .

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 , we enclose herewith transcript of the earnings calls held on November 8, 2023 for the quarter & half year ended September 30, 2023.
Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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SRIVASTAVADigitally signed by
MANOJ KUMAR SRIVASTAVA
Page 1 of 19MedPlus Health Services Limited Q2 FY'24 Earnings
Conference Call
November 08, 2023
MANAGEMENT : MR.MADHUKAR REDDY GANGADI MANAGING
DIRECTOR &CHIEF EXECUTIVE OFFICER,MEDPLUS
HEALTH SERVICES LIMITED.
MR.SUJIT MAHATO CHIEF FINANCIAL OFFICER,
MEDPLUS HEALTH SERVICES LIMITED.
MR.CHETAN DIKSHIT CHIEF STRATEGY OFFICER,
MEDPLUS HEALTH SERVICES LIMITED.
MR.PRASAD REDDY MEDPLUS HEALTH SERVICES
LIMITED.

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Moderator: Ladies and gentlemen, good evening and a warm welcome to the MedPlus Health Services Limited Q2FY24Earnings Conference Call.

As a reminder, all participant lines will be in the listen only mode. There will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing *then 0on your touchtone phone.
Please note that this conference is being recorded.

I now hand the conference over to Mr. Prasad Reddy from MedPlus. Thank you. And over to you, sir.

Prasad Reddy : Good evening everyone.

the MedPlus Q2 FY24 Earnings Conference Call to discuss the financial results of MedPlus for

the Second Quarter of Financial Year 24 which were announced earlier today. We have with us today the Senior Management team, represented by Mr. Madhukar Reddy Gangadi Chief Executive Officer and Managing Director; Mr. Sujit Mahato CFO and Mr. Chetan Dikshit CSO.

Before we begin, I would like to mention that

may be forward looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties are on Slide #1 of the Investor Presentation shared with all of you earlier. Documents relating to our financial performance have been circulated earlier and these have also been posted on our corporate website.

I will now handover the call to Mr. Madhukar. Thank you. And over to you Madhukar.

MadhukarReddy : Today I would like to talk to you about the four main features of the Indian Pharmacy retail industry as we see it and the opportunities these features actually present to us.

i) The industry itself is extremely fragmented.

ii) It is a big resilient, small and medium sized set of operators out there who still have nearly 85% of the overall market.

iii) The other factor would be the active involvement of government in promoting low cost generics.

iv) Marked absence of store brands in the country, you know like in U.S. and UK and all where Walgreens and CVS and all have their own store brands.

Just to come back to the main points out here, the Indian pharma retail industry as I said has over 900,000 retailers serving a population of 1.4 billion people. To put that into perspective Indian market has roughly 20 times as many pharmacies as in U.S. while our population is only four times that of U.S. The U.S. market has roughly 45,000 to 48,000 stores catering to their over 350,000 million people.

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So, 900,000 retailers for a market that is less than 200,000 crores makes the average per store any normal business standard

should be considered extremely low throughput and as a result unviable actually. The reason it still works in India is because of the prevalence of high margin trade generics. Some of the lower throughput stores have a 50% or more share of trade generics where often the margins are above 70%. This makes even a store that sells Rs. 20 lakhs per year viable with an average margin of 50% which takes care of the fixed cost and makes a salary level wage for the operator.

Then if trade generics is how the small operators survive the reason for the resilience of the mid to large size independent retailers is not really much different. As you all know the retailer and distributor margins in India are 20 and 10 for drugs not under DPCO and only 16 and 8 for those under price control.

enjoys both retail and distributor margins actually get a margin of around 20% or 21%. Despite this we see them competing with us and with the other large key pharmacy guys with a 20% discount. We have seen this happening in most South Indian markets, and we have seen them actually mushrooming, some of these stores.

What we have seen and what we have actually observed when we have shopped in these stores and what we have from industry knowledge is that most

of these operators, a large number of

them at least do substitution for the prescription which they get with trade generics. So, an average bill of Rs. 1500 if 20% of it is substituted for a trade generic product, that they would still make Rs. 150 at least on that. The trade generic of Rs. 300 basically fetching them at least Rs. 150, 50% margin, which still makes an overall 10%, and which is how most people today seem to be competing.

So, in a super competitive scenario where online players are willing to lose money, and whereas large retailers like us are using our buying power to actually give, choose discounts we actually see a lot of the small operators still continuing to give the same discount and still continuing to actually compete with us.

While the first two factors depend on selling trade generics at full price the other third big factor on the list has the potential of changing the industry in the opposite direction: selling generics at hugely discounted price. Government has introduced Jan Aushadhi Stores a while back and the fact that they are even selling anything despite them not being located in the best places and not carrying the full assortment is proof of the fact that the Indian consumer has started realizing that 80% of the drugs sold in the market are only variations of a limited set of actual formulations.

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While the effect of these stores on the sales of regular pharmacy retail is not big as of now, if things were not to change then the effect will actually continue to grow should not be in any

doubt. The recent NMC notification, although it was withdrawn after a short while asking all doctors to write only the generic name in the prescription is one more step in the direction of a generics market. This we can be sure will come in some form or fashion back to take the branded generics out.

Another key feature or lack of, I would say, in the Indian Pharma retail market is the absence of store brands. In U.S. for example customers get to buy either the innovator product and if the innovator product is not longer on patent or if it comes off patent, the store brand. So, customers could still continue to buy the innovator product even if it is off patent for a slightly lesser cost. Or they would buy the store brand which would either be by Walgreens, Rite Aid or CVS there is nothing in-between.

Indian market on the other hand does not have this, largely because till now there was no large chain that had a trusted brand that customers could buy. Or no large chain that had the scale to get a full assortment of drugs manufacturing. You require a certain number of stores to be able to get the MOQ manufactured for, the large set of drugs. And you could perhaps get away with maybe the Top 50 or Top 100 but to actually get the Top 500 drugs or the Top 800 it would require substantial scale and till now there was no one actually there in the Indian market.

This has led to a proliferation of a large number of me-too branded generics resulting in pharmacies being forced to carry several thousand SKUs to cover the prescription while essentially stocking a few hundred actual therapeutic types only. So, all the above-mentioned four resulted in us taking our next step in our path to becoming the largest pharma retailer.

End June 2023 MedPlus launched over 400 drugs in Hyderabad and these drugs which could potentially provide substitutes for over 55% of the medicine needs of our customers. We launched them with a discount of 50% to 80% on the MRP and the MRP Maximum Retail Price itself was tagged to the price of the marked leader for the corresponding drug composition. We were able to do this because we obviously don't have any marketing cost, that we own the channel, and we are able to pass on all the sales to the customer. And we were also able to convince the customer of the value proposition. Our aim in doing so was to

take advantage of

the fact that the Indian market has matured and has become knowledgeable of the fact that 80% of the drug sold in the market are off patent and their generic substitutes which work just as well as the innovator product as long as they are manufactured in a good factory.

This is also aimed at bringing in customers who otherwise shop at Jan Aushadhi Stores and their like. And our aim was not just to get these customers but also anyone who understood the fact that all the drugs are generic in nature, but who was looking for slightly better, I would say better quality manufacturing than what was available in the market. So, to these customers also, our appeal actually worked well.

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MedPlus making MedPlus Store brand available at 50% to 80% below MRP also makes it difficult for small stores to sell at full price trade generics. And if they are not able to sell it, as I said, stores which otherwise sell Rs. 20 lakhs a year or Rs. 1.5 lakhs a month or Rs. 1.6 lakhs a month, if there access to the margin which is afforded by trade generics is taken away they actually become unviable and we expect that will be one of the ways in which the market will consolidate as small operators basically stop doing this.

MedPlus also in future, plans to make trade generics from companies like Cipla, Alchem, Zydus and the likes available at the same price as MedPlus Store brands. This will now make it tougher for stores that are substituting promoted brands with trade generics to compete with us and to offer the 20% discount which we actually give on branded products.

This approach not only positions MedPlus as a cost-effective choice, but also enables the company to capture a significant portion of the market that was previously dominated by at least was populated by I would say a large number of smaller, independent players. Thanks to our comprehensive control over the entire supply chain MedPlus can strategically adjust pricing and discounts, thereby optimizing profitability. This grants the company a competitive advantage in effectively managing its profit margins in achieve a favorable financial position in the market. Additionally, as more and more customers take to our products, we expect our inventory cost to come down significantly. As the cost of stocking MedPlus brands is only a 5th of the cost of stocking branded generics. We also expect fill rates to get much better in our stores for customers who have adopted the MedPlus brand. Needless to say, it is much more easier to stock 500 to 600 products in sufficient quantities versus stocking 47,000 SKUs that we now stock across our network.

MedPlus products were introduced in Hyderabad and the rest of Telangana at the end of June 2023 followed by their launch in Andhra Pradesh in 2023. Notably witnessed a substantial 15% increase in September compared to June, a growth 3 times higher than that observed in Bangalore and Chennai. Out of the 15% portion a large portion 9% has been contributed by the MedPlus Store brands. Furthermore, the regions outside of Hyderabad

and Telangana and Andhra Pradesh have experienced remarkable growth than compared to similar territories in Karnataka and Tamil Nadu.

We are happy with the way the customers have taken to our brands and we have now launched them across all the states in October.

and meanwhile I will now hand it over to Mr. Sujit to give our quarterly updates.

Sujit Mahato : Good evening, everyone. As of September 30th, we have been serving the Healthcare and Household needs of our community in 599 cities across 9 states through our extensive network of 4089 pharmacy stores. During the current quarter, we have successfully expanded our MedPlus Health Services Limited

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presence to 18 additional cities. In addition to our pharmacy operations in Hyderabad, MedPlus operates four full service diagnostic centers, se

ven Level-II centers and 120 collection centers.

These facilities play a crucial role in our commitment to providing affordable diagnostic service to our customers.

An update on the network, our store expansion program remains on track as we continue to balance growth and profitability. Over the past 12 months, we have added a net total of 761 stores to which 139 stores opened in Q2. Notably West Bengal and Karnataka saw the highest number of store addition with 32 and 26 outlets respectively.

Of the store openings in Q2 48% were in Tier-II cities and beyond, reflecting our strategic focus on these markets. Currently out of our 4089 stores around 1819 stores reflecting 44% are located in Tier-II cities and beyond. We recognize the potential of these markets and aim to further expand due to the maturity of our operations and robust supply chain capability. During Q2 we experienced 25 store closures. Considering both openings and closures we achieved a net addition of 114 stores in Q2 compared to 153 stores in Q1.

In terms of the age of our store network approximately 21% of our store are less than a year old. Around 26% of our stores are in their second year of operation and the remaining 53% of our less than two years age bracket are still in their ramp up stage. From a financial perspective they continue to have a negative impact on our operating EBITDA. However, as these stores mature we anticipate them contributing positively to our profitability.

We closely monitor the time it takes for our new stores to breakeven. For stores open between October 2022 and March 2023 approximately 54% of them achieved breakeven within six months of operations. Additionally, as a cohort all the stores combined achieved breakeven in just five months.

In terms of the network store size, as at the end of the quarter, our network has grown to 4089 stores. We are 2.1 million plus square feet compared to 3320 stores and 1.8 million square feet at the end of September 2022. The average store size is 538 square feet. To give you a sense of spread in store sizes, we have 2958 stores that are less than 600 square feet and 1131 stores that are greater than 600 square feet.

In terms of the revenue mix with our expanded scale, we have strategically positioned to enhance our revenue share from private label products. Our private label range is designed to offer customers high quality product at affordable prices. Currently MedPlus offers around 1400 thoughtfully curated SKUs spanning across pharmaceutical and non-pharmaceutical category. Private label sales accounted for around 14% of our total revenue. Furthermore, our expanding presence in Tier-II cities and beyond is making a significant impact on our revenue mix. Sales from these markets accounted for 34% of our pharmacy revenues in the current quarter demonstrating an increase from 31% in the same period last year.

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Financial numbers -

Consolidated Revenues

Our Consolidated revenue was Rs. 14,086 million with growth of 25.7% year-on-year and 9.7% sequential basis quarter-on-quarter. Our consolidated operating EBITDA stood at Rs. 410 million representing 2.9%.

Pharmacy

Around 99% of our revenues is from our pharmacy operations. The pharmacy operating like to update on our store older than 12 months, revenues from these stores in Quarter 2 was Rs. 12,520 million or 91% of pharmacy revenue. These stores had a store level EBITDA margin of 9%. The store level ROCE of these stores stood at 50.6%, a word here on the store level EBITDA margin by vintage.

While stores greater than 12 months had a margin of 9% this was 9.8% for stores greater than 24 months and 6.5% for stores in the 13 months to 24 months age bracket. If the allocated non-store related cost than the operating EBITDA of stores greater than 12 months would be Rs. 530 million which translates to a margin of 4.2%.

Diagnostics

Our diagnostic numbers,

Diagnostic revenues has grown to Rs. 182 million in Quarter 2 compared to Rs. 58 million in Quarter 2 FY23 which is primarily due to launch of the new centers in Hyderabad. Diagnostic segment recorded an operating EBITDA loss of Rs. 29 million compared to a loss of Rs. 53 million in Q2.

Working Capital

Our networking capital for Q2 was 64 days. The inventory in our warehouse was 37 days. As you are aware because of the sales trajectory of new stores, their inventory turnover is lower in the first year. In Q2 the inventory level of our first year stores was 110 days. In comparison for our stores older than 12 months, the inventory was 41 days.

Now I request Chetan to update on our Diagnostic business. Over to you Chetan.

Chetan Dikshit: Good evening, everyone. To recap what Sujit has already said, in our pilot market of Hyderabad we have four full-service diagnostic centers, eight Level-II centers and 120 plus collection centers. In July 2023 we had increased the price of our plans by Rs. 150. This was a 15% increase. On earlier occasions we had modified prices but at the test level. So, this was our first instance at testing price increase at scale and it has not affected the offtake of our plans. This is positive.

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In July we sold 348 gross plans per day. In August and September this was 381 and 408 respectively. As on 30th June, we had 105,000 active plans and 186,000 underlying lives covered under our plans. As of 30th September we have 117,000 active plans and 207,000 underlying lives. Our current observed on-time renewal rate is 15%, but in 6 months post-expiry we have observed the renewal rate to be 40%.

Moderator : Thank you very much. We will now begin the question and answer session. The first question is from the line of Tanmay Gandhi from Investec. Please go ahead.

Tanmay Gandhi : My first question is on profitability, so if we look at the operating matrices for this quarter, so we have probably the best quarter in last 8 quarters right, in terms of store aging. So, we have only 20% of the stores less than 12 months; in terms of private labels, we are very close to all time high of 14.1% or 14.2%, right. And still in terms of operating EBITDA margins we are nowhere close to the recent peak of 5%. So, what exactly is dragging our EBITDA margins?

Madhukar Reddy : In terms of, in the Investor Presentation we have guided on the second last slide that the drag from the new stores and the stores which were opened in the last 12 to 24 months, they are still a drag on our P&L and on a matured state basis we should see those inching northwards.

Tanmay Gandhi : But even if we look at the EBITDA margins for stores which are more than 12 months old and within that also we have a very favorable mix in terms of stores which are more than two years old. Still, it is only 4.2% versus the 5% which we had seen. And we have a very high contribution coming from gross margins, what exactly is dragging our margins even in case of older stores?

Madhukar Reddy : Tanmay for us the overall maturity is only at the level of 24 months and more, 24 to 30 is when we actually hit the 10% gross margin. And below that then we have the apportion of cost of the

So, there, the mix is not quite there. So, that we definitely need these to come to the level of that Rs. 15 lakh odd sales per month, per store kind of number for it to actually get to that number.

Tanmay Gandhi : No sir actually my question was --

Madhukar Reddy : But we are going in the right direction.

Tanmay Gandhi : --More than two-year-old stores are at 53% which is 5 quarter highs and still we are not 5 quarter high in terms of margins. But anyways I understand. And secondly in terms of store additions, we had initially guided for 1000 plus stores and now I think we are at 800 to 1000 stores, so just wanted to understand that do you want to adjust your guidance given that we have only added a

bout 260 stores in the first two quarters of the year.

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Madhukar Reddy : So, in the beginning of the year we said 800 to 1000, we were looking more at 800 stores. I really think we will actually be able to ramp up in the Q4. We may be somewhere close to that but if there is any need for us to adjust the number,

Tanmay Gandhi : So, have we seen any, are we facing any challenges in terms of store opening, maybe some regulatory challenges or some bandwidth related challenges because in last two quarters it has meaningfully come down. And again, this coincides with our new initiatives of diagnostics, of generic or rather MedPlus branded generic side. So, are there any constraints in terms of

bandwidth or these are just timing issues in terms of regulatory approvals.

Madhukar Reddy :

have been some regulatory issues here and there, those we expect are normal. While we should have basically had something like 400 stores, we are at some 260. But these numbers are usually adjusted towards the end of the overall year anyway. Last four years our Q4 has been the best and so we expect that we will continue to ramp up going forward. Definitely not a bandwidth issue. Yes, we are going into some new states, that is taking us a little bit of time. Kerala, Madhya Pradesh and Chhattisgarh but that is to be expected. I would say whatever adjustment we have made is mainly to make sure that we can just get to the guided number of 800 odd stores, all.

Tanmay Gandhi : Last question from my end, so we have recently obtained an enabling resolution for QIP and as of September 2023, we already have about Rs. 220 crores of cash on books. And anyways our quarterly store additions have come down. So, can you give us some color on where will you deploy such kind of funds? And what is the exact number of fundraise you are looking at in the near term?

Madhukar Reddy : No, as you said this is mainly an enabling resolution only. If we do decide to basically go get the money then it will be only to bolster the overall balance sheet and we will use the money in the best way, but yes, I would say at this point it is just being ready to raise if necessary. And again just to clarify, if at all we end up actually doing a raise then it will be for growing the pharmacy faster, I would say one. And second definitely on the diagnostic side. And of course we will require some money for working capital as we go forward.

Moderator : Thank you. The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed : Can you talk a little bit about the seasonality in the business, the second half versus the first half? When I look at our quarterly margins for the pharmacy retail segment, last year second half was much stronger. So, should we expect a similar trend this year as well?

Madhukar Reddy : So, there is definitely a little bit of seasonality, not sure if extends all the way to the second, third and fourth quarters and all, but first quarter typically we have seen a softening normally during

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the peak summer months and if especially the summer month gets extended then you definitely y because people obviously tend to

too much seasonality on this, Harith. There is always the festival factor which comes in during the extended Dusshera, Diwali periods and all and the vacations period, that is not a time in which people typically tend to go to hospitals or to doctors and all, nothing elective gets done during those days. Doctors themselves are on vacations and all. So, we see a small dip during those times, but otherwise there is not much I would say.

Harith Ahamed : Couple of questions on the MedPlus branded private label that we have launched. So, when I look at the contribution of our revenues this quarter. And it has come d

probably leading to this lower share. But we were also ex

pecting the higher volumes to kind of

offset the impact of discounts, right. So, how should we look at the contribution from private label pharma, should it inch back to the previous level of close to 9% that we saw in the September quarter FY23?

And second one on the same foray would be on our plans to expand beyond Hyderabad and rest of AP, Telangana into our other key markets?

Madhukar Reddy : Obviously the second part of the question answers the first one. We have seen a significant increase in volumes where we have introduced private label on the 50% to 80% discount. In Hyderabad, rest of Telangana and in Andhra where we did it successively, we started in Hyderabad in end of June then after that in July, we did it in rest of Telangana and in September we did it in Andhra. We have actually seen on a GMV basis close to 15% pharma private label in these states. So, if we have not seen the overall thing, it is because Hyderabad and rest of Telangana are a portion of the entire country. But we are now going to the reporting on a GMV basis all our sales, on this overall to show you the effect of the overall private label and all. And you will see a significant increase in these.

Just to give you an idea here Harith, one second, so we introduced roughly around 433 products in the end of June in Hyderabad. Those account for 55% of all the medicine needs which we have, which is you know if we say that 80% of all my sales are medicines and out of that 55% that 44% of overall sales. We had substituted 44% of the medicines which we sold. What we saw at the end of the quarter was that 15% of our sales now are coming from these discounted products. So, which means 15% out of 44%, which is a third of whatever we offered, was coming in from sales of private label products in Hyderabad and rest of Telangana, and slightly higher in rest of Telangana and Andhra actually.

Harith Ahamed : And then our preparedness to go beyond Hyderabad and AP, Telangana with this --?

Madhukar Reddy : So, we have now launched except in Karnataka and Maharashtra where we had a little bit of hurdle but otherwise, we have launched across the rest of the country. And probably we will be

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doing that also in those two states also, shortly but yes. So, as of at least now, as of November 1st at least you can assume that we are there across the country with our private label, in all the states in which we are working, that is, with our private label offering.

Moderator : Thank you. The next question is from the line of Vilina Jain from Perpetuity Ventures. Please go ahead.

Vilina Jain : So, firstly on the pharmacy business, there were 14 store closures for which in the presentation you have mentioned that the reason is others. So, what is basically involved in this?

Madhukar Reddy : So, for us the others are mainly due to various reasons it could have been lease which has expired or we have changed it because of location problems or whatever which have come up in the middle. So, as we can see, the age of the other store is also 5.6, the average age is 5.6, so they are not stores which we just took and made a mistake and had to move, these are stores which where we found better locations to actually move to, so that would largely be the cost. I can get back to you with the exact details after the call though. But the fact that the average age of these stores is 5.5 years should tell you that these were regular stores which we had to move for other reasons.

Vilina Jain : Secondly on the diagnostic business when do we expect the business to breakeven? And what are the target active subscriptions by the end of FY'24?

Chetan Dikshit : Vilina, we want to take advantage of Q3 and Q4, Q4 tends to be slightly strong seasonally. But at this stage we are not giving firm guidance. And on the second part of your question was in terms of what are the active plans goals that we have set, see we

kind of look at the active plans

as well as the active lives the target that we are currently looking at is 250,000 lives, we expect to close the year below 150,000 plans sold, active plans. Again we are not in a position to give guidance at this stage.

Vilina Jain : Any plans to take the diagnostics business to other cities or states?

Chetan Dikshit : Well at this stage we do, see we do have residual legacy diagnostics business in few cities. These are legacy from our earlier phase. What we talk about as the current experiment that is being carried out in Hyderabad, there is no plans to go outside of Hyderabad as of now.

Vilina Jain : Lastly of the active subscriptions that is 117,000 currently, how much of this is renewed and how much of this is new subscription base?

Madhukar Reddy : Of this roughly 20,000 of those which have gotten renewed.

Moderator : Thank you. The next question is from the line of Lokesh Manik from Vallum Capital. Please go ahead.

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Lokesh Manik: My question first was a clarification in the DRHP it is given store payback period is 3 years, maturity is 4 years. In your presentation you are reporting numbers at 2 years plus. The presentation shows stores greater than 12 months are operating at 8.89% that is the margin. So, I am just confused as to what is the exact or common average and what is the maturity period for a store that you consider?

Madhukar Reddy : So, the maturity period continues to be the same which is around 24 to 30 months or so. The payback period is 3 years, is what we say. If we are getting stores more than one year at 8.9%

Lokesh Manik: Yes

Madhukar Reddy : No, . I

Anyway, just

to walk you through that, we expect our stores to breakeven anywhere from three to six months, which is when they have 0% EBITDA. At the one-year timeframe they probably have around 3% to 4%. And at the end of two years, they should have roughly around 7% to 8% or 10% and that could go on up to being a 30-month thing, in which they are around 10% EBITDA at the store level for an average.

Lokesh Manik: My second question was on store closures, what we have ideally seen in the retail industry is the store closures, the age of these stores is usually younger. In our case these are older. And what is happening is that stores with an average age of six years will get closing, means significantly impacting our return matrix given these are maybe 60%, 80% according to the ROC generating higher EBITDA margins stores, greater throughput stores. Given that we are operating on thin margins so what are the steps we have taken to avoid this? As far as my understanding store closure is 5% of the number of the stores, roughly get closed every year. And these are in the higher 5, 6 years age bracket. So, just your view on what we are doing to avoid this?

Madhukar Reddy : So, it is not as high as 5% but we will get back to you on that. The reasons for store closures for us are the average lease for us is around 9 years. So, if we feel or at the 9-year period if you see

that the landlord is not renewing the lease, for various reasons or we are not happy with the location, we could tend to move so there in that case, we will probably move. And those movements which happen because of landlord not renewing or we not being happy with the rent and all, they could significantly increase the overall age of the store closure rate

Second, for us unlike most store, we open at the rate of around 50 to 100 stores a month, sometimes, right. So, what that means for us is --. So, in the way we do it and largely because of the way the economics of the store are structured. We set up a store at roughly around Rs. 25 lakhs to Rs. 30 lakh cost, out of which Rs. 2 lakhs to Rs. 3 lakh goes towards the rental deposit and Rs. 6 lakhs to Rs. 7 lakh goes towards the build of the cost store and rest is all inventory.

So,
fact that we have great availability in

our stores, and we have the best value pricing for the

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customers we are confident that we could actually go into any market in which there is a reasonable density and even if there is competition, we could actually grab business from those guys.

So, hence we go location by location, market by market and set up the store in the best available location at that time. And we constantly look to upgrade the location as our business increases or as new locations open up in the same market. And the reason we do this is because inventory, store deposit and everything, all of those are completely recoverable. 80% to 90% of the cost of our fit-out is modular furniture and the stuff which can be moved from one store to another. So, we actually lose less than Rs. 2 lakhs when we move the location. So, it's a big, let us say a problem if we have to move the store, especially when we come across a location which is better than what we currently have.

Lokesh Manik: So, in the DRHP it is mentioned the attrition rate was about 20% to 30%, DRHP is about 21%, so are we seeing the drop from there now at that attrition level?

Madhukar Reddy : See most of the people are working at minimum wage down anytime soon. We are figuring out different ways of solving this problem, but yes we would be happy to solve it, no, attrition has not come down.

Lokesh Manik: You did because of competitive intensity, or they are opening up their own after getting --?

Madhukar Reddy : Neither, see obviously there is always different kinds of things out there. It is definitely not competition. Some of the people, after coming to Hyderabad to setup or to any other big cities to actually work with us for a few years, may end up going to the small towns to go setup a store. It is neither this nor that, it is

just that, people today definitely are --. Seeing retail is almost like the first step for most people who come in from small towns into big cities. They come in, get some experience and then try to move on to better jobs as they go forward. We also expect that they work with us only for two to three years we did for longer than that.

Lokesh Manik: In diagnostics business, when do you plan to breakeven, if I am not mistaken we have invested over the last few years about Rs. 200 crores in terms of CAPEX --

Madhukar Reddy : No, I don't, we have invested only around Rs. 110 crore odd, you probably are seeing some other lease old things out there as capitalized expense. So, our investment is only Rs. 110 crore. We are now more or less breaking even at all the central level I would say. At the company guidance on the exact time of breaking even. But our sales continue to grow up and our losses continue to go down. And we are seeing a huge uptick by all the, I would say citizens of Hyderabad, we expect that if the value proposition of what we have is really good and we are seeing a really good uptake. So, we feel confident about the prospect of the old business.

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Lokesh Manik: On the strategy you are getting operational leverage through scale, so if I understand correctly a fixed cost is about 2%, so there is not much fixed cost left to create operating leverage. So, then are you then looking at expansion of gross margin because the chain that you are competing with that is a retailing distributor, gross margin level is about 28% to 35%. We are at 21.4% so then is that where you are expecting operating levels to kick in closing to that gap?

Madhukar Reddy : No, I still think operating leverage is possible as we move forward and automate our warehouses and all, that has taken us slightly longer than what we expected, but that will happen. So, today 3% of the cost goes towards warehousing and 2% on corporate. As we grow from Rs. 4,500 crore to whatever number it is this year cost or the warehousing cost will grow in tandem

with that. There is still a lot of room for

leverage out there, I think.

Gross margin, definitely but that is going to come on the back of better adoption by customers

or at least higher penetration of private label among our customers for us. So, private label is where we expect even on the discounted private label model which we have today, we still expect margins to be better as we go forward.

Moderator : Thank you. The next question is from the line of Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee : I wanted to understand the growth rate that you mentioned in the 12 month plus at around 15%. So, for those stores are growing?

Madhukar Reddy : Saion, we are currently not sharing those numbers whatever is there in the presentation is what it is. We expect the older stores to grow slightly slower than this obviously. But given the fact that we now have two levels out there private label in non-pharma products which will continue to gain traction, second, our own private label which is now going to go at that 50% to 80% discount, we expect growth is going to be much higher than inflation, much higher than the cost

Saion Mukherjee : On this experience on generic, Hyderabad you have launched it in June and there is some month of experience. So,

now with the kind of transition, you know is it more than 100, how much more than 100 or less than 100 if you can provide some color on that. And how we should think this number going forward?

Madhukar Reddy : So, I will give you a longish answer for this. I explained the four main kind of features of our Indian industry and all, and I also said how we are going to go and take more top-line. I expect our EBITDA to grow both as a result of higher HSG as we go forward as we take market share from the smaller independent retailers and all (1).

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And 2) also as the private label grows from the current 15% pharma which is in Hyderabad to 20% and above. We have already seen a pretty significant growth in the overall top-line and I expect that to continue. The margin growth though, will take a little while, because although the private label is more profitable than the branded generic, it has also in the first stage replaced the slightly more profitable, full cost private label which we had earlier. So, right now what we have is a MedPlus brand but earlier to that we had a MedPlus subsidiary brand which we used to sell at full price. So, cannibalization of that has resulted in a slight degrowth of I would say of a margin of there, what has been more than made up by the increase of overall sales, and by the increase from 8% to 15%. So, for us is Hyderabad trending at a better EBITDA, definitely, compared to earlier. The exact numbers, I think you will probably be able to see it as we go forward in the next quarter, when we give the results for the full country.

Saion Mukherjee : And finally one question is the plans that you have, is there any scope for acquisition both at the distributor level or at the retail level, is that something which makes sense given the plan for expansion that you have at this point?

Madhukar Reddy : We are always willing to look at acquisitions out there. But as of now we have nothing in mind. We are not really seeing any great targets for acquisition. But I'm pretty sure at some point we will continue to evaluate as they say and if there is a good deal out there on table we will take it.

But as

Saion Mukherjee : But you would pursue, I mean this is mainly because of valuation --

Madhukar Reddy : Absolutely .

Saion Mukherjee : So, the issue is valuation because if the industry is consolidating, I would assume that you would have target which we would like to consolidate with the largest player. And you could then add value. So, I was wondering if something which we can see in the near term.

Madhukar Reddy : See there are couple of things 1) Valuation, definitely, but 2)

also the kind of models, right. If

their business philosophy is completely different and the way they actually go about getting their customers and their value proposition to customers is completely different than it may not really blend very well with our own model. Then the only way in which we could basically run it is have two independent brands supplied by the same backend. And that could be one possible thing, but we have not really seen any I would say immediate opportunity out there. But yes, if we see

for complimentary models we obviously work at, we are a value player and we basically do discounting and everything else but there could be someone out there who caters to a different audience and who is profitable and which can be acquired at a decent price, we could perhaps look at it, at that time.

Moderator : Thank you. The next question is from the line of Madhav Marda from FIL. Please go ahead.

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Madhav Marda : My question basically was that as we see this mix of private labels, new private label which we have launched, the MedPlus Store Brand Generics I think if I picked up right, you are saying

gross margins are better. And we are seeing higher volumes in the stores where we have launched them which means there should be better operating leverage. Is it fair to understand as we go ahead the EBITDA that we make per store should structurally improve because we get better gross margin and operating leverage? I am taking like a slightly longer view if you could think from like a one-or two-year

Madhukar Reddy : 100%, as I said earlier, the reason why we have so many stores out there is because a lot of them depend on trade generics for their viability. Store which does Rs. 5000 and Rs. 10000 per days is selling 30%, 40%, 50% of its overall thing making around 50% to 60% margin. Now that we not a promoted brand, then we are there with a 50% to 80% discount on that. And in Hyderabad and rest of Telangana we are also selling trade generics at the same price. And we are testing out to see how this works. You know if trade generics are available at that number, at 50% to 80% discount that automatically means lot of the smaller stores will find it very difficult to sell it at that full price. And if they tried to sell, at a price which matches with us, the low throughput will not allow them to actually survive. We will see a consolidation, we will see the benefit of that coming to us, that is for sure. And again, as I said the Indian consumer is also getting, I would say more knowledgeable about the overall medicines and all, we are also seeing a bunch of s not a big deal, we know MedPlus, we know all these brands are same, now that you guys have something to offer, we are more than happy to actually take it and save money. So, we are seeing a lot of those people come too. So, between both, between some of the stores not giving up and sending their customers over to us and people switching to our thing, both in terms of top-line and on the margins, I expect we will see a positive outcome as we go forward.

Madhav Marda : So, payback period for store that we introduced obviously just mathematically should come down right, over the period of time as these unit economics of a store increases, our sort of ROIC profile I mean I am just 1+1 but that should happen, I would assume?

Madhukar Reddy : No, long run definitely, mainly because of this, the cost of private label is 1/5th of what it cost for us to buy a brand. So, at the sale of Rs. 18 out of Rs. 30 lakhs basically going into inventory for us. As the inventory cost comes down, as our overall investment in the store comes down definitely the ROCEs will improve. And as we continue right, today we are making let us say after all the discounts which we give and after all the incentives which we give our employees for selling our private label, we still make at least I would say 75% to

a 100% more than what

we make on branded generics after all the discounts.

Now this number where it is can only go up from here, because today the average discount in Hyderabad is roughly around 58% to 59% or even 60% on the MedPlus private label. This need not always be that number, it can easily move anywhere from 60% to 55% or even one lever which we have and that will actually improve our overall margin.

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And the normal inflation which the government allows every year of 10% right on the drug price, that will also, some of it we will pass to the customer, because you know that 10% does get passed on us, 50% to 55%

expect that the price of manufacturing it will go up as much.

Moderator : Thank you. The last question of today is from the line of Sayantan Maji from UBS. Please go ahead.

Sayantan Maji : My question is regarding the private label portfolio. So, if I look at the private label portfolio and considering that there would have been some cannibalization to the earlier trade generics that we were selling. So, is it fair to assume that say if I just look at the 14% portfolio, 14% revenues that are coming from private labels to the gross margins sequentially could have come down and considering that the pharmacy gross margin has increased so in the branded pharma business probably the gross margins would have increased quarter-on-quarter and hence the average discounting levels would have come down? Is that a fair assumption or something is not correct?

Madhukar Reddy : No, so you are saying our margins because of the cannibalization of our earlier higher margin private label will have come down that is right. Earlier in Hyderabad we had roughly around 7% to 7.5% So, we assume that all 7.5% has been

substituted by this, then we would have lost roughly around 2% on that. But the additional 7% which we actually got here would have basically secured another I would say another 1% would have basically been made up by the extra private label which we sold and that would have replaced the regular branded generic. Now what we are also not seeing is the fact that our sales also grew significantly because of this. So, a growth of even 10% extra at a 20% margin more than makes up for all this. And we expect that we will continue to grow as we go forward. From now on, I would say in Hyderabad whatever margins we have lost because of merging both the old and new ones that effect is long gone, so we will continue to grow from here.

In the other states though, in Karnataka and all these other places, we have now, till now kept

these separate so we continue to operate the old private label as it is. We are seeing some cannibalization but not complete so to that extent we will see some loss, but overall since the private label is growing out there we expect to make more money out of that.

Sayantana Maji : Another question related to private label is when you had chosen 400 SKUs which cater to 55% of the customers. So, are these the ones who have probably the sale base were lower earlier or are these top selling SKUs that you see in branded pharma. And the 15% which is as you explained 1/3rd of the addressable market that you were able to convert. So, do these customers had higher awareness or is there some below the line activity which has been done to create awareness which led to this uptick in private labels?

Madhukar Reddy : Two things, everyone is aware because the private labels was sold only to people who had MedPlus membership and MedPlus membership was sold at some price, super discounted price,

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original price which we thought of was Rs. 499 but we ended up selling it for Rs. 49 as initial sell. But they had to actually become a member, pay that money to actually obtain this discount. So, they were told. And in the stores when they came in and we also had some marketing m

aterial

which went outside we also did a little bit of advertisement and all. We talked about only this, that 80% of all the drugs in the country are off patent, all these are just branded generics or generics only. And we basically just said that MedPlus with a 4000-store network and a Rs. 4500 crores as of last year as the size and capability of getting these drugs manufactured in the same places and to the same level of quality as any other larger brands. And given that we did not have any marketing cost and all and we made it very clear that the discount was coming to them ost. And so

customers bought that and a significant portion of them ended up actually buying as I said a 3rd of whatever we put out on offer basically got taken up. And these are fast moving products by the way, these are not products in which the fill rate is a problem or anything like that, these are the, the fastest moving products.

Sayantana Maji : So, that is substitution,

were able to fulfill 8 out of 10 and the 2 was the gap and you available for the customer.

Madhukar Reddy : Exactly yes.

Sayantana Maji : So, your CAPEX if I just do a CAPEX per store it comes to around Rs. 1.4 million which is Rs. 14 lakhs so we have fit out cost of Rs. 6 lakhs, Rs. 2 lakhs to Rs. 3 lakhs of deposit, so where is the remaining Rs. 5 lakh to Rs. 6 lakhs coming from assuming that none of the CAPEX is going into diagnostics for this quarter?

Madhukar Reddy : --; capitalized or something

little bit of a maintenance, we actually did tell the investors last time that we are going to refurbish I would say refit roughly around 1000 stores to accommodate the private label products, non-pharma private label products. which would now start going from being shelved from behind the counter to in front of the counter. So, it is going to be, it should have change it so much. Happy to come back to you with more details on this please reach out to us after this.

Moderator : Thank you. Ladies and gentlemen, that was our last question for today. I now hand the conference back to the management for their closing remarks. Thank you. And over to you.

Madhukar Reddy : Thank you everyone for the call.

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Moderator : Thank you very much. Ladies and gentlemen, on behalf of MedPlus Health Services Limited that concludes this conference. Thank you for joining us today and you may now disconnect your lines.

Call: Feb 2024

Medplus Health Services Limited

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February 7 , 202 4

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Scrip code: 543427 Bandra (East), Mumbai - 400051
Symbol: MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Earning Call Transcripts for the quarter ended December 31, 2023 .

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 , we enclose herewith transcript of the earnings calls held on February 5 , 202 4 for the quarter ended December 31, 2023 .

Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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KUMAR SRIVASTAVADigitally signed
by MANOJ
KUMAR
SRIVASTAVA
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"MedPlus Health Services Limited
Q3 FY'2 4 Earnings Conference Call "
February 05, 202 4

MANAGEMENT : MR. MADHUKAR REDDY GANGADI – CHIEF
EXECUTIVE OFFICER AND MANAGING DIRECTOR –
MEDPLUS HEALTH SERVICES LIMITED
MR. SUJIT MAHATO – CHIEF FINANCIAL OFFICER –
MEDPLUS HEALTH SERVICES LIMITED

MR. CHETAN DIKSHIT – CHIEF STRATEGY OFFICER –
MEDPLUS HEALTH SERVICES LIMITED
MR. PRASAD REDDY – ASSISTANT FINANCIAL
CONTROLLER – MEDPLUS HEALTH SERVICES
LIMITED

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Moderator: Ladies and gentlemen, good evening, and a warm welcome to the MedPlus Health Services Limited Q3 FY '24 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Prasad Reddy from MedPlus. Thank you, and over to you, Mr. Reddy.

Prasad Reddy : Thank you, Michelle. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q3 FY '24 Earnings Conference Call to discuss the financial results of MedPlus for the third quarter of financial year 2024, which were announced on 2nd February 2024.

We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, Chief Executive Officer and Managing Director; Mr. Sujit Mahato, CFO; and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties are on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit, the CFO. Thank you, and over to you, Sujit.

Sujit Mahato : Thank you, Prasad, and good evening, everyone, on this call. We are pleased to share that as of December 31st, we have been serving community in over 600 cities across 10 states, including the 3 new states of Madhya Pradesh, Chhattisgarh and Kerala through our network of 4,233 pharmacy stores. Additionally, in Hyderabad, MedPlus operates 4 full-service diagnostic centers, 8 level 2 centers and over 120 collection centers offering essential diagnostic services at affordable rates.

Let me cover the network in terms of openings and closures of our outlets. As informed during the last quarter, we have taken a thoughtful approach towards store expansion, prioritizing scale-up of MedPlus-branded pharma and non-pharma products and also maintaining a balance between overall growth and profitability.

Over the past 12 months, we have added a net total of 676 stores. On a gross basis, this was 758 stores, with 164 stores opened during Q3 alone. Most of the new stores were opened in Tamil Nadu and Telangana with 55 and 24 stores, respectively.

This strategic focus of growth reflects our commitment to meet the evolving needs of our community while maintaining a focus on sustainability and financial health. We expect the new store openings for the fiscal year 2024 to be around 600 stores.

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In the third quarter, 59% of our store openings were in Tier 2 cities and beyond, underscoring our strategic emphasis on these markets. Presently, out of our 4,233 stores, 1,908 stores, representing 45%, are situated in Tier 2 cities and beyond. We acknowledge the growth potential inherent in these markets.

Throughout Q3, there were 20 store closures. Taking into account both openings and closures, we achieved a net addition of 144 stores during the current quarter, a notable increase compared to the 114 added in the previous Q2 quarter.

In terms of the network age, approximately 18% of our stores are in the first year of operation, around 27% of our stores are in the second year of operation and the remaining 55% of our stores have been operational for 2 years or more. It is noteworthy that all stores in the less than 2 years

are

age bracket are still in their ramp-up phase. From a financial standpoint, they continue to exert a negative impact on our operating EBITDA.

However, as these stores mature, we anticipate a positive contribution to our profitability. We closely monitor the timeframe for our new stores to reach breakeven. For stores opened between January 2023 and June '23, approximately 59% of them achieved breakeven within 6 months of their operations.

As a cohort, all stores combined reach breakeven in just 4 months. In terms of the store size as at the end of the quarter, our network has grown to 4,233 stores, covering 2.2 million square feet compared to 3,557 stores over 1.9 million square feet as of the end of December '22. The average store size was 535 square feet. To give you a sense of spread in store sizes, we had 3,082 stores less than 600 square feet and 1,151 stores that are greater than 600 square feet.

In terms of revenue mix, with our expanded scale, we are strategically positioned to increase our revenue share from our private label products. Our private label range is crafted to provide customers with high-quality products at competitive prices.

Presently, MedPlus offers over 1,000 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical category. Private label sales currently constitute 14.5% of our total net revenue. However, our growing presence in Tier 2 cities and beyond is significantly impacting our revenue mix. Sales from these markets comprise 35% of our pharmacy revenue in the current quarter, marginal increase from 33% in the same period last year.

I'm pleased to provide an update on the impact of the launch of MedPlus-branded products in Telangana state. In quarter 1 FY '24, prior to the launch, the share of private label pharma stood at 6.61% of the total GMV in Telangana. However, following the launch and in quarter 3 FY '24, the gross merchandising value notably increased to 15.03% of the total GMV in Telangana. Following the promising results observed in Telangana, the company initiated the launch of MedPlus branded products in other states starting October '23 onwards. Prior to the introduction of MedPlus-branded pharma products, sales of private label pharma products accounted for 8% of the gross merchandise value in quarter 2 of FY '24.

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However, subsequent to the launch and throughout the reported period, the private label share of GMV surged to 11.2%. The increase in private label GMV indicates the positive reception from customers and validate our commitment to delivering high-quality products under the MedPlus-brand umbrella.

Now coming to the financial numbers. On our quarter performance, our consolidated revenue was INR14,415 million with a growth of 21.1% year-on-year and 2.3% quarter-on-quarter. Our consolidated operating EBITDA stood at INR466 million, representing 3.2%. A round 99% of our revenue is from our pharmacy operations.

The pharmacy operating EBITDA is INR505 million, representing 3.6%. In terms of our store performance, I would like to update on our stores older than 12 months. Revenue from these stores in quarter 3 was INR12,953 million or 91% of pharmacy revenue.

These stores had a store level EBITDA margin of 9.6%. The store level operating ROCE of these stores stood at 52.5%. A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 9.6%, this was 10.1% for stores greater than 24 months and 7.2% for stores in the 13 to 24 months age bracket.

If we allocated the non-store related costs, then the operating EBITDA of stores greater than 12 months to the INR588 million, which translates to a margin of 4.5%. Quick numbers. Diagnostic revenue has grown to INR196 million in Q3 '24 compared to INR97 million in Q3 of FY '23, which is primarily due to launch of the new

centers across Hyderabad.

Diagnostics segment recorded an operating EBITDA loss of INR34 million compared to a loss of INR33 million in quarter 3 of fiscal year '23. Our net working capital for quarter 3 was 66 days. The inventory in our warehouse was 38 days.

As you are aware, because of the same trajectory of new stores, their inventory turnover is lower in the first year. In Q3, the inventory level of our first year store was INR111 million. In comparison for our stores older than 12 months, the inventory was 42 days.

Now I request Chetan to share an update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. As you know, Q3 is a seasonally weak quarter for Diagnostics in great part due to the festival and holiday season. However, we have 8% quarter-on-quarter growth in Q3. In October, we sold 405 gross plans per day. In November and December, this was 369 and 357, respectively.

As on 30th September, we had 117,000 active plans and 207,000 underlying lives. As of 31 December, we had 127,000 active plans and 228,000 underlying lives. Our current observed on-time renewal rate was 19% in Q3 versus 15% in Q2.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question

is from the line of Ashita Jain from Nuvama. Please go ahead.
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Ashita Jain: My first question is on your store opening guidance. I think for this year, your store down to 600. What is the store opening guidance for the next 2 to 3 years or 3 years for FY '25? Can you give color on that?

Madhukar Gangadi : Sure. Thanks, Ashita. So as Sujit said earlier in this call, we have focused primarily on increasing the overall private label, both in pharma and non-pharma. That's going to be our focus for the first 1 or 2 quarters this year also. We've just introduced over 600 products. And even as the product line is expanding, we are also making sure that products are available across the entire network and all.

And we're also ensuring that our employees are fully trained and are able to sell and everything else. So as I said, the primary focus is going to be on this. We will most likely end up doing only around 600 to 700 stores the next year, the coming year, to o. But we will keep you guys updated as soon as our figures are finalized.

Ashita Jain: And secondly , you just spoke about last few quarters about entry into the newer market. What's the update on that?

Madhukar Gangadi: It is still early going for the three states, one is Kerala, Chhattisgarh and Madhya Pradesh, but we are, I would say, pretty encouraged by the response in all these places. We will put up a small -- a very small working kind of warehouse to make sure that we are able to service all our stores properly and all.

But in general, both Madhya Pradesh and Chhattisgarh are doing well. We expect to grow faster than that. There are -- I would say, the growing is a little bit slower in Kerala for us, but we continue to expand in that state also.

Ashita Jain: Any particular reason for this Kerala -- slower uptick in Kerala?

Madhukar Gangadi: So see, a lot of the initial store opening depends on the regulator over there. There are some regulators who are slightly more sticky about every single detail. And in some states, it is not as much. It is a state-wise thing. So it is a slow growing in the beginning stages, I would think. But yes, that's about it is. Otherwise, it's not an issue. The market itself is pretty good in Kerala.

Ashita Jain: Understood. And I think phenomenal job on the private label side, now we're close to 14.4% pharmacy presence. How do you think this number would settle in the next year, given our initiatives on the MedPlus branded labels?

Ma

dhukar Gangadi: It's really difficult to say. We are very encouraged with the way the initial two states have responded with the initial two regions where we started in June have responded. And also under the early listing out there in the other states, it is following the same path. Just on the pharma side alone.

I think given that 80% of our sales comes from pharma right now, and we will be able to hit or at least we'll be able to, I would say, have substitutes for at least 60% of the entire inventory, which is 70% at least of the medicines. I think for us to get to 20% will not be very tough on the pharma side over the next one year.

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Moderator: Thank you. The next question is from the line of on Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee: Sir, on the private level, the MedPlus brand. So I mean what you see at a GMV level, how the ratio has moved? If you get your share at an overall company level like how much it was before, let's say, when you launched in first quarter and now?

Sujit Mahato: Yes, I'll take that, Saion thank you for the question. In quarter two, in terms of the GMV, the private label pharma was 8.96% overall. And end of quarter three, the same percentage on the same basis of GMV reflects at 11.78% If this is helpful.

Saion Mukherjee: So this is for the overall, entire pharmacy...

Sujit Mahato: Overall. Only for the private label pharma.

Saion Mukherjee: Understood. Okay. And the other question I wanted to check with you is that you have provided stores less than 12 months revenue split. How has been the revenue growth for stores, which are, let's say, more than two years old? I mean, is that growth very slow? I mean, is that a right assessment that it's in single digits?

Sujit Mahato : In fact close to the inflation number. If we get all stores put up to FY21 end, it is inflation -plus.

Saion Mukherjee: Okay. But generally, the pharma market growth is around 9%, 10%, right? And if there is price increase that is also there in pharma. So I was wondering why this number is a bit lower? Is there

sort of a de-stabilization with the new store opening? How do you sort of read this number?

Madhukar Gangadi: Yes. So Saion, while definitely there is some price inflation and unfortunately the DPCO rules which kick in once every while, also reduce the prices a little bit. So there's also that impact. For us, I expect that the growth will be slightly over inflation as we go forward. There may be some cannibalization in some existing states, but I don't really expect that to mute the sales any more than it should.

For us, the other main reason could be also this. We are not really -- except for places where we're reporting the private label, we are not taking the GMV route to actually explain the sales growth. So in some of those cases where private label medicines have actually taken off, you would see a slight dip in the top line because of the lower value at which we sell the discounted private label products, the new pharma private label products.

Saion Mukherjee: Right. Sir, is it possible to share the growth numbers on a like-to-like basis at the GMV level? Is that possible to -- like you reported 21% growth, how much would have been the growth at a like-to-like basis?

Madhukar Gangadi: Sujit, do you want to take that?

Chetan Dikshit : Madhukar, I'll take that. Saion, our reporting what we have done is we've benchmarked it to peers. But we'll take your feedback on board. And see if there's more disclosure that we can give going forward.

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Saion Mukherjee: Okay. And finally, are there any changes in the discounting in your normal branded products due to competition and behaviour of online players? Any changes that you have witnessed in the recent past?

Madhukar Gangadi :

We have not increased the discounts anywhere. And if your question was, if the competition is actually coming down, are you taking by decreasing it? No, not as of now.

Saion Mukherjee: Okay. Thank you.

Moderator: Thank you. We'll take the next question from the line of Gaurav Nigam from Tunga Investments. Please go ahead.

Gaurav Nigam: Sir, first question was on, I was looking at your presentation, you used to give this private label number of pharma SKUs. That number from Q2 to Q3 presentation is dropped from almost 1,100 to 783. Just wondering, is this a correct number to look at? And what's the reason if this is correct?

Madhukar Gangadi: We will have to come back to you. I don't think we have really decreased the overall number. If anything, given that we have been continually increasing the pharma private label and also looking at other product, I guess not to decrease the overall number of products. Sujit, do you want to add anything else?

Sujit Mahato: I will come back in a while.

Gaurav Nigam: Understood, sir. Another question was on, can you provide the average level of pharma discounts that was there in Q3? And has it quarter-on-quarter, has it increased or reduced, any sense of direction?

Madhukar Gangadi: Pharma discounts on branded products has not really increased significantly. There may be a little bit of increase in 1 state, West Bengal, where the earlier hurdle was -- after the earlier hurdle we used to give a 10% discount, we moved it to 15%, but that was at least 2 to 3 quarters ago. Otherwise, I see no reason for any increase in discount.

If as a blended this thing it is coming out a slightly higher discount, it could be because of our private label products. Our private label products, the new ones sell at an average of 50%. So if you combine the entire pharma products, then that could increase the discount. But I think if you look at purely the branded ones, there should be no increase or there should be very little increase.

Gaurav Nigam: Understood, sir. And sir, what is the guidance for diagnostics capex in FY'24 and '25, sir?

Madhukar Gangadi: Do you want to take that Sujit or Chetan?

Chetan Dikshit : Yes, sure. I can take it. Apart from maintenance level capex, we don't have anything really planned in terms of big machines or new centers. If there is a change in view, we will definitely update you.

Gaurav Nigam: In FY'25, sir, as well, you're saying?

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Chetan Dikshit : Yes. For the foreseeable horizon, our capex program has been rolled out. In some of the centers, we may have minor capex maybe an additional ultrasound, ambulance. So which is why I cannot

spoke about it like maintenance capex. But there is no plan on the cards for new centers or new cities or new big machines. We will update as and when anything like that is on the cards.

Gaurav Nigam: Understood, sir. And sir, just a last question. Is there any update on the QIP? I think you have taken a shareholder approval. So just wondering if there is anything in the plan going forward.

Chetan Dikshit : Thank you for asking that question. We maintain our stance that what we have taken was shareholder enabling provision just to keep that optionality and that is for a full duration of 12 months. As and when the circumstances require us to raise additional funds, we will then have the option of both looking at equity and debt. As of now, it is an enabling provision, and there is no action expected on it in the near-term.

Gaurav Nigam: Understood, sir. Thank you for answering my question.

Moderator: Thank you. We'll take the next question from the line of Kunal Randeria from Axis Capital.

Please go ahead.

Kunal Randeria: Thanks for taking my question. Madhukar, your private label sales contribution on the pharma side has actually gone down on a year-on-year basis, right? But your GMV has gone

up, I

believe, so does that mean Wynclear MHS had a lot less discount than made by MedPlus brand?

Madhukar Gangadi: Yes. So earlier Wynclear and even a little bit of the trade generics which we sold, were sold at the same price as branded products. Since then, we have decided to basically go with a store generic thing, promote MedPlus brand and give it the full discount. So while a response to this has been very good, there has also been a little bit of cannibalization of our original Wynclear brand. And so in some places, that has come down a little bit. And so overall margin will have come down because of that.

Kunal Randeria: Okay. So do you still sell under Wynclear.

Madhukar Gangadi: Sorry?

Kunal Randeria: Do you still sell under the Wynclear brand or have you stopped it?

Madhukar Gangadi: No, we do. So we will continue that at least in the acute kind of therapies and all. In Telangana, the entire state of Telangana, we have reduced the price of Wynclear to the level of the MedPlus brand. So we sell it at the same price, all the products have been sold at the same price. But in the states in which we have started after this, in the rest of the country in the seven states, Wynclear continues to sell at the old price.

It is some of the chronic -- for some of the chronic therapies, the sales has come down. Most likely it will come down to next to nothing or zero in a while, we'll let that happen over a period of time. But we will continue to actually sell the Wynclear private label at the full price for acute therapies.

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The idea is that for those who are not very price sensitive, those who are not members of the MedPlus discount plan and all, and those who need the -- and those who are seeking early convenience, from them, we will get the full price. Otherwise, anyone else who is price sensitive will get the MedPlus brand at the full discount.

Kunal Randeria: Got it. And just one last one on this. So this is more like your -- it's been a few months since you launched Made by MedPlus. Let's say, as it gets bigger and people recognize the brand, do you plan to reduce discounting slightly, let's say, from 70% to a 60% or something like that?

Madhukar Gangadi: So today, our overall discount is at around 60%. We would say 50% to 80%, but the average falls at 60%. That is something we will see as we go forward, depending on how the market is evolved. Our primary thing right now concerns to grab market share and increase the top line. Once it is fully established, people understand the quality of the product and people are buying MedPlus -- are asking for it by name and then coming and buying it. Then we could always play around either with the top line, either with the MRP price, increase it slightly or perhaps decrease the discount to a slightly lower number. But that is not something which we are going to decide right now.

Kunal Randeria: Got it. Just one more if I can. Have you introduced it across the states or is it just Telangana at the moment?

Madhukar Gangadi: No, it is across all the seven states right now. As I said, only in Telangana, we have killed our old private label. Everything is now available at the new MedPlus private label brand rate. But in the other states, both co-exist.

Kunal Randeria: Got it. Thank you and all the best.

Moderator: The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: I joined the call a bit late, so my apologies if this was asked before. So my first one is on store additions. I see that we are tracking around 400 stores added year-to-date FY'24. So I remember that previously, we had guided for roughly 1,000 stores addition, 800-plus. So what would be the updated number for FY'24? And is there a revision in the guidance for FY'25 and beyond in terms of net additions?

Madh

ukar Gangadi: Sure. The number of stores, the gross additions this year will be close to 600, around the 600 mark, the net additions may be slightly lower. And for the coming year though, the number could be around the same number. The focus this year is going to be on private label. We want to make sure that we get it to the 20% kind of mark on the pharma side. So that's going to be the thing -- both by increasing the range, increasing the awareness and also increasing the education level of our own employees in being able to sell this product.

Harith Ahamed: So based on your experience so far on the MedPlus branded private label side, can you share some of the lessons or challenges that you've encountered in this foray? Is it on the sourcing side or is it in terms of getting subscribers? And if you can comment on the willingness that you're

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seeing among patients to switch from the brand to the private label, so if you could share some of the takeaways on these areas, that would be helpful.

Madhukar Gangadi: Sure. So you have seen a pretty good response to our products in Telangana where we launched it first. The results from this are only a quarter. Actually we started only in the middle of October or late October in the rest of the country. So the numbers are still not there. But in Telangana, we have seen a very good response.

What I think is that the early adopters, people are buying the fact that all the products available in the market are generic only. And MedPlus is able to give a very good price only because it does not have any marketing or distribution costs. So people are getting that. So some of the early adopters have come on board.

We think as we go forward -- but there still remain some questions. I think we have our work cut out in trying to educate the customer. The common question we encountered is this.

Whenever our guys have gone and said 50% to 80% discount on our brands, they have said customers have come back and said is this a generic product?

So we are taking the time to explain that all the products in the country are generic. There's really no difference. It's all the same product anyway. The only way you can differentiate one from the other is on the quality of its make. So we are countering that using several different explanations out there. And we are seeing a lot of people converting once that happens.

So this will happen not just through our stores, but also through media, influencers and all sorts of other testimonials and all. Again, to come back to your question, how is the response? Pretty good. The early adopters are on board. People like the fact that they are saving a lot of money. We're seeing a steady increase. But I think if you want to hit the next, let's say, level of growth, we'll have to answer some of these questions, which we are now in that process today.

Harith Ahamed: Okay. Just a clarification, Madhukar, you mentioned that you've launched it across all the states that you operate in, but in select stores. So I was just wondering that from an operations standpoint, wouldn't it be easier to launch it across all the stores in a certain market versus launching it in select stores?

Madhukar Gangadi: No, I may have communicated it wrongly. We have actually launched it across all states, all stores.

Harith Ahamed: All states, all stores.

Madhukar Gangadi: I don't know why it came out like that. Yes, yes, now it is all states, all stores.

Harith Ahamed: And last one, I didn't get 1 number that you shared previously, the store level margins for stores of more than 24 months of maturity?

Madhukar Gangadi: Yes, Sujit, do you want to take that?

Sujit Mahato: Yes. It's around 10.1, Harith.

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Harith Ahamed: Alright. Okay. That's all from side. Thank you.

Moderat

or: Thank you. The next question is from the line of Madhav Marda from Fidelity International. Please go ahead.

Madhav Marda: My first question basically was that as we target this 20% private label mix in the pharma portfolio, how do you see the gross margins for the company trending, say, in the next -- I think you said that you can hit the 20% mark over the next year -or-so. So if you look a couple of years out once we have that Mark, how do you see the gross margin profile? Like I think we're trending at about 21.5%, 22%. How do you see this changing for the company?

Madhukar Gangadi: So Madhav, we'll try and give you a guidance as we go forward on that. So it's going to be a

function of 2 things. One, as we replace the branded product, we will see an increase of around 0.1 or 14%, overall...

Madhav Marda: Sorry, I didn't get you, you said half...

Madhukar Gangadi: No, no. As we replace the branded product, right, on the branded product, post -GST, post everything and all, we probably get around 13% overall margin and this one basically gets us around 22% to 23%. So there'll be that increase as we replace that. But in the initial phase, though, we may end up replacing some of our own private label, which is Wynclark. While we continue to basically run both of those, I think the consumers who are buying chronic product, the medicines for chronic ailments and who are buying the Wynclark brand on that, may switch over to MedPlus brand.

And there, for every one person, we will probably end up losing around 0.3%. So in the conversion -- if you convert maybe 2% or 3% out there, you'll probably end up losing 1% gross margin on the Wynclark side. But after that, it will all be a gain.

Madhav Marda: Okay. No, sir, you're saying Wynclark...

Madhukar Gangadi: So Madhav, sorry to interrupt there. See, our goal in trying to actually get this MedPlus brand popular and get the word out to everyone about the quality and the pricing of that is not to convert the existing customers into the MedPlus brand, but we actually get customers to increase their top line by bringing in customers from the other mom-and-pop and other organized stores. We are seeing a lot of people actually doing that. So we expect the benefit to us is going to be more on the top line than on actual margin side.

Madhav Marda: Okay. Okay. Understood. So -- got it. So would that mean that if earlier, like you said the store level EBITDA margin for 24 months plus stores were actually 10.1%. Essentially, what we're saying is that there is gross margin and then there's EBITDA margin. So in your view, if we get higher volumes at the existing stores, would you have a view on where the store level margins for the mature stores could move? Because I guess that's what we are finally trying to target?

Madhukar Gangadi: Sure, sure. So for us, I mean just -- if we were to increase our sales by, let us say, 10%, over the current level, right, the average store, let's say, is doing around INR15 lakh out there, if you were

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to increase the top line without really changing anything else by 10%, that's -- that will increase sale by INR1.5 lakhs. The margin will be INR30,000 extra. And that would actually increase the overall EBITDA by 2%.

Madhav Marda: Okay. Okay. So basically, the sort of older version versus the new version of stores, you could get 200 basis points higher margin, as this mix of private label goes up because we'll sell more basically at the same stores, right?

Madhukar Gangadi: The idea is to be able to sell more because if it was purely for margin accretion, then we will just continue and the old private label and just continue that. What we feel is introducing a store generic concept for the first time in India, getting people to -- see, our ability to convert a normal customer into the Wynclark customer for a higher

margin is only so much given that our pricing was not any lesser than the other brands.

Now when we talk about it and when we tell people about the MedPlus brand and its quality and we also talk about the price advantage to them, we see a lot more people, a lot more receptors. So, A, conversion of our existing customers; but B, more importantly, we have only about 15% to 20% of any other markets in this city, even though we are really big.

We feel the balance 80%, do you feel we'll be able to win over at least 10% to 20% of the balance 80% who are not shopping with us based on the price. And that's what we're looking for. Yes.

Moderator: The next question is from the line of Sayantan Maji from UBS Group.

Sayantan Maji: My first question is on the portfolio of private labels. How much of it -- sir, how much of the current pharma sales are these private label seeing? So I remember a number that you had given earlier was somewhere around 55%. So has it increased now, now that we have launched it nationally? Or is it still somewhere in that range?

Madhukar Gangadi: No, it has increased slightly. So earlier when we talked about it, it was 80% of our overall sales comes from medicines and for 55% of that we had a substitute. So on the overall scale, we had substitute for 44%. Now that 55%, I think, has now moved to around 60% or 62%, but we have also placed an order for a lot more medicine. So I expect that it will give you up to 70% as we go forward.

Sayantan Maji: Okay. So if I say, let's say, 70% of 80%, which is, say, 55%, so out of this 50%, 55%, you are expecting a 20% share of pharma private label or is it on the total sales?

Madhukar Gangadi: No, we are expecting on the total thing. See, on this 56%, the 20% is already there almost because even leaving aside Telangana, the rest of the country is at 11.2%. So out of the 56%, 20% would be only 11.2%, right? So we already have that. And in Telangana it is actually 14%. So the number is slightly higher. So when I'm talking 20%, I'm talking 20% on the overall number, so

which will actually become 20% and 56%.

Sayantana Maji: Okay. Okay. So this is basically the share of GMV, not the net revenues?

Madhukar Gangadi: Yes.

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Sayantana Maji: Okay. Got it. And also, I think in the opening remarks, you had mentioned that the share of pharma private labels has increased from 8% to 11.2%. And later in the call, you said 8.96% to 11.78%. So is it the difference between the timing? Or is it -- can you just explain like where is the difference coming from?

Madhukar Gangadi: Sure. Sujit, do you want to take that?

Sujit Mahato: Yes. We had the first number, 6.61% to 15% was an update on only Telangana, which we have launched end of June. And the second one, 8%, going up to 11.2% is all states put together, excluding Telangana. It's the only once during October, and that's relevant to the quarter.

Sayantana Maji: Okay. Understood. Okay. That's clear. And my second question is on Diagnostics. So in Diagnostics, we are ramping up in Hyderabad. So -- and I remember you're saying that it is a pilot. And once the pilot is successful, you will expand to other cities. So when do we -- when -- at what point will the decision be taken to expand into other cities based on the results of the pilot in Hyderabad.

Madhukar Gangadi: Sure. Chetan, I'll take this. So yes, no, this continues to be a pilot still. We today are at around 120,000 subscribers serving roughly around 220,000 people out there. The goal is for us to get to at least 200,000 number before we can take a call on expanding in the current situation.

Sayantana Maji: Okay. Thank you so much for taking the questions.

Moderator: We'll take the next question from the line of Tanmay Gandhi from Investec.

Tanmay Gandhi : Hi sir. Thanks for taking my questions. Sir, firstly, if you can clarify the p

private label pharma

sales on the right. So is it possible to split it between MedPlus branded and the normal private labels which we used to have even earlier on a net revenue basis?

Madhukar Gangadi: Sure. We'll see if you can do that. But for Telangana, that is Hyderabad and rest of Telangana state, all of them are lumped into 1 category right now. To the customer, it doesn't matter because he's getting the same price. To us also, it doesn't matter. So we're just not differentiating them on there, but we should be able to give you the actual split.

In the other places out of the 11.6% or 11.7%, I think around 5% is probably the old Wynclear.

I'm not sure, Sujit, do you -- if you have the right numbers, you can just go ahead.

Sujit Mahato: So it's around 11.2% on a net revenue basis, the old and the new put together.

Tanmay Gandhi : No, sir, actually. No, no, sir, actually, I want it on a net revenue business which we report, right? So I want to split off...

Sujit Mahato: I mentioned it is on a net revenue basis. I'm mentioning on a net revenue basis. But both the old as well as the new put together, as Madhukar explained that in the state of Telangana, we have not differentiated between the erstwhile model and the current MedPlus brand. And therefore, on a total basis, this is around 11.2% net.

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Tanmay Gandhi : Okay. Got it. Okay. And secondly, now that we are -- the pace of store addition has come down. So how should we look at the corporate level operating margins? So should we see a meaningful uptick or you plan to invest in some other initiatives like Diagnostics or the MedPlus -branded generics which you have launched?

Madhukar Gangadi: Okay. See, on the Diagnostics, as we said earlier, we're not going to be doing any other investments. We want to -- I think we are fully invested, except for some small maintenance capex, as Chetan said earlier. We will be wait to see how it is shaping up. We are doing well on the B2C side. We also want to see some traction on the B2B and post that we will figure out how to actually expand either on investments in the same place or in going to new cities.

On the private label side, there is no significant investment for us. The cost of the medicines is not very high, and it is just replacing one part from the other that's all. So I think the benefit of adding lesser stores, should be, I would say, more on allowing us to focus on the private label and possibly on the profitability also as we go forward.

Tanmay Gandhi : Sure. So sir, if we look at your operating EBITDA bridge that you provide, right, what are the various drags on the reported numbers. So I think the biggest drag comes from the new store additions and given that now we have a bigger base and obviously, the addition will also be lower in FY '25. So is it fair to assume that there will be a significant uptick in EBITDA margins? Or it will be steady expansion on this?

Madhukar Gangadi: No please. assume for now it is going to be a steady growth only in that. We will see how the private label and all the top line and all go up. I know the cost will be slightly lower, but I don't think that will -- I think the bigger driver for us is -- are the first 2 which is top line and private label component.

Chetan Dikshit : Tanmay, I think you have the modelling inputs over the last many quarters. The reason why you heard Madhukar say what he said was because we do see the new private label having a fairly large opportunity. And we may have additional non store operating costs like marketing, etcetera, which is what Madhukar said, what we said.

Moderator: Mr. Gandhi, we were not able to hear sir.

Tanmay Gandhi : Sorry, I lost you Chetan, sir, but my question was that given that we are right now just giving our store addition guidance, should we assume good uptick in margins because as Ma

dhukar

had said that we are not spending a lot on private label side? Sorry, I missed you. I didn't expect [inaudible].

Moderator: Sir I would request you to kindly unmute yourself and speak. Chetan sir and Madhukar sir.

Chetan Dikshit : I am afraid we couldn't comprehend what was being said. At least at our end it was quite garbled.

Moderator: Mr. Gandhi I would request you to kindly use your handset to ask question please.

Tanmay Gandhi: Yes, is there any better?

Moderator: Yes.

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Tanmay Gandhi: Sir, so my question was that, so because we are reducing our store additions for the next year, and obviously, now we have a better base as well, right, larger base, so should we assume a significant uptick given that we are not spending much on marketing as Madhukar has said, right? So I just wanted to clarify that what are the various drags on the margins?

Because if we are lowering our store additions, then ideally there should be -- the gap between 12-month -plus stores margins and the reported margins for -- at the company level should be narrower, right?

Madhukar Gangadi: No, what you're saying is right, Tanmay. Logically, if we have lesser new stores, yes, definitely. But for us, the drag of that was never very big anyway. We will continue to report stores, which have more than one year old and all. I think it should definitely be slightly better as we go forward.

Moderator: The next question is from the line of Neelam Punjabi from Perpetuity Ventures. Please go ahead.

Neelam Punjabi: My question is on the Diagnostics business. So if I look at the latest quarter, we have already reached an INR80 crores annualized run rate for the diagnostic business. However, we are still reporting our losses segmentally. So by when can we turn profit in our Diagnostic franchise?

Madhukar Gangadi: Yes, sure. Chetan, do you want to take that?

Chetan Dikshit: Yes, sure. I can take the first crack at it. Neelam, the Diagnostics business also has a similar structure like pharmacy in the sense that a lot of operating costs sit both at the center and outside the centers. So because the Diagnostics business has costs which are at the HO level, which are currently not diversified across many cities, the segment continues to be dragged by those costs.

The improvement in the performance of the centers naturally, will make up for some of that.

And we do expect to report the segment as profitable. But what we haven't disclosed so far is the center level profitability for diagnostics as we do in pharmacy which is why if you can just keep that framework in mind that what's ramping up is the revenue and the profitability of the diagnostic center.

But the HO level costs, which are actually at a multicity level that degree of preparation, whether it is procurement or absorb various lines of sub business, that continues to be a drag because our current operations are limited to Hyderabad.

Neelam Punjabi: Okay. But however, then can you help us understand what's the center level EBITDA margin, at least like for the mature center, the oldest -- first the Gachibowli center, if that will be helpful for us to understand then?

Chetan Dikshit: Sure Neelam. If you allow us, we'll do that in the next quarter.

Neelam Punjabi: Sure. Okay. And 1 clarification I needed. For the private label for our pharmacy business, we gave a target of about 20%, right? So is that on the net revenue or GMV basis?

Madhukar Gangadi: Sujit?

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Sujit Mahato: Yes. I think we are talking on a GMV basis, please.

Moderator: We'll take the next question from the line of Lokesh Manik from Vallum Capital. Please go

ahead.

Lokesh Manik: I have just one question from my end is being on the store addition in this quarter is net addition of 144 stores, but the rent expe

nse is flat at INR53 crores. So can you please help me reconcile this?

Chetan Dikshit: Yes, I'll take that. So if you observed last year for the similar period, over the period, we opened around 1,100 outlets at various points in time during the year. And in the current year, during the nine months, we have opened 400 -plus outlets. So that is the full impact taking care of the timing issue. But for that, there is no -- but for that, I would say we have the 5% to 7% annual contract base increase. So that's the major reason for the variance in the rental expense.

Lokesh Manik: Okay. But for this quarter, we should have expected a higher rent expense I mean because we are in line for the stores which are not there in the last quarter. So quarter -to-quarter, it's flat Q2, Q3 it's INR53 crores.

Chetan Dikshit: Yes?

Lokesh Manik: Yes, Q2, Q3 is flat at INR53 crores, and there is an increase in store addition of INR144 crores. I'm just trying to understand how the rent expense is...

Chetan Dikshit: Yes. It's in line with the contracts which we have entered the lease rent till and there's no exceptions in those numbers.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you, and have a great day.

Moderator: Thank you, members of the management. Ladies and gentlemen, on behalf of MedPlus Health Services, that concludes this conference call. We thank you for joining us, and you may now disconnect your lines. Thank you.

Call: Feb 2024

Medplus Health Services Limited

Regd Off . H No. 11 -6-56 Sy. No. 257 & 258/1 , Opp: IDPL Railway Siding, Moosapet, Kukatpally - Hyd-500037, TS India.
040-6724 6724

CIN No: L85110TG2006PLC051845 | Website: www.medplusindia.com | Email: medplus@medplusindia.com
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Dear Sir/ Madam,

Sub: Submission of Earning Call Transcripts for the quarter ended December 31, 2023 .

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015 , we enclose herewith transcript of the earnings calls held on February 5 , 202 4 for the quarter ended December 31, 2023 .

Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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by MANOJ
KUMAR
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"MedPlus Health Services Limited
Q3 FY'2 4 Earnings Conference Call "
February 05, 202 4

MANAGEMENT : MR. MADHUKAR REDDY GANGADI – CHIEF
EXECUTIVE OFFICER AND MANAGING DIRECTOR –
MEDPLUS HEALTH SERVICES LIMITED
MR. SUJIT MAHATO – CHIEF FINANCIAL OFFICER –
MEDPLUS HEALTH SERVICES LIMITED

MR. CHETAN DIKSHIT – CHIEF STRATEGY OFFICER –
MEDPLUS HEALTH SERVICES LIMITED
MR. PRASAD REDDY – ASSISTANT FINANCIAL
CONTROLLER – MEDPLUS HEALTH SERVICES
LIMITED

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Moderator: Ladies and gentlemen, good evening, and a warm welcome to the MedPlus Health Services Limited Q3 FY '24 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Prasad Reddy from MedPlus. Thank you, and over to you, Mr. Reddy.

Prasad Reddy : Thank you, Michelle. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q3 FY '24 Earnings Conference Call to discuss the financial results of MedPlus for the third quarter of financial year 2024, which were announced on 2nd February 2024.

We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, Chief Executive Officer and Managing Director; Mr. Sujit Mahato, CFO; and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties are on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit, the CFO. Thank you, and over to you, Sujit.

Sujit Mahato : Thank you, Prasad, and good evening, everyone, on this call. We are pleased to share that as of December 31st, we have been serving community in over 600 cities across 10 states, including the 3 new states of Madhya Pradesh, Chhattisgarh and Kerala through our network of 4,233 pharmacy stores. Additionally, in Hyderabad, MedPlus operates 4 full-service diagnostic centers, 8 level 2 centers and over 120 collection centers offering essential diagnostic services at affordable rates.

Let me cover the network in terms of openings and closures of our outlets. As informed during the last quarter, we have taken a thoughtful approach towards store expansion, prioritizing scale-up of MedPlus-branded pharma and non-pharma products and also maintaining a balance between overall growth and profitability.

Over the past 12 months, we have added a net total of 676 stores. On a gross basis, this was 758 stores, with 164 stores opened during Q3 alone. Most of the new stores were opened in Tamil Nadu and Telangana with 55 and 24 stores, respectively.

This strategic focus of growth reflects our commitment to meet the evolving needs of our community while maintaining a focus on sustainability and financial health. We expect the new store openings for the fiscal year 2024 to be around 600 stores.

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In the third quarter, 59% of our store openings were in Tier 2 cities and beyond, underscoring our strategic emphasis on these markets. Presently, out of our 4,233 stores, 1,908 stores, representing 45%, are situated in Tier 2 cities and beyond. We acknowledge the growth potential inherent in these markets.

Throughout Q3, there were 20 store closures. Taking into account both openings and closures, we achieved a net addition of 144 stores during the current quarter, a notable increase compared to the 114 added in the previous Q2 quarter.

In terms of the network age. Approximately 18% of our stores are in the first year of operation, around 27% of our stores are in the second year of operation and the remaining 55% of our stores have been operational for 2 years or more. It is noteworthy that all stores in the less than 2 years

are

age bracket are still in their ramp-up phase. From a financial standpoint, they continue to exert a negative impact on our operating EBITDA.

However, as these stores mature, we anticipate a positive contribution to our profitability. We closely monitor the timeframe for our new stores to reach breakeven. For stores opened between January 2023 and June '23, approximately 59% of them achieved breakeven within 6 months of their operations.

As a cohort, all stores combined reach breakeven in just 4 months. In terms of the store size as at the end of the quarter, our network has grown to 4,233 stores, covering 2.2 million square feet compared to 3,557 stores over 1.9 million square feet as of the end of December '22. The average store size was 535 square feet. To give you a sense of spread in store sizes, we had 3,082 stores less than 600 square feet and 1,151 stores that are greater than 600 square feet.

In terms of revenue mix, with our expanded scale, we are strategically positioned to increase our revenue share from our private label products. Our private label range is crafted to provide customers with high-quality products at competitive prices.

Presently, MedPlus offers over 1,000 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical category. Private label sales currently constitute 14.5% of our total net revenue. However, our growing presence in Tier 2 cities and beyond is significantly impacting our revenue mix. Sales from these markets comprise 35% of our pharmacy revenue in the current quarter, marginal increase from 33% in the same period last year.

I'm pleased to provide an update on the impact of the launch of MedPlus-branded products in Telangana state. In quarter 1 FY '24, prior to the launch, the share of private label pharma stood at 6.61% of the total GMV in Telangana. However, following the launch and in quarter 3 FY '24, the gross merchandising value notably increased to 15.03% of the total GMV in Telangana. Following the promising results observed in Telangana, the company initiated the launch of MedPlus branded products in other states starting October '23 onwards. Prior to the introduction of MedPlus-branded pharma products, sales of private label pharma products accounted for 8% of the gross merchandise value in quarter 2 of FY '24.

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However, subsequent to the launch and throughout the reported period, the private label share of GMV surged to 11.2%. The increase in private label GMV indicates the positive reception from customers and validate our commitment to delivering high-quality products under the MedPlus-brand umbrella.

Now coming to the financial numbers. On our quarter performance, our consolidated revenue was INR14,415 million with a growth of 21.1% year-on-year and 2.3% quarter-on-quarter. Our consolidated operating EBITDA stood at INR466 million, representing 3.2%. A round 99% of our revenue is from our pharmacy operations.

The pharmacy operating EBITDA is INR505 million, representing 3.6%. In terms of our store performance, I would like to update on our stores older than 12 months. Revenue from these stores in quarter 3 was INR12,953 million or 91% of pharmacy revenue.

These stores had a store level EBITDA margin of 9.6%. The store level operating ROCE of these stores stood at 52.5%. A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 9.6%, this was 10.1% for stores greater than 24 months and 7.2% for stores in the 13 to 24 months age bracket.

If we allocated the non-store related costs, then the operating EBITDA of stores greater than 12 months to the INR588 million, which translates to a margin of 4.5%. Quick numbers. Diagnostic revenue has grown to INR196 million in Q3 '24 compared to INR97 million in Q3 of FY '23, which is primarily due to launch of the new

centers across Hyderabad.

Diagnostics segment recorded an operating EBITDA loss of INR34 million compared to a loss of INR33 million in quarter 3 of fiscal year '23. Our net working capital for quarter 3 was 66 days. The inventory in our warehouse was 38 days.

As you are aware, because of the same trajectory of new stores, their inventory turnover is lower in the first year. In Q3, the inventory level of our first year store was INR111 million. In comparison for our stores older than 12 months, the inventory was 42 days.

Now I request Chetan to share an update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. As you know, Q3 is a seasonally weak quarter for Diagnostics in great part due to the festival and holiday season. However, we have 8% quarter-on-quarter growth in Q3. In October, we sold 405 gross plans per day. In November and December, this was 369 and 357, respectively.

As on 30th September, we had 117,000 active plans and 207,000 underlying lives. As of 31 December, we had 127,000 active plans and 228,000 underlying lives. Our current observed on-time renewal rate was 19% in Q3 versus 15% in Q2.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question

is from the line of Ashita Jain from Nuvama. Please go ahead.
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Ashita Jain: My first question is on your store opening guidance. I think for this year, your store down to 600. What is the store opening guidance for the next 2 to 3 years or 3 years for FY '25? Can you give color on that?

Madhukar Gangadi : Sure. Thanks, Ashita. So as Sujit said earlier in this call, we have focused primarily on increasing the overall private label, both in pharma and non-pharma. That's going to be our focus for the first 1 or 2 quarters this year also. We've just introduced over 600 products. And even as the product line is expanding, we are also making sure that products are available across the entire network and all.

And we're also ensuring that our employees are fully trained and are able to sell and everything else. So as I said, the primary focus is going to be on this. We will most likely end up doing only around 600 to 700 stores the next year, the coming year, to o. But we will keep you guys updated as soon as our figures are finalized.

Ashita Jain: And secondly , you just spoke about last few quarters about entry into the newer market. What's the update on that?

Madhukar Gangadi: It is still early going for the three states, one is Kerala, Chhattisgarh and Madhya Pradesh, but we are, I would say, pretty encouraged by the response in all these places. We will put up a small -- a very small working kind of warehouse to make sure that we are able to service all our stores properly and all.

But in general, both Madhya Pradesh and Chhattisgarh are doing well. We expect to grow faster than that. There are -- I would say, the growing is a little bit slower in Kerala for us, but we continue to expand in that state also.

Ashita Jain: Any particular reason for this Kerala -- slower uptick in Kerala?

Madhukar Gangadi: So see, a lot of the initial store opening depends on the regulator over there. There are some regulators who are slightly more sticky about every single detail. And in some states, it is not as much. It is a state-wise thing. So it is a slow growing in the beginning stages, I would think. But yes, that's about it. Otherwise, it's not an issue. The market itself is pretty good in Kerala.

Ashita Jain: Understood. And I think phenomenal job on the private label side, now we're close to 14.4% pharmacy presence. How do you think this number would settle in the next year, given our initiatives on the MedPlus branded labels?

Ma

dhukar Gangadi: It's really difficult to say. We are very encouraged with the way the initial two states have responded with the initial two regions where we started in June have responded. And also under the early listing out there in the other states, it is following the same path. Just on the pharma side alone.

I think given that 80% of our sales comes from pharma right now, and we will be able to hit or at least we'll be able to, I would say, have substitutes for at least 60% of the entire inventory, which is 70% at least of the medicines. I think for us to get to 20% will not be very tough on the pharma side over the next one year.

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Moderator: Thank you. The next question is from the line of on Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee: Sir, on the private level, the MedPlus brand. So I mean what you see at a GMV level, how the ratio has moved? If you get your share at an overall company level like how much it was before, let's say, when you launched in first quarter and now?

Sujit Mahato: Yes, I'll take that, Saion thank you for the question. In quarter two, in terms of the GMV, the private label pharma was 8.96% overall. And end of quarter three, the same percentage on the same basis of GMV reflects at 11.78% If this is helpful.

Saion Mukherjee: So this is for the overall, entire pharmacy...

Sujit Mahato: Overall. Only for the private label pharma.

Saion Mukherjee: Understood. Okay. And the other question I wanted to check with you is that you have provided stores less than 12 months revenue split. How has been the revenue growth for stores, which are, let's say, more than two years old? I mean, is that growth very slow? I mean, is that a right assessment that it's in single digits?

Sujit Mahato : In fact close to the inflation number. If we get all stores put up to FY21 end, it is inflation -plus.

Saion Mukherjee: Okay. But generally, the pharma market growth is around 9%, 10%, right? And if there is price increase that is also there in pharma. So I was wondering why this number is a bit lower? Is there

sort of a de-stabilization with the new store opening? How do you sort of read this number?

Madhukar Gangadi: Yes. So Saion, while definitely there is some price inflation and unfortunately the DPCO rules which kick in once every while, also reduce the prices a little bit. So there's also that impact. For us, I expect that the growth will be slightly over inflation as we go forward. There may be some cannibalization in some existing states, but I don't really expect that to mute the sales any more than it should.

For us, the other main reason could be also this. We are not really -- except for places where we're reporting the private label, we are not taking the GMV route to actually explain the sales growth. So in some of those cases where private label medicines have actually taken off, you would see a slight dip in the top line because of the lower value at which we sell the discounted private label products, the new pharma private label products.

Saion Mukherjee: Right. Sir, is it possible to share the growth numbers on a like-to-like basis at the GMV level? Is that possible to -- like you reported 21% growth, how much would have been the growth at a like-to-like basis?

Madhukar Gangadi: Sujit, do you want to take that?

Chetan Dikshit : Madhukar, I'll take that. Saion, our reporting what we have done is we've benchmarked it to peers. But we'll take your feedback on board. And see if there's more disclosure that we can give going forward.

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Saion Mukherjee: Okay. And finally, are there any changes in the discounting in your normal branded products due to competition and behaviour of online players? Any changes that you have witnessed in the recent past?

Madhukar Gangadi :

We have not increased the discounts anywhere. And if your question was, if the competition is actually coming down, are you taking by decreasing it? No, not as of now.

Saion Mukherjee: Okay. Thank you.

Moderator: Thank you. We'll take the next question from the line of Gaurav Nigam from Tunga Investments. Please go ahead.

Gaurav Nigam: Sir, first question was on, I was looking at your presentation, you used to give this private label number of pharma SKUs. That number from Q2 to Q3 presentation is dropped from almost 1,100 to 783. Just wondering, is this a correct number to look at? And what's the reason if this is correct?

Madhukar Gangadi: We will have to come back to you. I don't think we have really decreased the overall number. If anything, given that we have been continually increasing the pharma private label and also looking at other product, I guess not to decrease the overall number of products. Sujit, do you want to add anything else?

Sujit Mahato: I will come back in a while.

Gaurav Nigam: Understood, sir. Another question was on, can you provide the average level of pharma discounts that was there in Q3? And has it quarter-on-quarter, has it increased or reduced, any sense of direction?

Madhukar Gangadi: Pharma discounts on branded products has not really increased significantly. There may be a little bit of increase in 1 state, West Bengal, where the earlier hurdle was -- after the earlier hurdle we used to give a 10% discount, we moved it to 15%, but that was at least 2 to 3 quarters ago. Otherwise, I see no reason for any increase in discount.

If as a blended this thing it is coming out a slightly higher discount, it could be because of our private label products. Our private label products, the new ones sell at an average of 50%. So if you combine the entire pharma products, then that could increase the discount. But I think if you look at purely the branded ones, there should be no increase or there should be very little increase.

Gaurav Nigam: Understood, sir. And sir, what is the guidance for diagnostics capex in FY'24 and '25, sir?

Madhukar Gangadi: Do you want to take that Sujit or Chetan?

Chetan Dikshit : Yes, sure. I can take it. Apart from maintenance level capex, we don't have anything really planned in terms of big machines or new centers. If there is a change in view, we will definitely update you.

Gaurav Nigam: In FY'25, sir, as well, you're saying?

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Chetan Dikshit : Yes. For the foreseeable horizon, our capex program has been rolled out. In some of the centers, we may have minor capex maybe an additional ultrasound, ambulance. So which is why I cannot

spoke about it like maintenance capex. But there is no plan on the cards for new centers or new cities or new big machines. We will update as and when anything like that is on the cards.

Gaurav Nigam: Understood, sir. And sir, just a last question. Is there any update on the QIP? I think you have taken a shareholder approval. So just wondering if there is anything in the plan going forward.

Chetan Dikshit : Thank you for asking that question. We maintain our stance that what we have taken was shareholder enabling provision just to keep that optionality and that is for a full duration of 12 months. As and when the circumstances require us to raise additional funds, we will then have the option of both looking at equity and debt. As of now, it is an enabling provision, and there is no action expected on it in the near-term.

Gaurav Nigam: Understood, sir. Thank you for answering my question.

Moderator: Thank you. We'll take the next question from the line of Kunal Randeria from Axis Capital.

Please go ahead.

Kunal Randeria: Thanks for taking my question. Madhukar, your private label sales contribution on the pharma side has actually gone down on a year-on-year basis, right? But your GMV has gone

up, I

believe, so does that mean Wynclear MHS had a lot less discount than made by MedPlus brand?

Madhukar Gangadi: Yes. So earlier Wynclear and even a little bit of the trade generics which we sold, were sold at the same price as branded products. Since then, we have decided to basically go with a store generic thing, promote MedPlus brand and give it the full discount. So while a response to this has been very good, there has also been a little bit of cannibalization of our original Wynclear brand. And so in some places, that has come down a little bit. And so overall margin will have come down because of that.

Kunal Randeria: Okay. So do you still sell under Wynclear.

Madhukar Gangadi: Sorry?

Kunal Randeria: Do you still sell under the Wynclear brand or have you stopped it?

Madhukar Gangadi: No, we do. So we will continue that at least in the acute kind of therapies and all. In Telangana, the entire state of Telangana, we have reduced the price of Wynclear to the level of the MedPlus brand. So we sell it at the same price, all the products have been sold at the same price. But in the states in which we have started after this, in the rest of the country in the seven states, Wynclear continues to sell at the old price.

It is some of the chronic -- for some of the chronic therapies, the sales has come down. Most likely it will come down to next to nothing or zero in a while, we'll let that happen over a period of time. But we will continue to actually sell the Wynclear private label at the full price for acute therapies.

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The idea is that for those who are not very price sensitive, those who are not members of the MedPlus discount plan and all, and those who need the -- and those who are seeking early convenience, from them, we will get the full price. Otherwise, anyone else who is price sensitive will get the MedPlus brand at the full discount.

Kunal Randeria: Got it. And just one last one on this. So this is more like your -- it's been a few months since you launched Made by MedPlus. Let's say, as it gets bigger and people recognize the brand, do you plan to reduce discounting slightly, let's say, from 70% to a 60% or something like that?

Madhukar Gangadi: So today, our overall discount is at around 60%. We would say 50% to 80%, but the average falls at 60%. That is something we will see as we go forward, depending on how the market is evolved. Our primary thing right now concerns to grab market share and increase the top line. Once it is fully established, people understand the quality of the product and people are buying MedPlus -- are asking for it by name and then coming and buying it. Then we could always play around either with the top line, either with the MRP price, increase it slightly or perhaps decrease the discount to a slightly lower number. But that is not something which we are going to decide right now.

Kunal Randeria: Got it. Just one more if I can. Have you introduced it across the states or is it just Telangana at the moment?

Madhukar Gangadi: No, it is across all the seven states right now. As I said, only in Telangana, we have killed our old private label. Everything is now available at the new MedPlus private label brand rate. But in the other states, both co-exist.

Kunal Randeria: Got it. Thank you and all the best.

Moderator: The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: I joined the call a bit late, so my apologies if this was asked before. So my first one is on store additions. I see that we are tracking around 400 stores added year-to-date FY'24. So I remember that previously, we had guided for roughly 1,000 stores addition, 800-plus. So what would be the updated number for FY'24? And is there a revision in the guidance for FY'25 and beyond in terms of net additions?

Madh

ukar Gangadi: Sure. The number of stores, the gross additions this year will be close to 600, around the 600 mark, the net additions may be slightly lower. And for the coming year though, the number could be around the same number. The focus this year is going to be on private label. We want to make sure that we get it to the 20% kind of mark on the pharma side. So that's going to be the thing -- both by increasing the range, increasing the awareness and also increasing the education level of our own employees in being able to sell this product.

Harith Ahamed: So based on your experience so far on the MedPlus branded private label side, can you share some of the lessons or challenges that you've encountered in this foray? Is it on the sourcing side or is it in terms of getting subscribers? And if you can comment on the willingness that you're

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seeing among patients to switch from the brand to the private label, so if you could share some of the takeaways on these areas, that would be helpful.

Madhukar Gangadi: Sure. So you have seen a pretty good response to our products in Telangana where we launched it first. The results from this are only a quarter. Actually we started only in the middle of October or late October in the rest of the country. So the numbers are still not there. But in Telangana, we have seen a very good response.

What I think is that the early adopters, people are buying the fact that all the products available in the market are generic only. And MedPlus is able to give a very good price only because it does not have any marketing or distribution costs. So people are getting that. So some of the early adopters have come on board.

We think as we go forward -- but there still remain some questions. I think we have our work cut out in trying to educate the customer. The common question we encountered is this.

Whenever our guys have gone and said 50% to 80% discount on our brands, they have said customers have come back and said is this a generic product?

So we are taking the time to explain that all the products in the country are generic. There's really no difference. It's all the same product anyway. The only way you can differentiate one from the other is on the quality of its make. So we are countering that using several different explanations out there. And we are seeing a lot of people converting once that happens.

So this will happen not just through our stores, but also through media, influencers and all sorts of other testimonials and all. Again, to come back to your question, how is the response? Pretty good. The early adopters are on board. People like the fact that they are saving a lot of money. We're seeing a steady increase. But I think if you want to hit the next, let's say, level of growth, we'll have to answer some of these questions, which we are now in that process today.

Harith Ahamed: Okay. Just a clarification, Madhukar, you mentioned that you've launched it across all the states that you operate in, but in select stores. So I was just wondering that from an operations standpoint, wouldn't it be easier to launch it across all the stores in a certain market versus launching it in select stores?

Madhukar Gangadi: No, I may have communicated it wrongly. We have actually launched it across all states, all stores.

Harith Ahamed: All states, all stores.

Madhukar Gangadi: I don't know why it came out like that. Yes, yes, now it is all states, all stores.

Harith Ahamed: And last one, I didn't get 1 number that you shared previously, the store level margins for stores of more than 24 months of maturity?

Madhukar Gangadi: Yes, Sujit, do you want to take that?

Sujit Mahato: Yes. It's around 10.1, Harith.

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Harith Ahamed: Alright. Okay. That's all from side. Thank you.

Moderat

or: Thank you. The next question is from the line of Madhav Marda from Fidelity International. Please go ahead.

Madhav Marda: My first question basically was that as we target this 20% private label mix in the pharma portfolio, how do you see the gross margins for the company trending, say, in the next -- I think you said that you can hit the 20% mark over the next year -or-so. So if you look a couple of years out once we have that Mark, how do you see the gross margin profile? Like I think we're trending at about 21.5%, 22%. How do you see this changing for the company?

Madhukar Gangadi: So Madhav, we'll try and give you a guidance as we go forward on that. So it's going to be a

function of 2 things. One, as we replace the branded product, we will see an increase of around 0.1 or 14%, overall...

Madhav Marda: Sorry, I didn't get you, you said half...

Madhukar Gangadi: No, no. As we replace the branded product, right, on the branded product, post -GST, post everything and all, we probably get around 13% overall margin and this one basically gets us around 22% to 23%. So there'll be that increase as we replace that. But in the initial phase, though, we may end up replacing some of our own private label, which is Wynclark. While we continue to basically run both of those, I think the consumers who are buying chronic product, the medicines for chronic ailments and who are buying the Wynclark brand on that, may switch over to MedPlus brand.

And there, for every one person, we will probably end up losing around 0.3%. So in the conversion -- if you convert maybe 2% or 3% out there, you'll probably end up losing 1% gross margin on the Wynclark side. But after that, it will all be a gain.

Madhav Marda: Okay. No, sir, you're saying Wynclark...

Madhukar Gangadi: So Madhav, sorry to interrupt there. See, our goal in trying to actually get this MedPlus brand popular and get the word out to everyone about the quality and the pricing of that is not to convert the existing customers into the MedPlus brand, but we actually get customers to increase their top line by bringing in customers from the other mom-and-pop and other organized stores. We are seeing a lot of people actually doing that. So we expect the benefit to us is going to be more on the top line than on actual margin side.

Madhav Marda: Okay. Okay. Understood. So -- got it. So would that mean that if earlier, like you said the store level EBITDA margin for 24 months plus stores were actually 10.1%. Essentially, what we're saying is that there is gross margin and then there's EBITDA margin. So in your view, if we get higher volumes at the existing stores, would you have a view on where the store level margins for the mature stores could move? Because I guess that's what we are finally trying to target?

Madhukar Gangadi: Sure, sure. So for us, I mean just -- if we were to increase our sales by, let us say, 10%, over the current level, right, the average store, let's say, is doing around INR15 lakh out there, if you were

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to increase the top line without really changing anything else by 10%, that's -- that will increase sale by INR1.5 lakhs. The margin will be INR30,000 extra. And that would actually increase the overall EBITDA by 2%.

Madhav Marda: Okay. Okay. So basically, the sort of older version versus the new version of stores, you could get 200 basis points higher margin, as this mix of private label goes up because we'll sell more basically at the same stores, right?

Madhukar Gangadi: The idea is to be able to sell more because if it was purely for margin accretion, then we will just continue and the old private label and just continue that. What we feel is introducing a store generic concept for the first time in India, getting people to -- see, our ability to convert a normal customer into the Wynclark customer for a higher

margin is only so much given that our pricing was not any lesser than the other brands.

Now when we talk about it and when we tell people about the MedPlus brand and its quality and we also talk about the price advantage to them, we see a lot more people, a lot more receptors. So, A, conversion of our existing customers; but B, more importantly, we have only about 15% to 20% of any other markets in this city, even though we are really big.

We feel the balance 80%, do you feel we'll be able to win over at least 10% to 20% of the balance 80% who are not shopping with us based on the price. And that's what we're looking for. Yes.

Moderator: The next question is from the line of Sayantan Maji from UBS Group.

Sayantan Maji: My first question is on the portfolio of private labels. How much of it -- sir, how much of the current pharma sales are these private label seeing? So I remember a number that you had given earlier was somewhere around 55%. So has it increased now, now that we have launched it nationally? Or is it still somewhere in that range?

Madhukar Gangadi: No, it has increased slightly. So earlier when we talked about it, it was 80% of our overall sales comes from medicines and for 55% of that we had a substitute. So on the overall scale, we had substitute for 44%. Now that 55%, I think, has now moved to around 60% or 62%, but we have also placed an order for a lot more medicine. So I expect that it will give you up to 70% as we go forward.

Sayantan Maji: Okay. So if I say, let's say, 70% of 80%, which is, say, 55%, so out of this 50%, 55%, you are expecting a 20% share of pharma private label or is it on the total sales?

Madhukar Gangadi: No, we are expecting on the total thing. See, on this 56%, the 20% is already there almost because even leaving aside Telangana, the rest of the country is at 11.2%. So out of the 56%, 20% would be only 11.2%, right? So we already have that. And in Telangana it is actually 14%. So the number is slightly higher. So when I'm talking 20%, I'm talking 20% on the overall number, so

which will actually become 20% and 56%.

Sayantana Maji: Okay. Okay. So this is basically the share of GMV, not the net revenues?

Madhukar Gangadi: Yes.

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Sayantana Maji: Okay. Got it. And also, I think in the opening remarks, you had mentioned that the share of pharma private labels has increased from 8% to 11.2%. And later in the call, you said 8.96% to 11.78%. So is it the difference between the timing? Or is it -- can you just explain like where is the difference coming from?

Madhukar Gangadi: Sure. Sujit, do you want to take that?

Sujit Mahato: Yes. We had the first number, 6.61% to 15% was an update on only Telangana, which we have launched end of June. And the second one, 8%, going up to 11.2% is all states put together, excluding Telangana. It's the only once during October, and that's relevant to the quarter.

Sayantana Maji: Okay. Understood. Okay. That's clear. And my second question is on Diagnostics. So in Diagnostics, we are ramping up in Hyderabad. So -- and I remember you're saying that it is a pilot. And once the pilot is successful, you will expand to other cities. So when do we -- when -- at what point will the decision be taken to expand into other cities based on the results of the pilot in Hyderabad.

Madhukar Gangadi: Sure. Chetan, I'll take this. So yes, no, this continues to be a pilot still. We today are at around 120,000 subscribers serving roughly around 220,000 people out there. The goal is for us to get to at least 200,000 number before we can take a call on expanding in the current situation.

Sayantana Maji: Okay. Thank you so much for taking the questions.

Moderator: We'll take the next question from the line of Tanmay Gandhi from Investec.

Tanmay Gandhi : Hi sir. Thanks for taking my questions. Sir, firstly, if you can clarify the p

private label pharma

sales on the right. So is it possible to split it between MedPlus branded and the normal private labels which we used to have even earlier on a net revenue basis?

Madhukar Gangadi: Sure. We'll see if you can do that. But for Telangana, that is Hyderabad and rest of Telangana state, all of them are lumped into 1 category right now. To the customer, it doesn't matter because he's getting the same price. To us also, it doesn't matter. So we're just not differentiating them on there, but we should be able to give you the actual split.

In the other places out of the 11.6% or 11.7%, I think around 5% is probably the old Wynclear.

I'm not sure, Sujit, do you -- if you have the right numbers, you can just go ahead.

Sujit Mahato: So it's around 11.2% on a net revenue basis, the old and the new put together.

Tanmay Gandhi : No, sir, actually. No, no, sir, actually, I want it on a net revenue business which we report, right? So I want to split off...

Sujit Mahato: I mentioned it is on a net revenue basis. I'm mentioning on a net revenue basis. But both the old as well as the new put together, as Madhukar explained that in the state of Telangana, we have not differentiated between the erstwhile model and the current MedPlus brand. And therefore, on a total basis, this is around 11.2% net.

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Tanmay Gandhi : Okay. Got it. Okay. And secondly, now that we are -- the pace of store addition has come down. So how should we look at the corporate level operating margins? So should we see a meaningful uptick or you plan to invest in some other initiatives like Diagnostics or the MedPlus -branded generics which you have launched?

Madhukar Gangadi: Okay. See, on the Diagnostics, as we said earlier, we're not going to be doing any other investments. We want to -- I think we are fully invested, except for some small maintenance capex, as Chetan said earlier. We will be wait to see how it is shaping up. We are doing well on the B2C side. We also want to see some traction on the B2B and post that we will figure out how to actually expand either on investments in the same place or in going to new cities.

On the private label side, there is no significant investment for us. The cost of the medicines is not very high, and it is just replacing one part from the other that's all. So I think the benefit of adding lesser stores, should be, I would say, more on allowing us to focus on the private label and possibly on the profitability also as we go forward.

Tanmay Gandhi : Sure. So sir, if we look at your operating EBITDA bridge that you provide, right, what are the various drags on the reported numbers. So I think the biggest drag comes from the new store additions and given that now we have a bigger base and obviously, the addition will also be lower in FY '25. So is it fair to assume that there will be a significant uptick in EBITDA margins? Or it will be steady expansion on this?

Madhukar Gangadi: No please. assume for now it is going to be a steady growth only in that. We will see how the private label and all the top line and all go up. I know the cost will be slightly lower, but I don't think that will -- I think the bigger driver for us is -- are the first 2 which is top line and private label component.

Chetan Dikshit : Tanmay, I think you have the modelling inputs over the last many quarters. The reason why you heard Madhukar say what he said was because we do see the new private label having a fairly large opportunity. And we may have additional non store operating costs like marketing, etcetera, which is what Madhukar said, what we said.

Moderator: Mr. Gandhi, we were not able to hear sir.

Tanmay Gandhi : Sorry, I lost you Chetan, sir, but my question was that given that we are right now just giving our store addition guidance, should we assume good uptick in margins because as Ma

dhukar

had said that we are not spending a lot on private label side? Sorry, I missed you. I didn't expect [inaudible].

Moderator: Sir I would request you to kindly unmute yourself and speak. Chetan sir and Madhukar sir.

Chetan Dikshit : I am afraid we couldn't comprehend what was being said. At least at our end it was quite garbled.

Moderator: Mr. Gandhi I would request you to kindly use your handset to ask question please.

Tanmay Gandhi: Yes, is there any better?

Moderator: Yes.

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Tanmay Gandhi: Sir, so my question was that, so because we are reducing our store additions for the next year, and obviously, now we have a better base as well, right, larger base, so should we assume a significant uptick given that we are not spending much on marketing as Madhukar has said, right? So I just wanted to clarify that what are the various drags on the margins?

Because if we are lowering our store additions, then ideally there should be -- the gap between 12-month -plus stores margins and the reported margins for -- at the company level should be narrower, right?

Madhukar Gangadi: No, what you're saying is right, Tanmay. Logically, if we have lesser new stores, yes, definitely. But for us, the drag of that was never very big anyway. We will continue to report stores, which have more than one year old and all. I think it should definitely be slightly better as we go forward.

Moderator: The next question is from the line of Neelam Punjabi from Perpetuity Ventures. Please go ahead.

Neelam Punjabi: My question is on the Diagnostics business. So if I look at the latest quarter, we have already reached an INR80 crores annualized run rate for the diagnostic business. However, we are still reporting our losses segmentally. So by when can we turn profit in our Diagnostic franchise?

Madhukar Gangadi: Yes, sure. Chetan, do you want to take that?

Chetan Dikshit: Yes, sure. I can take the first crack at it. Neelam, the Diagnostics business also has a similar structure like pharmacy in the sense that a lot of operating costs sit both at the center and outside the centers. So because the Diagnostics business has costs which are at the HO level, which are currently not diversified across many cities, the segment continues to be dragged by those costs.

The improvement in the performance of the centers naturally, will make up for some of that.

And we do expect to report the segment as profitable. But what we haven't disclosed so far is the center level profitability for diagnostics as we do in pharmacy which is why if you can just keep that framework in mind that what's ramping up is the revenue and the profitability of the diagnostic center.

But the HO level costs, which are actually at a multicity level that degree of preparation, whether it is procurement or absorb various lines of sub business, that continues to be a drag because our current operations are limited to Hyderabad.

Neelam Punjabi: Okay. But however, then can you help us understand what's the center level EBITDA margin, at least like for the mature center, the oldest -- first the Gachibowli center, if that will be helpful for us to understand then?

Chetan Dikshit: Sure Neelam. If you allow us, we'll do that in the next quarter.

Neelam Punjabi: Sure. Okay. And 1 clarification I needed. For the private label for our pharmacy business, we gave a target of about 20%, right? So is that on the net revenue or GMV basis?

Madhukar Gangadi: Sujit?

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Sujit Mahato: Yes. I think we are talking on a GMV basis, please.

Moderator: We'll take the next question from the line of Lokesh Manik from Vallum Capital. Please go

ahead.

Lokesh Manik: I have just one question from my end is being on the store addition in this quarter is net addition of 144 stores, but the rent expe

nse is flat at INR53 crores. So can you please help me reconcile this?

Chetan Dikshit: Yes, I'll take that. So if you observed last year for the similar period, over the period, we opened around 1,100 outlets at various points in time during the year. And in the current year, during the nine months, we have opened 400 -plus outlets. So that is the full impact taking care of the timing issue. But for that, there is no -- but for that, I would say we have the 5% to 7% annual contract base increase. So that's the major reason for the variance in the rental expense.

Lokesh Manik: Okay. But for this quarter, we should have expected a higher rent expense I mean because we are in line for the stores which are not there in the last quarter. So quarter -to-quarter, it's flat Q2, Q3 it's INR53 crores.

Chetan Dikshit: Yes?

Lokesh Manik: Yes, Q2, Q3 is flat at INR53 crores, and there is an increase in store addition of INR144 crores. I'm just trying to understand how the rent expense is...

Chetan Dikshit: Yes. It's in line with the contracts which we have entered the lease rent till and there's no exceptions in those numbers.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you, and have a great day.

Moderator: Thank you, members of the management. Ladies and gentlemen, on behalf of MedPlus Health Services, that concludes this conference call. We thank you for joining us, and you may now disconnect your lines. Thank you.

Call: May 2024

(no extractable text — likely a scanned image PDF)

Call: Aug 2024

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Dear Sir/ Madam,

Sub: Transcripts of Analyst / Earning Call / Conference Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the earnings conference call held with analyst on August 5, 2024 for the quarter ended June 30, 2024.

Kindly take the same on record. The same is available on Company's website at www.medplusindia.com

Thanking You,
For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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"MedPlus Health Services Limited
Q1 FY'25 Earnings Conference Call "
August 05, 2024

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer
and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. Chetan Dikshit - Chief Strategy Officer
Mr. Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day, and welcome to the MedPlus Health Services Limited Q1 and FY '25 Earnings Conference Call. As a reminder, all participant lines will be in the listen only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Srinivas. Thank you and over to you sir.

Srinivas: Thank you, Shajal. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q1 FY '25 earnings call to discuss the financial results of MedPlus for the first quarter of FY '25 which were announced on the 2nd August 2024. We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, CEO and MD; Mr. Sujit Mahato, CFO and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward looking in nature, and may involve risks and uncertainties. Please note, the disclaimer mentioning these risks and uncertainties on slide 1 of the investor presentation shared with all of you earlier, documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website. I would now hand over the call to Sujit. Thank you. And over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and a very good evening to everyone on the call. We are pleased to share that as of June 30, we have been serving communities in over 650 cities across 10 states through our network of 4,444 pharmacy outlets. Also, the company operates four full service diagnostic centers, eight level two centers and over 110 collection centers, offering essential diagnostic services at affordable rates.

I would now brief upon the openings and closures of our outlet. Over the past 12 months we have added a net total of 469 gross 568 store editions with 66 stores opened during quarter one. However, planned new store openings for FY '25 remains at 600. In the first quarter, 44% of our store openings were in tier 2 two cities and beyond. At present, out of our 4,444 stores, 2,031 representing 45% are situated entire two cities and beyond. We continue to acknowledge the growth potential inherent in these markets.

Throughout Q1, there were 29 store closures. Taking into account both openings and closures, we achieved a net addition of 37 stores during the quarter, compared to the 174 stores added in Q4. In terms of our network, the age of the network is around 36 % of our stores are operational for less than two years and the remaining 64% of our stores have been operational for two years or more. It's noteworthy that all stores in the less than two years age bracket are still in their ramp up phase.

As these stores mature, we anticipate a positive contribution to our profitability. As a guardrail, we closely monitor the timeframe for our new stores to reach breakeven. For stores opened between July 2003 and December '23, approximately 64% of them achieved breakeven within six months of operation. As a cohort, all stores combined reach breakeven in just four months.

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In terms of our store size, as at the end of the quarter, our network has grown to 4,444 stores with 2.3 million plus square feet compared to 3,975 stores representing 2.1 million square feet at the end of June 2023. The average store size was 529 square feet. To give you a sense of spread in store sizes, we have 3,269 stores, less than 650, and 1,175 stores that are greater than 600 square feet.

On our revenue mix, we are strategically positioned to increase

our revenue share from private

label products. Our private label range is crafted to provide customers with high quality products at competitive prices. Presently, MedPlus offers over 880 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical categories.

Private label sales for quarter one FY '25 constitute 15.8% of our total revenue. Moreover, our

growing presence in tier 2 cities and beyond is significantly impacting on our revenue mix. Sales from these markets comprised 36% of our pharmacy revenues in the current quarter, marking an increase from 34% in the same period last year.

The following is the impact of the launch of MedPlus branded product across our network. In quarter one FY '24, prior to the launch, the share of private label pharma stood at 7.9% of total GMV compared to 15% during the current quarter. The increase in the private label GMV share indicates a positive reception from customers, and validates our commitment to delivering high quality products under the MedPlus brand umbrella.

Now on the financial numbers. On our quarter performance, our consolidated revenue was INR14,888 million with growth of 15.9% YoY and remains flat quarter-on-quarter. Our consolidated operating EBITDA stood at INR435 million representing 2.9%. Around 99% of our revenue is from our pharmacy operations. Revenue from pharmacy operations grew by 24% year-on-year on GMV basis and by 15.3% year-on-year on net basis. The pharmacy operating EBITDA stood at INR432 million representing 3%. The pharmacy operating EBITDA adjusted for special marketing spend of INR95 million stood at INR527 million, representing 3.6%.

Now on our stores performance, I would like to update on our stores older than 12 months. Revenue from these stores in quarter one was INR13,604 million or 94% of pharmacy revenues. These stores had a store level EBITDA margin of 9.3%. The store level operating ROCE of the stores stood at 46.1%.

Award year on the store level EBITDA margin by age, while stores greater than 12 months had a margin of 9.3%, this was 9.7% for stores greater than 24 months and 6.8% for stores in the 13month to 24month age bracket. If we allocated non-store related cost, then the operating EBITDA of stores greater than 12 months would be INR525 million, which translates to a margin of 3.8%.

On our diagnostic numbers, diagnostic revenue grew to INR242 million in quarter one FY '25 compared to INR139 million in quarter one of FY '24. Diagnostic segment recorded an operating EBITDA of positive INR3.3 million compared to a loss of INR46 million in quarter one FY '24.

However, central level operating EBITDA stood at INR31 million.

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Working capital. Our net working capital for Q1 was 69 days. The inventory in our warehouse stood at 40 days. In quarter one, the inventory level of our first year store was 104 days. In comparison, for our stores older than 12 months, the inventory was 47 days.

Now I request Chetan to update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you Sujit, and good afternoon, everyone. As you know, Q1 is a seasonally weak quarter for diagnostics. With that context, in April we sold 406 gross plans per day. In May and June, this was 408 and 442 respectively. As on the 31 March, we had 133,000 active plans and 247,000 underlying lives covered under our plans. As on the 30 June, we had 141,000 active plans and 270,000 underlying lives. Our current observed on time renewal rate was 24% in Q1 versus 21% in Q4. That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. The first question is from the line of Tanmay Gandhi from Investec.

Please go ahead.

Tanmay Gandhi: Hi, sir, the quarter looks a relatively weaker one, and I understand that this is a relatively weaker quarter on

a seasonal basis, but I just want to understand.

Moderator: Sorry to interrupt you sir. May I request you to please use your handset?

Tanmay Gandhi: Yes. Sir, my question is that, you know, whether do we need to revisit our guidance or are we reiterating our guidance of 20% plus top line growth and, you know, maintaining 4Q level margins?

Madhukar Reddy Gangadi: No, I don't think we need to change anything out there. As we mentioned earlier, Q1 is typically slow, and that's what it is. I don't think there's anything more to look beyond that honestly.

Tanmay Gandhi: Sure. And the 16% growth looks very weak, given specifically if you look at pharma companies' growth, even the largest player had reported almost 16% year-on-year growth. So just wanted to understand that, have you plateaued in terms of market share against even our same store sales growth also looks a little tepid. Just wanted to understand that, you know, is it something, is it a market specific trend or there is someone off in our numbers?

Madhukar Reddy Gangadi: No, no, Tanmay, it's actually 23%. You know, given that significant portion of our sales is now coming from MedPlus private label, and these are all selling at, you know, half the price of our regular brands, right. So, any growth in this is going to definitely affect the overall top line. So, if you look at it MRP to MRP, and if you compare like to like out there, our growth is actually 23%.

Tanmay Gandhi: Understood. So GMV growth is 23%. Got it. So, the 20% plus growth which you have guided for, that is for reported revenue or that is for GMV revenue?

Madhukar Reddy Gangadi: No, that's actually for normal reported revenue only for this year.
Tanmay Gandhi: Understood. And sir, you know, our store generics are relatively better in terms of gross margins, right, but still, we have seen a good 50 bps decline sequentially. So just wanted to understand that, you know, is there any inventory write off during the quarter?
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Madhukar Reddy Gangadi: See, while the private label margin is definitely better than the regular branded margin and all, there are two things happening here. One, we are cannibalizing into the old private label, which had a slightly higher margin.

And this one, as you know, while it is better, it is not as good as the old private label. We actually have 52% to 55% overall discount on this product. So, the margin is not going to be as accretive.

So, if you look at a INR100 MRP equivalent for private label and for a branded generic, the regular one, if after the discount on a branded generic, we get something like 13 or 0.13 or 13% or something like that.

On the new private label, you end up getting anywhere from 23 to 26. This is on the INR100 MRP. If you look at on the net sales, it is going to be around 60, 64. So, it's not going to be as accretive. On the inventory and all. I'll let actually Sujit answer that, to see if there's any other adjustment. As far as I know, the private level growth is definitely accruing to us. The margins.

Sujit Mahato: Yes. In addition, what I would like to highlight is since we are speaking on the sequential quarter, Tanmay, in terms of the other business income and the data fee which we had recorded in quarter four, we had also highlighted during that time that to the extent of 0.3% of the gross margin.

There is a shortfall, which means that that was a Q4 event, which we also expect during the current fiscal. To that extent it has impacted our gross margin, and some amount of inventory provisioning has also impacted, which is another 0.3%.

Tanmay Gandhi : Sure. Got it. That's all from mine. Thank you.

Moderator: Thank you. The next question is from the line of Adrit Chaturvedi from Nomura Financial Services and Securities. Please go ahead.

Adrit Chaturvedi: Hello. Good evening. Yes, I was just trying to gauge the baseline

margin performance of the

stores. So, what I'm looking at is in 1Q FY '24, the overall retail pharmacy had an operating EBITDA margin of 2.7%. And on the like for like basis, if I look at 12 month plus operating EBITDA margin that is reported in this quarter, that's 3.8%.

So that's roughly like 110 bps of increase on these stores, the baseline stores, excluding the stores that were not added particularly. So, I was just trying to understand if you could qualitatively let me know how much of this 110 bps is just your efficiency in terms of margin profitability, and how much is it just ramp up of a lot of stores that were on the platform at 12 months. Or if you could just give me a number at how much is the operating EBITDA margin for 24 month plus stores as of now?

Sujit Mahato: Yes, sure. So, in terms of year-over-year, the way we have to look at is due to the change in the private label pharma mix, which has increased from 7.98% to 9.5%, there was an additional margin. Net of the cannibalization of our old private label, we were able to increase the gross margin by 60 basis points.

Additionally, we had an impact, what I just mentioned a few moments ago, we had an unusual inventory provision around 0.3% which impacted our gross margin. In addition to that, on our private label non-pharma products, due to the mix change from 5.7 up to 7%, we had a positive

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gross margin impact of another 25 basis points. So, with this, you will be able to broadly reconcile the year-on-year gross margin.

Adrit Chaturvedi: Okay, so what is your sense, just to follow up quickly, that out of this 110 bps increase that we are seeing on the baseline stores, how much could be attributed to just organic margin improvement and how much could be attributed to the ramp up of the new stores that were there on the platform last year? So, if you could, like out of the 110 bps, is there some sort of attribution that could be done?

Sujit Mahato: I think we'll take that up offline, please. I don't have that handy.

Adrit Chaturvedi: Okay. And just finally for me, could you share the private label at MRP growth for the new and the old?

Madhukar Reddy Gangadi: Yes. So as of last quarter, the private label stands at 15%. This is on GMV. I think it moved from 13.5% to 15%. So, the growth over the quarter was 1.44.

Adrit Chaturvedi: And like a YoY growth? Could there be a YoY growth in the private label, new and old?

Sujit Mahato: Last year, it stood at 7.9% overall and now it stands at 15.

Adrit Chaturvedi: Okay. And how much of this could be the new one, the new private label? And how much is old? Are you only talking about the new private label because there's obviously cannibalization?

Sujit Mahato: Yes. Out of this, I think, Yes. The old private label is at around 1.8% and the balance is new private label.

Adrit Chaturvedi: Okay, thanks a lot. Thank you. That'll be it.

Moderator: Thank you. The next question is from the line of Madhav from Fidelity Investments. Please go ahead.

Madhav: Hi, good evening. Thank you so much for your time. I wanted to understand that given that there's a big product mix, category mix change which is playing out in the company, is it more accurate to look at the company on a per store? Like should we look at EBITDA per store or gross profit per store?

Would that be a better reflection of the profitability? Because if you look at that special marketing spend which we're doing of INR9.5 crores in quarter one, and if you assume that that's not going to be a recurring expense, EBITDA up a sort of, in my understanding, probably improves quite well on a year-over-year basis. So firstly, am I thinking in the right direction? If you could share your thoughts.

Sujit Mahato: Sure, Madhav. See, as we had said earlier, we were running a pilot in Tamil Nadu, and that on that ground we ended up spending

ending INR9.5 crores. While it is, you know, we didn't get what we expected, I don't think we're going to go and spend any more money or at least to this level in the next in the coming quarters. So, this is not a recurring expenditure.

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So, if you take that out, definitely from a two -- I think from a 3, it goes up to 3.6 for MedPlus pharmacy and EBITDA. And as your question about, you know, should we do it store by store? That, Yes, we can take it offline. I don't think -- we don't have the data right now, but, Yes, that would be one way of looking at it.

Madhav: Got it. And then just a second question was, I think you all said that the GMV growth was about 24% versus 15%, 16% on a sales basis. So that 24% basically reflects volume growth at the network level, right? Is that how we should read into it?

Sujit Mahato: Absolutely. Absolutely. Yes. It is just the Delta between 83, which is the selling price for us at INR100 MRP for a branded generic, versus 42 to 43, which is for the MedPlus brand. So, every time you do that, the percentage of these drugs which are sold at 42 INR100 MRP are going to be reflected in a slightly higher GMV growth.

Madhav: Got it. And just the last question was that in some of the stores, like in Tamil Nadu or West Bengal, where the net plus private label mix, you know, has probably, like, you know, is higher versus some other parts of the network. If, you know, maybe, if not now, maybe in the quarter two call, if you could give us some more color in terms of how the profitability is in those stores, given that, you know, you would have like, probably good product mix change that played out. So just let me know directionally how the network would look two, three years out for the company as a private label.

Madhukar Reddy Gangadi: We will try and, you know, do it as we go forward. But right now, while the growth has been very encouraging and it is something which we are banking on, I can't give you the exact numbers, like model but certainly we can work out at least.

Madhav: Thank you.

Moderator: Thank you. The next question is from the line of Vinod Chandra, who is an Individual Investor. Please go ahead.

Vinod Chandra: Thank you for the opportunity, sir. I have two questions. One is generally we receive a notice about our store suspension, and there are a lot of notices about come in a month. So, is there a specific reason behind those store suspension or can we not control well in advance before receiving these notices? So that is the first question. Should I ask the second one as well? Okay, Yes. The second question is about like, I know, like you are reporting and showing the store level operating EBITDA, right. Or ROCE, we say around 46%. So, is it like something, if you can give us the ballpark figure about like what would be the store level ROE figure rather than operating ROCE? So, these are the two questions I have. Thank you.

Madhukar Reddy Gangadi: Okay, great. And answer to your first question, it's see the suspensions, unfortunately, some of them are at least a part of the business, given a large network of 4,444 stores. And as you know, healthcare, especially pharma retail is a highly regulated business. And there's always some small issue or the other for which you receive a small suspension.

For us, it is not material and it never came up earlier because it was not, any nonmaterial stuff was not supposed to be. We were not asked to report any nonmaterial, you know, kind of events.

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Now, of course, without changing the rules, we're doing everything out there. But again, as an answer, these are largely documentary issues, documentation kind of issues. So, this is something which we try to actually keep pace and try to reduce them. And other thing is, in a bunch of the suspensions which we received, which were for slightly

higher number of days, we have actually gone to the court and gotten a stay. So, I don't think that's going to really impact us too much. We will fight that and get it out.

But Yes, it is, I would say, unfortunately, something which we have to live with, not material, and we are trying our best to basically keep it under control as much as possible. We will definitely evaluate the store level ROE figure. We'll try and come back to you. Right now, we have -- we are reporting what we see RPI supported right now, but we will consider this, then come back.

Vinod Chandra: Okay. Thank you.

Moderator: The next question is from the line of Akansha Gupta from Solidarity Investment Managers. Please go ahead.

Akansha Gupta: Hello. Yes, so my question is around the gross margins. Could you please give us the breakup of the gross margin segment wise? That is the private label pharma, the private label non-pharma and the branded pharma and brand FMCG businesses.

Sujit Mahato: Yes. As a reference, for branded pharma, we will guide between 13% and 13.1%, private label Pharma, which was our old private label in the range of 74% to 76%. The newly launched MedPlus store level brand in the range of 64% to 66% gross margin. Branded non-pharma products in the range of 10.5% to 11%, and private label non-pharma product in the range of 16% to 18%.

Akansha Gupta: Okay. Okay, thanks. That's all from my side.

Moderator: Thank you. The next question is from the line of Harith Ahamed from Avendus Spark. Please go ahead.

Harith Ahamed: Hi, good evening. Thanks for the opportunity. In terms of store additions, this quarter seems to be on the lower side. While we had guided for around 500 stores, 600 stores for FY '25, are we still maintaining that number?

Madhukar Reddy Gangadi: Yes, Harith, so there's no change in guidance. If you have seen the last couple of years also, we usually do slightly better in the second, third and fourth quarters usually. This year, Q1 was unduly, I would say, kind of depressed because there were a couple of reasons. One of the main reasons was elections, I would say.

We actually had a lot of attrition and we had a lot of people -- and we were finding it a little difficult to hire the right kind of people during that month. And so almost everything, due to various things, actually, it got a little delayed. And I had already mentioned that our overall focus MedPlus Health Services Limited

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on MedPlus brand has increased a lot. And while that is not the main reason, that's where the company has been focused. For us, the guidance remains the same, one.

We are looking at our complete network. And in addition, so this year, while the net will remain more or less the same, we could have a few more closures than usual, because we have added quite a few stores in the last three years, could be anywhere from 30 to 50 more than usual, if we are looking at the entire network out there, and so that may change the number a little bit, but I don't think the addition itself will be affected too much.

Harith Ahamed: Okay. And this 15% GMV share of the pharma private label, how should we think about this number progressing over the next few quarters? And probably where do you see this reaching by FY '26?

Madhukar Reddy Gangadi: I think we'll probably be able to see the trajectory a little bit more clearly after this quarter. You know, when we started initially, obviously there were a lot of early adopters who came in immediately, and after that, it has been moving steadily at around 1.5% to 2% per quarter. If you look at it, I think last quarter was at around 13.56% and today it is 15%. That's 1.44% growth. We expect as we go forward, it will maintain at this level and also go up slightly, but I will come back to you guys with the guidance on growth quarter on quarter maybe in the next

quarter or

so. Probably have a more clearer idea.

Harith Ahamed: Thanks, Madhukar. One question for Sujit, in the slide where you've shown the IndAS adjustments in the interest income line for the quarter, the IndAS, adjusted interest income is

lower by around INR2 crores versus the reported interest income of around INR4.3 crores. So exactly is this adjustment, if you could give some color.

Madhukar Reddy Gangadi: I would say these adjustments are for the security deposits, what we have placed for the property.

But I will just look into this and review the details offline.

Harith Ahamed: Right. Thank you very much. That's all my side.

Moderator: Thank you. The next question is from the line of Aejas Lakhani from Unifi Capital. Please go ahead.

Aejas Lakhani: Yes, hi. So firstly, could you call out what was the entire ad and marketing expense for the quarter versus same thing last year and last quarter?

Madhukar Reddy Gangadi: Sure. As this year, we ended up spending around INR9.5 crores in this quarter versus Q1 of last year was INR1.3 crores.

Aejas Lakhani: Okay. And so, what was the same number for the previous quarter?

Madhukar Reddy Gangadi: Previous quarter was 8.5.

Aejas Lakhani: Okay. So, you know, I just want to understand that, you know, you made these spends in a timeframe when, you know, you just said on call that elections and there were some challenges and delays and attrition on account of that. So how do you measure the effectiveness of the spends?

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Madhukar Reddy Gangadi: No, no effectiveness so, I said the store openings did not happen because of that. Keep that aside.

That has nothing to do with the effectiveness aspect, right.

Aejas Lakhani: Okay. But, sir, could I understand that when you think of these, you know, these larger spends, which is more than normal course of business, how do you measure the effectiveness? And then what is the outlook for this line item for FY '25?

Madhukar Reddy Gangadi: We are not going to be doing any more of this in the next two quarters for sure. We actually spent this money in two different states. One was in West Bengal and the other was Tamil Nadu.

Largely the money was spent in Tamil Nadu. And for us, we are internally looking at a couple of things.

One, the number of new customers we have acquired and the growth of private label, MedPlus plan private label. So, those two would be the three key criteria for us to actually decide if the ad is worth spending on or not. But obviously, you can only get the, I would say you can only assign so much value to the immediate growth, the benefit to the brand and the benefit to the MedPlus brand. It is, you know that is going to come over a period of several quarters, much more difficult to actually much more difficult to measure.

Aejas Lakhani: Got it. And sir, the private label pharma, which is 9.1, could you just tell me that what is the split of 9.1 in old and new?

Madhukar Reddy Gangadi: Sure. So, the 9.1, which you're talking about is the net sale return number. I think the old one is around 1.8 and the balance goes to a new one.

Aejas Lakhani: Okay. And sir, when do you expect this runoff of the 1.8? I mean, do we expect it during the course of FY '25 or do you expect a sharper rundown?

Madhukar Reddy Gangadi: We will most likely see a slightly more sharper rundown as we go forward. Maybe in the next two quarters it should actually go down, but it will probably be always there to some extent. But from now, I think we'll probably end up going to slightly under one in the next two quarters.

Aejas Lakhani: Got it. And sir, I just want to understand more at the strategy level that why this decision to run down that old brand and replace it or replace it with MedPlus, because as you would I mean, there has been an impact on margins on account of this entire movement. And why not just

introduce the MedPlus brands, and let that automatically or naturally run down or progress with time? Why have we done this intervention which has been sort of forced to run that down and increase the MedPlus? Could you just help me triangulate that at a strategy level?

Madhukar Reddy Gangadi: No, no. It is not a question of running it down intentionally. We always had two different brands, but when customers walked in and they saw the entire set, the customers who are buying the old private label were people who are willing to take a substitute. The customers who are willing to take the new one are also the same set. Now, given that there is an alternative where they can get a discount for a small membership fee, most of them have chosen to go here.

Aejas Lakhani: Got it. Okay. Got it. So, sir, from here on, we are expecting, your growth guidance is 20% for, on a reported basis for FY '25, correct?

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Madhukar Reddy Gangadi: That's right.

Aejas Lakhani: And sir, operating margins. What is the outlook there?

Madhukar Reddy Gangadi: So, we'll continue to grow from here. Q1 typically is our slowest quarter normally. So, it should definitely be better than what it was last year. So, Yes, that's all I can tell you.

Aejas Lakhani: Okay, thank you and all the best.

Moderator: Thank you. The next question is from the line of Kunal Randeria from Axis Capital. Please go ahead.

Kunal Randeria: Hi, just one question, continuing on the previous participant's question. Madhukar, do you expect a sharper impact from canalization of your private label brand the next one or two quarters? And after that, that impact should completely go?

Madhukar Reddy Gangadi: I think so. Kunal. It will be there for a while because we have inventory. There are customers who are willing to buy. As you remember, as we told in the past, our previous set of private label also had the full set of chronic medicines too. So, some of them where, you know people who are on drugs like atenolol, telmisartan and all, they'll probably continue.

We have the stock. We'll probably switch them over to this at some point of time or they may themselves basically ask for this. So that part, that tailed stuff will continue, but most of the acute stuff will probably end in the next two quarters or so. But for us, whatever the impact may be of cannibalizing, this slightly more profitable portion of our business will most likely start coming down in the next two quarters.

As we continue to grow this. This is also accretive to us. This also gives us more margin, but as long as it comes at the cost of branded generics, it is good for us. If it comes at the cost of our old private label, not obviously. So that impact, I think will go down in the next two quarters.

Kunal Randeria: Sure. And just with the older private labels, have you changed the discounting practice or is it still, you know, trying to get in line with MedPlus discounting or same old method?

Madhukar Reddy Gangadi: So, if you're talking about my old private label, in the state of Telangana the old private label has always been sold at the same price as MedPlus as a new one. But in the rest of the states, we'd actually continue to sell it at the old rate out there. So, there was some confusion. We'll probably end up, you know, taking out some of that and, you know, redoing it in the MedPlus side as we go forward. Was that the question, Kunal?

Kunal Randeria: Yes, that was the question. Basically, I just wanted to understand whether on the old label also, are you giving a 50% to 80% kind of discount as you are giving it on made by MedPlus?

Madhukar Reddy Gangadi: Yes. In Telangana, yes, we are. We started on that note. We started off with that value proposition earlier to the customers. But in the rest of this country, while we started with maintaining the old private label at the old

discount. There are some products, maybe out of the

600 odd products, probably around 200 odd, which we will end up actually moving to the new pricing, the MedPlus pricing. This is just to expedite the overall sale of the inventory and to MedPlus Health Services Limited

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make sure that nothing goes into expiry. For us, obviously, we have both the inventories. We want to make sure that everything sells and we are left with nothing end of the day. And so part of that will be moving into a new price.

Kunal Randeria: Got it. Thanks.

Moderator: Thank you. The next question is from the line of Vilina Jain from Perpetuity Ventures. Please go ahead.

Vilina Jain: Hi. In your opening commentary, you mentioned INR95 million of special marketing spend. Could you elaborate on that? Also, secondly, you have been talking about Q1 being seasonally weak. If you could highlight some of the reasons for the same.

Madhukar Reddy Gangadi: Sure. The INR95 million got spent in Tamil Nadu and West Bengal, largely in Tamil Nadu. I think around INR7 million to INR7.5 million probably got spent there and INR2 million to INR2.5 million got spent in West Bengal. This advertisement was spent on, mostly on newspapers, some bit on TV, very little, some on radio and little bit on the social media, YouTube and everything else, and a little bit on podcasting and influencers and all those other things. That's one.

And I think, Yes, on the seasonality side, see typically April and May, the sales actually fall for two reasons. People end up actually leaving large cities in which we are largely placed. They move out, they go out for vacations, they go out of town and everything else and all. Even the senior citizens do end up actually, or, you know, people who are in their 50s also end up going back to their home towns and all once a vacation start for kids, that's one.

And second, in general, the overall burden of, I would say, disease on the infection side is much, much less. And so that whole set of antibiotics and painkillers and fever tablets and everything else, which are sold normally throughout the year and especially so in June, July and August are almost nil in April and May. And that's why Q1 is usually weak.

Vilina Jain: Okay, so regarding the marketing spend, is this more to do with the private label increase that is happening and going forward, how should we see the marketing spend increasing?

Madhukar Reddy Gangadi: So, we are probably bringing it down to a maintenance level of around something like INR10 million a quarter, which is INR1 crores, INR1.2 crores. But Yes, all the money has been spent on private label only. So, the whole idea is trying to get educate customers as many as them as possible, to kind of make them aware that all medicines sold in India, 90% of medicine sold in India are just generics, we're just buying them under different brand names, MedPlus now makes the same medicines.

And because MedPlus doesn't have any marketing spend or any, let us say, the channel spend, we sell it through our own stores, we are able to offer this big discount, and that's the value proposition to the customer, and that's what we have been pitching, and that's something which will continue all the way to a much, much lower level.

Vilina Jain: Understood. Thanks a lot.

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Moderator: Thank you. The next question is from the line of Vinayak Mohita from Axis Bank. Please go ahead.

Vinayak Mohita: Hello? Yes, hi. Good evening. I just have this one question on the store numbers for the greater than 12 months. So, accounting for the INR9.5 crores of advertisement expense, one time advertisement of expenses, if we removed that, then is it fair to assume that the operating margin for the greater than 12year, 12months stores would be more towards the 4.5% to 5% range like from th

e 3.8% that you have reported in the current quarter?

Madhukar Reddy Gangadi: Actually, it will go up by 0.6%.

Vinayak Mohita: Understood. 0.6% or 4.4%, perfect. And sir is it fair to assume that -- so when we look at the metrics on a standalone basis from what the quarter has been reported, they look to be deteriorating. But it would be fair to assume that x of those one time expenses and considering how the quarters would pan out going forward now on a YoY basis, we should continue to see an improving trajectory on the margins front for the stores, on a store level, that would be a right assumption to have?

Madhukar Reddy Gangadi: Definitely.

Vinayak Mohita: Understood, and so just wanted to understand from a longer term perspective, while your aspiration would be to be at 100% own brands, maybe, maybe not. But just wanted to understand over the next three to five years, where do you see this own brands for yourself settling down in the entire mix of your business? What's your aspiration out there for us to be able to visualize? How could the mess as a whole look like maybe three to five years forward from respect to private brands specifically?

Madhukar Reddy Gangadi: See, it is not yet one year since we have launched the brand across the country. We are already at around 15% overall. I feel very confident that this number will go up significantly, but to give you the actual trajectory, I think we'll have to wait a couple of quarters. So, it is possible that the early adopters have all come in and basically bought while we are seeing a gradual increase every quarter.

I just want to wait for a couple of quarters before I can predict that. But just to give you an answer, while it looks like store brands and all are typically capped at around 20%, 30%, 40% and all, the one number we should actually bear in mind is that even in a country like U.S. and places like Walgreens and CVS and all, 90% of what the store actually fills prescriptions are generic, and these are all, you know, the law, of course, basically allows them to actually substitute.

And the store then basically decides whether it wants to do its own private label or it has a preferred player, which is also a generic. There's no brand, right. So, the aspiration would be to go to that level over a period of time, but that will mean, you know, a lot of education of customers and maybe at some point the government comes in and basically changes the rule a little bit for us to do it.

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But, Yes, even if it were not to go to that number, I see no reason why you cannot settle somewhere in the 30s or 30% plus kind of number. But again, I will reserve that overall, this thing for at least another couple of quarters before I actually can make prediction out there on that.

And the reason why we are super excited about the whole thing is we are making slightly lesser

margin than our old private label, but please bear in mind that people are now buying the MedPlus brand. They are seeing it. It's not a substitute, it's not something else which we are just giving them. It is the MedPlus brand.

And now, as they get used to it and as they start buying it, and all, we have in our hands an ability to change the MRP every year and the government allows us to do that. You are allowed to do it by 5% or 10% odd. And we also have a chance to basically move the discount around a little bit to make it closer to 50, today it's 50 to 80, we can move it closer to 50, or we can even shift it. So, for those two reasons, we feel that the entire thing is going to be based around this whole strategy of private label.

Vinayak Mohta: Understood. And is there any reference in the global context wherein pharmacy has been able to replicate what you are trying to do? Because please do correct me if I'm wrong. From what I had read a couple of on a couple of pharmacies

ies, none of these pharmacies were able to cross the 20%, 22% mark on the private label side. So, do you have any reference on the similar company that I might have missed out on? Maybe my data is not correct. So, if you could just help me out there.

Madhukar Reddy Gangadi: Okay, so two things here. So one is, you know, is the store selling it on its own name? Right, then, Yes, maybe 20%, 30% probably is the number. Walgreens and CVS and all do that much, and I'm pretty sure other pharmacies across the world also have at least that much. But the number to actually bear in mind is that while these people may not sell on their name, when the patient walks into the store and hands in a prescription, all the pharmacist is asking him is, okay, your insurance says it has to be, it can be generic or your doctor has written, dispensers is written. If the doctor says dispensers written, then it is exactly given, the brand is given, but then the copay is very high and all that sort of stuff. But in 90% of the prescriptions, the pharmacist is filling a generic only. And the generic is not, let's say X company's generic or Y company generic or Z company generic. It is the generic which is chosen by Walgreens. And as far as the customer is concerned, it is the store generic only. It could be made by whoever it is, right? It is a store generic. So, yes, you know, you could actually go all the way up to 70%, 80% also in such cases.

Vinayak Mohta: Got it. And one last question. Have we in the recent one year stage, any kind of regulatory problems or something on the MedPlus brand products, like any complaints or any action on that front on the products part?

Madhukar Reddy Gangadi: No, no. Obviously what we are doing is, you know, doesn't go very well with the thousands and hundreds of thousands of modern part retailers, so they've all got a complaint. You know, drug authorities have come, they picked up the samples, they've tested them, everything is fine.

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So, for us, obviously, you know, we are disrupting a big market. It is going to have some people who are not going to be super happy about it. So, there have been people who have been calling on us and, you know, checking out all the things and all, but, Yes, that's about it. Because whatever you're doing is 100% worship.

Vinayak Mohta: Perfect, perfect. Great. Thank you so much and all the best.

Moderator: Thank you. Ladies and gentlemen, we will take that as the last question. I would now like to hand the conference over to Mr. Madhukar for closing comments.

Madhukar Reddy Gangadi: Thank you. I thank all participants on this call for your interest in the MedPlus journey. Our investor relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Nov 2024

MedPlus Health Services Limited

040-6724 6724

Regd. off. H. No: 11 -6-56, Survey No: 257 & 258/1, Opp: IDPL Railway Siding Road, Moosapet, Kukatpally, Hyderabad – 500037, Telangana, India

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November 16 , 2024

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BSE Scrip Code: 543427 Bandra (East), Mumbai – 400 051
NSE Symbol: MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcripts of Earnings Call held on November 13, 2024 :

In continuation of our earlier letter dated October 25, 2024 and November 12, 2024 regarding the investor call on November 13, 2024 and pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the earnings conference call held with analyst on November 13, 2024 for the quarter and half year ended September 30, 2024.

Kindly take the same on record. The same is also available on Company's website at www.medplusindia.com

Thanking You
Yours faithfully
For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer
FCS 7460

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"MedPlus Health Services Limited
Q2 FY25 Earnings Conference Call"

November 13, 2024

Management : Mr. Gangadi Madhukar Reddy - Chief Executive Officer
and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer ,
Mr. Chetan Dikshit - Chief Strategy Officer ,
Mr. DRN Srinivas - Sr. Manager Finance

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Moderator : Ladies and gentlemen , good day and welcome to the MedPlus Health Services Limited Earnings Conference Call.

As a reminder, all participant lines will be in the listen only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" and then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Sreenivas. Thank you and over to you, sir.

Sreenivas: Thank you. Good evening , everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q2 FY25's Earnings Call to discuss the Financial Results of MedPlus for the 2nd Quarter of FY25, which was announced on 12th November 2024.

We have with us today the senior management team represented by Mr. Madhukar Reddy – CEO & MD; Mr. Sujit Mahato – CFO, and Mr. Chetan Dikshit – CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on slide one of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier and these have also been posted on our corporate website.

I will now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas. And good evening, everyone, on this call.

We are pleased to share that as of September 30th, we have been serving communities in over 680 cities across 12 states through our network of 4,552 pharmacy stores. Also, the company operates four full service diagnostic centers, eight level two centers, and over 100 collection centers offering essential diagnostic services at affordable rates. In terms of the update on our network, over the past 12 months, we have added a net total of 463 stores, gross 561 store additions with 132 stores open during quarter two and 198 stores open during the first half of the current fiscal. In the 2nd Quarter , 59% of our store opening were in tier 2 cities and beyond. At present, out of our 4,552 stores, 2,102 stores are situated in tier 2 cities and beyond. We continue to acknowledge the growth potential inherent in these markets. Throughout Q2, there were 24 store closures. Taking into account both opening and net of closures, we achieved a net addition of 108 stores during the quarter compared to 37 stores added in quarter one. In terms of our store network age, around 30% of our stores are operational for less than two years and the remaining 70% of our stores have been operational for two years and more. It's noteworthy that all stores in the less than 2-years age bracket are still in the ramp up phase. As these stores mature, we anticipate a positive contribution towards profitability. As a guardrail, we closely monitor the timeframe for our new stores to reach breakeven. For stores open between October
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23 and March 24, approximately 59% of them achieve breakeven within six months of operation. As so far , all stores combined reached breakeven in just five months.

In terms of the store size, as at the end of the quarter, our network has grown to 4,552 stores with 2.4 million plus square feet compared to 4,089 stores and 2.1 million square feet at the end of September 23. The average store size was 529 square feet. To give you a sense of spread in store sizes, we have 3,357 stores less than 600 square feet and 1,195 stores that are greater than 600 square feet.

In terms of revenue mix, we are strategically positioned to increase our revenue share from private label products

. Our private label range is crafted to provide customers with high quality products at competitive prices. Presently, MedPlus offers over 1,100 carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for Q2 constitute 18.4% of our total revenue. Moreover, our growing presence in tier 2 cities and beyond is significantly impacting on our revenue mix. Sales from these markets comprise 36% of our pharmacy revenues in the current quarter, marking an increase from 34% in the same period last year. The following is the impact of the launch of MedPlus branded products across our network. In quarter one, FY24, prior to the launch, the share of private label pharma stood at 7.9% of total GMV compared to 16.3% during the current quarter. The increase in the private label GMV share indicates a positive reception from customers and validates our commitment to delivering high quality products under the MedPlus brand umbrella.

Now on our quarter performance in terms of the financial numbers, our consolidated revenue was Rs. 15,762 million with growth of 11.9% year-on-year and 5.9% quarter-on-quarter. Our consolidated operating EBITDA stood at Rs. 739 million representing 4.7% pre IndAS. Around 99% of our revenue is from our pharmacy operations. Revenue from pharmacy operations grew by 18.4% year-on-year on GMV basis and by 11.9% year-on-year on net basis. The pharmacy operating EBITDA stood at Rs. 717 million representing 4.6%.

Our stores performance, I would like to update on our stores older than 12 months. Revenue from these stores in Q2 was Rs. 14,438 million, 94% of pharmacy revenue. These stores had a store level EBITDA margin of 10.2%. The store level operating ROC of these stores stood at 58.1%.

A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 10.2, this was 10.6 for stores greater than 24 months and 7.7 for stores in the 13 to 24 month age bracket. If we allocated non-store related costs, then the operating EBITDA of stores greater than 12 months would be 752 million which translates to a margin of 5.2%. On our diagnostic segment, diagnostic revenue grew to Rs. 283.1 million in quarter two compared to 181.5 million in quarter two of this last fiscal. Diagnostic segment recorded an operating EBITDA of 21 million, 7.4%, compared to a loss of 29 million in Q2 last year. However, central level operating EBITDAs stood at 62 million. We did a restructuring of our legacy diagnostics MedPlus Health Services Limited
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operations in Nagpur, Bengaluru, and Vijayawada. This has resulted in a one-time expense of Rs. 19.7 million, of which Rs. 11.3 million is non-cash in nature. The net operational savings from closure of these centers is around Rs. 7.5 million per quarter. If we exclude the one-time expenses from Q2, then the revised operating EBITDA would be 11.1%, or Rs. 31.5 million instead of the 7.4% mentioned earlier.

An update on our working capital, our net working capital for Q2 was 61 days. The inventory in our warehouse was 38 days. In Q2, the inventory level of our first year stores was 95 days. In comparison, for our stores older than 12 months, the inventory was 40 days.

Now I request Chetan to update on our diagnostic business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. Q2 is a seasonally positive quarter for diagnostics. In July, we sold 459 gross plans per day. In August and September, this was 466 and 461, respectively. As on 30th of June, we had 141,000 active plan and 270,000 underlying lives. As on 30th of September, we had 148,000 active plans and 299,000 underlying lives. Our current observed on-time renewal rate was 25% in Q2 versus 24% in Q1.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you. We will now begin the question-and-answer session. We have the first question from line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: A couple of questions from my end. No as we scale our private label and expand stores what kind of working capital cycle can we envisage because if I see last few years inventory days have been around 83 days on an average and payable days 18 days, is there a scope of improvement on this. If yes then how will that happen, that's my first question. Private level contribution has been increasing. So, if you could give us some more sense is it scaling across pharma, wellness, FMCG all parts of the business, where do you see the margin improvement trajectory if this contribution increases? And lastly of the 600 stores which we are planning to add, is it all leased? Is there franchisee of the current 4,500 stores? What is on lease or

franchisee? And incrementally, how do we look at free cash flow generation this year and say next year considering our expansion plans? So, these were my three questions. Thank you.

G. Madhukar Reddy: I'll take the first two questions. I think the first one was on the overall inventory and payable and working capital. So, inventory levels are also a function of the number of new stores. If I open, let's say, 200 stores in a quarter, each store is going to have at least 18 plus 18, that is 36 stores.

And given that the sales in the first quarter are going to be less, it's going to look like a lot of inventory days for the amount of sales out there on that day. We haven't opened as many stores in the last two quarters, so working capital would have gone down slightly. As we go forward, obviously, that is not the only thing only lever on the working capital, we also have been optimizing inventory in warehouses, as well as in the stores. That is something which we will
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continue to improve. But that said, one should remember that MedPlus, as with any pharmacy store, the success of such a store depends purely on the fill rate or your reputation of getting the right fill rate. So, you can't cut it too fine and not have enough stock and turn away customers.

The cost of lost sales is far higher than the cost of inventory. So, we are always mindful of that. So, we try to optimize it. We don't, we try to make sure that the fill rate is right. So, that's all.

The impact of private label on the other hand, is going to be really good, but it'll take a while for it to happen. The reason is that the private label basically costs us on a product or on a typical product which costs Rs. 100 in MRP, PL cost is roughly around Rs. 12 versus branded pharma costs around Rs. 68. So, if you were to replace the branded pharma sales by private label pharma sales over a period of time, you would definitely see a huge decrease in the working capital. But we are not at the stage as of now. We started only last year in June. And in fact, across the country, we only launched it in October or November. So, right now, whatever private label we have, is in addition to the branded pharma. So, we really don't know which brand, see while we know what category we are selling, we are selling around 700 odd products and for the top 100 products we have a significant, in fact for almost everything which we have, we probably are the largest sellers in that category. But we don't know, we are probably taking up sales of all the 10 brands which are out there. So, we can't really start stocking any of those. It will be a while before we understand and we get to a new model in which we can decrease the inventory. So, the impact of private level inventory while it is going to be there, it's going to take a while for it to actually clear out.

Prakash Kapadia: Private level contribution will increase over a period of time, so what could be the margin trajectory for us with increased private level?

G. Madhukar Reddy: So, private level is two components one is the pharmacy side, I think of which the 17% which you see out there that is a net sales. Pharmacy component would be around 10 or 10.5 and the rest would be non-pharma part which is 10.5 and 7.9, that's the split for private label right now. 10.5 is private label pharmacy, and 7.5 is the FMCG products out there. Both of them are going up. We expect that on a net sale basis, this will increase by maybe around 0.5% to around 0.16:27 every quarter. And additional margin which you get on that is 50%. So, if you increase it by 0.7, you probably increase your margin by 0.3 or 0.35. The margin of private label is 50%. Now, but we also look at these numbers on a MRP basis. The reason is that a product which we would otherwise sell at 83. On a Rs. 100- MRP, where we give a discount of 17 and sell it at 83. Now the same product is getting sold at 45 on a discount of 50% or 55% out there. So, obviously the net sales is going to be lower when you do this, so the percentage grows slower. But to get an apple-to-apple kind of comparison, we usually do a MRP thing. And right now, I think we have MRP, which counts the volume, is at around 16%. We ended the quarter with a 16% sale, and we expect that we will grow this by 1% to 1.25% every quarter. So, that's on the pharmacy side. And obviously, every quarter, we are going to see a small increase in margin because of this. The second thing is, on the general goods, while we are increasing the overall sales, the margin on those is actually the same as the rest of the company. It is not hugely margin accretive. The
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way we see it is, it adds to our customer base. There are a lot of products which we sell, it's obviously it's our own brand and our strategy in that area is to make sure that we sell those

products at a price which is unavailable in the market. For instance, and the reason we do that is because we feel that a customer has got several different options to go and buy a soap or shampoo or toothpaste from. And the reason they would actually pick it up from our store is because, A, it is convenient, they come to us and B, if they pick it up, we want them to come back for the product. We don't want it to be a one-time purchase. And we want them to come back because the product is great and it is available at a price at which no one else sells. So, our private labels actually sell at a price which is lesser than any other private label in the market. So, much lesser than the brands. And that's one reason why it has been growing. But there it brings us customers, loyal customers. It adds to our top line, but the margin is still 20 % or 21%. So, it really doesn't add too much to the margin.

Prakash Kapadia: And lastly, the free cash flow and the operating metrics in terms of franchisee or lease stores? How do we look forward and what kind of free cash flow can we expect this year and in the medium term?

G. Madhukar Reddy: So, all our stores are leased. A very small portion of our stores are franchised. We don't know how that will play out. We will see as we go forward. But in the past, we have basically franchised our stores only in locations where it was difficult for us to reach and service that place. So, we would actually do it in smaller locations. But we are now trying it out in some of the larger cities, too. But it is a very small portion of our overall stores. Every one of our stores is leased. And on the free cash flow, I'll actually let Sujit answer that question on how the free cash flows will play out.

Sujit Mahato: I think based on our network we have presented, you would have observed 1,425 million for the quarter. And this has the components of two things, one, less number

of new stores opened, so that has helped us accrue cash. But otherwise, at least a significant portion of this will continue to accrue as we move forward.

Prakash Kapadia: You alluded to cash flow being better because of slightly lower stores which were added during the quarter. If I were to take the broader annual guidance of say around 600 stores, will this free cash flow accretion happen despite the addition is what we are saying?

G. Madhukar Reddy: I think with lower editions we were able to do this. If we continue with the planned additions, this will be a bit lower.

Moderator: Thank you. We have the next question on the line of Yash from Stallion Asset. Please go ahead.

Yash: My first question is that you've seen almost 200 basis point gross margin expansion right, 23.7 So, this number sustainable going forward?

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Sujit Mahato : See this is on the back of the growth in private label. I don't see any reason why it should not continue, the gross margin.

Yash: And how many stores in terms of percentage are able to do the less than two-hour delivery?

G. Madhukar Reddy: Right now, it is across 3 or 4 cities, Hyderabad, Bangalore, Chennai, and Calcutta. But we have been facing a little bit of a problem in getting people as delivery boys and all. So, we have actually decreased it to a little, to a small extent in some of the cities. So, I think what we typically do is have roughly around 30 to 40 hubs in each city, around 10% of our stores. And these are designated as delivery hubs and from there we do the two-hour delivery. I think I will have to get back to you, but we may have decreased a few hubs in the recent past.

Yash: And since you've opened slightly less number of stores, so any sort of guidance on could be the store opening run rate going forward, are you satisfied with the number of stores that you've opened this quarter or will this sort of be maintaining this sort of run rate?

G. Madhukar Reddy: So, we opened around 145 stores, we shut down around 30 -35. So, that would mean around 108 stores which we opened net this quarter. And I think the overall thing for this year is probably around 145 right now. We had actually guided to a 600 store number, but we could not achieve our usual number in the first quarter because of elections and a couple of other things. So, that will be lost. We will most likely in this year do a store count of anywhere between 400 to 450

stores of net addition. That's the plan for us.

Yash: And last question, the tax rates have been very volatile. So, is there some sort of a sustainable tax rate number that you can sort of let us know?

G. Madhukar Reddy: I think the tax rate what we are talking into the quarter will sustain.

Yash: Because I think 19% is something that's sustainable, going forward ?

G. Madhukar Reddy: That's a factor of the extra deduction what the company claims on section 80JJAA, which is directly linked to the additional number of employees which we hire for our store opening. And that's been a factor of our new store opening s as well .

Moderator: Thank you. We have the next question from the line of Jatin Chawla from RTL Investments. Please go ahead.

Jatin Chawla: My first question is, you spoke about the fact that on a GM P basis, your private label share is 16% and you see scope for a 1% increase every quarter. Till what level do you see this scope? How do you see this evolving, let's say, 18 to 24 months from now? Do you see it going to like over let's say 8 quarters 24 %-25%.

G. Madhukar Reddy: I see no reason why it should not. 1% is a fairly conservative kind of estimate, I would think. Today we have substitutes for roughly around 75% of the products which we sell as medicines.
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We will try to increase this a l

ittle bit. But more importantly, we are finding that some of the smaller towns in all filed a little late. But with all of them coming on board, and in some of the newer states obviously where MedPlus is not as well known , we saw that the uptake was not as high, while the rest of the countr y, there are places where we have as high as 20% private label contribution on MRP basis. We saw some of the newer states at around 8% to 10% and all. We are putting in efforts to get the w hole message out about the brand and everything else in those places. We see as they also continue to pick up over a period of time, the overall number will definitely go up. And 8 to 10 quarters, 1% each, I think obviously difficult to say that far out. But I see no reason to believe that it will not be done.

Jatin Chawla: And it's been a year now, so what are your learnings on this? Your hits and misses and any areas of improvement on this? And second question related on the subscription price that the customer pays for getting private labels. So, any thoughts on how that could shape up?

G. Madhukar Reddy: We were always reasonably sure that the chronic patients, the guys who are buying medicines for their chronic ailments would actually buy these and that has been actually proven. But we are also, I would say pleasantly surprised why those who would otherwise buy only the acute, they also are buying in reasonably large numbers. So, across the board, sales of our products are reasonably high. For us, I think the only thing we need to do is to just make sure that the product is available in every one of our stores, and we continue to increase the range of products available. Today, I think we are probably around close to 700, but we could add another 150 products as we go forward to increase the overall percentage. On, I think the second ques tion was?

Jatin Chawla: Subscription price and how do you see that shaping up ?

G. Madhukar Reddy: We started off at 49 in Telangana and I think a couple of months back, we increased it to 99 in Tamil Nadu, Rs. 99, so we left it there. So, this quarter we've added up a couple places, but Yes, we are going to slowly experiment with this, increasing the subscription price as we go forward.

Jatin Chawla: Just one last question , on the sustainability of margins that we have seen this quarter, so when I look at your last few years of results that you have shown, 2 Q-3Q, it's not as if the revenues go down materially. They are either like flat tish or marginally up, and then 4Q is normally seasonally a very strong quarter. So, going forward, looking out over the next two quarters, do you see this 2Q kind of margin sustainable or are there some one -offs that you would like to call out?

G. Madhukar Reddy: No, I don't think there's anything out there. The gross margin is going to be equally good, if not better. I see no reason for it to come down. And the sales, as you mentioned, should be flat or

slightly up. No reason for it to be any worse than the 2nd Quarter .

Moderator: Thank you. We have the next question from the line of Umakant Sharma from SAFE Enterprise.
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Umakant Sharma: So, I have a few questions Firstly, could you just through some color that there are you know? Many of these quick commerce companies are planning to enter the pharmacy retail space. So, could you just throw some color as to how do you plan to compete with them and do you also have any plans to establish an online presence for yourself? Secondly, in terms of brand building standpoint, could you just highlight what activities are we undertaking in terms of getting that brand awareness, building that brand awareness, brand recall and all of that? Here are the first two questions.

G. Madhukar Reddy: So, I don't think we'll actually do anything on the quick commerce side as of now. We are going to wait and watch and see how that comes along. So, we have been online for a long time, since 2014. 7% of our business now I think comes from online side. We have not given any additional discount or given any additional benefits for online buyers. Hence it has been where it is. On the quick commerce side, if pharmacy is tough for e-pharmacy or for the internet side. I think it'll be even more tougher for quick commerce , but we will see. We'll wait and see how that plays out. Honestly, we don't want to really jump onto anything right now. On the brand building side, no, actually, whatever we are doing, we are doing in the stores, we are actually creating a little bit of testimonial kind of thing. We are trying to give as much information as possible to the customer. We are talking about the fact that 90% of all medicines sold, even in a country like US, are generic. And what we have in India is all generic. So, we believe that our best bet is for people to get educated on the whole aspect. We are not really doing anything on no extra spend for us, outside of that.

Umakant Sharma: But are we picking up any marketing efforts from our side to educate them about the private labels and all of that?

G. Madhukar Reddy: We are doing a little bit of the influencer stuff, some podcasts, a lot of information going out to our regular customers, very pinpointed kind of thing, but nothing ATL. I don't think we spent anything , I think we spent around a crore or so last quarter. So, I don't see us spending a little more than that, a couple of crores last quarter. So, that's going to be the extent of the effort for us.

Umakant Sharma: And sir, on our private label pharmacy side, could you just, like you said, on a GMV level, we are at 16% right now, correct? Could you just split that number between the discounted private label and the non -discounted private label?

G. Madhukar Reddy: So, everything is discounted, but some of it is to members who are, and that is the 50 %-55% discount. That is roughly around 14.6 and the other is probably around 1.5 -1.6.

Umakant Sharma: And sir, would it be, like I believe you mentioned that incrementally we should see the scaling up of the private label discounted fees going up incrementally as well , could you share some insights, let's say, from the stores which we had launched in the very initial stages? What would
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be the level of penetration within those stores, let's say in Hyderabad, Telangana side? What would be our private label penetration within those stores?

G. Madhukar Reddy: I think in Hyderabad it's probably north of 17 and Telangana it is over 20% , rest of Telangana.
Umakant Sharma: And sir just one book keeping question which is rental cost per store per month I am talking about, per store per month, and this quarter it has gone to about 43,800 levels. So, is there any one-off or is there, could you just throw some color around those rental costs?

G. Madhukar Reddy: Now there's no one off, but we will look at it and come back to you if there's anything out there.

Moderator: Thank you. We have the next question from the line of Vijay Shah from RCL Investments.

Please go ahead.

Vijay Shah: My question is the MedPlus brand which you are primarily selling only at your stores, can you expand it to other stores like primarily the stores which are not chain and they are like mom & pop stores?

G. Madhukar Reddy: No, we are not thinking about it right now. We'll give it a thought. But no, I believe people have access to different kinds of generics out there. They probably buy from Cipla, one of the good brand anyway. The MedPlus brand is sold at a 50 %-55%-60% discount rate. And most of the small independent operators sell generics at full cost. And that's how they survive. They have low throughput,

Rs. 3 or 4 lakh a month. They sell 50% of their stuff at a full margin of 60% or 70%. Now, if they were to take MedPlus product and that is openly available in MedPlus stores and we advertise it extensively as being available at a 50% to 80% discount, that won't really work well for an independent operator, so I doubt if anyone would actually pick it up.

Vijay Shah: And secondly, I mean, see quick commerce has shown a lot of success across many categories. So, what would you be looking for in quick commerce to actually go on that platform? So, what are the data points which will convince you decisively that whether you should be on that platform or not?

G. Madhukar Reddy: We'll see how the customers are responding to it because there is a need of prescription. Someone has to upload a prescription, right? And that takes a while. And that doesn't really make it possible for people to deliver in 10 minutes. So, that automatically increases the time. And then again, the range of products required in medicines are far too many. They are not like the generic 500 or 1,000 brands which are or 1,000 SKUs which are sold in quick commerce across all locations. So, for us, it is several hundred brands, and each one, each zip code has a different one and fill rate is a very critical thing for a lot of people. So, if you're buying medication for your monthly needs, then your long list of medication, every one of them has to be there, otherwise they won't really go out there, and they are not really looking for that very quickly. The only guys who are looking for that really quick 10 -minute delivery would be the guys who are probably suffering from headache or diarrhea. And those are all mostly OTC products. I am not really sure if the need for the commerce is as high as it is when you are missing a critical
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component of something you're cooking out there in your home or some urgent need. But that said, we'll look at it. We don't know. We could be surprised by how the customers take to it. So, we are monitoring it closely. We'll see what happens.

Vijay Shah: Just being a customer for my parent, one of the things which we find is that they can't go down many times. When you are sick, you don't want to go down to the medical store. Even the medicine definitely comes in 30 minutes, it is a really good proposition because most of the local stores do not deliver even in 30 minutes and you have to call them so many times. So, it actually the delivery of medicine for sick people and older people might be a very good proposition.

G. Madhukar Reddy: Absolutely I agree. We will see how the people will react.

Moderator: Thank you. We have the next question from the line of Raghavendra, who's an investor. Please go ahead.

Raghavendra: Madhukar Reddy sir, so a special question for you regarding diagnostics. So, in one of the conference calls, you have mentioned that you're exploring B2B channels for diagnostic penetration. So, now given the OPD policies are common across IT employers and as well as all are linked to the TPAs. Any plans of seeing collaboration with the TPAs and ITAs in place for the diagnostic process? It's a very complicated.

G. Madhukar Reddy: We are exploring all options, including working with brokers, insurance brokers, insurance companies, TPAs, and going directly to companies and having this as an add-on product to whatever they have. So, typically, an OPD product costs around Rs. 5,000 to 6,000 for each company. A lot of companies find it very difficult to pay that money, given that their overall outflow on the basic insurance now is around 25,000, and that's quite an expense. And the OPD then is either a hit or miss in the sense that there could be some people who use it, and they could use it to the extent of 10,000, but what they are seeing is a large portion of

people don't use it at

all. So, it's actually a last -minute proposition for the employer who pays that money. So, I am not sure how many people are really taking it up at that expense. There are obviously some very, I would say, profitable companies who will do it. But we feel that if we were to offer our product, which actually costs Rs. 2000 for a family of two plus two to get a 75% discount on every single diagnostic test they can think of for the year and that 2000 then is also converted into wallet points which they can use at a full price , MRP. If that product were to be available to companies at maybe at a discounted price of 30 % to 40 % or even 50% discount for depending on the size of the employer and all that . We feel that there'd be a pretty good reason for companies to actually buy. We have approached a bunch of them. We have a pretty decent set of small to medium sized co mpanies based in Hyderabad who have actually taken it up and have given that option to their customer, to their employees. We think as we go forward, more and more people will sign up.

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Raghavendra: To follow up on this, so on the diagnostic full-service centers, we still miss OPG . So, and only for the OPG, for example, myself, I am a subscriber, so I need to mark the other competitors do. So, is there a plan of when you actually say it as a full-service diagnostic center,

G. Madhukar Reddy: We are looking at that, but Yes, not an immediate thing.

Raghavendra: So, we found that the majority of the interiors are actually done by your own company, CustomFurnish. So, just wanted to understand the relationship between CustomFurnish and also by MedPlus and how much we are actually paying as competitive pricing?

G. Madhukar Reddy: I was the founder of CustomFurnish and a majority shareholder of that company. And that company used to supply lot of the furniture needs for Me dPlus at a time when we were not growing in that space . Now Me dplus has set up its own furniture making unit. It's a small unit, but it supplies all of Me dPlus' needs and going forward, it will also supply all of the diagnostic needs. So, I am not sure exactly what is the value, but I can tell you this for sure. The price at which Custom Furnish sold Me dPlus is not a price you can find anywhere else in the market. It is the least possible listing out there.

Moderator: Thank you. The next question is from the line of Mehul Sheth from HDFC Securities. Please go ahead.

Mehul Sheth: One question on gross margin side. So, when you are saying that you will be able to sustain the gross margin, so should we consider that as an H1 gross margin or the Q2 gross margin?

Chetan Dikshit: Historically as you know, we've been used to talk about 21, 22. Currently this is 23 plus. If you want to be conservative, you could take the average for Q1 and Q2 as H1. But if you want a management sense, we are actually optimistic above the H 1 number.

Mehul Sheth: One more question on your private level when you say you have around 16% of G MV contribution can you split between acute and chronic side? Even if chronic can be more of a like a stick y in nature , so can you split between that too?

G. Madhukar Reddy: Chronic is actually 50% or more and the reason why that is , while one would expect that people who are buying medication for a long -standing kind of disease are a little less open to changing over, the fact of the matter is they are the people who are actually spending a lot of money, month -on-month. And they are probably coming to our store regularly and they know our store employees well. And so when we talk to them about the product and the fact that it is made in the best factories an d it is exactly the same as everything else out there and given the fact that the spend is much higher per month and they can save a lot more by

switching, they are the ones

who actually listen, who take the time to actually understand and basically switch. So, the chronic for us is actually slightly more than 50%. So, that's where it is and that's the good part. Again the question which 22.7 is actually split into the MedPlus pharmacy and the diagnostic margin. So, I don't know if the diagnostic margin can go much further, mainly because we

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already are optimized out there. Now it's just going to go on the topline. The MedPlus is actually

22.7. So, you will see the pharmacy component continuing to grow.

Mehul Sheth: And one last question on your expansion. So, from your presentation, we can see you recently opened a store in Uttar Pradesh. So, are you looking to expand it further in that state, so given it's a much larger state ?

G. Madhukar Reddy: No, we actually 3 stores. one in Delhi, one in Noida, and one in Gurgaon. So, it is Delhi, NCR. While it is in UP, it is more or nearly the NCR area. So, we are just testing it out. We are looking to see how well they grow and everything else and all. Delhi is the last of the remaining big seven cities in which we are not there, in the top seven cities. So, we said , let's just open one and figure out how things are right now and all. All the three stores are actually doing well. We will monitor it a little bit more and then maybe do 4 or 5 or 6 stores in each of those regions to slowly start gaining, let's say a foothold in that area. Two reasons actually, one, it is the last of the big cities in which we are not there. And second, as we, at some point, whenever we decide to do ATL, if at all we decide to do ATL for Me dPlus we want to make sure that we are there in every one of the capitals so that we can supply online in that state. So, being in Noida, if we were to get an order from anywhere in UP, allows us to tie up with the courier and send the MedPlus products out across the state. So, that's the reason for it. But Yes, we are not really immediately looking to expand in any big way out there.

Moderator: That was the last question. I 'd like to hand it over to the management for closing comments.

Srinivas: Thank you. I would like to thank all the participants on this call for your interest in the Me dPlus journey. Our investor relations team can be contacted at ir@metplusindia.com. Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services that concludes this conference. Thank you for joining us and you may now disconnect your line.

Call: Feb 2025

MedPlus Health Services Limited

040-6724 6724

Regd. off. H. No: 11 -6-56, Survey No: 257 & 258/1, Opp: IDPL Railway Siding Road, Moosapet, Kukatpally, Hyderabad – 500037, Telangana, India

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February 06, 2025

The Listing Department The Listing Department
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BSE Scrip Code: 543427 Bandra (East), Mumbai – 400 051
NSE Symbol: MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call :

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst / institutional investors on February 03, 2025 for the quarter ended December 31, 2024.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively .

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

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“MedPlus Health Services Limited
Earnings Conference Call ”
February 03, 2025

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer ,
Mr. Chetan Dikshit - Chief Strategy Officer ,
Mr. DRN Srinivas - Sr. Manager Finance

Moderator: Ladies and gentlemen, good day, and welcome to MedPlus Health Services Limited Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Srinivas. Thank you, and over to you, sir.

DRN Srinivas : Thank you, Sagar. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q3 FY '25 Earnings Conference Call to discuss the financial results of the company, which were announced on January 31, 2025. We have with us today the senior management team represented by Mr. Madhukar Reddy, Gangadi, Chief Executive Officer and Managing Director; Mr. Sujit Mahato, CFO; and Mr. Chetan Dikshit, CSO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note, the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I will now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas and good evening, everyone, on this call. Our company remains committed to balance growth with profitability, while continuously enhancing our operational efficiency.

We are strategically strengthening our back-end operations and infrastructure to support long-term scalability and ensure seamless execution.

As we prepare for the next phase of expansion, we remain focused on optimizing our existing network, while laying the strong foundation for opening new stores across the 13 states in which we operate. As an update, we have added four additional warehouses to enhance their availability at our existing outlet and further supporting our endeavour in opening new stores. This disciplined approach will enable us to drive sustainable growth and enhance value for all the stakeholders.

On our network, we opened 87 stores during the current quarter. Over the past 12 months, we have added a net total of 379, gross of 484 store additions. Throughout quarter 3, there were 27 store closures. Taking into account both openings and closures, we achieved a net addition of 60 stores during the quarter, compared to the 108 stores added in quarter 2 and 37 stores in quarter 1, totaling 205 stores on YTD basis. We expect a total of 300 net store additions in the current financial year.

In terms of our network -- in terms of our stores' network age, around 27% of our stores are operational for less than 2 years, and the remaining 73% of our stores have been operational for 2 years or more. As a guardrail, we closely monitor the time frame for our new stores to reach breakeven.

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For stores opened between January 2024 and June 2024, approximately 55% of them achieved breakeven within 6 months of operations. As a cohort, all stores combined reached breakeven in 6 months. These include stores which have been opened in the new states.

In terms of our store size, at the end of the quarter, our network has grown to 4,612 stores with 2.4 million plus square feet compared to 4,233 stores and 2.2 million square feet at the end of December '23. The average store size is 528 square feet.

On the revenue mix, presently, MedPlus offers over 1,200 carefully selected SKUs spanning across pharmaceutical and nonpharmaceutical categories.

Private label sales for quarter 3 FY '25

constitutes 19.6% of our total revenue. The following is the impact of the launch of MedPlus brand products across our network. In Q1 FY '24, prior to the launch, the share of private label pharma stood at 7.9% of total GMV compared to 17.7% during the current quarter.

On the financial numbers, now on our quarter's performance, our consolidated revenue is INR 15,614 million with growth of 8.3% on a Y-o-Y basis and a degrowth of 0.9% on a quarter-on-quarter basis. Our consolidated operating EBITDA stood at INR 799 million, representing 5.1%. Around 99% of our revenue is from our pharmacy operations. Revenue from pharmacy operations grew by 12.3% year-on-year on GMV basis and 7.9% year-on-year on net basis. The pharmacy operating EBITDA stood at INR 780 million, representing 5.1%.

On a store performance, I would like to update on our stores older than 12 months. Revenue from these stores in quarter 3 was INR 14,388 million, representing 94% of pharmacy revenue.

These stores had a store level EBITDA margin of 11%. The store level operating ROCE of these stores stood at 61.7%.

A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 11%, this was 11.3% for stores greater than 24 months and 8% for stores in the 13 - to 24 -month age bracket. If the allocated non -store related cost, then the operating EBITDA of stores greater than 12 months would be INR 815 million, which translates to a margin of 5.6%. On our Diagnostics numbers, Diagnostics grew -- revenue grew to INR 274.7 million in quarter 3 compared to INR 196 million in quarter 3 of the last year. Diagnostics segment recorded an operating EBITDA of INR 22.1 million, representing 8.1% compared to a loss of INR 34.1 million in quarter 3 last year. However, central level operating EBITDA stood at INR 49 million. On working capital, our net working capital for Q3 was 61 days. The inventory in our warehouse was 36 days. In Q3, the inventory level of our first -tier stores was 88 days. In comparison, for our stores older than 12 months, the inventory was 40 days ...

Moderator: Sorry, sir, we have lost the audio from your line. Ladies and gentlemen, the line for the management seems to have disconnected. Please stay connected while we reconnect the line back.

Ladies and gentlemen, we have the line for the management reconnected. Please go ahead, sir.

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Sujit Mahato: Our update on the working capital. Our net working capital for Q3 was 61 days. The inventory in our warehouse was 36 days. In Q3, the inventory level of our first year stores was 88 days. In comparison for our stores older than 12 months, the inventory was 40 days.

Now I request Chetan to update on our Diagnostics business. Over to you, Chetan.

Chetan Dikshit: Thank you, Sujit, and good afternoon, everyone. Q3 is a seasonally weak quarter for Diagnostics. In October, we sold 413 gross plans per day. In November and December, this was 433 and 419, respectively.

As on 30th of September, we had 148,000 active plans and 299,000 underlying lives. As on 31st of December, we had 152,000 active plans and 315,000 underlying lives. Our current observed on-time renewal rate was 26% in Q3 versus 25% in Q2.

That concludes our update for this quarter. I request the host to open the line for questions.

Moderator: Our first question comes from the line of Saion Mukherjee from Nomura Securities.

Saion Mukherjee: So my question is around the growth in the Pharmacy business. So you mentioned, the GMV growth of 12.3% this quarter, now, obviously, there is some slowdown in new store addition that might have played a role. And also the -- if I look at the branded business, right, branded pharma number, probably, we are growing in low single digit, which is obviously much lower than what the pharma market

growth is.

So can you just throw some color on how the dynamics with respect to private label is playing out? Is there a very significant substitution that's happening from private label to branded, which is leading to very slow growth in branded? And now going forward, how are you thinking about the overall GMV growth in the pharmacy in the quarters ahead, please?

Gangadi Madhukar Reddy: So if you look at the growth in the pharma...

Moderator: Sir, we have lost the audio from your line once again.

Gangadi Madhukar Reddy: I think you have to reconnect.

Moderator: Ladies and gentlemen, the line for the management seems to have been disconnected. Please stay connected while we reconnect the line for the management back. We have the line for the management reconnected. Please go ahead, sir.

Gangadi Madhukar Reddy: Thank you. So as I was saying, we actually look at the overall pharma sales. And for us, it is not -- there's no real division between private label and brands and all. Growth is growth. We offer the customer an alternative when it comes to the store. We tell them, "Okay, we have got what you requested for." And we also have an option in the private table, and that is at a 50% to 80% discount.

Now one thing may get substituted for the other, depending on the customer's comfort with the overall thing and all. So for us, as long as we are growing the overall pharma pie, we are fine.

Now the growth itself, I know, was a little muted actually this quarter, and that is partly because

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the actual net sales would be slightly muted, I would say, mainly because the private label sells at a much lower rate and all.

But yes, outside of that and a little bit of seasonal, I would say, effect having plenty of holiday

season and all, especially in January and all, we did see a slightly, I would say, lesser sale than expected, but I expect that to get back.

Saion Mukherjee: Are you willing to guide any number for the GMV growth? Or how should we think about it over the next year or so?

Gangadi Madhukar Reddy: So it should be the usual listing of inflation plus kind of thing or it should be in line with the typical pharma market growth out there. So as we keep saying that for us, it is more important that we maintain the store -level EBITDA of 10% or 11% rather than just trying for same -store sales growth and all.

Some amount of cannibalization is always going to be there when we do this when we set up so many stores. But I'm fairly comfortable in saying that we should growth on the 2 -plus year stores also at; inflation and slightly above that.

Saion Mukherjee: Right. And my second question would be on your private label and gross margin. In this quarter, we have seen a significant pickup in private label, which is also reflected in gross margins, much higher than what you had guided in the previous quarter. So how should we think about that going forward?

Gangadi Madhukar Reddy: So think about the private label as going up by roughly around 1% every quarter on the volume basis, which is on MRP basis, which means additional margin of roughly around 15 to 20 basis points maximum per quarter. We could have 1 or 2 slightly better than normal kind of quarters and all, but I'm fairly comfortable we should be able to do 1% every quarter.

Moderator: The next question comes from Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: Yes. So continuing on the previous participant's question, apart from the fact that your share of private label will kind of go up, what are your thoughts about the discounting policy in the private label?

I know right now, you are at 60, 65. At what levels do you think that maybe you could take a lower discount into consideration? And additionally, adding to this point, so let's say, the final brand that you are discounting against takes a hike, does that g

et reflected in your discount price also?

Gangadi Madhukar Reddy: Okay. About our private label discount itself, we advertise a 50% to 80% discount based on the molecule. And I think the average discount tough comes out to around 50%, 51%, that's all, because most of the higher selling drugs are all around the 50% discount.

On -- I don't expect that we'll really reduce this at any point of time. It may go down slightly over a longer period of time, I would think. But as of now, given where the discounting is on MedPlus Health Services Limited

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various generic products and all, I don't think we'll be able to reduce this much lower than what it is right now.

I think the second part of the question, if I understood it right, was, is the discount reflective of what the original brands we're substituting for. Not -- if that is what -- that was your question, no, not necessarily. We set the price based on the average price of the top 2 or the leader -- the brand leader out there because most of the people would end up actually buying those.

And in places where the thing is fragmented a lot more, the market is fragmented a lot more, and there are 1 or 2 or 3 players, all of them having more than 20%, 25% market share, then we will probably take an average outcome somewhere towards the middle of that and then set the price -- set the discount such that the net price to the customer is around that. The discount is off the average of these 3.

Sudarshan Agarwal: Got it, got it. And one more additional. While I understand the net store addition this year you have trended. For next year, are you guiding any number that we should kind of take into account, 500, 600 or it could be lower than that?

Gangadi Madhukar Reddy: No, I think it will be at least 600 stores for -- I don't think it will be less than 600 stores. This year, we have had a few things which we had to actually take care of because of our focus on private label and a couple of other things.

We also are -- as Sujit spoke in this call, we are also in the process of strengthening the back end. We have actually launched a bunch of warehouses across the state, in which we are there right now.

These will help us actually reach the stores faster and help us to actually grow faster, too. So a lot of that work is on. I expect that we'll be done in this quarter and we'll be able to start with the usual growth next year onwards.

Moderator: The next question comes from Tanmay Gandhi from Investec.

Tanmay Gandhi: Congrats on a good set of numbers. Sir, my question is again on private labels, right? So on the GMV, which is they are already at around 20% level, right, for the Pharmaceuticals, so my question is that do we need to put in more efforts to actually increase the contribution of the private labels from here on? Or do you think that, that it is going to be more organic?

Gangadi Madhukar Reddy: Tanmay, if you mean are we going to spend a lot of money advertising, I don't think so. What we'll end up doing is probably increasing the number of products which we have in private label. We are today, for the overall set of medicines, we probably are there at roughly around 68%. I expect insulins, which cover around 10% of the overall medicine sale, and some branded products will be another 5%, so which means that leaves at least around 85% to be targeted. I expect, if not 85%, it will probably take the 68% to maybe 75% to 77% or so. So increasing the number of products would be one thing. Making this available on a much better level than what we have been able to would be another thing.

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And third, as the number of people who buys these continue to increase, I expect that over a period of time, the word will spread, and we will basically see a lot more people asking for the product. It may not be -- we may

not really need to push as much as we are doing right now at this point.

Tanmay Gandhi: Yes. So sir, is it fair to assume that for the next year, the core focus area for the management will be store addition?

Gangadi Madhukar Reddy: The focus will be definitely on adding new stores also, 600 stores odd. And then same -store sales growth. Private label will be -- will continue to grow anyway. So that's going to be 1% per quarter is what we're aiming at. It could be slightly better, but I think we'll be actually able to hit that number easily without too much of a profit.

Tanmay Gandhi: And sir, one more question. The 4 new warehouses which you have launched, right, so that is largely for the upcoming stores, which you are planning to add in FY '26. Are these also to cater to your own private labels and overall increase the volumes at your existing stores?

Gangadi Madhukar Reddy: No, no. These are actually to increase the overall operational efficiency. For instance, that we have actually added around 3,000 stores in the last 4 years. In some of the states, we have got now around 1,000 stores each. Karnataka, we got 1,000 stores. Tamil Nadu, we have 1,000 stores. And AP Telangana combined, we have more than 1,200 stores.

So some of states are big, and we have not been able to reach the extreme parts as easily. And so supplies have not been as good as we wanted it to be. And so sales have suffered a little bit. So now we're setting up a warehouse in Hubli, one in Madurai. And 2 in Agra, 1 in Vijayawada, 1 in Vizag and 1 in Kapi and so on. So dividing up the states into 2 parts now. At least, our main area from where we operate and manage around 60%, 70% of the stores and a subarea from where you manage around 25% to 30% of the stores. So West Bengal, we probably have 1 warehouse in Asansol area and so on.

So this is -- we'll do 2 things. It will supply our current stores better. And with the operational heads sitting out there in that area, we probably would be able to launch new stores faster in those areas, too.

Tanmay Gandhi: Understood. So what is the plan for warehouse addition for next couple of years?

Gangadi Madhukar Reddy: So I think I pretty much told you where all we're going to put. We are adding a few more, let's say -- actually, there's one more coming up in Nagpur. In the new states, we're adding warehouse, a small one in Raipur and 1 in Indore. That will help us actually go deeper into Madhya Pradesh as we go forward.

But I would think most of these, we have already acquired. So you're probably already seeing the rents in the cost already, but we probably will add 3 or 4 more warehouses. I think a total of 10 smaller warehouses, anywhere between 30,000 to 50,000 square foot kind of leases. That includes the ones I've already talked about.

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Moderator: The next question comes from Madhav from Fidelity.

Madhav: So I had a slightly longer -term question. So now that our new MedPlus private label has been in the market for about 6 or 7 quarters, maybe 6 quarters, if I'm not wrong, so I think -- and it reached about high teens or almost 20% of our sort of volumes. So my sense is that it's kind of stabilizing within our network over the last few quarters now.

Now that some of this has been absorbed into the system, could you give us some sense on a 3 - year view if you think about network expansion, if you think about how gross margins would change for us?

And EBITDA margin has crossed 5% for the Pharmacy business, which is quite healthy. So is this a sustainable way to work with for us? As we think 2, 3 years, can it go a bit higher? Could you give us some sense because we've done a big transition in terms of the product mix, but how

are we thinking 3 years will be really, really helpful?

Gangadi Madhukar Reddy: So Madhav, as I said earlier, even if you take it for the

next 8 quarters, I fully expect that we will

be able to grow the level of 1% per quarter on the MRP basis, right? I expect -- see, this is something which we feel we can achieve without too much of a problem.

Now we could get benefited from the network efforts of these. A large number of people using it, being happy about it, maybe talking about it, one. Two, government pushing the generic products even more aggressively. And three, possibly competition also taking these up and talking up the generic -- the value of generic products out there. So all these could see, I would say, a disproportionate increase.

It is possible for it to happen that way, but I'm not -- I wouldn't bank on it. I would say, by our own thing, just by increasing the product -- increasing the range of products and also pushing it a little bit, we should be able to get that 1% for every quarter easily.

And that would mean possibly -- and if nothing else changed, I would say 1% growth in gross margin because every 1% in MRP, I think, increases our margin by around 20 basis points.

Madhav: So essentially, gross margins from here only have scope to move as the private label or MedPlus brand private label sort of improve. I understand that bit. If you're adding 600 stores, it seems like we can grow our network at, let's say, 12% -- 11%, 12% network expansion each year. So that's low double digit.

So is it fair to assume that if you have some SSSG as well, we're looking at 15%, 16% sales growth with margins expanding for us from here? Or do you think the 5.1% margin has any elements, which can bring it down? I understand seasonality of the business, which would be a different quarter would have different seasonality. But are we at a fairly sustainable base now? Or how should we think about margins going ahead? Can you be 5.5%, 6% in 3 years?

Gangadi Madhukar Reddy: No, I think we are at a fairly sustainable kind of margin level for the company out there. Now the only reason which can change it is if we add a ton of stores -- new stores. But as you said, the number of stores, which we add are going to be much lesser than the base which we have.

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So I really don't see a significant dip out there. What we can do is we can call out that number as we go forward. But the current set of stores should only be at this level or should get better -- sorry, it's not going to be a significant dip. There will always be a drag as we add stores, but it's not going to be significant.

Madhav: And if the Pharmacy GMV growth, which is 12%, which is kind of proxy for volumes, do you think that should grow in line with our store expansion? Or can it grow a bit faster? I don't know how -- is that the right way to think about the Pharmacy GMV growth?

Gangadi Madhukar Reddy: It's a little difficult to tell you, but I am sure that as the new warehouses come into play and as our supply chain also become slightly more efficient, I think we should be able to get some benefit. But I can really commit anything. We will maybe guide you in the next couple of quarters on any efficiency coming from that side.

Madhav: Sir, how much cost can these warehouses save for us? Is it any incremental cost savings which will come through or it's only sort of to kind of help the next leg of growth for the company?

Gangadi Madhukar Reddy: It is only to help the next level growth, that's all.

Moderator: The next question comes from Harith Ahamed from Avendus Spark.

Harith Ahamed: I joined the call a bit late, so my apologies if it's repetitive. The first question is on Pharma private label GMR. What exactly was the share of Pharma private label as percentage of our total GMV for the quarter.

And Madhukar, your comment that we are looking to increase this by 1 percentage point every quarter, is there a level that we're targeting and beyond which probably any further

increase

would be difficult given that branded generics will still be a significant part of our overall business and a significant part of the overall market?

Gangadi Madhukar Reddy: Yes. So the private label itself on a GMV basis is around 17.6% right now on the Pharma side. If you look at the net sales side, I think it's around 1.6%. The non-Pharma is also pretty significant now, around 8% overall. So we are a little under 20% right now for the overall private label on a net sales basis.

On our aspiration for growth, I have always been saying this. If the customer actually starts to believe that the product is same and if we can actually put in substitutes for all of products, I know there are some which we will not be able to do for a long time, insulin and some -- obviously, leave the patented products aside. Other than that, only insulin is something which we will not be able to do a substitution for.

But outside of that, if you are able to put everything out there and over a period of time, over the

next 2, 3, 4, people are not going to be, I would say, completely blind to the fact that these are all the same products being made in the same factory. And across the world, this is what actually sells. If you look at a country like U.S. and all, 90% of what they sell is generic, and that's all store generic. That's just one -- any one store basically selling their products.

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So people are not going to be completely blind to all this. I expect as we go forward, this will go all the way up to whatever numbers there are across the world. I don't see any limit to it. But over the next 7 or 8 quarters, I would still say, maybe 1% every quarter.

Harith Ahamed: Okay. Got it. My second question is on discount rates. So earlier, before we launched the branded Pharma private label, we used to share that number of around 16%, 17% as the discount rate -- blended discount rates across our various formats. Now my first question is, what is the current discount -- blended discount rate in the Pharma private label? And what is that rate for rest of the business, which used to be at that 16%, 17% level in the past?

Gangadi Madhukar Reddy: Okay. For the Pharma private label, it is around 51% right now. For the rest, I don't see any reason why it should be any different. It used to be around 17% or 18%. It will probably be the same number even today. But I'll tell you -- I'll come back to you with the numbers.

Harith Ahamed: Okay. Last one on the provision, the MRP level growth that you've shared for the quarter, it's around 8.5% and a decline versus Q2 where we had 13%, 14%. So any reason -- compared to the lower store addition could be reason that, that was the case even in 2Q? So I'm just wondering what has led to this lower growth rate on a sequential basis.

Gangadi Madhukar Reddy: A little bit of effect of seasonality, holiday season and everything else. And honestly, I can't think of any other reason out there. Possibly, some of the stuff which we're doing on the warehousing and everything else will add a little bit of efficiency and all. Possibly that our super quick addition of stores may have compromised the supply chain a little bit. And that also could have cost it, but I'm not sure. But I feel that -- I believe it's largely seasonal effect, nothing more.

Moderator: The next question comes from the line of Prateek Poddar from Bandhan Asset Management Company.

Prateek Poddar: I was just wondering if you could give me SSSG basis GMV? What could be SSSG growth basis GMV? Hello? Am I audible?

Gangadi Madhukar Reddy: Yes, yes. No, we can hear you. Outside of what is there in the slides we have given, we have to come back to you with the more granular breakdown. I think on the GMV slide you're seeing, the growth is year-on-year 8.3, but we will come back to you with the actual SSSG.

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Prateek Poddar: Yes, please. The second question is distribution of private label, right? Obviously, the average is 10.7 for the system. But maybe you could give us a sense in terms of the median number or, let's say, what's the difference between the highest store and the lowest store? Just trying to get to -- get an understanding as to where are we trending. I understand you called out 1% per quarter, but just that would help.

Gangadi Madhukar Reddy: Yes. Sure. So if you see across the board, the smaller towns and the rural areas are going to have

a higher private label than the bigger cities, one.

Second, the older states in which we have been functioning are going to be higher than the newer states in which we have gone to. For instance, Madhya Pradesh and Chhattisgarh are going to

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be at a slightly lower private label number than AP, Telangana or West Bengal or Orissa, where we've been for a long time.

To give you an idea, I think Maharashtra, which is as urban as you can get, it's probably at around 7% or 8%. I think Bombay and all are at 7% or 8% for us on the Pharma side private label or maybe slightly less. On the other hand, a smaller town in Telangana or in Orissa or West Bengal is probably close to 20%.

Prateek Poddar: 20, 20, right?

Gangadi Madhukar Reddy: Yes.

Prateek Poddar: And this is on a net sales basis, correct?

Gangadi Madhukar Reddy: Yes. It's on net sales basis.

Prateek Poddar: Okay. And the last question. Look, when I see your gross margins, a sequential improvement of 130 basis points. And when I look at your operating EBITDA margin for stores older than 12 months, only a 40 basis point of improvement, can you help me understand where does this 80 basis point go? Or where have you reinvested?

Gangadi Madhukar Reddy: Sorry. Could you just repeat the question?

Prateek Poddar: No. So when you look at -- Madhukar, when you look at your gross margins, right, on an overall basis, 130 basis points of improvement on a sequential basis, correct? But when I look at store operating EBITDA margins, which is post all the unallocated -- I mean, corporate costs, et cetera, went up from 5.2% to 5.6%, so 40 basis point improvement. I'm just trying to understand why a lot of it didn't flow down. What am I missing here?

Sujit Mahato: Sure. In terms of the movement, I would say around 60 basis points came in because of our increase in the private label Pharma share mix. 20 basis points came in because of the non - Pharma private label share mix.

And due to lower inventory provision, we had a 60 basis positive impact during the current quarter. And your observation is right. The entire thing did not flow down to EBITDA as certain costs also went up.

For example, in Tamil Nadu, the minimum wages increased by 28%. This has also eaten away some portion of our store -level EBITDA. And we also set up, as we had explained earlier, certain warehouses, which has costed us some amount of money.

Moderator: Next question comes from Prakash Kapadia from Spark BMS.

Prakash Kapadia: A couple of questions from end. What exactly is the store -level revenue growth which is reported in the PBT? Because if I take the absolute revenue growth of mature stores, it doesn't come to 4.5%.

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Typically, most of the retail companies break sales down into SSSG growth, new store addition and sales from new stores. So if we can do that, it will give a lot of clarity to investors. As you know, retail, that's the key metric. Because here, it's very difficult to understand the number, 4.3, 4.4. So if you could give some insight, that would be helpful.

And secondly, as I see net store additions were 60 during the quarter, which brings us to around 205 stores. So what has changed? Because in every quarter, we are scaling down the

expected

store addition. So those are my 2 questions.

Gangadi Madhukar Reddy: Yes. So for us, I know this year, we have been a little up and down on the new store opening and all. A lot of our bandwidth has been taken up by focusing on the private label, one. And two, on setting up the new warehouses and everything else, we have seen that some of the supplies have suffered a little bit because of the super fast growth, which we have had in the last few years. So some of that, we are already getting done.

On the EBITDA, it's -- on the same -store sales growth itself, for us, we actually -- and this is something which we have been talking about even before the IPO and all. For us, we always look at the overall store -level EBITDA. And to us, that is the most important thing.

The reason I say that is in markets like Hyderabad, Bangalore and Chennai and all, where we already have 300, 400, 450 kind of stores, we have to put in new stores because new markets keep coming up. And no matter how much of a different catchment it is, it ends up cannibalizing some of our stores anyway. And so there is always that effect.

So if you were to basically look at it, a store which is doing, let's say, INR 1 lakh per day, if we -- is probably bringing in more customers from slightly farther than what you would want. And so if you go and set up a new store in that catchment, maybe 500 -- 750 meters away, we will always see some decrease in sales. We're not really concerned about that.

When we set up a new store, we try to figure out whether it will breakeven in 3 or 4 months or not, and whether it will actually get to the expected level of EBITDA within 2 or 2.5 years, which is around 10% at the store level.

And two, on the existing stores, we -- let us say, even if the sales were to dip a little bit, our goal would be to make sure that these are -- the store is doing 10% or more.

So overall, we will continue to gain market share and everything else in that area. But for us to mention exclusively SSSG would be a little bit more tough given how we operate. So for us, it's more the market level and the 10% EBITDA kind of thing, which we actually look at.

Prakash Kapadia: I do understand the point of profitability. But typically, that's the standard retail metrics, which is the key performance indicator. And it becomes difficult to evaluate businesses and cycles over a period of time. So think about it, I think maybe the SSSG breakup, new store addition and store -- sales from new stores would really be helpful to investors. That's my suggestion.

Gangadi Madhukar Reddy: We'll definitely consider that.

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Moderator: The next question comes from Akhil Parekh from B&K Securities.

Akhil Parekh: Congratulations on a good set of numbers and healthy operating cash flow generation. On the penetration level of private label, you did highlight that the penetration levels are different across small versus larger towns. But going forward, so next 2, 3 years, how are you seeing the private labels? Will it be because of the increasing depth of private label in the existing stores? Or more by increasing the breadth of presence of private label in the stores?

Gangadi Madhukar Reddy: So both. We definitely increased the range of products, which we have out there. And we definitely try and convert more customers into our private label. But I also hope that some of the positive benefits, which the private label has on consumer population out there will get talked about and more new customers will walk in from neighboring stores to our stores. And so that will also help us in basically increasing our overall sales growth.

For us, it's going to be a combination of all 3. And I think the longer we are in the market, the more likely it is that we continue to actually grow.

Akhil Parekh: Second question on store expansion, right?

t? In answering the previous participant, you highlighted that the focus is more on the profitability rather than growth and more on gaining the market share within that particular geography.

So we have seen a reduction or decline in SSSG over the last 4 or 5 quarters over Q3 of FY '24 versus Q3 of '25. I believe it's part to do with the increasing sales contribution from private label. But are we monitoring if there is a cannibalization of sales because we are expanding aggressively within a similar cluster?

Gangadi Madhukar Reddy: So we are, but the thing which concerns us even more is that each individual store is basically sticking to the profitable -- profitability numbers we should expect to have. So that would be the first thing we should actually look at, one.

Second, one of the things which probably is showing up as less efficiency and all for us is also this, that we've always been talking about stores, which are more than 1 year. The number of stores which are between, let me say, 12 months to 24 months have been decreasing constantly. And that number -- the -- those number of stores -- the stores between 12 and 24 are going to be growing much faster. And so decrease in those number of stores would have probably shown a decreasing overall number. But I don't think the overall mature stores themselves are behaving any differently from what they were earlier.

Akhil Parekh: Okay. Sir, should we assume that SSSG of, whatever, 4%, 5%, is that a sustainable rate the next 2, 3 years perspective? Because SSSG certainly helps in terms of absorption of fixed cost, right? At one end, we want to improve the profitability. And at other end, if we're not focused on SSSG, it may not help us in terms of absorption of fixed costs, right? That's how typical retail model works. So what would be a normalized SSSG growth rate if we were to expand at 500, 600 outlets every year for next 2, 3 years?

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Gangadi Madhukar Reddy: So I think we'll be at least at the level currently where we are. I don't really see dipping any further and all. I don't think that's got to be a problem because we are not going to be adding -- so we are looking to add additional stores next year. So around ...

Akhil Parekh: Sorry. Just to clarify. You're saying 3% to 4% is the base -- new base at least.

Gangadi Madhukar Reddy: 4% to 5% is what we are looking at, actually. That is a number at least we can be at a steady state as far as the profitability is concerned.

Moderator: The next question comes from the line of Prolin B. Nandu from Edelweiss Public Alternatives.

Prolin B. Nandu: While you have talked a lot about private label, but my question is also on the same. So Madhukar, you mentioned about the difference between metros and nonmetro -s, but are you positively surprised by how fast the private labels are taking off? Can you give us some instances as to what are the products within Pharma, which are tough to crack? And have you had customers come asking for private labels in those kind of products as well?

And in some of these stores where you have introduced private label much earlier than rest of your network, are you probably surprised by, let's say, that number from a net sales point of view reaching maybe 25 -odd percent? Can you give some qualitative color on private label and whether it is surprising your internal estimates there?

Gangadi Madhukar Reddy: Not really. We expected this growth. I actually expected a slightly higher growth, honestly. But yes, this is not out of -- I would say, it's not surprising. And as I said earlier, the numbers are smaller towns to slightly better and all. And across the country now, we are almost at the same level, I would say, because it's now been more than, I would say -- we started in June of '23, it's almost 1.5 years right now. The full country started in November. So

even for the full country,

it's still more than 4 quarters right now. So not much out there.

But to give you an idea, the number of -- I would say, the private label share of chronic versus acute is exactly the same as it is for the regular brands. So I think 60% to 70% of our sales are private label comes from the chronic segment. And so people are buying all those Telmisartan, Atenolol and everything else also at the same level. The top 10 molecules probably will have at least 6 chronic and 4 acute molecules.

Prolin B. Nandu: Okay. That's clear. Now the second quest is more on competition, let's say, from quick commerce side. While what is appreciated that a typical quick commerce versus a quick commerce for pharma requires a very different kind of a setup.

But in case if the competition were to ramp up and you were to match their service levels in terms of 2 hours or whatever for all your stores, which you are doing for some part of your stores, how much of a drag will it be on margin? How soon you will be able to ramp up, right, to probably meet the standards of quick commerce?

Let's say, even if you were to launch 2 -hours delivery from your stores, right, how soon can you do it? And what kind of a drag will it create on your margins?

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Gangadi Madhukar Reddy: Yes. As long as the customer is willing to pay the delivery fee, I don't think there will be a drag at all. If the delivery were to be 100% free, today we charge around INR 20. If we shift to a 0 delivery fee and also try to kill ourselves by doing a 10 -minute delivery, then, yes, it would be a drag. But I don't foresee that happening. If we decide to do a 10 - or a 20 -minute delivery, it would bring on delivery partners. We're looking at the prices right now. The prices are not something which a customer will not pay. So I don't really expect this to be a drag.

Prolin B. Nandu: Right. But how soon will it take, Madhukar, for you to ramp up, let's say, where you are offering a 2-hour delivery all your stores?

Gangadi Madhukar Reddy: No, no. We'll never do all our stores at any point of time because customers across all the 600, 700 small towns in which we are there don't really expect this service and don't really want to pay that kind of delivery fee also. It will always be there in the urban areas, in the highly dense networks where we can actually go and supply. I don't think it's going to -- so we already have a 2-hour thing.

Now do we want to ramp it up to a much faster delivery time or not? That's something which we have not yet decided. But we are monitoring the situation right now, and we'll figure out the best way forward in the next...

Prolin B. Nandu: Sure. But Madhukar, a couple of quarters back, you had probably ramped down this, right, in the number of stores or whatever your reach was. And right now, what I understand is that you're not sensing any reason for you to ramp up, right? Am I correct in terms of competition and in terms of what the market is doing?

Gangadi Madhukar Reddy: See, we are monitoring the place -- space right now. We will see, depending on how the whole -- so it all depends. If we are able to get some delivery partners or able to do it at a very good price -- at a price at which the customer is willing to pay, then we would do it all right. But yes, we will have to figure that out.

Moderator: Ladies and gentlemen, we would take that as our last question for today. I now hand the conference over to Mr. Sujit for closing comments.

Sujit Mahato: Thank you, Sagar. Thank you, ladies and gentlemen. I thank all the participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you for joining us. You may now

disconnect your lines.

Call: May 2025

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May 30, 2025

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Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst / institutional investors on May 28, 2025 for the quarter and year ended March 31, 2025.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively .

Kindly take the same on record.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

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“MedPlus Health Services Limited
Q4 FY'25 Earnings Conference Call ”
May 28, 2025

MANAGEMENT : MR. GANGADI MADHUKAR REDDY - CHIEF EXECUTIVE

OFFICER AND MANAGING DIRECTOR

MR. SUJIT KUMAR MAHATO - CHIEF FINANCIAL OFFICER ,

MR. CHETAN DIKSHIT - CHIEF STRATEGY OFFICER ,

MR. DRN SRINIVAS - SR. MANAGER FINANCE

MedPlus Health Services Limited
May 28, 2025

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Moderator: Ladies and gentlemen, good day, and welcome to the MedPlus Health Services Limited Q4 FY '25 Earnings Conference Call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch -tone phone.

I now hand the conference over to Mr. Srinivas. Thank you. And over to you, sir.

Srinivas: Thank you, Sagar. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q4 FY '25 earnings conference call to discuss the financial results of MedPlus for the fourth quarter and full year FY '25, which were announced yesterday. We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, CEO and MD; and Mr. Sujit Mahato, CFO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward -looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website. I would now hand over the call to Sujit. Thank you. And over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone on this call. As we prepare for the next phase of expansion, we are strategically strengthening our backend operations and infrastructure to support long -term scalability and ensure seamless execution. We remain focused on optimizing our existing network while laying the strong foundation for opening new stores across the 13 states in which we operate.

As an update, we have added around 10 additional warehouses in the last financial year to enhance our availability at our existing outlet and further supporting our endeavour in new store expansion. This disciplined approach will enable us to drive sustainable growth and enhance value for all the stakeholders.

An update on the network, we opened 113 stores during the current quarter. Over the past 12 months, we have added a net total of 305 stores, gross 398 store addition, and we closed down certain stores. Throughout Q4, there were 13 store closures. Taking into account both openings and closures, we achieved a net addition of 100 stores during the quarter compared to the 60 stores added in Q3, 108 stores added in quarter 2 and 37 stores in Q1, totaling 305 stores on YTD basis.

We expect a total of 600 net store additions in FY '26. In terms of our stores' network age, around 22% of our stores are operational for less than 2 years and the remaining 78% of our stores have been operational for 2 years or more.

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As a guardrail, we closely monitor the time frame for our new stores to reach breakeven. For stores opened between April 2024 and September 2024, approximately 58% of them achieved breakeven within 6 months of operation. As a cohort, all stores combined reached breakeven in 5 months.

In terms of our store size, as at the end of the quarter, our network has grown to 4,712 stores with 2.5 million plus square feet compared to 4,407 stores and 2.3 million square feet at the end of March 2024. The average store size is 527 SFT. In terms of the revenue mix, presently, MedPlus offers over 1,200 carefully selected SKUs spanning across pharmaceutical and nonpharmaceutical categories.

Private label sales for Q4 FY '25 constitutes 23.3%, pharma 13.6% and non -pharma 9.7% of our total revenue. The following is the impact of the launch of MedPlus branded products across our network. In Q1 FY '24,

prior to the launch, the share of private label pharma stood at 7.9% of total GMV compared to 20.9% during the current quarter.

Our consolidated revenue stands at INR15,096 million for the quarter and INR61,361 million for FY '25. Our consolidated operating EBITDA stood at INR803 million, representing 5.3% for the quarter and INR2,776 million, representing 4.5% for the full year FY '25. Around 99%

of our revenue is from our pharmacy operations.

Revenue from pharmacy operations grew by 6.2% on Y-o-Y basis on GMV terms and 1% Y-o-Y on net basis. On the full year revenue from pharmacy operations, it grew by 14.8% on GMV basis and 8.6% on net basis. The pharmacy operating EBITDA stood at INR769 million, representing 5.2%.

On the update on our store performance, I would like to update on our stores older than 12 months. Revenue from these stores in Q4 was INR14,011 million, representing 95% of pharmacy revenue. These stores had a store level EBITDA margin of 11.5%. The store level operating ROCE of these stores stood at 59.2%.

A word here on the store level EBITDA margin by age, while stores greater than 12 months had a margin of 11.5%. This was 11.7% for stores greater than 24 months and 8.2% for stores in the 13 months to 24 months age bracket. If we allocated the nonstore-related costs, then the operating EBITDA of stores greater than 12 months would be INR817 million, which translates to a margin of 5.8%.

On working capital, our net working capital for Q4 was 63 days. The inventory in our warehouse was 37 days. In Q4, the inventory level of our first year stores was 108 days. In comparison for our stores older than 12 months, the inventory was 43 days. Our Diagnostics numbers. Diagnostics revenue grew to INR280 million in Q4 FY '25 compared to INR232.4 million in quarter 4 FY '24. Diagnostics segment recorded an operating EBITDA of INR34.3 million compared to a loss of INR11.3 million in quarter 4 last year.

In January, we sold 420 gross plans per day. In Feb and March, this was 462 and 478 plans sold per day, respectively. As on 31st December, we had 152,000 active plans covering 315,000 underlying lives. As on 31st March, we had 157,000 active plans and covering

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327,000 underlying lives. Our current observed on-time renewal rate was 25% in Q4 versus 26% in the last quarter.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. Our first question comes from the line of Saion Mukherjee from Nomura Securities. Please go ahead.

Saion Mukherjee: Yes. Sir, this question on GMV growth at pharmacy, which you mentioned is at 6.6%. Given that the pharma market itself is growing at around 7%, and we would expect MedPlus to grow faster, what is -- what in your assessment is impacting the overall GMV growth in pharmacy?

Madhukar Reddy: So a couple of things, Saion. The one main one would be the mix of private label and branded products. Our private label, of course, sells for a much lesser price than the branded products.

So the top line is going to be slightly lesser. That's one. Second, and I know we will come to this question going forward.

Our focus on private label and the incentives which we have set up for our employees have also probably dampened the overall or muted the sale of branded things to a small extent, right? I don't think it is significant. I think what we did was absolutely necessary to get the entire organization enthused about selling private label and everything else. So I think we have achieved what we set out to achieve. Now we're basically again aligning everyone to both private label as well as branded products. So I think we'll probably recover it again.

And the third thing, which probably muted our overall sales and maybe did not give us the SSG, which we really required was because of the rapid

id expansion, we had a few constraints,

both on manpower side on the back end and also on the warehouses, warehouses in particular.

We have now started the process of getting the warehouses. We're seeing the benefits in just West Bengal where it is launched, but the rest are still to come. So I think going forward, we'll probably see the benefits of all those things as we go forward.

Saion Mukherjee: Yes. Actually, I was mentioning about the GMV, which shouldn't have got impact. I can understand the transition and the mix change in favor of private label. But GMV growth ideally should have been sort of higher than where the pharma market growth is. I mean is there any pricing issue with respect to private label? Like, I mean -- or is there market share gain by competition or online players of quick commerce? Any such structural dynamics at play here, you think?

Madhukar Reddy: I don't think so, Saion. Actually, the GMV growth is 9%, not 6%, right, for the year. No, for the year is 9% for the year. So it is not 6%. So we have grown slightly, I mean not that it is a big thing. We expect to grow much faster. But the other reasons are also there, which basically accounted for whatever happened out there, which is our own constraints and everything else and all. And on the GMV side, it's definitely not lower for the year.

Saion Mukherjee: And sir, what's your expectation going forward? Like how should we think about this growth to play out now?

Madhukar Reddy: I think we will be back on track given that the new store numbers are going to be much less compared to the overall size of the company itself and stores also will sort of get phased out throughout the year. I don't expect that to be a big factor. So whatever growth will come in will probably come in through the existing stores just growing. And that I expect it to be, let's say, high single digits to low double-digit kind of number on the SSG.

Moderator: Our next question comes from the line of Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: Yes. On the gross margin front, you have seen a strong expansion on a Q-on-Q basis and on a Y-on-Y basis. If I'm not wrong, every 1 percentage point of your private label expansion adds around 30 to 40 bps, right? So is there some additional benefit that kind of came into this quarter on the gross margin front? Was there some inventory provision, et cetera, that kind of was taken? Or -- and also, what would be the steady-state margin going ahead for this on the gross margin side? Yes.

Sujit Mahato: So in quarter 4, Sanjay, what happened is we had an inventory provision release of around INR3 crores. This was in connection with the provision which the company had made as part of its abundant caution accounting policy on the Wynclear or the old private label pharma products when we had transitioned to our MedPlus brand. And there has been a release only to the extent into the books on actual liquidation or sale. And during Q4, we were able to sell worth INR3 crores of this inventory, and we got a release of around INR3 crores in the gross margin. That would have contributed to around 20 bps.

But the major surge what you have seen is for the private label increase between Q3 and Q4, both on pharma as well as non-pharma. Your other question on the steady-state gross margin, we would look at in the range of 24.5% to around 24.75% going forward. And once again, the private label growth journey commence, we should again start seeing an incremental growth in the gross margin.

Sudarshan Agarwal: Okay. So my follow-up question is that given that you saw a strong ramp-up in this particular quarter for private label share, I think you had alluded to, let's say, 1% Q-on-Q expansion per quarter for, let's say, 6 to 8 quarters. Do you still see that from current levels? Should it moderate? Can you provide

some guidance on that?

Madhukar Reddy: Yes, sure. So we probably grab the Q1 growth also in the Q4 side. So I expect this quarter to be slightly flattish as far as private label growth is concerned. But going forward, I think we will definitely be able to maintain the 1% growth pretty easily. I don't think that's an issue.

Moderator: The next question comes from the line of Sanjay from Bastion Research.

Sanjay: Yes. Sir, my question would be in the morning interview, you have guided the revenue growth to be at around 15%. And I understand you have said that the older retail, older stores which we have, which commands 80% of the stores, they will grow at inflation plus. So that -- is the understanding correct that the additional 600 stores, which we are going to open in the next year, they will deliver or the remaining below 12-month stores will deliver greater than 20% growth going forward? And how you are seeing that, how can we should see the revenue growth drivers going forward, if you can explain us?

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Madhukar Reddy: See, revenue growth is going to come from regular same-store sales growth as more customers come to our store and everything else as they mature and all that. I actually said closer to, let us say, the low double-digit kind of number. I don't think the 600 stores which we add is going to really add a lot in terms of top line.

So most of it is going to come through SSG only. The 600 stores which we add at 50 per month typically will start with only around INR2 lakh to INR3 lakh or INR4 lakh per month. So it is not very significant to the overall growth of the company. So going forward, what are the drivers?

There could be several, as we continue to make the private label popular. We're not really spending any money on advertisement actually. But as word of mouth spreads and more people benefit from the savings of private label, we expect at some point, there will be people walking into a store to take advantage of that. But I don't think that time is now. I think we'll probably need to wait for a while before the commission starts happening in a major way.

Sanjay: Okay. Sir, my second question would be on -- so in Q4 FY '20, our revenue growth for stores greater than 12 months has degrown by 1%. So it is because of private label mix increasing? Or is there something else attached to that?

Madhukar Reddy: No. As I summarized in the first answer, which I gave out, 2 things. The combination of private label mix and everything else obviously drives it down a little bit. Our focus on private label also kind of, I would say, maybe put -- muted the overall growth on brands and everything else.

And finally, as I said, our growth of number of stores in the last 2, 3 years has constrained our supply chain systems and everything else. We're still in the process of easing that situation. It will take a little while. But that also has caused a little bit of, I would say, dampening of sales. So it's a combination of various things.

Sanjay: Okay, sir. So sir, since we have added 10 additional warehouses and since the ramp -up is also on the stage. So in terms of growth, you have alluded that you are seeing high single digit of the overall revenue or of the greater than 12 -month stores revenue?

Madhukar Reddy: I don't think it really is going to change that much either ways because the new stores, which we added this year are only 300. The base is going to be really low out there. So you could take it as the full set, high single digits to low double -digit kind of growth.

Moderator: Our next question comes from the line of Madhav Marda from FIL.

Madhav Marda: First, wanted to understand that, just a clarification, when we say that the pharmacy sales growth was about 9% for FY '25, how much was the GMV growth for the full year FY '25?

Madhukar Reddy: The full year FY '25 was 14.8%

on GMV.

Madhav Marda: Sir, which is a proxy for volume, right? So we can say 15% revenue growth?

Madhukar Reddy: Yes, 15%. Yes.

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Madhav Marda: And the 6%, which is sort of the difference is more of a mix impact from more private label sales, right? So that's the way we should read it very broadly?

Madhukar Reddy: Yes, absolutely.

Madhav Marda: The same thing -- could you read the similar thing for quarter 4 as well, where we reported 1% sales growth. How much is the GMV growth for quarter 4?

Madhukar Reddy: Can you hold for one second, Madhav? Can I get back to you on that regarding the growth of GMV?

Madhav Marda: Sure. Sure. Sure. Then sir, the second question was on the margin side. So I think you just guided for a gross margin of 24.5% to 24.8% going ahead. This is for the pharmacy business, right?

Madhukar Reddy: Yes.

Sujit Mahato: Yes. It will be plus 1% compared to Diagnostics.

Madhav Marda: Sorry, sir, I didn't get the last part.

Sujit Mahato: I said you can add 1% for the Diagnostics, so that comes to the consol number.

Madhav Marda: Correct. Correct. Yes. So the pharmacy business where the guidance is 24.5% to 24.8%, that number was 23.3% this year. So we are basically indicating 120 to 150 basis points of expansion of gross margin in next full financial year. That's how we should read it, right?

Sujit Mahato: So for the current year, Madhav, it's in the range of 24.1% in the -- for the full year. That's what we are saying it should move up to 24.5% to 24.8%.

Madhav Marda: Okay. At the consol level basically. Okay. Maybe I'll just check it with you offline. Maybe there's some confusion here. And sir, then on the margin side, the last follow -up was that it seems like this year, we have spent some money for expansion of our warehouses, which probably would have led to some front -ending of some cost. So is it fair to assume that next couple of years, the opex growth could be more controlled and we could see some operating leverage benefit from there? Is that the right thing?

Madhukar Reddy: The opex may or may not be controlled. We may continue to add some warehouses here and there, some automation and everything else that we will see. But definitely, the benefits of these will come in, in the next year only because of all the warehouses which we actually kicked off, only one has become functional as of now in Calcutta. Otherwise, almost everything is going to come online in the next few quarters.

Madhav Marda: Sure. Sir, any guidance for the pharmacy operating EBITDA margin, which was at 4.5% this year. Any outlook for that over the next couple of years? Do you see that crossing 5% in the next 2 years or so? Or could you give some sense there?

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Madhukar Reddy: Yes. I think so, Madhav, because the number of stores we are adding are not very significant. So the drag of the new stores is not going to be significant. But I think we'll continue to get the

benefit of private label. So that should basically help us expand the margin, I think.

Moderator: Our next question comes from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: Yes. Madhukar, this revenue growth with all the private label shift happening, are we looking at a relatively low revenue growth increasing margin sort of strategy going forward?

Madhukar Reddy: Not as much as we have seen in the past. The main reason is I think the early adopters have basically come in and bought the private label already. So the growth from now on the private label is going to be just 1%. So the substitution of the brands is not going to be very high. So whatever growth which comes in through the brands will continue to accrete to it.

And the -- so the substitution is not going to be high. I don't expect that the drag because of substi

tution is going to be very high on the top line. But the margin will continue to grow, mainly because we will at least continue to grow every quarter on the mix.

Bino Pathiparampil: Got it. So things get stabilized now more. Second question on store addition, the 600 that you mentioned, would that be net or gross?

Madhukar Reddy: That will be net.

Bino Pathiparampil: Net 600. Okay. And yes, finally, on your tax rate, why is it below the 25% level? And how you see that moving over the next 2, 3 years?

Sujit Mahato: Yes. On the income tax, there is a deduction which the company continues to claim. It's the Section 80JJAA, which is linked with the employment created for new employees. So as we continue to expand our network and new employees get added to the network, there is an additional deduction which is available over the next 3 years for the amount spent on salaries for these new employees, and that is driving down the tax numbers. And we expect this to continue in the same manner provided this section is available in Income Tax Act.

Moderator: The next question comes from the line of Aejas Lakhani from Unifi Mutual Funds.

Aejas Lakhani: Yes. Madhukar and Sujit, nice to see the operating EBITDA number truly reflect the efforts that the team has been putting in. My first question, Madhukar, to you is that at the start of the year, we had called out for the 600 store addition, and we've closed at about half of that. So could you just walk us through what really took place for us to drop this run rate? And how are you thinking at that point of time?

Madhukar Reddy: So, Aejas, a couple of things happened actually. One, the first quarter of last year was not great because of elections and everything else. And then by the time that we went into the second quarter and all, we realized that our workers were very strained. They were not able to supply all the stores, and we decided that we would actually slow down, especially in the periphery and all. So for instance, in Tamil Nadu, Karnataka and all, we now have set up warehouses in Hubli and Madurai and likewise in other places in West Bengal and all.

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So the places which would be supplied by today's Madurai and Hubli were receiving the material very late and we were having a little bit of challenge in the main cities because of manpower attrition and everything. So because of all those reasons, we thought we would actually set up the back end properly and come back and actually start growing once again. And that's the reason we took a breather out there. We were thinking that we do around 400, but in the end, I think we guided towards a 300, 350 number, and that's where we ended. We did a total of 398 though, gross additions, but we shut down a bunch of stores, which were old and their characteristics kind of changed and renovated and all. So the net addition then ended up being only around 305 stores.

Aejas Lakhani: Got it. Madhukar, the second question actually is a leaf of what you just answered. So one is that could you just give some further color on the 10 warehouse that you have added in the year? Is this more from a densification standpoint? Or is it more expansionary towards Tier 2 and onwards?

And just double tapping into that question, I'm trying to understand how have fill rates been?

And if you can give me any color because have fill rates been affected in -- because there was supply chain from a warehousing standpoint constrained. Have fill rates been affected? And third, if you could just speak a little bit about the attrition aspect?

Madhukar Reddy: Yes, fill rates, definitely, they were affected, and they were affected largely in the remote parts of the states, which are getting supplied from the main warehouses. For instance, the Bangalore warehouse is supplying all the way up to Udupi, Mangalore and everything else. And th

at was taking a hit because of attrition at Bangalore. Attrition in the larger cities in South India has been fairly high.

So combined both warehouse which is not fully adequate and people -- and being short of people, we were barely able to supply the main cities. And so our sales -- our fill rates and then

consequently, our sales in the smaller towns took a hit. Yes, attrition fairly high.

We feel that as we go into the smaller towns, it will not be as high, and we'll probably be able to get slightly cheaper manpower, but at least the attrition will not be high even if the cost is not very different. Second, definitely, as we go forward, fill rates, we expect they will be better. And hopefully, they'll all get reflected in sales as we go forward.

Aejas Lakhani: Understood. And could you just share that the warehouse, was it more densification or expansionary towards Tier 2, Tier 3 cities?

Madhukar Reddy: Yes. Okay. By densification, you would mean, are we doing more in the, let us say, in Hyderabad or Bangalore or Chennai, no. We are definitely setting up warehouses in the, let's say, outer parts of the state, whereas in the initial stage, we basically had just 1 warehouse for one state. We're now doing anywhere from 2 to 3 for each state. So you could say this is definitely densification of the state itself. They're putting in more in the state, but away from the main warehouse and to increase the supply lines to the remote parts of the states.

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Aejas Lakhani: Understood. And just one last thing on the private label pharma as well as non-pharma. In your own mental construct, where are you from a category standpoint? Are you seeing white spaces there that you still wish to populate? Or the first wave of private label that you needed to introduce is all there, and it's just now about the push-through and conversion?

Madhukar Reddy: On the pharma side, I don't think there's that many gaps. The one big gap, I would say, is insulin, but that's not an easy one to actually fill. We will have to find a willing and a reliable partner out there for us to actually go forward on that. So there are not that many gaps. And I think right now, as we said, it is going to be more about conversion of more of the newer guys. And that's why we say 1% every quarter. We're not going to be really jumping up on those numbers anytime soon.

On the other hand, the private label and general goods, that's a wide open thing. There, we can add a lot more categories and probably a lot more -- I would say, we'd have a lot better chance of selling if the range was bigger so that the customers who walked into a store to buy the regular stuff now can see a variety of brands from us and appreciate the quality and the price difference and then start buying. So that would be all additional sales. And there we see a lot of scope, not as much in pharma, but definitely in the non-pharma side.

Aejas Lakhani: Understood. And finally, on Diagnostic, I'm sorry, Sujit, I missed the starting remark where you called out the count of paying subscribers that we have. And where are we on that journey? I know we had a number which was significantly higher. So what's the call out there from a Diagnostic standpoint? Are we just going to keep trying to increase that member base right now? And are we going to add those feeder diagnostic centers so that they feed into your larger centers? Just some color on Diagnostic.

Madhukar Reddy: Sure. The number was 157,000 active plans and 327 active plans -- underlying lives versus 152. So while we added 5,000 plans out there, it is definitely much lower than what we want. We need the number to be closer to 250,000 for us to actually take the leap of expanding to other states and all. So for now, we are going to continue to push forward and try to get this number up.

We are seeing obviously a bigger level

of acceptance out there. And one of the ways in which we expect to actually grow this number is by increasing the stand-alone collection centers across the city and increase the service level or increase the number of people available for home pickup of samples. So both these places, we will see some expansion and hopefully touch more people and increase the number of plans.

Aejas Lakhani: Understood. And Madhukar, the people who go for the home collection, so is my understanding correct that at the existing store only, you will have a technician who can double up as that and then go and collect it? Or are you talking about incremental hires and space required to execute the same?

Madhukar Reddy: No, no, no. So there is no space required. This is an online thing. You open up the slots based on the number of people you have. And typically, as the demand continues to grow, you keep hiring new people. So it is not in one shot. So rarely do we have a situation in which the people

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are not actually ready. In fact, today, we have a shortage of slots and some of the active members are actually wanting more slots to be opened up for home pickup.

So yes, there is -- and space, that's not space and that's not the people in the store don't double

up. The home service guys are different. All these orders are booked a day in advance and home service and the people in the stand-alone collection centers, they stay there between 7 to 10 to take care of any walk-in customers. They don't go out to pick up the samples.

Moderator: The next question comes from the line of Gaurav Gandhi from GCM Limited.

Gaurav Gandhi: Yes. Sir, how do you plan to reduce the drug license suspension issue? And is there any possibility of any bigger regulatory action if such suspensions happen regularly?

Madhukar Reddy: No, we are working with the drug departments and all to correct all the issues out there as much as possible. I can tell you for sure, we probably are more in line with the government's mandate than anyone else out there. It is just that we are a larger company, so we attract a little bit more attention and we probably -- and we have to actually report it. That's why it's probably in the news. But otherwise, our stores are way more compliant than anyone else out there.

Gaurav Gandhi: Okay. And the second question is, do promoters have any plans to cut down the stake in the company in the next 3 to 4 years?

Madhukar Reddy: 3 to 4 years is a long time. I'm not really thinking about it right now, but I don't expect to sell any shares in the near future at least.

Gaurav Gandhi: All right. All right, sir.

Madhukar Reddy: Yes, just to finish off that -- make that answer slightly more elaborate. If I do sell, it will be for mainly to clear the debt. And when it comes to refinancing, I may end up doing a little bit of that. But otherwise, it is not -- I'm not looking to exit any amount of my stake out there.

Moderator: The next question comes from the line of Lakshminarayanan KG from Tunga Investments.

Lakshminarayanan KG: Madhukar, a couple of things. I just want to understand what are your top 3 priorities for the next 3 years?

Madhukar Reddy: So I would say opening stores, 600 stores, that's the #1 priority. Also getting into, let us say, the -- making sure that the pilot for the franchisee store goes well. We'll try to make the franchisee thing as successful as possible. Given that the margins now have increased for us overall, we feel that this is the time for us right now where we can actually share a little bit with the franchisee and make it successful, at the same time, reduce the overall.

Let us say, burden of investment as well as the overall task of managing people, slightly less onerous for us by going to the franchisee side. So we feel if we can get the franchisee thing right, we will be able to grow

much faster. So outside of opening the stores and doing the

franchisee thing, I think the third one would be to continue to grow the private label and see if we can actually continue to increase the margins for us.

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Lakshminarayanan KG: Got it. And second, I see that you have been expanding in Kerala in the last 1 year. How successful it has been, because I could see a significant ramp-up in Kerala. And what kind of potential it will have in the next 2, 3 years?

Madhukar Reddy: We are actually in comparison to other states and all, we're still going slow in Kerala. Kerala has its own set of problems out there. It's not very easy to -- there are several issues out there.

We see a lot of potential. It's very dense. People are affluent. There are a lot of, I would say, need for medication out there and all. But it had its own set of problems. So we are still kind of doing a wait and watch kind of thing. We have put in a few stores. We want to make sure that they are all profitable and we find the right model for us to expand.

Lakshminarayanan KG: Got it. Got it. And you talked about 600 stores that is over a period of -- can you just tell me in the next 3 years, how do you want to ramp up your 600 stores?

Madhukar Reddy: The 600 stores are going to be in the next 12 months, so they will spread out. So we will continue to open. Maybe Q1 is not going to be as big as the other quarters typically. But yes, otherwise it will be throughout the year.

Lakshminarayanan KG: Got it. And this revenue mix which you actually show in your Slide 14, this -- I mean, I think this is calculated at the discounted price or at the MRP?

Madhukar Reddy: It is a net price, net of discounts and everything else.

Lakshminarayanan KG: It's a net of discount. So which means that normalized thing, our -- I mean I'm just trying to understand our own brand pharma should be -- what would be the normalized price because this is net of discount.

Madhukar Reddy: So if you actually compare it MRP to MRP, which is more this thing of the volume, the pharma private label in the last quarter would be around 22% and the non-pharma in the last quarter was 13%. So bearing in mind that 80% of our overall sales comes from pharmacy, given that we are already at 22%, that would mean roughly around 28% of all medicines sold today by GMV -- in GMV terms under the MedPlus brand.

Lakshminarayanan KG: And in terms of new state addition, which are the states you are planning to go for the next 3 years?

Madhukar Reddy: So the states are Madhya Pradesh, Chhattisgarh and Kerala that remains the same, not

changed.

Moderator: Our next follow-up question comes from the line of Saion Mukherjee from Nomura Securities.

Saion Mukherjee: Madhukar, this you mentioned 28% of pharma is on private label. So what's the -- and it will keep growing, as you mentioned. So do you have any sort of optimum traction in mind that where this number can settle eventually?

Madhukar Reddy: I would like it to go all the way to 90%. But the thing is it will probably settle down at a much lower number, mainly because of 2 things. One, despite our discount, there are still some

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brands which would be cheaper. There's still some naturally really inexpensive brands, which you can't anyhow substitute. So there's going to be those.

There's also going to be products which -- for which we don't have a substitute, the insulins and all and a bunch of other things. And as we go forward, we're also seeing an increase in share of patented products, and all coming in, if we start selling a lot of those, those will not have substitutes for a long time.

So between all these things and the fact that not everyone is going to switch, we ourselves are going to be obviously pushing a lot on emphasizing a lot more on inc

reasing the top line rather

than just the private label. I think over the next 3, 4, 5, 6 quarters, a growth of not more than 1% is to be expected on the private label side.

Beyond that, Saion, I'm not sure. As more and more companies get to the generic side and they start talking about it, as the government starts talking about it, the overall word of mouth spread to an extent where people may just -- it may reach a tipping point where they just say, that's it. We will end up taking just the generics and nothing more. And then it may just change. But right now, I don't see that. I'm thinking for the next several quarters, it's going to be 1% every quarter.

Saion Mukherjee: Understood. And just one question I was thinking, this whole GLP-1 opportunity, which is going to come up next year. In terms of supply chain, is that something different which sort of benefits in any way, MedPlus? Or you think it's like one of the other products like which goes off patent? And obviously, we shouldn't expect any private label play in the short term in that, right?

Madhukar Reddy: No, why would you not expect private label? Obviously, that is a great product to launch a private label. We'll definitely try and do it as quickly as possible. We're already talking to people about that. So that will happen. Now is there anything special in the supply chain? Not really. Obviously, the cold chain and all have to be maintained and everything else, but that's about it. I don't think there's anything -- I mean it's not really a big deal. I mean it's like selling insulins anywhere.

Saion Mukherjee: And Madhukar, like private label for that injectable, like when can you -- I mean, launch? I mean, is there any timeline you have in mind?

Madhukar Reddy: So we are talking to people. I can't give you the exact timeline, Saion, but I know it is coming out of patent, I think March '26. And I'm hoping that we should be ready on day 1 or maybe if not day 1, maybe month 1.

Moderator: Our next question comes from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed: The share of pharma private label, which you disclosed around 21% of GMV for the quarter.

Madhukar, can you give some color on how this number looks like across various states and across Tier 1 to Tier 4 markets, how different is the share?

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Madhukar Reddy: So I don't have the exact numbers out there, Harith, but I can tell you this for sure. Tier 4, Tier 3, obviously, are going to be much, much higher than Tier 1. But that said, Hyderabad, Bangalore and Chennai are not that bad, even Bombay, which is the lowest, I think, for us and private label is still at around 10% or 11% for us.

So smaller towns, the private label medicine could be as high as 30% or 32%. Yes. But state-wise, I don't think it really differs that much. It is more in line with our own entry into the state. So it depends on when we have actually gone to the place. So the older we are, the better the known we are, the higher the overall private label mix in the state.

Harith Ahamed: And you touched on opening stores and other franchisee models. Can you share a bit of color on the economics? Will you be booking the entire revenue? How -- what kind of share will go to the franchisees? And are these stores on top of the 600 stores that you guided for?

Madhukar Reddy: No, actually, they will be part of the 600 stores. Part of the stores which we are doing will be

franchisee. This will be like a wholesale kind of model where we sell the product to the store, and he then sells it downwards. And yes, it will be a MedPlus store where we act like a super distributor, you could say.

The only product that franchisee can sell is stuff which we sell at. So everything will be sold to them and that will be the price which we bill. So yes, the top line will come down slightly because of that, but th

e overall return on capital and everything will be much better.

Moderator: The next question comes from the line of Tarang Agrawal from Old Bridge.

Tarang Agrawal: Just quite a few questions actually. On bookkeeping, what are your number of stores in Tier 2 and beyond?

Madhukar Reddy: I don't have the exact number, but I think around 40% now of our stores probably are outside of the Tier 1.

Tarang Agrawal: Sorry, 70%, is it? I mean you used to call out this number in your opening remarks, right?

Madhukar Reddy: 40%. I think 40% of our stores, but I'll come back to you on that. So why don't you go ahead with your rest of the question.

Tarang Agrawal: Okay. In Q2, if I'm correct, I think you opened a store in UP. Is that right? I mean, is UP still there in your footprint or it's no longer there?

Madhukar Reddy: See, we opened a store in Noida, one in Gurgaon and one in Delhi. So this is largely to basically see if we could enter Delhi as a city and do online delivery out there. So we are still - that pilot is still running. Some of the stores are doing well, but it is not to enter the state of Haryana, UP or this one.

Tarang Agrawal: Okay. Got it. Got it. Got it. Madhukar, you said you guys have added about 10 warehouses in FY '25. What was this number in FY '24?

Madhukar Reddy: So 10 less.

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Tarang Agrawal: No, no. I mean, what's the overall warehouse number as on 31st March 2025.

Madhukar Reddy: It has actually doubled, I would say, close to double because we had one large warehouse for each state.

Tarang Agrawal: Okay. Got it. And in terms of working capital, as the store matures, what are the inventory days that you guys generally work with? I mean, for your reasonably matured store in your footprint, generally, what are the inventory days that, that store would work with?

Madhukar Reddy: Around 40 to 43 days.

Tarang Agrawal: But one would believe that -- I mean, that's the number that you work with for stores greater than 12 months, correct? But typically, you would make it even more efficient, right, as the store matures even further or 40 to 43 days is where sort of you hit the ceiling?

Madhukar Reddy: Not necessarily. The store sales go beyond INR2 lakh per day, it will probably come down a little bit. But anything below that, it's going to stay at around 40 to 43 days or maybe it will go down to 37. See, the reason is the store's reputation is built on fill rate. So if you're going to constantly cut the long tail, people will stop coming to us.

We have several products in our store, which are -- which we would have not sold even once in 6 months or once in 7 months, but they need to be carried because every month we see from the list of products which we sell, there are several products which we have never sold in 6 months sold as part of a customer overall basket. And if it were not for that product, maybe the customer would have walked. So the thing in pharma is not focusing a lot on inventory days in the store, but more on the fill rate and the top line, I would say.

Tarang Agrawal: Got it. Just out of curiosity, I mean, given the stock warehouse expansion that you've had, one would have presumed that your inventory days at the warehouse should have increased materially more, right? But that hasn't happened. So just wanted to understand, because logically, if you're adding a warehouse, you would typically add as much of SKUs and product that was there in the earlier warehouse, and that would have a significant drain on your overall inventory, but that hasn't played out. Is that my understanding correct, though?

Madhukar Reddy: Not really. But the thing is it is still to play out because the stores -- the warehouses are not yet functional. The only one warehouse is up right now. The others are actually getting ramped up. But in the initial stage, maybe, but afterwards,

it's not going to really change that much

because each of the warehouses is going to be taking care of at least 250 to 300 stores. So the inventory in, let's say, the Chennai warehouse is not going to be split between Madurai and Chennai.

Tarang Agrawal: Got it. Got it. And I mean, from a working capital standpoint, both on inventory and payable days, do you think there is scope of improvement or you've maxed out there?

Madhukar Reddy: I don't think there's a lot to be done out there.

Tarang Agrawal: Okay. And last, just if this number is handy with you, actually, to Madhav's question, what was the MRP rate, MRP growth rate for Q4? And second, if you could give us a sense on what were the bills cut through the quarter?

Madhukar Reddy: We don't have that number. The MRP growth, I think, was 0.4% or something.

Sujit Mahato: 6.4%.

Madhukar Reddy: 6.4%.

Tarang Agrawal: 6.4%. Okay.

Moderator: Our next question comes from the line of Vilina Jain from Perpetuity Ventures.

Vilina Jain: So I just want to understand our employee expense trend. So over the last 3 quarters, it has grown significantly. So firstly on that. Secondly, to grow private label pharma, are we using some kind of incentives in the stores?

Sujit Mahato: Yes, I think it's a great question. So it has both the components, as you rightly mentioned. As the private label sales increases, there is also the component of the employee incentive.

Technically, we would have wished that to go from the gross margin, but the way the accounting literature dictates, it goes as a selling cost, and therefore, it is shown as part of the salary expenses.

So that's the predominant thing for the increase. In addition to that, a couple of quarters ago, we had highlighted that we have brought in for the purpose of retaining employees, there is a retention plan for the store level employees. So that has also contributed to a little extent. And based on these reasons, the costs have gone up.

Vilina Jain: Sir, if you could highlight what would be this selling cost, which is like an employee incentive for private label? And how should we see this growing as we grow the private label?

Sujit Mahato: So it's again based on private label pharma and private label non-pharma and then there are -- during this year, we are also trying to align to the overall sales. So this is a dynamic thing, which we try to do it every quarter internally. So maybe offline, I can take that with you.

Moderator: Ladies and gentlemen, we will take that as the last question for today. I now hand the conference over to Mr. Sujit for closing comments.

Sujit Mahato: Thank you, Sagar. I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you for joining us. You may now disconnect your lines.

Call: Aug 2025

MedPlus Health Services Limited

040-6724 6724

Regd. off. H. No: 11-6-56, Survey No: 257 & 258/1, Opp: IDPL Railway Siding Road, Moosapet, Kukatpally, Hyderabad – 500037, Telangana, India

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August 07, 2025

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Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst / institutional investors on August 04, 2025 for the quarter June 30, 2025.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

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"MedPlus Health Services Limited Q1 FY'26 Earnings
Conference Call "

August 04, 2025

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer and Managing Director

Mr. Sujit Kumar Mahato - Chief Financial Officer

Mr. DRN Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day and welcome to the MedPlus Health Services Limited Q1 FY '26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the initial remarks from the management. Should you need assistance during the conference call, please signal an operator by pressing * then 0 on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. DRN Srinivas. Thank you and over to you, sir.

DRN Srinivas: Thank you, Sagar. Good evening, everyone. On behalf of MedPlus, it is my utmost pleasure to welcome you all to the MedPlus Q1 FY'26 Earnings Conference Call to discuss the Financial Results of MedPlus for the 1st Quarter FY'26 which were announced earlier.

We have with us today the Senior Management Team represented by Mr. Madhukar Reddy Gangadi – CEO and MD, and Mr. Sujit Mahato – CFO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide #1 of the Investor Presentation shared with all of you earlier. Documents relating to our Financial Performance were circulated earlier and these have also been posted on our Corporate Website.

I would now hand over the call to Sujit. Thank you and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone on this call.

As informed earlier, we continue to strengthen our backend operations and infrastructure to support long-term scalability and ensure seamless execution. We remain focused on optimizing our existing network while laying a strong foundation for opening new stores across the 13 states in which we operate.

As an update, out of the 10 additional warehouses, 6 warehouses have become operational. This disciplined approach will enable us to drive sustainable growth and enhance value for all stakeholders.

In terms of our network:

We have opened 124 new stores during the current quarter. Over the past 12 months, we have added a net total of 369 gross additions of 456 in terms of new stores. Throughout Q1, there were 23 store closures. Considering both openings and closures, we achieved a net addition of 101 stores during the quarter compared to the 100 stores added during the last quarter. Store closures also include 8 franchise conversions. We continue with the outlook of adding 600 new store additions in Fiscal '26. In terms of our store network age, around 21% of our stores have

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been operational for less than 2 years and the remaining 79% of our stores have been operational for 2 years or more. At the end of the quarter, our network grew to 4,813 stores with 2.5 million plus square feet compared to 4,444 stores and 2.3 million plus square feet at the end of June '24. The average store size is 527 square feet.

On the revenue mix:

Presently Medplus offers over 1,350 plus carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for Q1 FY'26 constitute 21.5%, pharma being 20.8% and non-pharma 8.7% of our total revenue. On a GMB basis, during the current quarter, the share of private label pharma sales stood at 20.4 % compared to 7.9% in Q1 FY'24 prior to the launch of MedPlus-branded pharmaceutical products.

On our financial numbers:

Our consolidated revenue is Rs. 15,426 million for the quarter. Our consolidated operating EBITDA stood at Rs. 728 million, representing 4.7% for the quarter. Around 99% of our revenue is from our pharmacy operations. Revenue from pharmacy operations grew by 6.6% YOY on GMB basis and 3.3% YOY on net basis. The pharmacy ope

rating EBITDA stood at 690 million, representing 4.6%.

Update on our store's performance:

I would like to update on our stores older than 12 months:

Revenue from these stores in Q1 was 14,188 million, representing 95 % of pharmacy revenue.

These stores had a store-level EBITDA margin of 10.9%. The store-level operating ROCE of these stores stood at 59.8%.

A word here on the store-level EBITDA margin by age:

While stores greater than 12 months had a margin of 10.9, this was 11.1 for stores greater than 24 months and 6.9% for stores in the 13-24 age bracket. If we allocated non-store related costs, then the operating EBITDA of stores greater than 12 months would be 727 million, which translates to a margin of 5.1%.

An update on our working capital:

Our net working capital for Q1 was 59 days. The inventory in our warehouse was 36 days. In Q1, the inventory level of our first-year stores was 97 days. In comparison, for our stores older than 12 months, the inventory was 39 days.

An update on our diagnostic business:

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Diagnostics revenue grew to 302.9 million in Q1 FY'26, compared to 242.4 million in Q1 FY'25.

Diagnostic segment recorded an operating EBITDA of 41.3 million, representing 13.6%, compared to 3.3 million in Q1 of the last fiscal. In April, we sold 457 gross plans a day. In May and June, this was 468 and 520 plans respectively. As on end of June, we had 1,64,000 active plans, covering 3,40,000 underlying life. As on 31st March, this number was 157,000 plans, covering 3,27,000 underlying life. Our current observed on-time renewal rate was 24% in Q1, versus 27% in the last quarter.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. Our first question comes from the line of Sanjay from Bastion Research. Please go ahead.

Sanjay: Hi, sir. Thank you so much for the opportunity. Sir, my first question would be, our growth has been quite lackluster in the last 5-6 quarters and in terms of SSG, it is degrowing. I understand we have scaled our private labor revenue and therefore the growth has been quite lower. But can you explain a bit more on that side when we expect growth to come forward? And the degrowth in the revenue is not just able to kind of think on that side. So, can you explain more on that?

Madhukar Reddy G: Sure. See, as we explained in the last call also, there were some issues out there on the supply chain and also on the warehousing and a couple of other things out there on the manpower and all. Most of them are getting addressed and as we go forward, you will see a growth in sales. We are already seeing that right now. Second, see, you also have to see that we grew from 7.9% to 20.1% by GMB on the private sector. And that has actually driven our margins, gross margins from something like 22-23 all the way to 26, right? So, it is, I would say, definitely there is a small trade-off out there. Whenever our employees go in and push this very actively, we want to see some customers who may not want to switch and may even get put off a little bit and go. So, that is something which we are addressing right now. But we were aware that something like this would happen. It is a trade-off, as I said. We'd rather take slightly flattish sales and go in for 2%-3% extra private label is what we thought originally and that's what happened. Now, we are again tempering the whole thing and basically holding the private label at that number. We probably still continue to grow, going forward at around 1.5% to 1% every quarter. Although we did promise 1% every quarter, I think most of it has already achieved in the Q4 for at least a couple of more quarters. So, we are going to be holding it out there, adjusting the overall incentives and everything else to our employees,

aligning it in line with the overall company

objectives of increasing top line, at the same time growing the private label. So, for us, not too concerned about this at this point. We just continue to focus on profitability and getting the stores to break even and all rapidly.

Sanjay: Sure, sir. Thank you for the answer, sir. Another issue would be, as in the last call also we said that, and you also alluded to right now as well, we will see private label to grow by 1% quarter on quarter. But when we see this quarter, the private label share has been dipping down and therefore the margin has also been dipped for the quarter. Is there is something to read on

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that line? Is this one off or we will see for a full year as you alluded that margin will improve 50-60 basis point from here on? Is the trend continuing and how should one think on that line?

Madhukar Reddy G: See, last quarter, I did say that we would actually have a slight dip or flattening of sales in private

label because Q4 had actually taken a lot of sales from the forward. So, their employees are heavily incentivized, there were all sorts of plans, and they ended up actually doing a little bit more on Q4. So, Q1 was always going to be a pullback and that is just the reason for that. Maybe Q2 will also be slightly flattish and everything else. But for us, there is nothing more to look into it. As I said, it is a trade-off, if I continue to push in private label, the sales will be flat, we do not want to do that. So, we will have 1% kind of growth. By the way, 1% growth or whatever is on the overall GMB sales, which means it is on the MRP value. Otherwise, the net sale value will always be around 0.5% growth quarter-on-quarter. So, I think most likely, I would say this quarter will probably be flattish or something like that. But beyond that, we will again continue to grow.

Sanjay: Great, sir. Sir, another question, we can see in breakouts of store closures this time that there are 8 stores which have been converted to franchises. So, what is the strategy regarding how we are identifying which existing stores to convert to franchises? Are these less performing stores or the strategy regarding franchising model? So, in the coming year, what proportion of stores can we expect from this model, like 4-5 years down the line?

Madhukar Reddy G: See, for us, some of the existing stores where we feel that a franchisee can do slightly better. In some places where these are slightly secluded kind of locations, where they have to I would say, will probably benefit from a franchisee who comes in and does the informal kind of labor thing in which he stays longer hours and everything else where we think they will benefit, we will most likely give it. There is no particular model out there. But going forward though, while this year, of course, as I said, we will only be doing piloting on the franchisee stuff. While going forward, that is the plan for us. I don't see any major changes. We will have some more conversions, but not to a major extent. Once the pilot is done, maybe in a quarter or two, we'll be able to guide you on the future, which is how many, what percentage will we end up actually going as franchisee versus company-owned stores. Right now, it's still not yet. I would say the numbers are not yet frozen.

Sanjay: Sure, sir. So, my last question would be, we have seen that our diagnostic segment has been ramping up and we were making losses at operating EBITDA level a year ago. Now, we make 13.6% operating EBITDA level. So, is this plan to not grow this business still intact or we can expect some expansion in this segment, at least in the States, we are already operational?

Madhukar Reddy G: No, we don't really expect to grow this at all at this point of time. While we were never in doubt that this would actually get to profit

ability and all, that is not the criteria which we have set ourselves or at least that is not the metric which will decide whether we end up expanding or not. The metric is the membership numbers. They are still at only 164,000 as of now. We need MedPlus Health Services Limited

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them to go beyond 200 and preferably closer to 250,000 members before we decide on expanding into new states or new cities.

Sanjay: So, thank you so much for answering. I will get back into the queue. Thank you.

Madhukar Reddy G: Thank you.

Moderator: Thank you. Our next question comes from the line of Sudarshan Agarwal from Axis Capital. Please go ahead.

Sudarshan Agarwal: So, on the branded generic side, I think in the last call, you did highlight that our incentive scheme on the private label side was kind of affecting the branded piece. This quarter, obviously, the decline has dropped a little bit to around 3% Y-on-Y. Could we expect that the branded piece to grow going ahead in future quarters?

Madhukar Reddy G: No, I do not think so. I do not think the percentage of private label is going to dip any further or it will go down too much. So, it will be there. But we expect as we go forward, as the incentives of the employees are aligned, not just for private label growth, but also for top-line growth, we will see an overall increase in topline. The absolute numbers of branded generics may go up with the overall increase in topline, but I would expect that they will end up being higher in the cost of private label.

Sudarshan Agarwal: Got it. And on the SSG point, I understand that it is better versus last year, last quarter, sorry. but when do we expect us to be back to, let us say, single-digit kind of SSG growth by?

Madhukar Reddy G: I think maybe one quarter or two. I think we still need a lot of work to happen in the back-end for us to actually really take advantage of whatever we have in the front-end. So, that has been slightly slower than what we expected. Although we did open somewhere else, we still need more. And so, we are getting that done, maybe a quarter or two is what I think, two quarters, maybe.

Sudarshan Agarwal: Got it. Thank you. This is from my side.

Moderator: Thank you. Our next question comes from the line of Lakshminarayanan K.G. from Tung a Investments. Please go ahead.

Lakshminarayanan K.G.: Hi. A few things. First is that how the branded fill rates have actually taken place in the last two

quarters, because you mentioned that there were some incentive schemes. So, I just wanted to check if the branded generics or the branded pharma fill rate dropped in those two quarters. And if so, was it the employee at the front-end did not do the fill rate or the company took a choice of actually pushing the private? What actually took place? And how do you track the brand fill rate? And where are we now?

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Madhukar Reddy G: No, it is actually nothing to do with the brand fill rate. While the fill rate is definitely an important criteria, it is more a function of the warehousing as well as the logistics and purchasing department out there. The employee has nothing to do with it out there, one. So, as I said, we had a little bit of a challenge on the back end and hence the fill rate was getting affected in some of the places, especially in the slightly more distant places from the main city. So, that is one challenge. The employee part comes in only where they are getting incentives for, let us say, a private label. And so, their interest then is only in selling that and nothing else. And so, in some cases, we have seen them almost being disinterested in some other products. So, that has changed. So, on their side, as long as the product is there, and which is not their work, the product being there in the store is the work of the supply chain guys, they will

now basically be more

than enthusiastic, or at least be not as enthusiastic about selling non-private label products.

Lakshminarayanan K.G.: And again, if I just look at your omni-channel phase, because last 2-3 quarters, it has been either stable or it has actually declined. And why is that so? Is it a conscious choice? And want to understand how many orders you actually do? What is the number of bill cuts that actually happened? And is it a decline in the per value of the bill or what has taken place here?

Madhukar Reddy G: Well, I think as a percentage, it may not have gone down too much, but it has not been growing.

We are now taking steps to make sure that we are able to do this. We are revamping some of our software in order to get this whole thing done. That is all internally done. So, I think maybe a quarter or two, we should be ready with a new plan and all that. Right now, no, we are not doing anything. So, the only thing which we do not do like most of the other companies out there is that we do not have any special discounts or we do any promotions and all. We have found that the cost of acquisition of customers is way too high. It is not really about the incremental business, so we have not done much on that. But I think just by, there may be a gap in some of our service levels. And that has come out of the huge demand for delivery boys across all the cities in which we have not maybe kept up with the salaries or whatever is there, this thing which has to be given to the delivery guys. So, some of those things are being addressed and I expect that as we go forward, we will definitely increase our sales. But no, we are conscious of the fact that some of the customers will want to go online. But we are not, I would say, against selling or we have nothing against the online channel.

Lakshminarayanan K.G.: And what is the average bill ticket for this online?

Madhukar Reddy G: The average bill value for us is north of Rs. 1,500 per bill, because we have a 20% discount above 1,000. So, a lot of the people who come to us are people who have chronic ailments and all and they will tend to buy slightly on the higher side.

Lakshminarayanan K.G.: You introduced a platform fee or something, is it meaningful or you intend to increase that also?

Madhukar Reddy G: No, I do not think we will increase it. The platform fee has been on all the online bills that is there, that is in line with most of the other companies. I do not really expect to increase that in anyway.

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Lakshminarayanan K.G.: Got it. Thank you.

Moderator: Thank you. Our next question comes from the line of Sidharth Sreekumar from iThought. Please go ahead.

Sidharth Sreekumar: So, my first question is, you said that going forward, you are seeing only 0.5% growth per quarter for your private label. Does that mean that the percentage of private label in your overall sales will come down and therefore, gross margin will also be not 26% going forward?

Madhukar Reddy G: No, why do you say that? I said it will grow from here. So, if I am growing from here by 0.5% every quarter, it should either be, it should actually grow. It is not going to come down.

Sidharth Sreekumar: But you are saying that 0.5% is the volume growth or is it like the share in your overall sales?

Madhukar Reddy G: Share in our overall sales.

Sidharth Sreekumar: Okay, it is not the growth number. Okay. Second question I have is regarding like when I went for a lot of, went to a lot of your stores, the one thing which was visible was that they do not have all the branded generics, which are not your brand and therefore, some customers are not buying from the store. So, do you plan to address this issue in any way?

Madhukar Reddy G: As much as possible. It is almost impossible to have 100% fill rate. It will take several crores of

inventory in each store

to do that. So, our goal is to be the best in the market out there. I believe we are still much better than most of our competition. But it is possible that in some places, in some stores, maybe some brands we may not have. It is a continuous process of improvement actually and we continue to get better and better.

Sidharth Sreekumar: Okay. So, one more question I have was regarding the warehousing. Like pan India level, how many warehouses do you have? And like how many stores can one warehouse cater to effectively without actually pressuring your supply chain?

Madhukar Reddy G: Sure. So, around 40 warehouses is what we are currently having, including the 10 which we have recently added. Roughly around 350 to 400, 450 max, we can service each of the warehouse to service our stores in that region.

Sidharth Sreekumar: Okay. So, 40 is the overall number of warehouses across the country and one warehouse can service how many stores? 300 stores?

Madhukar Reddy G: 350 to 450 range. Again, some of the warehouses are purely only pharma, some cater to the FMCG products as well. And that is how we think for our current network, this would service at least in some areas, we checked up to 500 stores. In some areas, this is between 300, 400, which means there is sufficient capacity even to grow new stores in those areas.

Sidharth Sreekumar: Okay. Thanks. Thanks for answering the questions.

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Moderator: Thank you. Our next question comes from the line of Raman KV from Sequent Investments. Please go ahead.

Raman KV: So, it is more like a clarification. My understanding is as your share of private label increases, your sole level MRP decreases mainly because the cost, and the average cost of private label medicine is comparatively less than branded generics. Is my understanding right?

Madhukar Reddy G: That is right.

Raman KV: So, if you are growing the private share label, there will be two things. There won't be the same store growth will be very hard to achieve and the store level MRP growth will be declining trend.

So, how are you planning to address this? One is that and a follow-up on that is you initially mentioned that you, the employees were incentivized to push private label products. So, and now you have talked about that the strategy has changed. What is the change in strategy? How are you planning to bring the same store growth across all the stores of MedPlus?

Madhukar Reddy G: Okay. So, first of all, if let us say the private label is sold more, even if the topline actually goes down, the absolute amount of money you make is much higher. So, if you are making X on a brand, after the discount, you actually take 2x on this, not as a margin or not as a percentage, but actually in absolute terms. So, it is actually good if you were to do it. So, there is nothing wrong. Now, but all the, so that is not the intention though, right? It is not to make private label 100% and not supply the customers what they want, because not everyone wants our private label, there will be people who will want the brands. So, the goal is to basically be a multi-branded guy, keep offering our private label and show the benefits of it and keep convincing some people to switch. That is all. Now, on the employee incentive, so where the earlier incentives were only aligned towards increasing the private label, now they have a mix where they are supposed to also grow the topline for them to get the private label incentive. So, it is different in different places. I cannot go into the full details, but yes, that is what it is generally. So, they are no longer linked only to one part of their performance.

Raman KV: Okay. And so, with respect to CAPEX per store, what is the CAPEX per store for us?

Madhukar Reddy G: Typically, Rs. 7 lakh crores to Rs. 8 lakh per store. It, of course, depends on the size of the store.

The rental advance is around 2 lakhs to 3 lakhs. So, you could say 10 to 11 lakhs overall.

Raman KV: And sir, with respect to the, you said now you will be aiming for increasing private label share products by 0.5% every quarter. So, which is effectively a 2% increase from current 13% to 15% by the end of this year. So, I just wanted to understand how will the gross margins and operating margins will improve.

Madhukar Reddy G: Yes, it is a little unlikely that this quarter will also grow. So, most likely, the quarter after this will start growing. The reason I say that is because we kind of achieved the growth of 2 or 3 quarters in the last quarter and we are stabilizing out there. So, after that, it will continue to grow.

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So, it is going to be a function of the growth of private label. 0.5 % on net sales value or 1% on MRP value, which is what we have been saying.

Raman KV: So, if you are increasing the half percentage on net sales value, how much will your gross margin

improve by? A ny ballpark number?

Madhukar Reddy G: Every 0.5% will probably give you around 0.2%.

Raman KV: Gross margin improvement. Okay. Thank you, sir.

Madhukar Reddy G: Thank you.

Moderator: Thank you. Our next question comes from the line of Madhav from Fidelity. Please go ahead.

Madhav: Hi, sir. Good evening. Thank you so much for your time once again. Just one question. I joined the call a little late. Sorry about that. Maybe it is a repeat question. But just wanted to understand going ahead, how is the management thinking about balancing the margins and the growth? I guess margins are being very strong and seems like we still have levers to grow the private label, which means gross margin should remain strong going ahead. But how do we think about just the topline and the gross margin or EBITDA margin equation for the next sort of few quarters or 1 or 2 years? That would be great, sir. Thank you. Just want to understand that.

Madhukar Reddy G: Okay. Sure. So, couple of things. One, the margin can come down if the number of new stores we open are going to be very high. The overall margin, I don't mean the gross margin, right? But we expect to open only 600 stores, so it's not going to be a huge impact on the overall EBITDA of the company. Now, on the gross margin, obviously, it is very beneficial for us to continue to grow the private label, and we are doing that. But at the same time, we don't want to put off customers who are coming for brands, and so that will be counterproductive. So, we are going to, as I said, we have already realigned some of the incentives and all. So, we expect that if not this quarter, from next quarter onwards, we will continue with the same kind of growth, which is either 1% on MRP sales or 0.5% on net sales.

Madhav: Okay. So, but the mix part is quite clear, actually. Just on getting the topline growth back, because I guess if you can balance that with the gross or the EBITDA margin, then sort of earning growth becomes pretty strong for the company. Even now, it's quite good, but just to get the topline moving, how do we set that equation here?

Madhukar Reddy G: Sure. So, we are doing a couple of things. One, making sure that the backend is strong, it is able to supply in time, and it's able to also do the fill rate properly and all. And also, we are aligning our employees into doing a little bit more of their topline sales and everything else. So, both those will help us catch up on the SSG. I don't think it's going to be a big concern.

Madhav: Just on the EBITDA margin side, now that Q1 we've done about 4.5%, 4.6%. And I guess this is easily one of the weaker quarters for the company. Correct me if I'm understanding this wrong, MedPlus Health Services Limited

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please. So, is it fair to assume that for a full year basis, our EBITDA mar

gin should be north of

5%, for sure, and maybe moving closer to 6% in coming times? Is that how we should think or that's being a little too optimistic?

Madhukar Reddy G: I think the latter may be a little too optimistic. I think we'll definitely grow from here as we move forward. Especially Madhav, since last year was a very, very strong growth year for us on the private label, and as I said, we want to balance it a little bit and grow slightly more moderately at around 0.5% or 1%, depending on how you look at it. I think the growth may be not as high as that. And last year also, we added only around 350 stores. This year, we plan to add at least 600. So, yes, that could also be a slight --

Madhav: So, any guidance on the topline growth there? Any guidance as to the full year, how that could end up for us?

Madhukar Reddy G: Not right now. We'll come back.

Madhav: Understood. Thank you so much, sir. Thank you.

Madhukar Reddy G: Thank you.

Moderator: Thank you. Our next question comes from the line of Aradhana Jain from B&K Securities. Please go ahead.

Aradhana Jain: Hi, thank you for the opportunity. I have two questions. The first is, if you see the total amount of inventory that we have as on June, it would be the lowest that we have seen in the last 7-8 quarters now. While we understand the upward push to the level of inventory should be the new store addition and the downward push is the shift from branded generic to PL. But if you could help us understand what is the level of inventory you hold in a store and where do you see the inventory settling? And would there still be room to see decline in absolute inventory in spite of new store opening? Yes, that is my first question.

Madhukar Reddy G: See, new store opening will basically increase the number of days of inventory, that is all. In absolute terms, it is always going to be around 17 lakhs to 18 lakhs of inventory per store. So, that is going to be the number. But when you, every time you open a store, you have to open it with a buffer inventory out there. Buffer meaning you really do not know what the customer asks there are the set of products which have to be there in the store. And initially, since the store usually starts with around Rs. 3 lakhs or Rs. 4 lakhs or Rs. 5 lakhs per month, you are going to see a much higher number of days of inventory. So, that is one. So, that is not going to change.

Second, on the private label, will it push down the inventory and all? It is still early days because we are not really looking to. So, we are actually carrying both private labels as well as our own right now. At some point, we may get a slightly better idea of which private label, sorry, which branded generic to drop or which not to stock. And then maybe you will see, start seeing the decrease of inventory days out there or at least the value, if I would say, for us as we go forward.

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Aradhana Jain: Understood. And the second question is, how is the acute versus chronic mix currently under PL? Is it similar to the historic levels of chronic more than acute of at 60:40 ratio or has there been any change there?

Madhukar Reddy G: No, it has always been chronic higher, mainly because chronic patients usually have a much higher need of medicines. Since they spend so much more, they are going to be affected a lot more by increase or decrease of discount. And so, we expect those guys to actually have them. And that has been the cause. That has been the case actually. Chronic is always higher for us.

Aradhana Jain: Understood. That is all from my side. Thank you so much and all the best.

Moderator: Thank you. Our next follow-up question comes from Sanjay from Bastion Research. Please go ahead.

Sanjay: Hi, sir. Thank you for the follow-up. Sir, my question would be, when we say back-end and supply chains are the challenges

for us, so what does that mean? What are the challenges we are facing on back-end? And you also mentioned that you are doing some work on supply chain side, so opening warehouses. What else are we doing on that side? If you can explain in detail, it would be really great to understand going forward how things will change from here.

Madhukar Reddy G: Yes. So, it is largely what you said earlier. It is on the warehousing side. Tamil Nadu basically had one warehouse and it was supplying 1,000 stores. And from Chennai, you are supplying all the way down to Madurai and Deep South. And that became a problem out there since until we set up the Madurai warehouse. Now, Madurai warehouse is slowly coming to speed and so we will see some of those problems going away as we go forward. Similarly, in Karnataka, we had one warehouse in Bangalore and it is supplying all the way to Bangalore and Udipi. Now, we have set up one more warehouse in Hubli. So, A, obviously, when you have something close by, you are able to, so that is one. Just the distance alone will basically come down significantly when you have a local warehouse and you are able to supply every day. Second part, since we had actually opened a lot of stores very, very quickly, the warehouses themselves were a physical constraint. They were overflowing out there and so we are not able to just put in enough stock and have it retrieved on time to send to the stores. These are getting addressed. Some of it is done and some of it continues to be there. So, we hope in the next one quarter or so, we should be all streamlined.

Moderator: Thank you. Our next question comes from the line of Omkar Hadkar from Mirabilis Investment Trust. Please go ahead.

Omkar Hadkar: So my question is on the private label. At this point in time, just wanted to understand in terms of availability of private label across the store network in terms of whatever SKUs we launched.

Is it fully available? And the second part is, are there any other kind of missing categories, like, for example, either insulin or GLP drugs or some other things, where there is an opportunity to

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kind of further introduce private label in the overall network? So, some color on that would be appreciated.

Madhukar Reddy G: Sure. So, today, I would say 80% of everything which sells in our store, we would have a substitute for that. The balance would be some of the patented drugs, a lot of the me-too, 80 or I would say 75, a lot of the me-too drugs would be the vitamins and small, I would say, very little sold combinations, which have got either a small change in the strength of one of the combination products or which has an additional mineral or additional kind of vitamin, multivitamin kind of complex kind of product. So, those, I would say, we have equivalents. We don't have the exact equal and we'll probably never have. But other than those products, I would say we probably have everything other than insulin. Insulin, we haven't really found very good, a reliable supplier for us to go forward on that. That's one. And is there scope for newer products? Definitely, I think GLP, I think both Wegovy and Ozempic are going to come off patent next year and we are getting ready to launch it in the first one month or so after the product comes off patent.

Omkar Hadkar: Got it. Sure. And I have a question on the diagnostic business. So, I saw that, right, at this point in time, you are adding about 7-8 thousand kind of lives per month, per quarter. So, just wanted to understand maybe the underlying phenomenon there because I guess you would have started

renewal cycles of some of the older subscribers. So, broadly, what are the retention rates like for that kind of a business? And also to reach your kind of stated goal of 2.5 lakh subscribers, at this current rate, it looks like it might take a while

to get there. So, are you kind of working on any strategies to accelerate the net additions in that business?

Madhukar Reddy G: Yes, we are looking at a couple of things, a little early to say, but the basic thing is going to be around B2B. And once B2B kicks in, then having, let's say, 5 or 10 thousand per month is not a big deal, or even more, you know, if you get a large company on board. So, B2B has to be the way we have to go, and that's why we have kind of put off all expansion plans till we can convince people to come on board. On the renewals and all, I think within the 1st Quarter, anywhere from 24% to 27% is the usual renewal rate for us. And after that, see, if it is B2C, people are going to come and renew when they need it. They're not going to renew immediately. A lot of the people are chronic customers. A lot of them may have gotten their test done in the last one month of their membership, and they want to wait for one quarter or more till their next cycle of test starts for them to come and renew it. But the renewals have been fairly steady.

Omkar Hadkar: Okay. And I missed part of the call, but if I can just summarize what you said about the inventory, you mentioned that the overall warehouse level inventory has come down because of the opening of the new warehouses and making it more efficient. And the store level inventory is more a function of the pace of the opening of the stores. Is that the right inference on the way inventory?

Madhukar Reddy G: No. The second part is right. The inventory level is not going to come down because of the opening of new inventories. It is just going to be split between, let us say, Chennai and Madurai

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where earlier it was only in Chennai. So, that is not going to come down. It is just, it is going to go faster into the stores and hopefully with better results in everything else as we go forward.

Omkar Hadkar: Okay. So, at the warehouse level, the turnovers have increased. That is what is reflected in the days. And at the store level, the metrics are pretty similar. There is not much change, right?

Madhukar Reddy G: Yes. It does not change much. In the first one year of its opening, it is always going to have a very high base of this thing out there. But the real matrix to see is the inventory of stores which are more than one year. That is usually being constant at around 35 days to 40 days. It is around 39 days, I think, right now.

Omkar Hadkar: And your old private label inventory, has that largely been cleaned up one way or the other?

Madhukar Reddy G: More or less. I think there is a very small, I would say, not a material amount still left. And this is all fully provided for.

Omkar Hadkar: Okay. Understood. Thank you. That is all I had for questions. I wish you all the best.

Moderator: Thank you. Our next question comes from the line of Sidharth Sreekumar from Ithought. Please go ahead.

Sidharth Sreekumar: So, you said that most of the private label sales is for the front side. So, I just want to know what would be the, like out of the customers you have in that segment, how many customers do repeat?

Madhukar Reddy G: Almost everyone. I am sure, there are some customers who do not. We do not have the exact numbers for it. But anyone who comes and takes a chronic medication, it is just going to repeat.

We have not seen, I would say, when we looked at it, it is not dissimilar to any branded product out there.

Sidharth Sreekumar: Understood. One more question I have is, you have said that 80% of whatever drugs you sell in your stores, you have your own brand for it. So, the 80% of the branded generics, right? And how many have you taken off from your stores?

Madhukar Reddy G: How many branded generics should we remove?

Sidharth Sreekumar: Yes, and replace with your own brand?

Madhukar Reddy G: No that is not how it works. We

do not replace any of the branded generics actually. The

assortment and the depth of that is completely a function of the sales of the product. Only if the sales goes down completely, does it actually go off the shelves. Otherwise, we do not take it off proactively.

Sidharth Sreekumar: Okay, understand. So, if I can squeeze in one more like a qualitative question, is that what are the critical earnings for you since the introduction of the private labels?

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Madhukar Reddy G: So, I would have wanted the adoption to be much faster, honestly, I thought we would right now be at close to 30% by GMB kind of terms. It is only at 20 out of 80 right now. So, it is around 25% of the overall sales. It probably, the early adopters after they moved in, I think the rest of the naysayers out there will take a little while to come on board. And we have seen that happen

gradually. So, slightly slower than expected, that is all.

Sidharth Sreekumar: Okay, thanks. Thanks for the opportunity.

Madhukar Reddy G: Thank you.

Moderator: Thank you. Our next question comes from the line of Neelam Punjabi from Perpetuity. Please go ahead.

Neelam Punjabi: Yes, thanks for the opportunity and congratulations on the margin execution that we have demonstrated over the last 12 months. My first question is that on a GMB basis overall, how was the growth YOY for the quarter for the pharma business?

Madhukar Reddy G: I think on a GMB basis, it is around 6% is what I remember, 6.8.

Neelam Punjabi: Got it. And my question is on the overall growth. So, while historically we were growing at a high double digit and in the last 4 to 5 quarters, we have slowed down to about mid to high single digit growth. And you mentioned that this is because of private label going up, which is at a steep discount to branded generics. However, given that this higher private label is now in the base, can we expect the growth to pick up and go back to the double-digit growth that we had historically, let us say in the next couple of years?

Madhukar Reddy G: Yes, definitely. It is going to be a function of two things. One, the number of stores we had and two, the overall growth itself. I fully expect that we will start growing out there. You see, one of the things is there is definitely, I would say, a trade-off between margins and growth, which is between private label and private label, which basically accounts for the gross margin and just growth, which is going to come from selling branded products. So, we choose to go the private label way. We wanted to really expand the market, form a solid base, and from there grow private label slightly in a slightly more consolidated fashion, but again, refocus on the branded stuff. So, we will now start growing because of that. But for us, it is a conscious call. We really wanted to expand the margin and take it to at least 25%.

Neelam Punjabi: That makes sense. But what I am trying to understand is that while that has happened in the last, let us say, 12 months or so, is this now a pivotal or an inflection point where we will be able to grow at a better rate compared to the last one year with keeping the margins at this current healthy rate? Like what is that inflection point? Is it today? Is it six months out where the growth would start picking up along with maintaining the kind of healthy margins that we have already demonstrated?

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Madhukar Reddy G: Sure. So, two things. One, of course, the reshifting of focus to, or re-aligning the incentives of all the employees to make sure that they sell both branded as well as private label, one. Second, as I said, streamlining of the overall supply chain and everything else, which will take a quarter or two completely to happen. That will also help us increase the fill rate and increase o

ur sales. So,

both those things will start happening and they will result in a topline growth. Thank you.

Neelam Punjabi: Thank you. Understood. Got it. And just one last question, in terms of our target of 600 store addition, what would be the broad split between the franchisee stores that we will be opening and our own stores?

Madhukar Reddy G: We are thinking at least 100 odd stores will be franchising. We are trying to see if we can do more, but we want to make sure that the pilot and all are going well before we really take off on that.

Neelam Punjabi: Got it. Thank you so much.

Moderator: Thank you. Ladies and gentlemen, we will take that as the last question for today. I now hand the conference over to Mr. Sujit for closing comments.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our investor relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services Limited that concludes this conference.

Thank you all for joining us and you may now disconnect your lines.

Call: Nov 2025

MedPlus Health Services Limited

040-6724 6724

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November 06, 2025

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Exchange Plaza,
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Bandra (East), Mumbai – 400 051
NSE Symbol : MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst/ institutional investors on November 03, 2025 for the quarter and half year ended September 30, 2025.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

Encl: a/a

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"MedPlus Health Services Limited
Q2 FY '26 Earnings Conference Call "
November 03, 2025

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. DRN Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day, and welcome to MedPlus Health Services Limited Q2 FY '26 Earnings Conference Call. As a reminder, all participant clients will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchstone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Srinivas from MedPlus Health Services Limited. Thank you. And over to you, sir.

Srinivas: Thank you, Swapnali. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q2 FY '26 Earnings Conference Call to discuss the financial results of MedPlus for the second quarter of FY '26, which were announced earlier. We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, CEO and MD; and Sujit Mahato, CFO.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the Investor Presentation shared with all of you earlier.

Documents relating to our financial performance were circulated earlier and these have also been posted on our corporate website. I would now hand over the call to Sujit. Thank you. And over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone, on this call. During the last 18 months, we have added 10 new warehouses across our network. We continue to strengthen our back-end operation and infrastructure to support the long-term scalability and ensure seamless execution. In terms of an update on our network, we have opened 145 new stores during the current quarter. Over the past 12 months, we have added a net total of 378, gross 469 store additions.

Throughout quarter 2, there were 28 store closures. Considering both openings and closures, we achieved a net addition of 117 stores during the quarter compared to the 101 stores added during the last quarter. We continue with the outlook for adding 600 new store additions in fiscal '26. In terms of our network age, around 22% of our stores have been operational for less than 2 years, and the remaining 78% of our stores have been operational for 2 years or more. In terms of our network size -- store size, at the end of the quarter, our network grew to 4,930 stores, with 2.6 million plus square feet compared to 4,552 stores and 2.4 million plus square feet at the end of September '24.

The average store size is 528 square feet. An update on the revenue mix. Presently, MedPlus offers over 1,450-plus carefully selected SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for quarter 2 FY '26 constitute 21.5%, pharma being 12.1% and non-pharma 9.6% as a percentage of our total revenues.

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On GMV basis, during the current quarter, the share of private label pharma sales stood at 19.8% compared to 7.9% in Q1 FY '24, which was prior to the launch of MedPlus-branded pharmaceutical products.

An update on the financial numbers. Our consolidated revenue for the quarter is INR16,793 million. Our consolidated operating EBITDA for the quarter stood at INR887 million, representing 5.3%. Around 99% of our revenues is from our pharma operations. Revenue from

pharmacy operations grew by 8.8% on Y-o-Y basis on GMV basis and 6.3% Y-o-Y on net basis. The pharmacy operating EBITDA stood at INR839 million, representing 5.1%. An update on our store's performance. Stores older than 12 months, revenue from these stores in quarter 2 was INR15,3

38 million, representing 95% of pharmacy revenue. These stores had a store-level EBITDA margin of 11.8%. The store-level operating ROCE for these stores stood at 68.6%. A word here on the store-level EBITDA margin by age. While stores greater than 12 months had a margin of 11.8%, this was 12.1% for stores greater than 24 months and 8% for stores in the 13 months to 24 months age bracket. On allocating all non-store-related costs, the operating EBITDA of stores greater than 12 months would be INR877 million, which translates to a margin of 5.6%. An update on our working capital. Our net working capital for quarter 2 was 53 days.

The inventory in our warehouse was 33 days. In quarter 2, the inventory level of our first-year stores was 100 days. In comparison, for our stores older than 12 months, the inventory was 37 days. An update on our diagnostics numbers. Diagnostics revenue grew to INR332.5 million in quarter 2 FY '26 compared to INR283.1 million in quarter 2 FY '25. Diagnostics segment recorded an operating EBITDA of INR50.9 million, which is 15.3% compared to INR21 million in quarter 2 FY '25.

In July, we sold 535 gross plans a day. In August and September, this was 485 and 531 subscriptions, respectively. As on 30th September, we had 170,000 active plans, covering 351,000 underlying lives. As on 30th June, we had 167,000 active plans, covering 345,000 underlying lives. Our current observed on-time renewal rate is around 24% in Q2 versus 24% in Q1.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: The first question comes from the line of Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: On the store addition part, right, I think H1, we are trailing a little below what half year mark we would like to be at, I would assume 300. So, we are confident of this? And on the franchisee front, I think you guys had said you would add around 100 out of the 600 stores this year from franchisee base. So, what is the update on that? Can you just let us know?

Madhukar Gangadi: Yes. We will be able to make up the balance in the next half. And franchisee also is on target for us. I don't think that's a problem.

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Sudarshan Agarwal: So can you just tell me how many have till date for this year been via franchisee model, if that is available?

Madhukar Gangadi: I don't have the exact numbers right now. We'll come back to you. But again, the 100 stores and franchisees shouldn't be a problem. We should be able to get that easily.

Sudarshan Agarwal: Okay. And your SSG growth, I think you had rejigged your private label incentives, right? So, our branded business has started seeing growth once again. And private label pharma has come down. So, what do you expect the trend to be for the quarters ahead, I mean, for the private label pharma and also on the FMCG side? FMCG actually has seen quite a big jump sequentially also.

So on that side, can you just update us?

Madhukar Gangadi: Yes. I think given the fact that we had actually grown a little more in the last quarter in franchisee -- in the pharmacy side, private label, we wanted to actually tone it down a little bit. But I think we'll be able to pick up the growth and start growing. For this quarter, it may be more or less at the same level. But after that, we should start growing at around 1% every quarter on an MRP basis.

So, that's the basic plan for us. Most likely, we will restart the -- press the accelerator on it only in the next quarter or so, not this quarter. On the private label general goods side, that is completely in our control in the sense that, that doesn't really replace any product. We don't have to actually tell the customer to buy one thing instead of the other. We're just expanding the range of products which we have.

We're taking advantage of the fact that we have

so many stores and we are able to bring in whatever products we want to without too much of a problem. So, that will continue to grow.

And even as, let us say, we kind of mute the private label pharma side, we will continue to push the accelerator on the general goods side.

Sudarshan Agarwal: Okay. And last thing. So when you say the subscription number, right, I assume that this is for the diagnostic or the pharmacy plan as well. I think you have rolled out MedPlus Advantage pharmacy plan, which allows another 2% discounts, right? So is that number -- the subscription including that?

Madhukar Gangadi: I think the 175,000-odd number, which you talked about, that is for the diagnostic side. The pharmacy subscription is a different number altogether.

Sudarshan Agarwal: Do you have that? And can you share that?

Sujit Mahato: It's upwards of 4 million.

Madhukar Gangadi: Yes. It's upwards of 4 million.

Sujit Mahato: We will connect, Sudarshan, offline on this.

Sudarshan Agarwal: Yes. Sure.

Moderator: The next question comes from the line of Jayshree Bajaj from Trinetra Asset Managers.

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Jayshree Bajaj: First of all, congratulations on a great set of numbers. My question is that diagnostics segment saw improved operating EBITDA margin, like a 15.3% in this quarter than the previous quarter, it was 13.6%. So, what are the current capex and expansion plans for the diagnostic services, including adding Level 2 and for service centers? And what is the revenue contribution we are expecting from the diagnostic centers in the next 1 year or 1.5 years?

Madhukar Gangadi: See, whether it is 15 or even 20, our plan will not change as far as the diagnostic centers are concerned, unless we cross 250,000 subscription numbers on the subscription side for the diagnostic side.

So for us, in the current model, which is capex intensive, where we put in INR120-odd crores to set up the 12 diagnostic centers, 4 of which are the large ones and all. We are not going to expand until we get the numbers we want. So that's it for that model right now. We are still looking to tweak the model to get those numbers.

And when we get them, then we will come back and basically tell you what we are planning to do. That's one. Second, given that this is a small portion of our overall business, less than 2% of our overall business, I don't think it is going to be material to us given that we are not planning to spend any more capex on this.

Moderator: The next question comes from the line of Avnish Barman, an Individual Investor.

Avnish Barman: Okay. So, this is Avnish. This is Avnish. I have one. I just wanted to understand the impact of the GST change on the business. And it will help if you can just segregate the impact on each player on the supply chain side.

So, for example, what was the impact on the manufacturer side, on the distributor side and on the retailer side? And if you also can comment on whether things are completely back to normal or there's still some time before things get back to normal?

Sujit Mahato: Sure. Sure, Avnish. Thank for the question. This is Sujit here. So, on the GST implementation, firstly, we went ahead with the broad intent of the Central Government, and we passed on the entire GST benefit to the customers right from 22nd of September when this got effective.

In terms of the impact, as you rightly said, for all stocks procured by the company prior to 22nd of September before the GST notification got implemented, we had incurred the higher input credit. So, there is an accumulation of input credit by an average of 7% for the pharma sector. And therefore, to that extent, the utilization got delayed. For example, we used to buy product at 12% GST prior to this change, and now it is reduced to 5%. So to that extent, there is a blockage of input tax credit by maybe a month or so in our case.

And on the retailer side, because all of them -- all of the stores are folding into the company, that's the only impact what we have. Additionally, what we have done, we have reached out to all our suppliers and we had negotiated an extra credit period, a one-time extra credit period, which you also see currently as a benefit in the net working capital change in our presentation, where we got some extra credit days for all the purchases made during the last couple of months.

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Because there is a blockage of our input tax credit on the inventory, which the company is currently holding. So, that's the very high-level impact on the GST change for us.

Avnish Barman: Okay. So from what I'm understanding, Sujit, it's like on one side, you've got like 1 month blockage of the ITC. And on the other hand, you've got like a one-time discount -- one-time credit leeway from the supplier. On the balance, has it helped the working capital? Or has it made transient working capital a little inferior?

And second, did you also get some discounts from the supply chain from the manufacturers?

And third, whether things are completely back to normal or there is still some time to go?

Sujit Mahato: See, from the impact on a wholesome basis, I think for a quarter, we have seen a positive impact. Going forward, it should come back to square zero because it's a zero-sum game going forward.

In terms, if you are thinking to look at what's the impact on volume, I think, yes, when the notification got implemented, we did not see any surge in volume.

It remained constant for us. And therefore, going forward, and you also mentioned, is there any discount from manufacturers? There are here and there, but it's not a standard kind of a discount

because most of them have helped us by giving us extra credit period as a one-time benefit.

Moderator: The next question comes from the line of Harith Ahamed from Avendus Spark.

Harith Ahamed: Sir, last quarter, we had talked about certain measures on the supply chain front, including addition of warehouses. So, can you talk about where exactly we are in that process? And if you can also comment on fill rates, which, again, we mentioned that there was some impact in the past as we rolled out the private label. So specifically, on fill rates for branded generics, have you seen an improvement in the last quarter or so?

Madhukar Gangadi: Yes. On the supply chain side, the new warehouses have been added and some of them are already functional, and some are still slowly coming to their full listing out there. They have all started functioning, though, most of them. On the fill rate, definitely, yes, we are slightly better, as you can see from the impact of there and all, both what we have done on rejiggering the entire incentive plan for private label versus brands.

And on the supply side, both of them have actually resulted in a slight increase in the branded generic sales. So it's a continuing process for us. I'm not sure, I don't think we are quite there on the fill rate completely, but it has improved.

Harith Ahamed: And on the pre-operative costs, there's been a marginal increase. It's higher than what we've seen in the previous quarters. So any reasons? Is this in line with the store additions planned in the second half?

Sujit Mahato: Absolutely. Absolutely, Harith. This is Sujit here. So, this is for the property bank and the number of apprentices, which we hired or the people which we put up for the initial training and those costs, we classify them as pre-operative. So, we have a property bank of 400 plus as we speak.

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So those 400 plus as and when the rental starts before we launch the stores gets accumulated under the pre-operative head. You are right.

Harith Ahamed: Okay. And last one on the margin

ins on the pharmacy retail operating EBITDA margins.

Historically, we've seen second half being stronger versus the first half. Should we expect a similar trend this year?

Madhukar Gangadi: I think where we are -- from where we are, it may not really go up by much. We'll probably be flat, maybe a slight increase, but I won't really count on it. So, a little early to tell you right now.

Moderator: The next question comes from the line of Madhav Marda from FIL.

Madhav Marda: Sir, my first question was on the -- if you could just give some sense on the growth -- top line growth versus margin metrics going ahead, if you had to think from a next, let's say, 2-, 3-year perspective, how is the sort of company thinking about driving top line growth for the pharmacy business along with the margins? If you could give some color in terms of how they could trend?

Madhukar Gangadi: Madhav, it's a little difficult for me to give you that right now. Our franchisee thing still is being piloted. Most of our growth should come from that side. Our normal growth, of course, would be to the tune of 600 to 1,000 stores, which we can put in without too much of a problem, given the overall market size and our own ability.

And the fact that we have money in the bank and everything else. So, please give us a little bit more time to really give you some proper color on what is going to happen in the next 2 to 3 years. I'd not be able to tell you right now.

Madhav Marda: Sorry, 600 to 1,000 stores is -- I mean, our own additions annually or over the next 2 years? I didn't follow that, sorry.

Madhukar Gangadi: That's annually.

Madhav Marda: Okay. Annual addition, 600 to 1,000 stores, you want to add to the network, which will be a mix of owned stores and franchisees in the next few years?

Madhukar Gangadi: And franchisee. But yes, this depends on a kind of a lukewarm kind of thing for franchisee. If franchisee really takes off, it could be much higher.

Madhav Marda: Okay. And how has the feedback been initially from some of the franchisees you've added, say, in the past year? Like if you could give some sense around the unit economics and how they are sort of faring, if you could give some sense there?

Madhukar Gangadi: A little early to speak about that, Madhav. I think we'll probably need at least another quarter or 2 for us to really draw any conclusions on that. The numbers are very low for us to really give you any meaningful kind of guidance on that or at least information on that.

Madhav Marda: Okay. Got it. And just one more question was on the opex. If you look at the pharmacy opex number, I mean, I just subtracted the EBITDA and the gross profit for the pharmacy business.

That opex number has grown a fair bit.

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So, have we front-ended some warehouse-related costs for like the people that we've hired for the new stores? Or is there some front ending of costs which has happened in 2Q for the expansions?

Sujit Mahato: Madhav, actually, the costs, if you look at are predominantly in the employee benefits line. As we had guided you earlier, the company had implemented a retention plan for the entire store-level workforce at least for the key cities.

And that's the major impact where we have seen where the first-year payment has just happened, where we had the plan was if an employee continues with us for more than 12 months, he or she is eligible for INR18,000 one-time cash bonus.

And if the employee rolls that for 1 more year and rolls it forward instead of encashing, that benefit increases to INR50,000. And if that is rolled for 1 more year, it goes up to INR1 lakh.

And that's the major chunk of increase what you're seeing other than the normal annual increment.

Madhav Marda: So, this sort of happened in 2Q. So 3Q, this number can actually come down a little bit like because then that payment is not going to

continue every quarter? Or how does that work?

Sujit Mahato: So the first year, for example, this would work more or less like an ESOP plan with various tranches. So for the first year, you have an estimate. But after a year, you start getting the true-ups or the actual catch-up depending on how many people exercise the option of roll forward versus encashment.

Once an employee encashes and the plan is still on, they go back into the queue and again start getting accrued for INR18,000 for the next year. Or if the employee rolls forward, then on a cumulative basis, you have to do an accounting for the INR50,000.

Moderator: The next question comes from the line of Tanya from Investec.

Tanya: Yes. So, congratulations for the set of numbers. My question is around the SSSG rate. So, what I can see is that it has made a turnaround in this quarter by registering a 2.2% growth as in the SSSG growth. What is your take on the same going ahead?

Sujit Mahato: Sir, we expect this to grow gradually because you saw the last couple of quarters, it did not show a positive traction. The company has changed the incentive plan and the other initiatives, adding supply chain capability. So, we expect this to grow gradually in the range and reach somewhere between the high-single digits of between 9% and 10%, but that should be a journey.

Tanya: And by what time frame do you expect it to reach the high-single digit?

Sujit Mahato: In the next 2 years, max, because this is above the entire network coverage. And as we speak, more than 80% of our network is in the greater than 24 months. So, we'll have to keep that at the back of our mind when we make this assumption.

Moderator: The next question is from the line of Amish Kanani from Knowic Investment Managers.

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Amish Kanani: Yes. Sorry. Partly my question related to GST and same-store sales have been answered. Just one question, which I just wanted to understand. Is there any benefit that we might get because of this GST rate cut on the FMCG part of our business?

And how does it work cross-sell and inherent demand, what type of products, if there is any -- some after-effects of the lower prices? Because I understand pharma, maybe the demand will not be elastic. But how do we see that?

Sujit Mahato: So on the FMCG or on the general front as well, directly, there is no impact because as you are aware, we report sales, net of GST. And also on the inventory, the inventory is accounted, net of GST. So, there is technically no impact on the P&L and balance sheet.

But as you rightly mentioned, if there are growth in the volume because of the overall uptick in the demand per se, we should get that fair share to our stores as well. But as of now, we have not seen any significant uptick on any volume.

Moderator: The next question is from the line of Saion Mukherjee from Nomura Securities.

Saion Mukherjee: Sujit, my question is on the expenses in general and employee expense in particular. So, we have seen almost 19% growth year-on-year with revenue growth at 7%. You mentioned about one-time bonuses scheme that you have launched. So, I just wanted to confirm whether this was launched in this quarter onwards?

And also, have you accounted for potential increases if the employee stays assuming some attrition rate? Has this been provided for? Basically, what I'm trying to understand is how should we think about this expense item sort of going forward?

Sujit Mahato: So, I think, on this question, Saion, we have been accounting this for the last 1 year, last 4 quarters. We have launched the scheme now close to a year ago. And I mentioned this is the first time where the payouts have happened, which means we have crossed the 12-month threshold. We have been doing an accrual basis of accounting on that. On a year-on-year, you see the impact now.

But otherwise, absolute

ly accruals have been factored with all estimates of how much do we expect will roll forward and how much we expect will encash. And only based on the due date, we are able to true up this number. But otherwise, we have factored this plan. Second, this is an ongoing plan as we speak. So it was not only a one-time, which means as and when people resign and new colleagues come in, they are also automatically getting enrolled in this plan. And therefore, the costs have been factored accordingly. So if I've answered you on this point.

Saion Mukherjee: So basically -- so there has been a structural change in the way you compensate the employee, which we have been doing it for a year now. So, going forward, this kind of a growth will come down, right? Because today, what we are seeing on the P&L is 19% growth year-on-year for employee expenses. So, the base is still adjusting.

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Sujit Mahato: Absolutely.

Saion Mukherjee: So next quarter onwards, it should start to moderate. The year-on-year growth in employee costs will start to moderate. Would that be a right assumption?

Sujit Mahato: Yes, it will start to moderate. Yes, that's the right assumption, Saion.

Saion Mukherjee: Okay. Okay. And on the pre-op expenses, so how large is that? And I mean, is it a meaningful dent on the P&L currently?

Sujit Mahato: One second. So in this quarter, it was close to around INR3.4 crores, INR3.3 crores. So, this was a factor of 2 things primarily. The employees who have been hired, not yet deployed anywhere, but are during the training program, which is for that 1-month period and rentals of premises, which we have leased, but not yet launched.

So, this should not exceed maximum of INR4 crores, Saion, because we continue to grow and therefore, there is always this number, which we classify as pre-op. But on a steady-state basis, this should continue to go down.

Saion Mukherjee: Okay. Okay. Just one more question on the employee. Have you seen any meaningful reduction in attrition after you rolled out the scheme? And if you can share the attrition rate now versus what it was a year back?

Sujit Mahato: So, I'll look into that. But at a very high level for in the major 3 cities where we launched this scheme, we have seen a significant amount of reduction because a couple of things, which we evaluated when the team looked at this model was more and more attrition pull from the, I would say, consulting model, be it a delivery point model or from a part-time store model vis-a-vis a full-time employment at our stores.

And therefore, we had upwards of 45% attrition in this section of employees. And after we implemented this, in some of the cities, we drastically saw a reduction of around 15%. So, this is being year 1. What we would like to see is how it plays out because there is also a ripple effect on the others. So, we want to see that how it plays out before we take any concrete steps on this plan.

Saion Mukherjee: Okay. And just one last question, sir, if I can ask. You mentioned about high single-digit growth rate, same-store kind of a growth rate 2 years down the line. Given that we are expecting the pharma market to grow at around 8%, don't you think you can get to double-digit growth, assuming you get market share from other retailers?

Sujit Mahato: No, I think, absolutely, but we did not want to sound so optimistic immediately. We want to go through this journey because you have seen now it's around 2.2% plus. So, I just want to be very cautious in terms of guiding. Otherwise, we would say maybe let's wait for a couple of quarters to be more optimistic on this number.

Moderator: The next question comes from the line of Aejas Lakhani from Unifi AMC.

Aejas Lakhani: Team, great job on execution and maintaining balance of margins this quarter. So, credit to you.

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Moderator: I'm so sorry to interrupt in between, sir. Your voice is not audible.

Aejas Lakhani: Yes. Is this better? Sorry. Is this better?

Moderator: Please proceed.

Aejas Lakhani: So team, I was just saying stellar performance in terms of the balance of growth and execution and margin maintenance. So, credit to the team for the same. I wanted to just understand a little bit more on the growth that you're seeing in private label, non-pharma.

So basically, your others, your branded segment and FMCG. Can you -- the growth there, I mean, if you trace it back to several quarters has been higher than that of private label pharma. So, could you tell me what exactly is driving the shift? Is it that the customer comes in, you don't have the product, maybe a branded product and they see your product?

And how do they sort of -- how is that buying decision and pattern there? Is it the price comparison that he's able to see, which helps him to pick it up? What is driving that decision?

And additionally, what are the margins on PL non-pharma and pharma? Could you just call those out?

Sujit Mahato: So on the PL non-pharma specifically, Aejas, what you rightly mentioned, couple of quarters ago, this was hovering around 7.5%, 7.8%. And we really grew steadily because one major aspect, if I can take you through is we have been guiding that the way we serve our customers in our stores, let's say, 18 months ago, we have changed that over period by bringing in the racks outside or the shelves outside wherever real estate allowed us.

So that there is some amount of display as well as buying by the customers themselves because pharma continues to be served from behind the counter, because most of the staff will have to pull out from our racks and give them based on the prescription.

But for non-pharma, since this restriction was not there, wherever real estate allowed us for more than, I think, 1,500-plus stores, we redesigned our outlets where we have now shelves outside, there's some amount of impulsive buying. And to add to that what Madhukar explained earlier that we have added a large assortment of new products on the FMCG side, which is constantly driving this growth.

To top that, there is technically no upper limit unlike in pharma, where once a doctor prescribes a regimen or a dosage for 3 days or 5 days, the customer is bound by that. But in a non-pharma, once the product is good and they see value in that, they can continue to buy those products. So, that has helped us, and I think the focus will continue on those lines.

Aejas Lakhani: Understood. And could you help on the margin build?

Sujit Mahato: Yes. So on the private label pharma, we are still close to 78% to 75%. This is before any store-level incentive is given out. And on the private label non-pharma, this would go up to 34%.

Aejas Lakhani: Okay. This is in gross terms, right?

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Sujit Mahato: This is on net revenue.

Aejas Lakhani: Okay. No, no. But in terms of an operating margin perspective, are you earning more on the non-pharma PL side versus the pharma PL side?

Sujit Mahato: No, no, absolutely.

Aejas Lakhani: That would be materially higher by, say, 5%, 10%. Could you quantify something like that?

Sujit Mahato: So, I mentioned the PL pharma would be close to 74% to 78% versus non-pharma would be in the range of up to 34%. So it's close to a 50% difference between pharma and non-pharma.

Aejas Lakhani: Okay. I'm sorry. Okay. I didn't catch that. Understood. And so could you just help me texturize this buying that is taking place in non-pharma? So let's say, it's a consumption item like, say, a toothpaste, Indians are indexed to 2 or 3 or 4 brands. So is it that the basis of purchase is non-availability? Or is it that they are able to see both the brands, compare the price and then price is the

driver of that decision? Could you help me with that?

Sujit Mahato: In terms of comparison for a customer, absolutely, for the brand, there is nothing more what MedPlus will do. But for our own products, the price and the first experience definitely drives the consumer demand.

So the pricing philosophy is very clear. It's cost-plus model rather than we doing a similar MRP model. So it is -- that is one. And then the customer experience, once they try out a product and they find good value for that, they do come back for that.

Moderator: The next question comes from the line of Ankit Singh from AB India.

Ankit Singh: Yes. Sir, my question is, sir, in this quarter comparison to last year and first quarter of this fiscal year, sir, inventory has been -- it has piled up a lot from -- I think in your P&L, it is showing approx. 150. Can you please tell me about that? Is there any change of trend are you seeing? What are you observing on that?

Sujit Mahato: I did not get your question. If you can please repeat?

Ankit Singh: My question is, sir, cost of inventories in your P&L section has been risen very significantly in this quarter compared to last year and compared to the first quarter of this fiscal year. Sir, what are you observing? Any change in trend or something else? It is going up very significantly.

Sujit Mahato: I did not understand this question. Are you talking on the inventory levels on our balance sheet?

Ankit Singh: Yes, yes, yes, yes.

Sujit Mahato: Okay. So if you see that, if you compare from a year ago, consistently, the inventory levels have been going down. So this is, again, a well-thought through plan internal monitoring where we are balancing between the private label inventory as well as in the branded, at the same time having a much sharper monitoring mechanism on our store level as well as on the warehouse

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level inventory, which I think over a period of time, the team has been able to optimize this

number. As such, there is no exception in these numbers, if I have to clarify.

Ankit Singh: Okay. Okay. Sir, what are your plans, sir? I've been listening it from last year, last 1.5 years from the listing, plans in expanding in north part of India, sir, Delhi side in North India. There is nothing you are telling about it. What are your plans now as you are listed for 2.5 -- 2 years of listing? Sir, what are your plans now north side of India?

Sujit Mahato: So currently, we have not expanded directly into the northern part of India, but we have started adding stores in Chhattisgarh and Madhya Pradesh. We are seeding our initial stores, and we will progress slowly because as you are aware, since the company is established more in the southern part of -- southern and eastern part of the Indian continent, there's still a lot of headroom to go deeper.

But at the same time, we take your point. We will internalize this because there is a plan to continue to grow. And as and when we add more states or adjacencies, we will come back and report to you.

Ankit Singh: Okay. Okay. Sir, one question more. Sir, are you getting competition from generic pharmaceuticals like I have been studying here, Knoll Private Limited in north part of India is also expanding in your cities like Hyderabad, Telangana. What is the level of competition are you getting from these drug holders?

Like they are being just selling it at very low point that I have observed in north part of India. They are selling at very cheap rate because -- by selling it generic. Are you -- because of this specific company, I know it. That's why I'm saying that it has also expanded in your Hyderabad, Telangana. Are you getting competition from these drugs?

Sujit Mahato: See, we are a retail chain. So, I think we trade or we partner with more than 2,000-plus companies. So obviously, we will not be able to comment specific

call you on any pharmaceutical

company. But we being a retailer, we would definitely try to fulfil the demand, whatever comes to our customers. And if those products are relevant, we will also sell them. That's all I can tell you.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to Mr. Sujit Mahato for closing comments. Over to you, sir.

Sujit Mahato: Thank you. I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you, and good day.

Moderator: Thank you. On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you for joining us today, and you may now disconnect your lines.

Call: Feb 2026

MedPlus Health Services Limited

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CIN No: L85110TG2006PLC051845 | Website: www.medplusindia.com | Email: medplus@medplusindia.com

February 04, 2026

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BSE Limited
Phiroze Jeejeebhoy Towers
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Mumbai 400 001
BSE Scrip Code: 543427 The Listing Department
National Stock Exchange of India Limited
Exchange Plaza,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051
NSE Symbol : MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst/ institutional investors on February 02, 2026 for the quarter ended December 31, 2025.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Manoj Kumar Srivastava
Company Secretary & Compliance Officer

Encl: a/a

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"MedPlus Health Services Limited
Q3 FY '26 Earnings Conference Call "
February 02, 2026

Management: Mr. Gangadi Madhukar Reddy - Chief Executive Officer and Managing Director
Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. DRN Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day, and welcome to the MedPlus Health Services Limited Q3 FY '26 Earnings Conference Call. As a reminder, all participant clients will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchstone phone. Please note that this conference is being recorded. I will now hand the conference over to Mr. D.R.N Srinivas from MedPlus Health Services Limited for opening remarks. Thank you, and over to you, sir.

D.R.N Srinivas: Thank you, Ryan. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q3 FY '26 Earnings Conference Call to discuss the financial results of MedPlus for the third quarter of FY '26, which were announced earlier. We have with us today the senior management team represented by Mr. Madhukar Reddy Gangadi, Chief Executive Officer and Managing Director; and Mr. Sujit Mahato, CFO.

Before we begin, I would like to mention that some statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties. Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone, on this call. We have opened 228 stores during the current quarter, and there were 46 store closures. Out of 46 closures, 17 stores were relocated. 7 stores are in the process of getting converted into franchisee. 10 franchisees we saw withdrawals and 12 stores are closed for other reasons.

We achieved a net addition of 182 stores during the quarter compared to 117 stores added during the last quarter. During the current financial year, we have achieved net addition of 400 stores. We continue with the outlook for adding 600 new stores in FY '26.

Update on the network. In terms of our store network age, around 23 % of our stores have been operational for less than 2 years, and the remaining 77% of our stores have been operational for 2 years or more. In terms of the network size, at the end of the quarter, our network grew to 5,112 stores with 2.6 million plus square feet compared to 4,612 stores and 2.4 million square feet at the end of December '24. The average store size is 527 square feet.

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Update on the revenue mix. Presently, MedPlus offers a large range of SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for quarter 3 FY '26 constitute 22.2%, Pharma being 11.6% and FMCG being 10.6% of our total revenues.

On GMV basis, during the current quarter, the share of the private label pharma sales stood at 18.9% compared to 7.9% prior to the launch of MedPlus branded pharmaceutical products. That was in last year's quarter 1. An update on financial numbers. Our consolidated revenue for the quarter stood at 18,061 million.

Our consolidated operating EBITDA for the quarter after considering the nonrecurring charge of 70.59 million on account of the implementation of the new Labour Code stood at 96.8 crores, representing 5.4%. Revenue from pharmacy operations grew by 15.6 % year-on-year on a reported basis. The pharmacy operating EBITDA stood at 925 million, representing 5.2%.

An update on our stores performance, stores older than 12 months. Revenue from these stores in quarter 3 was 16,300 million, representing 96% of our pharmacy revenues. These stores had

a store level EBITDA margin of 12.4%. A word here on the store level EBITDA margin by age. While stores greater than 12 months had a margin of 12.4%, this was 12.6% for stores greater than 24 months and 8.8% for stores in the 13 to 24 months age bracket. On allocating all non-store-related costs, the operating EBITDA of stores greater than 12 months would be 967 million, which translates to a margin of 5.8%. On working capital, our net working capital for quarter 3 was 53 days. The inventory in our warehouse stood at 34 days. In quarter 3, the inventory level of our first-year store was 103 days. In comparison, for a store older than 12 months, the inventory was around 35 days.

An update on our diagnostic numbers. Diagnostics revenue grew to 326.7 million in quarter 3 FY '26 compared to 274.7 million in quarter 3 FY '25. Diagnostics segment recorded an operating EBITDA of 50.7 million compared to 22.1 million in quarter 3 FY '25. This translates to 15.5% as an operating EBITDA margin. In October, we sold 506 gross plans per day. In November and December, this was 529 and 528, respectively.

As on 31st December, we had 1,80,000 active plans covering 3,68,000 underlying lives. As on 30th September, we had 1,70,000 active plans covering 3,51,000 underlying lives. Our current observed on-time renewal rate is 23% in Q3 versus 24% in the previous quarter.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: We take the first question from the line of Riddhansh Chandak from Unifi AMC. Please go ahead.

Riddhansh Chandak: Hi, team. Congrats on a stellar number. My first question is that, Madhukar, could you explain that 10% plus SSSG, we've seen that after some time now. And you also spoke about in your opening comments, Sujit mentioned the 18.5% private pharma GMV sales. So could you give some more color about what's driving that?

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Sujit Mahato: So I think that's a great question. What we had articulated earlier, while we had seen a slowdown in the greater than 12-month stores. The company tweaked the incentive arrangement at the store level, and we are seeing that paying off dividends, and this is in the right direction. So at least there are 2 reasons for this SSSG growth. One, we started with a bit lower base of the previous quarter in the last year. And two, the impacts of the changes in the incentive structure and a bit of availability improvement due to the implementation of the new warehouses.

Riddhansh Chandak: Understood. Sujit, so you have alluded to the incentive structure change, but I thought that you had reached the desired level of private label -- private pharma sales and you were hoping that branded would again start to pick up. So did we make those tweaks in this quarter or the old -- or the revised incentive structure continues, which is continuing to drive the private pharma sales?

Sujit Mahato: No. As we had indicated earlier, we have tweaked the incentive structure so as to consider the total sales growth at our store level, which is paying off dividends. We are clearly seeing the improvement in the branded pharma uptick as well as the uptick in the private label non-pharma, which is helping us to achieve these numbers.

Riddhansh Chandak: Understood. Second is, could you speak about the margin profile a gain has been very strong this quarter and congrats on excellent execution. So how do you expect gross margins to continue to trend for 4Q and next year along with the operating EBITDA?

Sujit Mahato: Yes, I think we do not generally give guidance, but on the gross margin level, we expect it to remain at the same level.

Riddhansh Chandak: Understood. Thirdly, could you just speak a little bit about what's happening on your franchisees that the expansion on that? Is this an opportune time to start speaking about it? Or should we

Id we

expect more color after 4Q?

Sujit Mahato: Yes. I think after the full year, we would like to get into that. But as you rightly picked up, that initiative is ongoing. And we are working with a couple of models, a couple of partners to see which is the right fit and how does it fit into our overall strategy. But that being said, that initiative is currently ongoing.

These numbers include the impact of franchisee and distribution sales, which is captured in the presentation in the other segment, which currently, if you see the others would be in the range of 5.7% of the total revenue. And this alone is now 3.7%. As a comparison, earlier, this was 1%.

Riddhansh Chandak: Understood. So incrementally, to understand the run rates of the franchisee, you'll be reporting it under others so that we get more clearer sense. Is that broadly correct?

Sujit Mahato: Yes.

Riddhansh Chandak: Understood. And could you just call out quantitatively how many franchisees were there this quarter? How many -- any color on those things, at least if you can quantify?

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Sujit Mahato: I think going forward, we would like to do a comprehensive disclosure on that. But for the moment, I think we will continue with this, yes. We can connect offline.

Moderator: We take the next question from the line of Umakant Sharma from Viansh Ventures. Please go ahead.

Umakant Sharma: Congratulations on a fantastic set of numbers. A couple of quick questions. Could you just talk a little bit about the GMV growth that we have seen in the quarter on a consolidated basis?

Sujit Mahato: I have to take it offline. I do not have it handy.

Umakant Sharma: Okay. Sure. I can take that offline. Parallely, just on a thought process standpoint, this quarter, we saw substantial growth in your non-branded non-pharma business, right? We saw growth of about 48%. Could you just walk us through how does the margin profile look along the branded pharma versus the non-pharma on the branded side? And parallelly, the unbranded pharma and the unbranded non-pharma, what is the margin profile across the spectrum look like on a net revenue basis?

Sujit Mahato: Sure. So on the reported basis, branded pharma would give us around 13% to 14% gross margin.

Branded non-pharma would be more or less in the similar range. For us, the private label pharma, the old as well as the new MedPlus brand put together would anywhere be between 65% to 70%.

And the private label non-pharma would be around 25% to 28% gross margin.

Umakant Sharma: 25% to 30% you said?

Sujit Mahato: Yes.

Umakant Sharma: Okay, great. And after a very long time, we are seeing a very good growth in terms of the store expansion side. Could you just throw some color on what the guidance could be for next year on FY '27 as well. So you may be guiding the target of 1,000 number on a full year basis for FY '27?

Sujit Mahato: Sure. Currently, we are not guiding. But yes, once we finish our internal AOP process, maybe at some point in time or next quarter or beyond that, we will definitely let you know.

Umakant Sharma: Okay. But any ballpark numbers that you are looking at for next year, sir?

Sujit Mahato: It should at least be the similar numbers of this year.

Umakant Sharma: Okay. Got it. Sure. And structurally for next year, do we have any guidance in terms of the top line that we're expecting. How should we be seeing the mix change between the four key areas between the branded, unbranded side?

Sujit Mahato: We do not provide such guidance, I'm sorry, actually.

Moderator: We take the next question from the line of Sudarshan Agarwal from Axis Capital. Please go ahead.

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Sudarshan Agarwal Congrats on the great set of numbers. So one of my questions is regarding SSSG. So you mentioned that base year growth was kind of low. So just kind

of extrapolating Q4 was actually

a decline for you in SSSG. So that should mean that SSSG at least for the near 3 to 4 quarters will remain at current levels or kind of improve also? Is that the right kind of conclusion to be taken away?

Sujit Mahato: I think, Sudarshan, your observation is right. At least for the next 1 quarter, I have a very clear visibility, but we will see as we move forward.

Sudarshan Agarwal Okay. And on the gross margins, I understand -- so on the gross margins, I understand that the private label pharma has kind of gone down a bit. But on the other hand, you have your non-pharma private label that has gone up. So the delta in gross margins, I think we used to allude that every 50 basis points adds around 10 to 20 basis points of gross margin. That is relative to pharma private label or overall private pharma label would also add that kind of level?

Sujit Mahato: Relative to the pharma private label, but you are right, since there is no substitution being done on the private label non-pharma, the overall increase in the sales and the gross margin will be accretive to that extent. Pharma, there is a possibility that there could be a switch from the brand to the private label. But in non-pharma, we see that very low. That could be only incremental sales and incremental margin.

However, from an optics, the gross margin stood at the similar level, but that had a small impact of our franchisee as well because we sell to the franchisee at a lower margin. And that after netting of that, and the increase in the private label non-pharma, we were able to continue with a similar gross margin level.

Sudarshan Agarwal Got it. Got it. One more request. So on the private label, I understand that you are still kind of working around with the incentives, etcetera. But let's say, 2, 3 years out the line, do you have a visibility right now, you would want it to be, let's say, 25% or 30%. Is there some number that we still are looking to kind of get to either pharma or non-pharma?

Sujit Mahato: As this is a growth journey, Sudarshan, you'll also agree, our numbers will be aspirational, but we are not giving any guidance at the moment because non-pharma by design, there's no cap because at least on the pharma, there is a cap theoretically because there is a doctor's prescription,

which we need to fill.

But on the non-pharma, if the experience is good, the product is good, the availability is good and the quality is good, there is no cap per se in terms of how far this could go. And the company continues to add new assortments, new categories, which is helping the sales and the gross margin.

Moderator: We take the next question from the line of Gaurav Nigam from Tunga Investments. Please go ahead.

Gaurav Nigam: Just one question. When I was looking at non-store expenses, they seem to have gone up materially and not only on year-on-year, but also on the quarter-on-quarter basis also. Can you please explain? And is this run rate inching up going forward? How should we think about that?

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Sujit Mahato: So in corporate expense line, there includes a one-off nonrecurring expense also we'll have to be mindful of that 70 million or INR7 crores, which was the impact of the past service cost post the implementation of the new Wage Codes. So that's a one-off. Going forward, it should be more or less in the same range because, one, we'll have to be mindful of what could come, those notifications and the rules are still not yet final.

So we are looking, like others, we are keeping -- closely monitoring this space. And maybe by end of March, end of April, we should have more clarity. But as we speak, we are on a very good footing.

Gaurav Nigam: This INR7 crores is onetime? Or is it expected to continue?

Sujit Mahato: It is onetime, you're right.

Gaurav Nigam: Okay. And sir, all the warehouses a

nd the other logistics improvement that we were doing, is that all open and that cost is in the base now?

Sujit Mahato: At least 60% to 70% of the new warehouses have been operationalized. The rest will also get operationalized in the next two quarters. The manpower recruitment for all these warehouses are complete. So we do not expect any significant ramp-up on those expenses on those lines.

Moderator: We take the next question from the line of Madhav Marda from Fidelity International. Please go ahead.

Madhav Marda: On the FMCG side, could you give a little bit more color in terms of what are the new assortments or categories that we're adding? And what's sort of helping this growth which can sustain in the coming times?

Sujit Mahato: I think, Madhav, we'll take this offline. But yes, what we have been adding now is more of the food category, the wellness category, some cold pressed oils, which are in -- again in the wellness category. So these are the few categories we have started adding, and we are seeing good traction.

Madhav Marda: Understood. Got it. And sir, just on margin side for us, I think this quarter, if we adjust for that one-off of the 70 million, pharmacy operating margins are about 5.5%, 5.6%. Like you said, a lot of the cost of the warehouses is already in the base now. So are we sort of within striking distance of getting to 6% in the next couple of years? Is that a fair way to think about it, given some of the operating leverage plays out with the sales growth. And given SSSG is sort of now improving at a good pace as well?

Madhukar Gangadi: So, Madhav, our margins, whatever we actually start improving on will be largely a function of the private label. And on private label, I actually am now equally bullish about the non-pharma side. Non-pharma is just a matter of adding categories out there. We are using, let us say, getting the full benefit of the 5,000 stores, which basically means we have the minimum amount of quantity for almost any product.

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So as long as the product is something which people will buy in the regular convenience or wellness category, we are more than happy to actually put it out there. And I feel at least given the last couple of quarters' performance, I see no reason why we should not get to that number.

But yes, and again, on the pharma side, while we fully expect this to continue to grow up, I expect that to happen as a result of, let us say, the quality of the product, as a result of all the positive publicity we expect to get from customers who are already using it. So we have significantly reduced the push on the private label pharma, but we continue to be, let's say, bullish about the non-pharma side.

Madhav Marda: Okay. Okay. So, when you said we can get there, as you talked about the margins, that there is potential for us to reach the 6% over a period of time?

Madhukar Gangadi: See, I don't see any reason why not. It's a little early to say. There are a lot of pieces out there on the ground. But given that only 20% of our sales now come from general goods and most of our

competitors are in the range of around 30% to 40%, even 50 %, even a slight jump in the general route side and most of it coming from private label will not only -- will definitely increase the top line and also will improve the profitability. Yes, I feel confident about that, but I can't really give you an exact time line in which this will happen.

Moderator: We take the next question from the line of Raman KV from Sequent Investment. Please go ahead.

Raman KV: Sir, I just want to understand, you have mentioned that you have tweaked your incentive structure, which helped your SSSG growth to be in the range of 10.5 %. Can you throw some light on what are the incentive structure that you tweaked?

Sujit Mahato: So earlier, the incentive structure was predominantly working around the private label sales.

What we have tweaked is we have now included a component of total sales at the store level, which means the branded as well as the private label sales.

If the total sales are not achieved or let's say, there are targets linked to that, then the achievement percentage on the private label goes down by certain basis points, and that keeps motivating the store level employees to achieve both the private label as well as the branded total sales growth at the store level, and this is helping us in the overall journey as well.

Raman KV: Understood, sir. And sir, I just want to understand the nature of the goods or basket of goods under the non-pharma business like branded as well as private non-pharma business, what are the products which -- that you are launching? And is it the growth in this business is recurring.

Or is it because you are introducing new product and your total addressable market is expanding?

Sujit Mahato: So the product range in the non-pharma, both branded as well as private label is a large assortment of all our daily needs, right from your early morning, let's say, the needs of toothbrush, toothpaste, hair oil, cream, toilet cleaners, soap, shampoos. So there's a large assortment of, I would say, 1,300-plus SKUs, which we offer to the customers, great quality and great affordable price. So that is helping us.

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And to it, which what we mentioned earlier, we keep on adding new categories depending on what the needs are, what we identify at the store level, where there is a regular need and a good product available. So that is what is helping our incremental sales on the non-pharma products.

Moderator: We take the next question from the line of Sanjay from Bastion Research. Please go ahead.

Sanjay: Sir, I just wanted to have a bookkeeping question. So we have stopped giving store level MRP growth for stores within 12 months. Could you please share that data?

Sujit Mahato: So it was not making sense, Sanjay, because you would agree that since the change in the GST rate, there is a flat reduction of the GST by around 6.5%, 7%, which makes comparability a big issue. And that is the only reason we removed that. But otherwise, we would have to do a manual adjustment and then give you a number which is not directly comparable.

Sanjay: Understood, sir. Okay. Sir, also wanted to understand that we are going much deeper into the states which we are already present. So I see the store expansion are largely into our states which we are present and have enough presence on that specific. Do we plan on that side. Or going forward, how is the store expansion we are looking forward? I understand that we follow a cluster-based approach. So -- but I wanted to have your sense on how we are planning to new store or new geography or new city, so to say?

Sujit Mahato: So the priority always would be going deeper into the existing states and then sweating the warehouse assets, which we have created for these states and then slowly getting into adjacent states. We had started Chhattisgarh, we have started Kerala. But yes, the main new stores where you can see now is part of the densification of the existing presence where we have. So it's an ongoing strategy, Sanjay.

Sanjay: Sir, you also shared in your opening remark or somewhere you shared the revenue for stores greater than 12 months. Did you share overall for the last quarters as well because I'm not seeing that. So if you can include for the quarter-on-quarter and Y-o-Y that revenue for greater than 12 months, it would be really helpful. Thank you.

Sujit Mahato: So at the overall level, we have given. But if you're only -- the specific thing we have added is for greater than 12 months as a bucket. But overall, we have anyhow given you both in the investor deck as well as in my call. But I did not get your question

if you're asking other cohorts.

Sanjay: Sir, I'm asking for greater than 12 months only because in the previous comments or previous call, we did not spoke to revenue greater than 12 months stores. So just wanted to have your Y-o-Y and quarter-on-quarter numbers on that side?

Sujit Mahato: So we'll look at it. I'll take this offline.

Moderator: We take the next question from the line of Divyansh Gupta from Latent PM S. Please go ahead.

Divyansh Gupta: A couple of questions. With the change in the incentive structure, the previous guidance that we

Does that still hold or that will -- that should also be -- is there any new reference point you want to share?

Sujit Mahato: I think we can continue with the same assumption. There's absolutely no change on that.

Divyansh Gupta: Got it. Got it. And the second question was, I think in the last con call, we had mentioned with for that employee retention, we had introduced a scheme and probably this quarter would be the first quarter where we would actually see the results of exercise. So if you can share some insights on that part. Is it working as per plan or...

Sujit Mahato: So it's an ongoing scheme, which is going to run for 3 years, which when we launched for only three cities. So that's again one initiative which we are very closely monitoring to see the effectiveness. Based on that, the company would then take a call whether to extend that across all the regions or curtail that further.

Divyansh Gupta: But my question was that as it -- given that there was -- the exercise timeline was in this quarter, has it panned out as per our expectations even in the final?

Sujit Mahato: There is an exercise, I'll explain to you. The first exercise was post 12 months an employee getting into this plan. And we had the first exercise last quarter, wherein we saw a segment of employees exercise their right to claim that first 18,000 and the segment of employees who have postponed their rights to claim 50,000 in year 2.

And we are seeing improvement in attrition rates in the three cities which we have implemented.

I also mentioned that the company is closely monitoring this initiative to see whether it would make sense to continue this, expand this or further curtail this. So all these are still open.

Divyansh Gupta: Got it. Understood. And sir, you had mentioned the gross margins for the various product categories on a gross basis. What would be the -- what can be the margin on a net basis?

Sujit Mahato: Whatever I mentioned is on net basis. It's all on net basis.

Divyansh Gupta: I thought private firm -- it's on net basis. Got it. Got it. Understood. That's all right. Thank you.

Moderator: We take the next question from the line of Lakshminarayanan from Tunga Investments. Please go ahead.

Lakshminarayanan: A few questions. I just want to understand what's the kind of mix of the repeats you have in your pharma side? And the second question is that in your old stores, in the pharma, what kind of repeat business you actually get in a sense that, let's say, the customer comes back and buys in the last 1-year or whichever way you qualify or quantify?

And the third, since we have a significant amount of private non-pharmacy also private label, what kind of inventory write-off or damages you actually occur every quarter and how you plan to bring it down? These are the three questions.

Sujit Mahato: Sure. Thank you. Thank you for your questions. To start with, in both the entire private label pharma portfolio consists of both the acute as well as the chronic segment. I would say we are MedPlus Health Services Limited

more to 55 chronic and 40, 45 on the acute. So we are all the top five therapeutic areas as listed by WHO

, we are all in all those top five categories.

You can take a diabetics, cardiovascular and the hypertension. So all these areas are completely covered by our chronic range. And the acute, as you are aware, a gain, around 40%, 45% of our sales comes from the acute range. Coming to the private label non-pharma, absolutely, there's a large assortment.

And you also mentioned about the inventory risk. You're absolutely right. On the private label, the inventory risk is completely on our book. As a trend, we have been providing close to 0.9% to 1% of our sales of private label as a provision for deterioration in inventory.

Lakshminarayanan: Got it. And in terms of repeat business, can you just give some views on that?

Sujit Mahato: Yes. As we speak, we have more than 46 lakhs to 47 lakhs of members on the pharma side itself. That actually gives us a very strong conviction of our repeat business. Customers become members to access the trade discount, which is offered by the company on the private label product as well as a higher discount on the branded pharma product.

And once they become members, they get full access to these discounts for 1-year. What we are seeing is on the both private label, pharma and non-pharma, a very good repeat business every once in 3 months. This is the exercise what we do internally. We are seeing complete repeat business of close to 90%.

Lakshminarayanan: Got it. Got it. And last one more question. I see that these days, you are able to provide or service the online pharma very quickly. Can you just help me understand whether this is like a one-off or you have actually consciously improved your serviceability in the online purchases?

Sujit Mahato: We are also looking at this space very carefully because you would appreciate the competition or the offerings by the quick commerce. So we have also ramped up our offerings on this space. But as and when needed, we can further improve it. But I think what you have experienced is a positive story for us if you have seen that improvement. We are also consciously working on that.

Moderator: We take the next question from the line of Madhav Marda from Fidelity International. Please go ahead.

Madhav Marda: Sir, just one other question. Our working capital has come down a fair bit, almost 10 days year-over-year. Could you explain why that has come down? Is it sustainable at these lower levels going ahead?

Sujit Mahato: See, I think a couple of things. One, the -- wherever we have opened the franchisee model stores, we are not carrying inventory on our books. It is sale on first day. To that extent, roughly 15 lakhs to 18 lakhs of inventory per store, that's a reduction which we have. Otherwise, we were carrying on our books. That's one.

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And on the second hand, we have been mindful of the inventory what we are carrying both on the store level as well as at the warehouse level. And that -- those improvements are very clearly getting reflected on the number of days. On top, we have added only 400 stores. To that extent, a year-on-year comparison shows you a better benefit because cash has been conserved.

Madhav Marda: Okay. Does the higher private label mix help as well. Because I remember in one of the earlier commentary you mentioned for private label structurally, we need to carry lower inventory at the stores because the SKUs come down. So is that helping at all or not really?

Sujit Mahato: So currently, we are carrying both the inventory, Madhav. So we have not yet what we had discussed earlier. At a certain point, we can then cut the tail or the long tail of the brand. But for the moment, we have not so that we ensure full availability to our customers.

Madhav Marda: Understood. Just last question on the gross margins for the pharmacy business. I think it's at about 25% odd. Sort of this should keep gradually moving

up as the private label mix goes up,

right? That's just the way to think about this line item?

Sujit Mahato: Absolutely. That's what Madhukar explained, you're right.

Moderator: We take the next question from the line of Akhil Parekh from B&K Securities. Please go ahead.

Akhil Parekh: Many congratulations on an excellent set of numbers. My first question is, I know you're not guiding for next year, but for our business model, what is a sustainable SSSG growth rate given that we are at 5,000-plus stores and our private label business has fairly stabilized. Any guidance or any ballpark thing basically which one should look at basically whether it's high-single-digit or low-double-digit SSSG is sustainable? That's my first question.

Sujit Mahato: I think that's the aspiration which we are internally working with. But we do not give any such guidance because, again, we have seen the store network getting matured over period. And that's where we are also looking at what measures we can take at our store level, including the back-end efficiency so that we can continue this healthy SSSG and aspire for much more. So at least for this moment, we are not giving any guidance.

Akhil Parekh: Okay, sure. Second and last question, if I look at our sales contribution from metros, right, has continued to decline over the last many quarters and non-metro and Tier 2, Tier 3 has gone up.

Is it largely to do with the -- our private label story because what I understand is the adoption of private label is much better in Tier 2, Tier 3, Tier 4 towns?

And hence, as we kind of increase our sales contribution from private label, kind of -- we'll kind of continue to see that expansion more into the non-metro cities. So is that understanding correct?

Sujit Mahato: I think it's not a straight answer, but what we could also keep in mind, we need to be mindful of the net realization on our private label pharma, especially. Earlier, we were able to realize around 83, 100 minus 17 of blended discounts, we were able to realize 83 vis-a-vis now 45, 46. So that has to play out for a couple of quarters to have a like-to-like comparison.

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Moderator: Thank you. Ladies and gentlemen, with that, we conclude the question-and-answer session. I now hand the conference over to the management for their closing comments.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services Limited, that concludes this conference call. Thank you for joining us, and you may now disconnect your lines.

Call: May 2026

MedPlus Health Services Limited

040-6724 6724

Regd. off. H. No: 11-6-56, Survey No: 257 & 258/1, Opp: IDPL Railway Siding Road, Moosapet, Kukatpally, Hyderabad – 500037, Telangana, India

CIN No: L85110TG2006PLC051845 | Website: www.medplusindia.com | Email: medplus@medplusindia.com

May 26, 2026

The Listing Department
BSE Limited
Phiroze Jeejeebhoy Towers
Dalal Street, Fort,
Mumbai 400 001
BSE Scrip Code: 543427 The Listing Department
National Stock Exchange of India Limited
Exchange Plaza,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051
NSE Symbol : MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst/ institutional investors on May 21, 2026 for the quarter ended and financial year ended March 31, 2026.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Gangadi Madhukar Reddy
Managing Director and Chief Executive Officer
DIN: 00098097

Encl: a/a

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"MedPlus Health Services Limited
Q4 FY26 Earnings Conference Call "
May 21, 2026

Management: Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. DRN Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day and welcome to FY26 Q4 Earnings Call of MedPlus Health Services Limited. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchstone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. DRN. Srinivas. Thank you, and over to you, sir.

DRN Srinivas: Thank you, Iqra. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to MedPlus Q4 FY26 Earnings Conference Call to discuss the financial results of MedPlus for the financial year FY26, which were announced earlier.

We have with us today the senior management represented by our CFO, Mr. Sujit Mahato.

Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties.

Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the investor presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone on this call. The update on our network with regards to openings and closures, the company opened 618 stores net additions during the year.

During the current quarter, we have opened 295 outlets and there were 77 store closures, including 18 related to relocation cases and 13 franchisee outlet closures.

We achieved a net addition of 218 stores during the quarter compared to the 182 stores added during the last quarter. For FY27, company plans to open 800 net new stores, including franchisee outlets. An update on our network. In terms of the age of our stores network, around 24% of our stores have been operational for less than 2 years and the remaining 76% of our stores have been operational for 2 years or more.

On the store size of our network, at the end of the quarter, our network grew to 5,330 stores with 2.8 million plus square feet compared to 4,712 stores and 2.4 million square feet at the end of March '25. The average store size is 528 square feet.

On the revenue mix, presently, the company offers a large range of SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for Q4 FY26 constitute 22%, pharma being 11.4% and non-pharma constituting 10.6% of our total revenues. Our consolidated revenue for the quarter is INR18,644 million.

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Our consolidated operating EBITDA for the quarter stood at INR1,076 million, representing 5.8%. Revenue from pharmacy operations grew by 23.4% Y-o-Y on a reported basis. The pharmacy operating EBITDA stood at INR1,022 million, representing 5.6%.

An update on our store performance. Stores older than 12 months. Revenue from these stores in quarter 4 was INR1,606 million, representing 96% of pharmacy revenue. These stores had a store level EBITDA margin of 13.1%. A word here on the store level EBITDA margin by age.

While stores greater than 12 months had a margin of 13.1%, this was 13.3% for stores greater than 24 months and 9.3% for stores in the 13 months to 24 months age bracket.

On allocating all non-store-related costs, the operating EBITDA of stores greater than 12 months would be INR1,133 million, which translates to a margin of 6.7%. An update on working capital.

Our net working capital for quarter 4 was 53 days. The inventory in our warehouse was 30 days. In Q4, the inventory level of our

first year stores was 112 days. In comparison for our stores older than 12 months, the inventory was 36 days.

There has been a corrigendum and would like to update. In Slide 3 of our investor presentation,

request you to please read operating cash flows as INR4,956 million instead of INR917 million for the full year FY26 and OCF by operating EBITDA ratio at 135.5% instead of 85.2% for the full year FY26. On our diagnostic numbers, Diagnostics revenue for the year is INR1,309.9 million and operating EBITDA is INR196 million. Diagnostics revenue for the current quarter grew to INR347.8 million compared to INR280.8 million in quarter 4 FY25.

Diagnostics segment recorded an operating EBITDA of INR53.1 million, representing 15.3% compared to INR34.3 million in quarter 4 of FY25. In January, we sold 537 gross plans per day. In Feb and March, this was 603 and 554, respectively. As on 31st March, we had around 2 lakhs active plans. Our current observed on-time renewal rate was 21% in quarter 4 versus 23% in quarter 3.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: Thank you very much. We will now begin the question-and-answer session. The first question is from the line of Sanjay from Bastion Research. Please go ahead.

Sanjay: Hi, sir. Thank you for the opportunity and congratulation on a good set of numbers. Sir, my first question would be our number of store closure is continuously on an increasing trend. So how one should view this? And also our franchisee in model store withdrawal is also on uptick. So can you throw some light on this or your input on this?

Sujit Mahato: Sure. Thank you. Thank you for your question. In terms of our store closures, this quarter, we had a few stores also closed down, which relates to the performance. We waited for three-year plus stores and where we felt that we were not able to revise them to the level of the company's expectation, around 26 to 27 stores we closed down. Rest all, we have given the reasons in the investor deck.

Sanjay: Okay. Sir, my another question would be in the previous calls, we have highlighted that we are rearranging our sales due to which our private shares had significantly declined in Q4 FY26.

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So going forward, as our previous guidance of increasing private label by, you know, 20 basis points to 30 basis points every quarter- on-quarter, is that guidance intact? Or how should we view the private label shares going up going forward?

Sujit Mahato: So I think that guidance continues to be intact. What we did for the last two quarters is stabilize the overall growth where we also had to balance out between the branded products as well as the private label sales where we had informed the investors that we were restructuring the incentives. Now we have seen that it's working the way we expect it, and we'll be back to the trajectory where we had promised that every quarter, we can expect that kind of a growth on the private label as well.

Sanjay: Thank you so much. My last question would be the current growth in the revenue and the margin, can we expect in the FY27 as well? Or how should we view that margin and growth revenue growth going forward?

Sujit Mahato: Yeah, as a principle, the company does not give guidance, but there is no reason to believe that we will not continue in the same manner.

Sanjay: Thank you so much, sir. Thank you. All the best. Thank you.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee: Yeah. Thanks for taking my question. The 800 new stores that you have guided for next year, how do you split between your own stores and franchisees? And what was the split for 600-odd stores that you opened this in FY26?

Sujit Mahato: So Saion for the current year, out of the 60

0 around 30-odd stores we have done the franchisee model. Again there, we had guided that we will do an experiment on this. We continue to do that and continue to refine the model.

The second question, the second part is on the next year. As we speak, I don't have a split of that, Saion, but as we progress, maybe we will share that information. But at the moment, it is 800, which includes both the company stores as well as the franchisee outlets.

Saion Mukherjee: Understood. Just to understand the impact of this. So overall, of the 5,000-odd stores you have, the total franchisee would be now just around 300, right? Because this is the first year you had meaningfully added franchisees stores. Is that correct?

Sujit Mahato: So the old ones and the new ones all put together should be in the range of 500 now 500 plus.

Saion Mukherjee: Okay. Okay. So this new addition of franchisees should not be a drag on your profitability? Is that a right assessment because I think on the books...

Sujit Mahato: The original model was that while the company stores continue to grow and we venture into franchisee as a faster growth adjacency, we will be able to, you know, make significant or sufficient margin what we were making with the company-owned stores at the store level. And

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we are in the same, I would say, trajectory or direction. The only additional good point is in our own company-owned stores, at least for the year one, we were making losses and then we were slowly climbing up. But in the franchisee model, right from day one, we are increasing.

Saion Mukherjee: Understood. Sir, one observation that you know, your margins have been increasing. So for stores greater than 12 months, if I call them mature stores, at store level 13% margin, at you know, operating level 6.7%. You also mentioned private label, which sort of got stagnated a bit will start to grow again. From a slightly long and then now you have franchisee also adding so where should we see operating EBITDA margin over the course of the next 3 years, 5 years, if you can help us think elaborate those things will show?

Sujit Mahato: See at least for the next one year, the way we have been discussing internally is we would first try to stabilize this and maybe deliver again the same 5.7% to 5.8% and then keep growing. So the plan is to add more assortment on the private label non-pharma side as well because there we believe the size and traditionally, we have been lower on that count when compared to our other peers and competitors. So that is one area which the company would focus in the current fiscal and going forward that on a rupee absolute basis, how can we keep adding the private label non-pharma, which will definitely add to the top-line as well as to the bottom-line.

Saion Mukherjee: Just one last question, if I can, you know, ask, is there any impact of GLP-1 launches or any comments that you have on that particular market? And anything for MedPlus, which can be a positive here?

Sujit Mahato: We did not track at that level, Saion. Maybe we will do a separate deep dive and maybe we'll come back to you.

Saion Mukherjee: Okay. Thank you.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Sudarshan Agarwal from Axis Capital. Please go ahead.

Sudarshan Agarwal: Yeah. On GLP, how is your cold chain storage kind of, you know, set up? Do you have enough, you know, in terms of warehouses and transport? Do you have that setup already, kind of, built up? Yeah, that would...

Sujit Mahato: So we always had this I think, Sudarshan, we always had that infrastructure in place because we were earlier also handling the injectables, the vaccines and other products.

Sudarshan Agarwal: Yeah.

Sujit Mahato: So there was no surprise element on that. But only thing as I mentioned just now to the other person that I

have not tracked that data separately. But beyond this, I would not be able to comment momentarily.

Sudarshan Agarwal: Got it. And in terms of your store addition plans, right? So you highlighted around 800-odd stores. So a couple of years back also, we had added quite a lot of stores and then we had to add MedPlus Health Services Limited

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warehouses. So where do you see the headroom till where you will have to add more and more warehouses from the current level?

Sujit Mahato: See, it depends again on what we had explained earlier. As we move into newer states, we would for sure need new warehouses. But for this set of 800 stores and if it is predominantly we concentrate in the existing state, then I believe not many additions of warehouse would be required.

We have enough headroom. But as we progress into the existing state of MP and Chhattisgarh where we started filling stores in the previous year, there we may need to plan a few warehouses that will be much lower when compared to the current year additions what we have done.

Sudarshan Agarwal: Got it. And on the accounting side, you had a 30 bps improvement in gross margins this quarter on a Q-on-Q basis. But if I look at your product mix in terms of private label, it has gone slightly down itself. So is there some what is driving the sequential improvement in gross margin?

Sujit Mahato: So absolutely, your observation is right. The other income component, which has gone up in this quarter and also a bump up on the pharma membership income. What we have observed is there are certain contracts with suppliers where we have an arrangement upfront where we can accrue over the years based on the performance.

Sudarshan Agarwal: Yes.

Sujit Mahato: But there are suppliers where there exists no contract, but based on the overall performance of the year when we negotiate with them, and we are able to get some benefit that is only taken again at the end of the quarter, but basis the units sold. So, for example, if we have procured 1 million units during the year and we have got x amount and out of that 60% has already been sold to the extent of 60%, we get the credit in the quarter. And that's been where we have seen some positive benefit, which we had to accrue in the current fiscal year.

Sudarshan Agarwal: Got it. Got it. And one last question. So given that you are kind of exploring this franchisee model, historically, you have followed more of a densification strategy in your 4, 5 states or 7-odd states. But with franchisee model, I believe you may be able to go more into pan-India basis. So should we kind of assume as and when you pick up more franchisees, the expansion in other states will kind of pick up pace?

Sujit Mahato: So I think immediately, we have not listed our any new state, but those states where we just started filling stores in the previous 1 year or 2 years, we would definitely like to expand there to the franchisee road. And we will continue the densification program as you rightly highlighted in the existing states because there's still room to be covered. And as we go to new states, we will inform the market.

Sudarshan Agarwal: Got it, got it. Yeah, that's it from my side. Thanks.

Sujit Mahato: Thank you. Thank you, Sudarshan.

Moderator: Thank you. Next question is from the line of Ahmed Madha from Unifi Capital. Please go ahead.

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Ahmed Madha: Yeah. Thank you for the opportunity. Similar question on the franchisee model. There are a lot of moving parts in the business. I'm assuming, first of all, you'll have certain investments to be made in terms of fixed assets and our partner will be making some fixed asset investment. And then there is inventory component, which probably will be lying on with the franchisee partner already, and you have to account it and maybe clean up some of it.

So can

you give a little more clarity in terms of your franchisee business model, how you are looking at asset turns, fixed asset investments and as well as inventory management. You spoke about the margins part, which is clear that your breakeven time line will be there's no breakeven time line in terms of margins and from first year will be profitable, but balance sheet side is a little unclear.

Sujit Mahato: So when we try setting up a franchisee outlet, as you rightly highlighted, the capex is being invested by the company. So let's say, we spend INR10 Lakhs per store for setting up the complete capex, including the EBITDA. And then on the inventory, the franchisee purchases that inventory from MedPlus on day 1. So that's an outright sale for us.

When we look at margins, the way we are looking at currently, let's say, the gross margin today at the pharmacy level is 24% that is distributed between the Franchisee and MedPlus. Say, MedPlus continues to retain 10% or 9.5% and the franchisee gives 14%, 14.5%. The way it is done is if earlier we were selling a product at INR100, we would now sell that product at INR85, for example, to the franchisee and the franchisee would sell at INR100 and make that INR15 margin. And we would be making that delta margin of 9.5% to 10% when we sell the inventory to the franchisee.

Ahmed Madha: And in terms of existing inventory with the franchisee partner, we don't have any connection with that then? Is that prime...

Sujit Mahato: If it is a new franchisee, the entire inventory is purchased. If it is an old franchisee, depending on case-to-case basis, there are instances we completely acquire the inventory and then completely replenish the entire inventory by a new sale. So those are case-to-case basis where there are conversions from a mom-and-pop to MedPlus outlet. But otherwise, as you rightly said, it is if it's a new store, complete inventory sold by us, there is no other inventory which the franchisees allowed to procure from outside.

Ahmed Madha: Sure. And for the existing stores where you get into a franchisee model, you will be sort of writing-off some inventories, I'm assuming because their way of valuing and our way of valuing may have some difference. Is that a fair way to look at it?

Sujit Mahato: We have not observed any such differences, just to clarify.

Ahmed Madha: Okay. And lastly, in terms of margins, gross margin is pretty clear. It is roughly 9.5, 10 percentage. But then the translation from gross to EBITDA, I'm assuming it will be much higher in franchisee model than the existing model.

Sujit Mahato: So the way to look at it is after the store level EBITDA of 9.5%, 10, there's no other cost associated with it. So that gets accretive to the company's margin directly.

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Ahmed Madha: Okay. Got it. Sure. Thank you so much.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Divyansh Gupta from Latent PMS. Please go ahead.

Divyansh Gupta: Hi, sir. One question on the franchisee, which you just mentioned. So 9% to 10% gross margin that we have is net of supply chain cost or supply chain cost to deliver it to them is separately charged and therefore, the EBITDA becomes, let's say, 6 to 8.

Sujit Mahato: Yes, you are right. It is separately charged, which is currently the model which we present, where we say that after the store level EBITDA there is the PSD, which is the warehouse related and marketing cost and then the corporate and other costs, which gives the company level EBITDA. Similarly, whatever we now articulated is the store level EBITDA in the franchisee model less the supply-related cost of, let's say, 2.5%, 3%. And then the net is the company level EBITDA, which gets added to the company bottom-line.

Divyansh Gupta: Got it. And there is no marketing fee that we charge them because let's say we advertise...

Sujit Mahato: It's all – impact. It's included in it.

Divyansh Gupta: All?

Sujit Mahato: Yes.

Divyansh Gupta: Got it. The second question was in the deck, we have mentioned a round 13 stores of franchisee had closed in this quarter. What would be the reason for them to withdraw?

Sujit Mahato: Various reasons, you know. Some of them are new business entrepreneurs. They get into it with a certain set of expectations. Very soon within the first two months, they understand maybe it's not meeting their expectations. So it is very normal that we will see such closures.

Divyansh Gupta: But the average age that we have mentioned is about almost 2 years.

Sujit Mahato: Yes.

Divyansh Gupta: So I understand 1 or 2 might be, let's say, 2 months experiment done by people and...

Sujit Mahato: Some are also the old model franchisee stores, which have closed down.

Divyansh Gupta: Got it. So for a new franchisee model, have we seen any, let's say, quick closures.

Sujit Mahato: A quick – we have seen. Yes, yes, we have seen.

Divyansh Gupta: Got it. But not let's say, significant to.

Sujit Mahato: No, no, absolutely.

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Divyansh Gupta: Understood. The third question was that I'm a consumer of MedPlus products. So I was looking through the app. So I saw your there is a DBH biscuits and there is non-pharma, non-eatable products. And the manufacturer was mentioned as optimal. So from being a, let's say, a trading company that's typically a pharmacist is, right? From a trading company, we also are becoming a manufacturing company. So what is the thought process of, let's say, manufacturing all of these zones, especially, let's say, here, at least there is no, let's say, regulatory FDA standards that need to be met. So the cost arbitrage will also my assumption would be lesser. So what's the thought process of manufacturing these items by ourselves?

Sujit Mahato: Yes. So some of the items, absolutely, you're right. Wherever possible, we completely get into a full backward integration mode. And in optical also, we have set up, I think if you are referring to the food items of the bakery products, we are doing the own manufacturing only. And some of the non-pharma products also like cleaning liquid, some toilet stuff and other stuff is also being manufactured via our own factory. Wherever we do not have the capacity to manufacture, we do go to the best-in-class contract manufacturers for both pharma and non-pharma and get our stuff done there.

Divyansh Gupta: So is there a, let's say, thought process as you were saying, like non-pharma expansion, there is a whole blue ocean available, blue sky available that we will over a period of time aim to manufacture all non-pharma private label products in-house.

Sujit Mahato: I will not say all, but wherever there is an opportunity, company is completely open to explore that. And as and when we do that, we will keep informing the market.

Divyansh Gupta: Got it. And just last question. The Q1, I think you partially answered, but let's say, the 12-month plus stores margin EBITDA margin is pointing six point some 6.7% versus, let's say, 5.8% for Q4 FY25. So there is no one-off that is leading to a jump here because that way, it's a very steep jump.

Sujit Mahato: Yes, there is no one-off. I can confirm that.

Divyansh Gupta: Got it. Got it. Thank you. I'll join back the queue.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Jitaksh Gupta from Tikri Investments. Please go ahead.

Jitaksh Gupta: Hello. Can you hear me, sir?

Sujit Mahato: Yes. Please.

Jitaksh Gupta: Sir, so my first question is regarding private label. So sir, just wanted to understand like why we are still at 22%, 23% of the total sales. And like what is the near-term target, let's say, for next year or next 2, 3 years for private label?

Sujit Mahato: So the private label has two

two components, both the pharma and the FM CG part. So on the pharma, we have been mindful the way we are growing. On the non-pharma, absolutely, the scope is

unlimited. On the pharma also that way the scope is unlimited. But there it is also linked to the customers' behaviour and their attitude to convert to a private label rather than only the brand. Whereas in the non-pharma, that uptick is very fast, and we have slowly continuously started adding more assortment of products, which is going to help us going forward as well. So that's the only reason. But otherwise, you are right. We do not have a specific number in mind for a target, but continuously, we would focus on that and keep growing.

Jitaksh Gupta: And sir, another part. Like how do we manage unsold inventory under the private label? Do we bear the cost entirely?

Sujit Mahato: Yes, your observation is right. On the private label, the entire inventory risk of expiry and write-off is on our books.

Jitaksh Gupta: Ok, sir. That's it for now.

Moderator: Thank you. Next question is from the line of Akash Shah from Investec. Please go ahead.

Akash Shah: Hi, sir. Good evening and congratulations on the good set of numbers. A couple of questions here. Sir, we saw that the gross and the EBITDA margins for FY26 were at 26% and 8.8%. Just wanted to have a bit of a color on what should be what should we look how should we look at the trajectory going forward? That was my first question.

And on the second question, sir, we noted the SSSG growth of 17.8 % for mature stores, that is stores greater than 12-months. Going ahead, on the for FY27 on an annual basis, where do you see the SSSG rate as well? Thank you.

Sujit Mahato: Thank you. Thank you for your question. On the first question, I think, the first question, what was the reference, sorry?

Akash Shah: It was on gross and EBITDA margin trajectory going forward.

Sujit Mahato: We do not give any specific guidance, but we expect at least to maintain the same level of gross margin. And as and when we see further improvements in the share of private label that should be accretive to our gross margins.

Akash Shah: Okay, sir. And sir, on the SSSG?

Sujit Mahato: On the SSSG, very clearly, we had articulated a couple of quarters ago that we would aim for 9% to 10% on an annualized basis, while we strongly believe that SSSG is not the only metric which we need to track at least for a pharmacy kind of a setup. But from a number perspective, we still believe that we will be able to continue to clock at least 9% to 10% on an annual basis.

Akash Shah: All right, sir. Thank you. Thank you, sir.

Moderator: Thank you. Next question is from the line of Jatin Chawla from RTL Investments. Please go ahead.

Jatin Chawla: Yeah. Hi. Good evening and thanks for the opportunity. My first question is on your capital allocation, how are you thinking about it? Because what I see you are saying is, you have stores which give you fantastic ROCEs and there you are, kind of, letting the franchisee invest. And then on private label kind of non-pharma, you know, side, you are you want to manufacture yourself, where I'm assuming the ROCEs will not be as attractive as your stores. So, you know, how is the thought process kind of doing.

Sujit Mahato: So while ROCE you are right, we'll have to track at the company level and the capital allocation is a dynamic part of that. Capital allocation will continue to be at what we think is where we can do a better job vis-à-vis contract manufacturer. Company would continuously focus on where we can add more value, add more products, bring in more assortment, bring in more reliability to the supply chain. That being said, we are completely very mindful of how we allocate capital so that we get the optimum return

for the shareholders.

Jatin Chawla: Kind of continuation of the same question. A few years back, 2 years, 3 years back, you were adding 800 stores on your own. And at that time, your balance sheet or even the company level ROCE was not as strong as it is today. So why is the thought process of now kind of going aggressively with franchisees versus what you doing it yourself?

Sujit Mahato: So that's the whole reason of your evaluation between adding, let's say, another 20,000 stores where you need to manage additional manpower of, let's say, 25,000 people vis-à-vis expanding aggressively through the franchisee route, getting in micro entrepreneurs who can manage, let's say, 5 stores to 10 stores on their own and thereby continuously growing this by making the MedPlus brand stronger, getting better product offering and making it more and more affordable to the customers.

Jatin Chawla: Got it. Got it. That makes sense. Just one more question. You said your gross margins will expand going forward with as the private label contribution increases, but EBITDA margins

you are looking to largely maintain at the current rate. So are there any specific areas where you are kind of intending to spend that the gross margin improvement will not translate to EBITDA margin improvement?

Sujit Mahato: No, I've not mentioned that. Very mindfully, we'll have to only calibrate that what I mentioned is we would first, you know, ensure or at least target to maintain the current profitability. If you look at our historical numbers, we have continuously improved. So first is we need to stabilize this and then continue to grow and that will be the journey forward.

Jatin Chawla: Got it. Thanks a lot. This quarter was fantastic. Hope to see similar results going forward as well.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Amish Kanani from Knowise Investment Managers. Please go ahead.

Amish Kanani: Sir, congrats on a good operating set of numbers. Sir, partly my question on asset allocation is, you know, addressed. But then the question is, sir with free cash flow coming in since last two years, net of lease payment liability as well. You know, what are your thoughts – you

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know, about kind of sharing this via dividend, which is one of the strongest signals that company doesn't need the capital and we are generating the free cash flow. So one, why are we not still there? That's question number one.

I understand you will have a lot of growth aspiration and growth capital. You know, we are making decent ROCE, but just your thoughts there, sir, as a signaling. And second, sir you know, given our omnichannel, you did mention about how we can do a better job vis-à-vis only online kind of a store.

The question is expanding our network in Tier 3 where maybe online may not make a lot of sense versus growing growth through online. If you can give us some sense and flavour of how we are planning to grow existing store online and going Tier 2, Tier 3, which obviously we can do both. But where is the growth prospects much better and much faster? Hello. Am I audible? Hello. Yeah, hi. Am I audible?

Sujit Mahato: Yes, please. Can you please repeat the question?

Amish Kanani: Yes. So I'll just quickly summarize my question. Sir, partly the growth, you know, the capital asset allocation on own store versus franchisee you have addressed. The question is then, sir, with free cash flow coming even after payment of lease liability, how about giving a dividend signal, which is one of the strongest we as an investor believe is a good signal that even after working capital and repayment and capex need, we are generating free cash flow and there are no major debts to take care because there are lease liabilities, which you know at least Ind AS shows it as debt. How about thinking on those lines, which we

have not yet kind of probably taken action?

And second, sir, quickly on online growth through the existing channels versus growing in Tier 2, Tier 3 towns where maybe online may not be probably so strong, we'll do maybe focus on more store level thing. Where is the growth more coming from getting more out of online in the existing store versus going in Tier 2, Tier 3, sir?

Sujit Mahato: Thank you. Thank you for the question. So on the dividend, for sure, we have not yet reached that stage is what we believe, but I'll take your question. We'll discuss it internally. The current juncture at which the company stands that there is huge headroom for the company to grow and invest.

So we have not taken that call yet, but I'll take your question back and discuss with the senior management and the Board. The second on the online growth versus the physical growth. For us, we continue to grow more in the Tier 2, Tier 3, if I can answer you.

Amish Kanani: Okay. And sir, just to understand the online thing, you know, in slightly more detail in slightly more flavour, how is it going? Is it something which is just an additional? How is it as a percentage of existing store level economics? How is it adding some flavour..

Sujit Mahato: So for us, online is not a direct focus or a channel where we drive sales, but it's been there for the company since 2014 as an additional convenience to the customers where the customers can order online, pick up while going back home, going from office, they pick up from our stores

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and some of them really take the full benefit of ordering online and getting them delivered at the home. It is five-plus percent of our current total sales.

Amish Kanani: Okay. Got it. So two-hour delivery is more like giving a comfort that you will get the thing that you want, if not immediately, then maybe within two hours or something like that?

Sujit Mahato: Absolutely, absolutely. Where our stores are physically available, online service is available. So

that's how we operate online. Where we do not have in a particular pin code, let's say, there is \ or in a particular city where we do not have a physical presence, we do not offer online.

Amish Kanani: Sure, sir. All the best and thank you.

Sujit Mahato: Thank you.

Moderator: Thank you . Next question is from the line of Raman KV from Sequent Investme nts. Please go ahead.

Raman KV: Hello, sir. Can you hear me?

Sujit Mahato: Yes, please.

Raman KV: Yeah. I will just two questions one just with respect to private label. In the ope ning commentary now you mentioned that you have achieved a good mix between branded and private label growth and you are looking to focus in the coming year on the private label products. So can we expect the initial guidance of growing your private label portfoli o, which is both your pharmacy and non-pharmacy 1% every quarter?

Sujit Mahato: No, we do not give such kind of guidance. But yes, as we grow, we had earlier indicated that for every 0.3%, we will have a 0.1% impact on the gross margin. For that, I said, yes, that guidance still remains. But we do not give guidance on how much we will grow because it has a couple of dynamic factors, one being the customers' behaviour ; and two, the company's efforts in getting more and more assortment and portfolio of products, both on the pharm a as well as on the non-pharma. As we continue to progress, we will keep updating the market. But as such, we do not provide any guidance.

Raman KV: Okay. Understood. Just a follow-up on this. So about two quarters ba ck, you were like you are planning to focus on overall growth of the on the stores over the gr owing of only private label, aggressively expanding into private label products. So how is the na rrative right now for the management? Now also are you sticking towards the overall balance growth? Or are you plannin

g to grow aggressively for the Private label.

Sujit Mahato: No, we will continue to be very balanced in terms of growth because we are being a multi-brand retail outlet to continue to offer the full scale of branded products. So we do not want a customer who walks in and ask for a particular brand be it on the pharma or non-pharma to say that we do not have that. So that behaviour is what the company worke d at in the last couple of quarters.

And I think we will continue that journey saying that private label will a lso grow, but will

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continue to be completely focused and overall growth makes more sense rather than only private label because that's been what we have learned in the last couple of ye ars.

Raman KV: Understood, sir. Thank you.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Aejas Lakhani from Unifi AMC. Please go ah ead.

Aejas Lakhani: Yeah, hi. Sujit, congratulations on a strong set. Sujit, firstly, I wanted to understan d that the SSSG that we have seen this quarter has been probably the strongest in 12, 13 quarters. So is there anything that you would have done that you can call ou t? Because in the middle, you guys have tweaked the incentives at the store level to incentivize the employ ees to sell private label, then you had rolled that back a little bit. So any color here ?

Sujit Mahato: Aejas, I think while the numbers speak for themselves, we shared our views with yo u earlier as well and with all other participants that really SSSG is not the number which we track. But yes, if you see the consequences of certain initiatives which the company has been pursuing, yes, we have seen numbers in this range. And that's the reason on an annu al basis when other participants has asked me what we said at best is what we guided earlier also is 9% t o 10% when the numbers were very low, if you remember that a few quarters ago.

So I think we did not do anything special, if I can answer you that. What we made in particular ensured is the availability of the full assortment of non-pharma prod ucts and pharma products wherever possible. And we saw a good uptick in the FMCG products as well, which helped us on the overall number.

Aejas Lakhani: Understood. Second, Sujit, could you just call out about is it pos sible that you can give the franchisee revenues that you have booked for the year? And go ing forward, is it possible to give the breakdown of the store addition owned stores and franchisee stores?

Sujit Mahato: I'll share with that maybe after this call because I don't have it hand y. But yes, once we reach a critical mass, we will again reevaluate what amount of subdivisonal data we c an share absolutely.

Aejas Lakhani: Understood. And thirdly, Sujit, in terms of you spoke at the start of the call about your desires to improve the PL non-pharma bit. So could you just tell me on thi s front, three elements. One is what categories?

Second is that any visit to your existing stores feels that there is no incremental space available for storage. So if there is a few of more PL non-pharma products, how do you intend to create incremental store space to manage some of this? And third is the margin p rofile for typically the

PL non-pharma, is it could you just give some colour about how it is compared to PL pharma?

Sujit Mahato: Sure. So in terms of the categories across every category in a retail store, we are relooking at the assortment and wherever possible, either we are bringing in more branded stuff or we are parallelly evaluating and adding more and more private label new products, be it in edible oils, any health sector, any health-related food supplements. So that is on e.

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Second, what you rightly observed, there are stores where our space is constrained, but there are many other stores where

we strongly believe that there's enough room to add more and more products to the same footing. And that's where I think even during the current year, we have a project to, I would say, upgrade or modernize at least 600-plus stores to accommodate more and more. So that is smart way of storing the rack system and the pharma and non-pharma demarcation right which can be serviced from behind the counter and which can be serviced or left out in the open. So those are the certain stuff we are again carrying out this year. And the last one, what you mentioned I missed your point, what was the questions.

Aejas Lakhani: Sir, margins.

Sujit Mahato: Yeah. In terms of the margin profile, for sure, the private label pharma commands a much higher margin. The private label non-pharma margins are in the range of 23% to 25% at the gross margin level, which are significantly better than the non-pharma branded products, which are in the range of maybe 9% to 10%.

Aejas Lakhani: Understood. And broadly, just one last thing is that has there been any broad change in your acute and chronic mix, say, over the last 2 years or 3 years?

Sujit Mahato: We have not seen that change yet, Aejas.

Aejas Lakhani: Understood. Thanks, and all the best.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Divyansh Gupta from Latent PMS. Please go ahead.

Divyansh Gupta: Hi, sir. Thank you for the follow-up. First question is the inventory days that you mentioned in our slide deck, right, for more than 12 months and less than 12 months. Just a formula check.

The inventory is what is in our balance sheet, but does the sale or the COGS is only for the COCO store or it includes franchisee or how should we understand?

Sujit Mahato: The inventory on our balance sheet only relates to COCO stores. Franchisee the date is sold, it is their inventory.

Divyansh Gupta: So when we compute the number of days in the denominator, do we like-to-like inventory that COCO but the denominator...

Sujit Mahato: In COCO stores. You are right. Here, only COCO stores numbers have been taken on both sides, just to clarify.

Divyansh Gupta: Got it. That makes that is helpful. Second is you mentioned that, let's say, earlier when we were discussing that 9% to 10% is what we get as gross margin when we do a franchisee sale. Right now, let's say, 500 stores are there. And if I just take 300 that were done last year and add another 300 for next year. So about 800 stores out of 6,000-odd so some stores becomes franchisee stores.

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Now the question is franchisee margin is 9% to 10% for us, whereas COCO will be at 24%. Now as this margin for as a percentage of franchisee store increase, the margin should come down, whereas you are expecting the margin to remain similar. So does it mean that the goodness from private label increase more or less gets sort of nullified by the increase in franchisee at least at the gross margin level.

Sujit Mahato: So the just only one point I would like to add so that it is apple-to-apple. What you rightly mentioned is gross margin currently is 24% and the store level EBITDA is at the company level, 11%, right? That level you should compare with 9.5%, 10%. And as the percentage of private label also increases in my franchisee outlets, this will also continue to improve. That's what we meant.

Divyansh Gupta: Got it. So you're saying EBITDA will remain same. Maybe the gross margin might show up lesser, but you are saving on other cost?

Sujit Mahato: Yes.

Divyansh Gupta: Got it. And the last question, GLP products have been launched on our, let's say, in our stores. What would be the revenue that, let's say, contributed in this quarter? I was not there in the opening slide if you had mentioned it.

Sujit Mahato: No, I had not mentioned. I said we are not tracking that immediately. But yes, from an available

liability all the products of all the brands are available at our store s.

Divyansh Gupta: Got it. And is there a time line for store generic of these products? Or right now, it's going to be branded generics only?

Sujit Mahato: As of now, we have not launched any store level for the GLP-1. But as and when we launch, we will surely inform the market.

Divyansh Gupta: Got it. Got it. Thank you. Thank you, sir.

Sujit Mahato: Thank you.

Moderator: Thank you. Next question is from the line of Rickanjeet Singh, an Individual Investor. Please go ahead.

Rickanjeet Singh: Hello. Am I audible, sir?

Moderator: Yeah.

Sujit Mahato: Yes, you are audible?

Rickanjeet Singh: Sir, my question is this, sir. We have FY26, we have paid an interest of INR120 crores, sir. That is a loan of INR1600 crores, sir. What should be the management's thinking about that? Should it be reduced or not?

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Sujit Mahato: So this interest that you are seeing in P&L, so this is all related to lease accounting. And there is no external loan in our book.

Rickanjeet Singh: Okay.

Sujit Mahato: So that is also being shown under lease liability Ind AS. So, there are no actual loans in the books. It's a zero-debt company. Yes, please say.

Rickanjeet Singh: So this is interest of INR120 crore?

Sujit Mahato: It is all about lease accounting where the lease rental is subdivided into interest and depreciation.

Rickanjeet Singh: Okay. Understood, sir. Going forward will the margin be in double-digit or how it will be, if you can give any guidance?

Sujit Mahato: So we don't provide guidance, as explained earlier, what we will ensure is to first maintain the current level of profitability and then continue to grow.

Rickanjeet Singh: Okay, sir. Thank you, sir. Thank you.

Sujit Mahato: Thank you. Thank you.

Moderator: Thank you. Next question is from the line of Govindarajan Chellappa from CSIM. Please go ahead/

Govindarajan Chellappa: Yeah, Hi. I just had one question. Your warehouse inventory has been continuously improving. What is the sustainable level of warehouse inventory, 30 days is what you reported last quarter.

That seems fairly, you know, industry best.

Sujit Mahato: I think, Govind this would be in the range of 30 days to 33 days. We have been dabbling, but we have not cut out on any tail brands as such. But at a point in time when private label really takes-off, we will relook at the entire SKUs, which we are carrying. For example, today, as you are aware, for every molecule, there are at least 200-plus brands available and depending on the physical locations of our stores, we try to fulfil the demands of those particular pin codes for the same molecules which are generated there.

And therefore, we are also forced to maintain all the tail brands plus the MedPlus brand. But a point in time, if, let's say, MedPlus reaches 40-plus percent, we would definitely relook at the tail brands and maybe reduce a few to reduce this number, which can help us also on the overall working capital. But for now, we think we will continue with the same level.

Govindarajan Chellappa: Okay. Sorry, if I could squeeze in one more question. Typically, what is the percentage of returns that you give back to pharma companies either because of expiries or damages? And what is the how long is it does it sit as inventory on your books? And how long does the payment come does it take for the payment to come back from the pharma companies?

Sujit Mahato: So generally, it is done only on expiries, returns to the pharma companies, especially on the branded products. And it's I would say, less than a month, we get reimbursed up to 95%.

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Govindarajan Chellappa: Okay. And you know, typically, how long is the inventory holding period for you? And what proportion of either value of sales or

whatever is typically return? I mean would it be 5%, 10%?

Would it be higher?

Sujit Mahato: It would be less than 1%, Govind.

Govindarajan Chellappa: It's less than 1%?

Sujit Mahato: Yes.

Govindarajan Chellappa: Okay, thank you.

Sujit Mahato: Thank you.

Moderator: Thank you. Ladies and gentlemen, we will take that as the last question for today. I would now

like to hand the conference over to Mr. Sujit Mahato for closing comments. Over to you, sir.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you, and have a good day.

Moderator: Thank you very much. On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you all for joining us today and you may now disconnect your lines.

Call: Jul 2026

MedPlus Health Services Limited

040-6724 6724

Regd. off. H. No: 11-6-56, Survey No: 257 & 258/1, Opp: IDPL Railway Siding Road, Moosapet, Kukatpally, Hyderabad – 500037, Telangana, India

CIN No: L85110TG2006PLC051845 | Website: www.medplusindia.com | Email: medplus@medplusindia.com

July 24, 2026

The Listing Department
BSE Limited
Phiroze Jeejeebhoy Towers
Dalal Street, Fort,
Mumbai 400 001
BSE Scrip Code: 543427 The Listing Department
National Stock Exchange of India Limited
Exchange Plaza,
Bandra Kurla Complex,
Bandra (East), Mumbai – 400 051
NSE Symbol : MEDPLUS

Dear Sir/ Madam,

Sub: Submission of Transcript of Earnings Call:

Pursuant to Regulation 30 and 47 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, we enclose herewith transcript of the Earnings Call held with analyst/ institutional investors on July 22, 2026 for the quarter ended June 30, 2026.

The same will be available on the website of the Company at www.medplusindia.com and also on the websites of BSE Limited and National Stock Exchange of India Ltd. viz. www.bseindia.com and www.nseindia.com respectively.

Thanking You
Yours faithfully

For MedPlus Health Services Limited

Shrenik Soni
Company Secretary & Compliance Officer
(M. No. F12400)

Encl: a/a

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"MedPlus Health Services Limited
Q1 FY27 Earnings Conference Call "
July 22, 2026

Management: Mr. Madhukar Gangadi- Managing Director and Chief Executive Officer
Mr. Sujit Kumar Mahato - Chief Financial Officer
Mr. D R N Srinivas - Sr. Manager Finance

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Moderator: Ladies and gentlemen, good day, and welcome to the MedPlus Health Services Limited Q1 FY27 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touchtone phone. Please note that this conference is being recorded.

I would now like to hand the conference over to Mr. Srinivas. Thank you, and over to you, sir.

Srinivas: Thank you, Manav. Good evening, everyone. On behalf of MedPlus, it's my utmost pleasure to welcome you all to the MedPlus Q1 FY27 Earnings Conference Call to discuss the financial results of MedPlus for the first quarter of FY27, which was announced earlier.

We have with us today the senior management represented by Mr. Madhukar Reddy Gangadi Gangadi, CEO and MD; and Mr. Sujit Mahato, CFO. Before we begin, I would like to mention that some of the statements made in today's discussion may be forward-looking in nature and may involve risks and uncertainties.

Please note the disclaimer mentioning these risks and uncertainties on Slide 1 of the Investor Presentation shared with all of you earlier. Documents relating to our financial performance were circulated earlier, and these have also been posted on our corporate website.

I would now hand over the call to Sujit. Thank you, and over to you, Sujit.

Sujit Mahato: Thank you, Srinivas, and good evening, everyone, on this call. An update on our store network, openings and closures. We have opened 222 stores at a gross level, and there were 52 store closures, including 9 relocation cases and 27 franchisee outlet closures.

Additionally, 24 stores are in the process of conversion from company-owned company-operated stores to the franchise model. We achieved a net addition of 146 stores during the current quarter compared to the 218 stores added during the last quarter. We continue with the outlook for adding 800 net new stores, including the franchisee stores in FY27.

In terms of our network age around 27% of our stores have been operational for less than 2 years and the remaining 73% of our stores have been operational for 2 years or more. In terms of our network and the store size, at the end of the quarter, our network grew to 5,476 stores with 2.9 million plus square feet compared to 4,813 stores and 2.5 million plus square feet at the end of June '25.

The average store size is in the range of 539 square feet. Update on the revenue mix. Presently, MedPlus offers a large range of SKUs spanning across pharmaceutical and non-pharmaceutical categories. Private label sales for Q1 FY27 constitutes 20%, pharma being 10.7% and the non-pharma being 9.3% of our total revenue. Update on the financial numbers. Our consolidated revenue for the quarter is INR18,796 million. Our consolidated operating EBITDA for the quarter stood at INR651 million, representing 3.5%.

Revenue from pharmacy operations grew by 21.8% Y-o-Y on a reported basis. The pharmacy operating EBITDA stood at INR588 million, representing 3.2%. An update on our store

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performance, stores older than 12 months. Revenue from these stores in quarter 1 was INR16,436 million, representing 94% of pharmacy revenue.

These stores had a store level EBITDA margin of 10.4%. A word here on the store level EBITDA margin by age, while stores greater than 12 months had a margin of 10.4%. This was 10.6% for stores greater than 24 months and 6.9% for stores in the 13 to 24 months age bracket.

On allocating all non-store-related costs, the operating EBITDA of stores greater than 12 months would be INR707 million, which translates to a margin of 4.3%. An update on the working

capital.

Our net working capital for Q1 was 54 days. Inventory in our warehouse represented 33 days. In quarter 1, the inventory level of our first-year stores was 100 days. In comparison for our stores older than 12 months, the inventory was at 36 days.

An update on our diagnostic numbers. Diagnostics revenue for the current quarter grew to INR370.8 million compared to INR302.9 million in quarter 1 FY26. Diagnostics segment

recorded an operating EBITDA of INR65.9 million compared to INR41.3 million in quarter 1 FY26.

In April, we sold 552 plans per day. In May and June, this was 557 and 644 plants respectively. As at end of June, we had around 2 lakh active clients. Additionally, we would like to inform that the capex proposals as approved by the Board has been put on hold by the company. We would evaluate and later inform, update the market on the utilization of the fund on the balance sheet.

That concludes our update for the quarter. I request the host to open the line for questions.

Moderator: We have our first question from line of Sudarshan Agarwal from Axis Capital.

Sudarshan Agarwal: I think you will have a lot of questions on this. But as you said, you have put the capex plans on hold. But just want to understand the rationale behind kind of putting that strategy forth and getting it approved. So you have INR 40 crores of capex for a food park and oil extraction unit and another one for the wellness services facility.

So just want to understand what are we thinking in terms of strategically going ahead with these plans that we have approved? Yes, we have put it on hold, but we have more insights into how the company is thinking on investing in these aspects going ahead? That would be my first question.

Madhukar Gangadi: Sudarshan, I'm happy to take you through the whole strategy if Sujit has already not announced that we are now putting it on hold. I don't think there's any point in discussing it, honestly. There was a thought and I thought that backward integration would really help, especially in areas where the products are new, supplies are not easy to find. And sometimes it is just easier to own the supply chain and all.

So but we have on further deliberation, decided that we'll probably put it on hold definitely put it on hold for now and come out with a plan on how we'll actually utilize the funds. So...

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Sudarshan Agarwal: This would be related to the food park. I guess you will be putting on hold the other one as well? Or is that investment continue?

Madhukar Gangadi: All capex spend for now, if we were to basically come back and yes, all capex, meaning the 2 main things, the food park and this one, those are the concierge plans.

Sudarshan Agarwal: So on the food park, I understood your thought process. On the wellness, I mean, this is something that we would we were thinking of trying as a new kind of foray, right, like we did with diagnostics, what I would kind of...

Madhukar Gangadi: Okay. I'll tell you what my thoughts were on that. But again, I have to reemphasize the fact that we are not putting it on hold. So, it probably does not is not relevant. But anyway, the thought was that all companies today are looking at diverse revenue streams in adjacent areas.

You can't just basically do just a pharmacy and. I know there's a lot of opportunity. But even as a vertical grows, you also sometimes have to bring in some horizontal areas also. We thought that, that was a good area to go in. Prevention was a great area to go in. Concierge medicine, wellness, prevention, longevity and all kind of go hand in hand, and we thought that is the logical next place for us to go given that we're already doing diagnostics.

But diagnostics has one piece left, which is when people come to a diagnostic center and they get themselves tested, since we are not offering a complete, let

's say, we're not closing the loop

on it. We're not telling them what he has to do. And a lot of times he ends up going to a doctor and doctors for reasons best known to them, will disregard any kind of lab, which brings a result which is not in their anyway just not marketed to them. Let me put it that way.

Right. So, we wanted to own that entire thing and basically do a complete out-of-hospital health care kind of stuff out of hospital. But anyway, that we have now put on hold. We're not going to do it. We will think about it. It's unlikely now we'll basically think about it in MedPlus, I would say.

Sudarshan Agarwal: Okay. Okay. Got it. Coming to your performance in the quarter. Yes, strong top line growth. But in terms of margins, we have seen yet again the private label share contracting in Q1. We had this issue last year. So can you just give some sense on what led to this drop? And how should we think of this private label share moving in quarters ahead?

Madhukar Gangadi: So for us, private label when we started off, we pushed very hard, and we made sure that everyone was aware of it and the employees are also heavily incentivized to sell it and all. So I think the early adopters very quickly shifted. And after that, maybe our employees would have gone a little overboard in trying to push it to every single customer out there.

And in the process, we may have basically, I wouldn't say annoyed, but at least put up some customers out there. So we try to put it back. And so now the stores are basically focused on see, while as a percentage, it has grown and degrown slightly, year-on-year, the growth is still there. Every single molecule which we have been selling, we're selling more of it. We continue to grow. As a percentage, it has degrown slightly because our overall sales has grown.

And we are now focused on making sure that the customer is completely served, whether it is for brand or whether he is looking for, let's say, slightly more affordable kind of medicine. So we offer it. If he takes it, great. If not, that's okay. But then what that has allowed us to do is allowed us to actually grow the customer base in the branded side. And that's why you're seeing a small, let's say, dip in the overall percentage.

Sudarshan Agarwal: Got it. And how should the share move?

Madhukar Gangadi: Yes. Going forward, it's going to be a function of two things. One, a little bit of our own advertisement outside the store, which is through influencers to other various other sources to bring people in, who are otherwise not coming into the store for seeking out the MedPlus equivalents of all the branded drugs. So that is how we plan to actually grow that whole share. And we'll also obviously try and get our employees to be slightly better salespeople, get the whole message out. So a combination of both making our employee slightly more skilled at selling private label and two, bringing in people who are otherwise not shopping with MedPlus at all to come into the store for the drugs. So that's the plan for us.

We think we will, with this quarter, be arresting the slight, I would say, degradation which we have had and from there on, start building up again at the rate of around 0.5% or 0.3%, 0.4% kind of stuff.

Sudarshan Agarwal: Okay. Okay. Got it. And a part of your gross margin was also affected by franchisees, right, 50 basis points. So can you just broadly give me a sense, private label, of course, will gradually come up, but this franchisee dilution will be part of our numbers going ahead on the gross margin side, right, at least for the near term as long as they don't scale up?

Madhukar Gangadi: Yes. So let me explain a little bit. Franchisee is still in an experimental kind of stage. We believe that India is a market where you have 20,000, 30,000, 40,000 stores. And we believe that long

term, that's the only way for us to get to that number.

So for us, we are working on that right now. One of the main things so for us, the way we see franchisees, that's a logical way for us to grow. Whatever margin we are getting is additional margin, yes, it will definitely be slightly lower than our regular store, but the thing is the return on investment on that is going to be higher. And once we track the model, we'll be able to grow way faster. That's how we see it.

Moderator: We have our next question from the line of Saion Mukherjee from Nomura Securities.

Saion Mukherjee: Sir, on this capex plan, you would have had some reasons to sort of plan for this, would have thought about it over many months on this. You got a Board approval. What has happened in the last, let's say, less than a few hours that you've decided to put it on hold?

Madhukar Gangadi: Difficult to say, Saion. Maybe it is the wisdom of the masses, I guess. But see, for us, we definitely are, let us say, going to be let's say, sensitive to the stock market for sure. We're not going to while I believe it's a good thing to do, create adjacency is the way the whole world is growing out there.

And that is what the Board also felt. But the fact that the market reacted so strongly and the fact that a bunch of our investors have also said the same thing and all, we feel that maybe we are not 100% right. So happy to put it I would say happy to put it on hold, but yes, we have taken the feedback, and we want to put it on hold.

Saion Mukherjee: Right. So sir, does that mean that we should not expect any sort of big capex plan in years ahead? I mean how do you plan to then sort of do this because public share holders could react the way they would. So how would you have that process in place to ensure that any capex decision, etcetera, in the future sort of doesn't have this kind of a fit?

Madhukar Gangadi: So I guess, public markets have not really said no to any kind of capex on our warehousing on the stores or anything like that. So I guess as far as long as we are focused on core business, I really don't see anyone cutting out with anything. And even if they did, then we wouldn't really give a call.

So core business will continue irrespective of what the market thinks or says, whether it's private label or regular stores and all that. So that will happen. Can we basically take a pause at this time on capex spending? Absolutely. But is that something which we will do continually? No, as long as it is and definitely not if it is about the core business.

Saion Mukherjee: Okay. Sir, coming to the core business, you have shared your annual operating plan. So based on your quarter 1 performance, how does this plan change? You had a INR400-odd crores of operating EBITDA on a consol basis. So what is your estimate now? Does that remain same or it undergoes a change based on quarter 1 number?

Sujit Mahato: This is Sujit here. So 2 things. One, maybe you're referring to the Board that which was by mistake uploaded, but the data remains as it is what you have rightly pointed out. And for the time being, numbers do not change because as Madhukar explained, we are working on getting back the private label back on track. We are working on certain costs, which we had not anticipated, which has got hit during the current quarter.

For example, effective 7 July, the company has already, I would say, tweaked the discount structure for all sales greater than 1,000 where the company gives 20% discount. We have already reduced to 19% for a large number of customers. We see that traction. And therefore, if there is any such other adjustments required so that we keep the annual operating plan as a basis and a guide, I would not say we are bound by that. We can either exceed that as well. So on top line, you would have seen we have achieved 99.9%. On

the operating EBITDA, there

is a gap to be covered. That's been the nature of Q1 as well. But we are pretty confident that coming quarters, we will be able to make that upside.

Saion Mukherjee: Okay. And sir, one last question on your private label traction. So the growth has been weak on a year-on-year basis. though there is some growth. So in terms of the number of people who are subscribed to this private label subscription, what is that number as an absolute number of people, like how has that changed on a quarter-on-quarter on a year-on-year basis?

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Sujit Mahato: Actively, there are close to 44 lakh, 45 lakh people who are on this plan. And every month, there is an annual pattern. Last quarter, if I remember the number right, we added 10 lakh plans, including renewals. The good part is, earlier we were realizing INR99. We have now increased that to INR149. So on membership alone, we should see a INR10 crore to INR11 crores impact on the top line for the whole year, which will flow through to the bottom line.

Saion Mukherjee: Okay. So today, how many you have 44 lakh, 45 lakh people on the plan as of June?

Sujit Mahato: Yes. Yes.

Saion Mukherjee: And how much was this number in the, say, the June of 2025 compared to...

Sujit Mahato: I don't remember it exactly, but it was in the range of INR39 lakhs to INR40 lakhs.

Saion Mukherjee: Okay. So around 40 lakhs has gone to 44 lakhs kind of a number.

Madhukar Gangadi: And not only that, we also actually increased the amount

Sujit Mahato: Yes. INR99 to INR149.

Madhukar Gangadi: Despite that it is gone up

Sujit Mahato: It has been lot of renewals going forward at INR149.

Saion Mukherjee: Okay. So this was implemented recently INR149?

Sujit Mahato: Yes, a month ago.

Saion Mukherjee: Month ago. Okay. And sir, one last question. I've seen your employee expenses also have gone up. So is there any impact because you had earlier mentioned about a lot of attrition and all of that. So as the pharmacy level employee compensations, those have gone up significantly?

Anything on that side, if you want to highlight?

Sujit Mahato: So on a year-on-year basis, the major impact is coming from people which we had added in the various warehouses. So we are getting the full quarter impact now. Additionally, we articulated on the labor force changes, especially in Karnataka and Telangana, where the increase has been more than, I would say, significantly more than the average of generally 4%, it was to be.

We have seen 60% increase in Karnataka and 25% plus in Telangana. We have factored that.

However, these were effective 1st June, which means going forward, in the Q1 quarter, we have the impact of only 1 month. Going forward, we will have the impact of the full quarter. We are taking steps to mitigate that in terms of, one, the retention bonus plan, which we had has been stopped effective 1st of March, which means there are no new additions because by design, the wages have gone up.

Two, we are also looking at the non-statutory payments, for example, the private label incentives and other incentives, how we can optimize and structure that in a manner to get a relief. And

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three, as we just articulated on the private label mix, that should also help us in addressing this cost trend.

Moderator: We have a next question from the line of Divyansh Gupta from Latent PMS.

Divyansh Gupta: First question on the private non-pharma products, the absolute revenue dipped from March to June. So just wanted to understand why would that happen? Is there a supply issue or a demand issue? Or is it just purely seasonality?

Sujit Mahato: So yes, absolutely, you pointed out right. From quarter 4 to quarter 1, which is a sequential quarter, we had an impact on the margin of around 100 basis points because of the

private label

mix, both on pharma and non-pharma. And non-pharma, we had a steeper fall from the last quarter.

For certain categories of our non-pharma products, especially the diaper range, there were disruptions in terms of supply. It's now coming back on track. By the end of this current quarter, we should see the full impact coming back. So we are very confident on the non-pharma contribution to sales, and we expect that to further grow in the coming quarters.

Divyansh Gupta: Understood. Understood. The second question was on the franchisees. You mentioned that 24 stores are moving from COCO to franchisee. So is it our staff there, which is taking it up or someone else is? I'm just trying to understand...

Sujit Mahato: Predominantly, it is the offering to the senior staff members who have been with the company for 3 years plus. So that's a retention strategy. And so that they also become individual entrepreneurs. They have learned with the company, they have been with the company. And as a model, we said that's the only way we can facilitate them to become individual entrepreneurs. And I think more than 95% of these stores will be taken up by the existing employees.

Divyansh Gupta: Got it. And the other question was the franchise stores, which were closed during the quarter. The average age was only 0.7 years.

Sujit Mahato: Yes.

Divyansh Gupta: Which is like 8 months. The previous quarter, you had mentioned it was the closure was of the older franchisee networks, which, therefore, had a higher age. Why would this be? Is it I now understand that franchisee is experimental, but any signals from the market that the franchisee model is not necessarily working out for them, and we will tweak to make it much better?

Sujit Mahato: You're absolutely right. That's the same observation we are having that look, as we speak, we have 600-plus franchisees. However, there is in some cases, what we are observing also is there is an expectation mismatch both from the franchisees who are first-time entrepreneurs, they get into this.

Their expectation is X, they'll end up getting Y. And very quickly, they are taking decisions, which we have to honor them. So that's what we are reporting that this is again, this will have to MedPlus Health Services Limited

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take some time to stabilize from both. One is from our end in tweaking the model. As we speak, we continue to look at this model to make it a win-win model.

We have made certain adjustments, for example, in helping the franchisees reach to a breakeven in a much faster pace, helping them with supporting them with fees in the first year. And therefore, we will have certain cases where it still does not double up to their expectations. And this is only an outcome of that.

Divyansh Gupta: And what were these expectations, just the ramp-up of it or like we were expecting that we will do more private label?

Sujit Mahato: It's a closure by the franchiser. And therefore, we are not directly involved. We do respect when they come back and say that, look, we did a mistake. It's not meeting our expectations. The numbers do not tie up. So we would like to go back.

In 99% of the case, we do relook at that and maybe help them with the exit or we find another franchisee who is ready to take up that unit and give them an exit. So this will continue to happen, Divyansh.

Divyansh Gupta: Got it. And sir, 2 more questions. Average store size you mentioned is 539 or 529.

Sujit Mahato: 539.

Divyansh Gupta: Given that, let's say, last quarter, it was more 520-something range. Is it fair to assume that the new stores are much, much larger in size?

Sujit Mahato: So there are as a strategy, we have been opening some large store formats because as a strategy, we would like to even test out that, which means that it is as good as 3 stores coming up as 1 store.

And then you can put in more people, more

focus, much better fill rate and a larger assortment

of both the brand as well as the private label offering to the customers. So that's one reason why you see an uptick in the average SFT per store.

Divyansh Gupta: But then that limits us penetration into like we'll be the neighborhood pharmacy.

Sujit Mahato: We will continue to do that, Divyansh. This is a very small portion. I think as we speak, around 47 to 48 stores have been launched under this format. So when compared to the 6,000-odd, this is a very small number, which also helps the company in garnering, I would say, local attention instead of spending good money for advertising.

Divyansh Gupta: Understood. So sort of a marketing, understood. And sir, the last question. You mentioned that, let's say, because of all the minimum wage hikes and everything, we are tweaking the we are not allowing any new sign-up into the retention plan that we had designed.

But then how does it address the attrition problem due to which we had launched the retention program in the first because if the minimum wages have gone up and let's say, they don't have

Sujit Mahato: I understand your question. I'll try to address that. The underlying issue itself was lower wages, especially when compared to employees in the quick commerce business. And now since the minimum wages have gone up, we expect and definitely, we would very closely monitor this that much better kind of employees will be available for this higher range of wages.

And therefore, sense should prevail, and we will closely monitor this how it works. And if something is required, this is again a non-statutory area which the company can very quickly address it if there is need in each of these pockets wherever possible.

So what we wanted to inform you is, one, there is no knee-jerk reaction, only continuously only the new guy from 1st March, if this plan is no more available for them. But for colleagues and employees who are already part of this plan, it continues. So whatever amounts have been accrued to them as on date will be absolutely paid by the company or honored by the company as and when time comes.

So therefore, we do not see a higher risk. But as you rightly pointed out, we continue to monitor this space and we will look at what interventions along with our HR, we need to do to attract and retain such talent.

Moderator: We have our next question from the line of Jasdeep Walia from Clockvine Capital.

Jasdeep Walia: That diagnostics business has been scaling up well. So what are your thoughts on scaling it further by way of either densification in the same market or maybe expanding into other markets?

Madhukar Gangadi: See, while it has been doing reasonably well and it is profitable also, it has not really gone to the level we wanted. That is a model in which the subscription plans should have come both from the B2B and B2C side. We have not had much success in the B2B and B2C also has languished around 220,000 or 210,000 kind of members.

By now, we should have had anywhere between 250,000 to 300,000 members. So for that reason, while we may do a little bit of maintenance kind of work here and there and replace 1 or 2 machines here or there, in on the whole, I don't think we'll be expanding that in a significant fashion. At least the radiology part.

Jasdeep Walia: Got it, sir. Sir, is there an inventory charge also this time in the on the expenses side, inventory provisioning charge?

Madhukar Gangadi: So the inventory provision charge this quarter is on the normal level. When compared to the previous quarter on a sequential basis, we had a release on the inventory provision line because whatever we had provided for, the company was able to liquidate. And therefore, it appears that this quarter has a higher charge, but the charge is a normal

number.

Jasdeep Walia: Got it. So what's the annual inventory provisioning charge for your company?

Madhukar Gangadi: Roughly 1%, 0.8% to 1% on the total sales.

Jasdeep Walia: I'm guessing that there will be no inventory obsolescence on the branded pharma or branded FMCG side. So this inventory charge is primarily linked to the private label, right?

Madhukar Gangadi: So a large portion is linked to the private label. There is also a small portion which comes as a charge on the branded as well. But you're right, predominantly more than 90%, 95% of our branded pharma and non-pharma products, we are able to successfully return back to the manufacturers.

Jasdeep Walia: Sir, then if we look at this expense only on the private label side, I think the annual expense would be somewhere close to 300, 350 basis points, right? That looks to be pretty high. So what are the reasons why you need to do such a large amount of provisioning.

Madhukar Gangadi: So, let's take it this way. For the branded pharmaceuticals or the branded non-pharma products, these are generally make-to-shelf products. They don't make for MedPlus, they make it for the whole market. Whereas if you take a private label product both on the pharma and non-pharma, it is made to order, which means the entire risk of obsolescence, damage, non-saleability, and slow moving is on the books of MedPlus.

So, it's a conscious call based on the margin availability, the overall economics of the product that up to 5% is what we consider as a very normal range on the private label. And as you highlighted, it is well within that number.

Jasdeep Walia: Got it, sir. And sir, as the business scales up, do you have any targets that this expense would be coming down in the future? Or this will remain close to...

Madhukar Gangadi: As a target itself, we said we do consider approximately up to 5% as a charge. But we obviously will continue to keep working to minimize this number because the amount, whatever we

minimize is straight bottom line.

Jasdeep Walia: Got it, sir. And sir, are there any plans on launching your own private label for GLP- 1?

Sujit Mahato: Not yet. If we do that, we'll definitely inform the market. Not for the moment. We have not yet launched.

Jasdeep Walia: Got it, sir. Sir, last question, sir, on the issue of promoter leverage, is there any time line to reduce leverage levels?

Sujit Mahato: So that is actively being looked at by the family office. But at present, there is no such plan. And as and when that happens, adequate disclosures will be made to the market.

Moderator: We have our next question from the line of Akhil Parekh from 360 ONE Capital.

Akhil Parekh: My first question is in terms of store opening, we still maintain our guidance of 800 store additions. That's at net level, right? And how would be the mix between franchisee versus total stores?

Sujit Mahato: Yes, Akhil. It's at the net level. The internal estimates what we had very broadly was 50%, 50%. But this quarter, it is completely on the franchisee side where we have increased on a net basis

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out of 146, 131 stores are the franchisee stores on a net basis what we added. And we continue to stick to the guidance of 800 store net store openings for the current fiscal.

Akhil Parekh: Sure. Second, on growth and the margins front, right now we have been growing heavily at the top line. So, should we continue to expect similar kind of growth trajectory in subsequent quarters in FY27?

And second, on the margin front, given the actions we have taken in terms of reducing the discount, increasing the membership fees, as Madhukar sir said, we'll increase our private label portion by 0.3% to 0.5% on a quarterly basis, starting probably from third quarter, is it safe to assume that our margin profile for

full year will go back to where it was in FY26? Or it might be still time in below that?

Sujit Mahato: It's too early, but what I would request is allow us one quarter. So that we also look at the outcomes of the actions which the company is currently taking. Maybe next quarter would be a much better period to see the benchmark and also look at the next balance 2 quarters of the current fiscal. We should be in a much better situation in quarter 2 to look at and project as well the latest estimates for the full year.

Akhil Parekh: Okay. And lastly, on the cash generation part, right? I mean we have a debt-free balance sheet, and we have almost INR600 crores plus cash and our operating cash flow, free cash flow also continues to be very strong by virtue of strong business fundamentals. So any plans to increase the dividend or do a share buyback because we'll have sufficient cash at the end of FY27...

Sujit Mahato: Your observation is absolutely right, Akhil. We are evaluating various options, and we would take the guidance and present to the Board various options. And if there is an outcome, we would definitely inform you next quarter.

Moderator: We have a next question from the line of Jitaksh Gupta from Tikri Investments.

Jitaksh Gupta: Sir, my question is regarding the revenue. So the other revenue under the total revenue segment, it is moving up. So can you understand the nature of this revenue portion and where it will move for the financial year?

Sujit Mahato: See the significant amount in that other is the franchisee sales, we still show it as others and that's in the range of INR830 million included in that number. That will be the largest number in that. And rest all are if you see it's in the same level of the previous quarters as well.

Moderator: We have our next question from the line of Bino Pathi from Elara Capital.

Bino Pathi: Madhukar, Just a follow-up from the initial question. I completely understand that the margin was down this quarter because of the lower mix of private label as well as increased contribution from the franchisee, which is very well understandable.

But then you also commented that all of these have grown, even the private label in absolute terms have grown compared to previous quarters, although the contribution has come down. In

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which case, the absolute EBITDA should have grown, although the margin comes down. So where exactly has the EBITDA decline happened?

Sujit Mahato: Bino, Sujit Here. I'll take this question. On a quarter-on-quarter basis, if you observe, the absolute sales for private label pharma and non-pharma have degrown by around INR33 crores.

Bino Pathi: Sorry, can we look at Y-o-Y, Y-o-Y also, is there a decline in...

Sujit Mahato: If you look at Y-o-Y, there is a 200 bps decline on the pharma alone, private label. As a mix, we had 13.8% and now.

Bino Pathi: No, no, I'm not talking about the mix. The mix change from margin comes down, that is logical

and I understand. But Y-o-Y in absolute sales, is there a dip?

Sujit Mahato: Y-o-Y on an absolute terms, INR197 crores versus INR200 crores on the pharma itself, it's flat.

Bino Pathi: Okay. So then why would EBITDA decline Y-o-Y?

Sujit Mahato: EBITDA has an impact of mix and the increase in the cost.

Bino Pathi: And the increase in cost is mainly the employee cost or some other costs?

Sujit Mahato: Predominantly the employee cost.

Bino Pathi: And where does it get recorded in the COGS or below that? Because I see that the gross margin has come down.

Sujit Mahato: The gross margin is due to the mix. The expenses go under the below the cost.

Bino Pathi: So see, I understand that the gross margin percentage comes down because of the mix change.

But if neither of the segments have degrown Y-o-Y, then the gross margin or EBITDA should not decline, gross profit or EBITDA should not decline.

e.

Sujit Mahato: Correct.

Bino Pathi: Hello, am I audible?

Sujit Mahato: I think we answered that question.

Moderator: We have our next question from the line of Ankit Bansal from AB Investments.

Ankit Bansal: Sir, my question, like you have just topped the opex plan. Rather than you're extending yourself to north or deep part of India in the pharmacy business, you are just hold on that investment.

What are your plans for the rest of India for your main business?

Madhukar Gangadi: So the plan is to actually grow contiguously in contiguous states. So we are there in Maharashtra right now in a significant number. And we have extended into both Chhattisgarh and Madhya Pradesh. And on the southern side, we have actually gone into Kerala.

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Once we get these states going fully and once they are profitable, then we'll go again northwards and westwards. That's the plan for us. The capex on our core business has not stopped, will not stop. I don't think there's any reason for it to stop. And that is not something on which we are looking for any kind of feedback from the market. This capex was for a slightly different thing. And so based on we rethought it a little bit. But it's nothing to do with our core business.

Ankit Bansal: Sir, my question is like how on the margin front, are the margins like now they have come down, are this fluctuation will continue for the coming years? Or will it get stable in that range or upper range? What will be the continuity for the investors to look for a stable kind of margins?

Madhukar Gangadi: See, once in a while, you're always going to have some issue like the new labor code was a pretty significant kind of impact, I would say. So whenever something like that happens, you are bound to see some kind of an impact.

But other than that, I see no reason why the margin should change significantly. The one thing which we will continue to focus on is private label. It will now, going forward, after some of the supply chain issues and all will stabilize, both on the pharmacy side as well as on the non-pharmacy side, the mix itself and then start growing. And that is what will drive the margins for us.

Ankit Bansal: Okay. Sir, the manufacturers from your manufacturing, are they of the all the auditing has been done by you? All the things are in place, you are satisfied with the kind of supply chain, all other services you're getting the products you are getting manufactured from them?

Madhukar Gangadi: On the pharma side, we actually get them manufactured from some of the biggest names in the country. The 3 or 4 people who supply us supply to all the top 30, top 40 of the big pharma. So we don't have any issues on the quality side with them.

And actually, what we go one step further, we also have our own NA BL accredited lab in our own campus. So, every batch, if not every other batch is also tested in addition to the standard testing which the labs do. So, we make sure that the quality is maintained for our customers.

Ankit Bansal: And the retail side, retail products?

Madhukar Gangadi: So I'm talking about the retail products, right? These are private label products. What comes from the brand is the brand's responsibility there. We have nothing to do with the quality and all. We sell what the doctor prescribes. And so it is the responsibility of the company to maintain its quality.

Moderator: We have a next question from the line of Axay Shah from VVD Asset Managers.

Axay Shah: Sir, my question is that quick commerce platform are scaling in some cities. And how do we see the competition from there because they are doing pretty well in a few cities?

Madhukar Gangadi: No, are you saying that quick commerce is doing well in some cities?

Axay Shah: Yes.

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Madhukar Gangadi: See, for us, if the customer

is actually willing to pay the cost of the delivery, we're more than happy to compete with quick commerce. I don't think it is something which we really want to focus on at this point of time because as of now, almost all of these businesses are bleeding cash. They are subsidized in the cost, and that is not something which we want to do. While they may take a little bit of our business or a little bit of the overall business, I don't think there will be any significant effect on the overall side.

Pharmacy is slightly different from the general grocery business and all. So, it had its own challenges and everything else. I don't think anyone has really succeeded. The day people figure out how to actually give a 20% discount, give a free delivery and give it in 10 minutes, we'll be more than happy to go to that model whenever it comes.

Moderator: We have a next question from the line of Swaraj Mehta from Perpetual Capital Advisors.

Swaraj Mehta: Am I audible?

Madhukar Gangadi: Yes.

Swaraj Mehta: I just wanted to understand you mentioned in your PPT that the stores that are greater than 12 months, the ROCE is around 63%. So but on a company level, our ROCE is very less. So I just wanted to understand why does not that reflect in the overall company numbers and where is this gap coming from?

Madhukar Gangadi: So significantly, your observation is right. Second, the way it is calculated, the store level ROCE is calculated based on store level investments and store level inventory. The company level, once it goes there, it also includes the warehouse level inventory, which is approximately 30 to 32 days of the company level sales.

So that's the only difference what you have in calculating the company level ROCE. And then you have apart from the store level computation, even if you look at the profitability, you have the corporate expenses, the warehouse logistics expenses and the sales and marketing related expenses. So, after netting that off and taking into consideration the inventory at the back end, which is the warehouse level inventory, we compute the company level ROCE, which is published. Net of the cash what we hold.

Moderator: We have a next question from the line of Anil Sarin from K16 Advisors.

Anil Sarin: A lot of my questions have already been answered. But I wanted to know what was the SSSG this quarter?

Sujit Mahato: So, as we have been informing the market, one, we do not really track SSSG per se because we strongly believe that while SSSG is a good metric for a retail store, but in pharma retail purely doesn't work because, one, we continuously add stores, which is close by to our own stores beyond a certain densification. And when we look at possibilities that it could be cannibalized by other stores or other competition, we do cannibalize our own store. And therefore, as a model, we really do not track SSSG, Anil.

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Anil Sarin: Okay. Okay. Fair enough. So, I noticed that your revenue growth rate has become quite handsome in between it had slowed down. So, congratulations on the same. In the fourth quarter, there was very good movement. And now in the first quarter also, there is 22% improvement. However, in the latest quarter, the gross margin seemed to have suffered quite a bit. I just wanted your view, is it solely due to the mix in favor of branded pharma that we have suffered this decline in gross margin, roughly 200 bps, it has come off sequentially?

Sujit Mahato: Yes. Out of 200 you mentioned, 100 bps is due to the lower sales mix of private label products, both pharma and non-pharma put together. 70 bps is attributable to the lower inventory provisions when compared to the sequential quarter. There was a question earlier that in quarter 4 of the previous fiscal, the inventory provisions were significantly lower because we had a release of provisions. In that period, we

were able to liquidate products which had earlier been provisioned.

Now that same number is actually a like-to-like a similar number, but Q4 was an exception. And therefore, we are seeing a larger dip and that impact is 70 basis points. And additionally, 40 basis points at the year-end, we were able to negotiate with our large supplier for both pharma as well as the FMCG products.

And we had some extra, I would say, supplier-related discounts at the year-end, which we do not have in Q1, but we are pretty confident that during the year, we will make up for that. So the 100 basis points, we will make up for that during the year. A significant portion, we should see a traction coming back in the next quarter and beyond.

And the 40 basis points on the full year to year, we expect it to get recovered fully. The decrease of 70 because of the inventory loss also will not be visible because on an ongoing basis, around INR12 crores is the inventory loss per quarter, and we should be on the same range on a quarter-

to-quarter basis going forward.

Anil Sarin: So, this INR12 crores equivalent, what was the figure in the first quarter?

Sujit Mahato: Fourth quarter was around INR4.5 crores, INR5 crores.

Anil Sarin: And that became INR 12 crores in the current quarter for which we are discussing.

Sujit Mahato: INR11 crores plus, yes.

Anil Sarin: Got it. So should I say that for the full year, when we close the books for fiscal '27, our gross margin would have recovered to the trend or if not exceeding it?

Sujit Mahato: I think we are shooting for that. Allow us a quarter. We will know the trajectory and the outcomes of the efforts what we are making. And post that, we can guide you for the whole year as well.

Anil Sarin: Great. Further, there are 2 developments which are somewhat concerning. One is that there seems to be a permanent increase, I mean, thanks to the regulations regarding labor, et cetera.

So there is a cost structure which has gone up permanently, at least in the states that you mentioned, Telangana and Karnataka. That is one.

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Second, there is what one hears is that there is continuing kind of discounting going on, both by the offline as well as by the online pharmacies. So that I don't know whether that is temporary or not. But on the manpower cost, there is a permanent kind of a hit. So is it possible to overcome that and go back to our older reported 9% odd kind of EBITDA margin that we saw?

Sujit Mahato: So I think in that direction, we earlier articulated that the company has already tweaked the discount structure a little bit effective 7th of July, and we have reduced 1% for purchases greater than 1,000 where the payment mode is other than UPI.

And therefore, we should see some benefit on that. And once we observe good traction and the impact on the consequential sale, the management would further evaluate and see if something else or additional tweaking is required. But our target also, again, is to go back to the earlier reported numbers and do it in a manner which is more sustainable.

Anil Sarin: Fair enough. Just one last thing. Sir, if you could just highlight the amount of the loan that has been taken against the pledge has been made, that one hears different figures. So there was a repayment and then there was a re-initiation of a debt. Obviously, it will go away only when the amount is repaid. But what does the amount outstanding stand at currently?

Sujit Mahato: Is this are you talking about the promoters' debt?

Anil Sarin: Correct.

Sujit Mahato: It's close to around INR1,150 crores.

Anil Sarin: Okay, INR1,150 crores.

Sujit Mahato: All Included, which is interest included.

Anil Sarin: So that would essentially I mean, if we have a great fiscal '27 without the price earnings multiple changing at all, I'm not talking about today's fall. I'm talking about the normalized

kind of price

earnings multiple. If the earnings were to go up at a certain level, the share price would consequently rise by an equivalent amount, and that should be at a level where you might feel comfortable to liquidate the debt?

Sujit Mahato: So that's the family office decision, nothing to do with the listco. But yes, you are right. Those calls and decisions are made by Madhukar and the family office. So I think I cannot comment anything beyond this.

Moderator: We have our next question from the line of Akash Shah from Investec Capital Services.

Akash Shah: Sir, am I audible?

Sujit Mahato: Yes.

Akash Shah: Okay. So as I can see from the presentation that was put on a second time, the SSSG has been reported at 15%, which is a fourth consecutive quarter of positive growth. Now in earlier quarter, we had alluded to the SSSG growth of 9% to 10% for the full year going ahead.

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So two questions here. First, do you see the SSSG growth rate stick to 9% to 10% for the full year? And second, as you had highlighted in the to an earlier question, you expect to arrest the degrowth of 100 basis points, which was due to the decline in share of private label.

So if your overall gross margins were to stay at the same level and EBITDA, would you would this mean that on an annual basis, the share will remain the same as it was earlier? These are the two questions.

Sujit Mahato: So as I mentioned on the SSSG, I think if you have followed my earlier response, it stays same. We do not report explicitly and track SSSG per se. But yes, if we are seeing the number it's only a number which as a consequence of various activities which we do on the ground, including the new store openings as well as the cohort of stores which is performing. But as the number to

shoot for is an overall number of 10%.

And on your second question, absolutely, we aspire to recover as much as possible on the 100 bps impact on the gross margins due to private label, but that will be over a period of a couple of quarters. Initially, we should see a comeback on the non-pharma and a bit on pharma. And subsequent quarters, we should continuously be around 0.25% to 0.3 % growth on overall private label, whereby we believe we will be able to recover the entire decline, what we have.

On a full year basis, as I had earlier mentioned, request you to be a bit patient, allow us a quarter, let us reassess the situation, and we will be in a better position to guide or even articulate what would be the full year numbers.

Akash Shah: All right, sir. And sir, just one last question. In earlier in one of the quarters, I think it was in quarter 4, you had said that every 20 to 30 bps increase in the share of private label in the overall sales leads to 10 bps growth in the gross margins. Is it correct? Or as per the current calculations in one of the so you had uploaded two presentations yesterday.

In the first presentation from the first presentation, what I could make is for every 30 bps increase in the share, there was 0.2% growth. So just on the numerical, if you could confirm.

Sujit Mahato: What we uploaded as the investor deck, my request is to stick to that because between the pharma and the non-pharma, depending on which category of customers would convert to our private label, that percentage play would always be in a particular range. What we had guided you in Q4, I would request we continue with the same guidance.

Moderator: Ladies and gentlemen, that would be the last question of the day. I now hand the conference over to the management for closing comments.

Sujit Mahato: I thank all participants on this call for your interest in the MedPlus journey. Our Investor Relations team can be contacted at ir@medplusindia.com. Thank you.

Moderator: Thank you. On behalf of MedPlus Health Services Limited, that concludes this conference. Thank you

for joining us, and you may now disconnect your lines.